



SURVEILLANCE HIGHLIGHTS

● International Health Event

■ Malaria within and near Point of Entry in Frankfurt, Germany

- On September 18, 2026, eight cases of *Plasmodium falciparum* malaria were reported among individuals working at or living in the vicinity of Frankfurt International Airport, Germany.
 - **Six cases occurred among airport workers**, with five developing symptoms in July and one in mid-August 2026.
 - **Two additional cases were subsequently reported among residents from different households living within 5 km of the airport**, who developed symptoms in early September 2026.
 - **None of the affected individuals had recently travelled to a malaria-endemic area**, suggesting possible local acquisition of infection.
 - **Three of the eight affected individuals died**, including one of the two residents living near the airport.
 - German public health authorities are conducting **epidemiological investigations, mosquito monitoring, and genetic characterization of parasites** to determine possible links among the cases and identify the likely source of infection.
 - The geographical and temporal distribution of cases suggests **multiple introductions of infected *Anopheles* mosquitoes** may have occurred. The possible involvement of *Anopheles* mosquitoes inadvertently transported by aircraft from malaria-endemic areas is also being investigated.
 - Airport workers and residents in the vicinity have been informed of the situation, while **healthcare professionals have been alerted to consider malaria among patients with unexplained fever or compatible symptoms**, even without a history of travel to malaria-endemic areas.
 - **Prompt testing and treatment are important**, as *P. falciparum* malaria can rapidly progress to severe disease and death if diagnosis and treatment are delayed.
 - Airport-associated malaria is **uncommon but has been reported sporadically in Europe**. A 2024 systematic review identified 145 airport malaria cases reported between 1969 and 2022.
 - Despite the increase in reported cases, **the risk to the general population remains very low**.
 - ECDC is in contact with the German public health authorities and is closely monitoring the situation.
 - For the Philippines, the available situation report does not provide a specific risk assessment for the country. However, malaria importation is also being experienced in the Philippines which are detected in the community amongst returning overseas Filipino workers, and not at international points of entry.

■ Bundibugyo Virus Disease (BVD) outbreak in the Democratic Republic of the Congo (DRC) and Uganda

- **As of September 16, 2026, a cumulative total of 7,475 confirmed EVD cases, including 3,605 deaths (CFR: 48.2%), has been reported across seven (7) provinces in the DRC. Ituri remains the epicenter, accounting for 77.5% of all confirmed cases.**
 - The outbreak continues to affect **62 of 167 health zones (37.1%) across seven provinces**: Ituri (28/36), North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34), Bas-Uélé (3/11), and South Ubangi (1/16).
 - **Contact tracing remains substantial**, with 29,873 contacts under monitoring and 26,181 (87.6%) successfully followed up during the previous 24 hours.
 - Epidemiological surveillance remains active, with 2,222 alerts reported in six affected provinces, of which 1,922 (86.5%) were verified. A total of 417 alerts were validated as suspected cases, all of which were investigated, with 202 transferred to treatment centers.
 - **Transmission remains high in Ituri, North Kivu, and Haut-Uélé**, while Tshopo and Bas-Uélé continue to report cases sporadically.

- **Population movement and points of entry remain important areas for surveillance.** Three alerts were notified at points of entry/points of control during the reporting period, while operational gaps, insecurity, limited personnel, and inadequate isolation measures were reported at some PoE/PoCs.
- **Response operations continue to face challenges**, including insecurity, insufficient mobility, gaps in infection prevention and control, limited treatment capacity, community resistance, and shortages of supplies and human resources. In North Kivu, for example, some ETCs have reached capacity, while in Bas-Uélé the absence of ETCs and ambulances continues to hinder case isolation.
- **Uganda officially ended its Bundibugyo virus disease outbreak on August 27, 2026 after completing 42 consecutive days without a new confirmed case.** A total of 20 confirmed cases, including two deaths (CFR: 10%), and one probable fatal case were reported. No community transmission was documented. Given the ongoing outbreak in neighbouring DRC, continued cross-border surveillance and preparedness are warranted.
- On August 18, 2026, following the second meeting of the IHR Emergency Committee, the WHO Director-General, based on the Committee's advice, determined that the ongoing outbreak remains a Public Health Emergency of International Concern (PHEIC).
- **WHO risk assessment:** The risk of further spread **within the DRC remains very high**, while the risk to **neighbouring countries sharing land borders with the DRC is high**. This assessment reflects sustained transmission, continued geographic expansion, high mortality, population mobility, insecurity, and persistent response challenges. **The risk is considered low elsewhere in Africa and globally, including the Philippines.**
- **Surveillance Updates**
 - **Event-based Surveillance and Response (ESR)**
 - **Increasing Dengue Cases in Albay (1,785 cases, 14 deaths)**
 - In April–August 2026, 14 dengue deaths were recorded in Albay, a significant increase compared with 3 deaths reported in 2025. On September 10, 2026, the Sangguniang Panlalawigan ng Albay approved the declaration of a State of Calamity due to the increasing number of dengue cases in the province of Albay. Cases involve individuals aged 2 months to 92 years old (Median: 15 years). Nine (9) cities/municipalities reported increases in dengue cases. The largest percentage increases were observed in Malilipot (884%), Daraga (357%), Bacacay (344%), and Sto. Domingo (300%). Laboratory tests showed 967/1,129 (86%) were positive for NS1, 672/765 (88%) were positive for Dengue Duo and 38/44 (86%) were positive for PCR. In response, The hospitals, RHUs, and CHOs managed the cases and dengue testing. The CHD RESU V conducted case monitoring, reporting, coordination, and specimen collection. The PHO, Dengue Program Coordinator, and Environmental Sanitation personnel conducted Dengue Caravans, larvicide provision, and LGU assistance. The PHO also conducted health promotion activities and distributed IEC materials.
 - **Case-based Surveillance**
 - **Leptospirosis**
 - As of Morbidity Week 37 (January 4 to September 19, 2026), 11,386 cases and 581 deaths have been reported (CFR: 5.1%). There is a 38% increase from MW34-35 (August 23 to September 5, 2026), with 3,480 cases reported, compared to MW32-33 (August 9 to August 22, 2026), which had 2522 cases. The two most recent MWs were not used for growth-rate comparison because reports may be delayed.
 - NCR, CAR, Regions I, III, IV-A, IV-B, and VI breached their epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026)
 - Clustering was reported in 426 barangays across 7 regions over the past four weeks, with a maximum of 154 cases recorded in a barangay. *Note: Link for the Clustering as of MW 37: <https://tinyurl.com/ClusteringMW37-Leptospirosis>*
 - **Dengue**
 - As of Morbidity Week 37, 161,925 dengue cases were reported, a 31% decrease from 233,155 cases reported in the same period last year. Among the reported cases, 62,837 (39%) were confirmed,

20,541 (13%) were probable, and 78,547 (49%) were suspected.

- Ilocos Norte and La Union (Region I), Aurora (Region III), Quezon (CALABARZON), Albay, Camarines Sur, Camarines Norte, Catanduanes and Sorsogon (Region V), Aklan and Iloilo (Region VI), Bohol (Region VII), Zamboanga Sibugay and the City of Zamboanga (Region IX), Basilan and Sulu (BARMM), Apayao and Mountain Province (CAR) Agusan del Sur (Caraga), and City of Mandaluyong (NCR) breached the epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026).
- Clustering of cases was reported in 3,441 barangays across all regions. The regions with the highest number of barangay clusters are Region III (497, 14%), Region IV-A (372, 11%), and Region VII (337, 10%). *Note: Link for the dengue clustering as of MW 37 2026: <https://tinyurl.com/MW37-ClusteringDengue>*

RECOMMENDED ACTIONS

● National Level

○ Disease Prevention and Control Bureau (DPCB):

- Provide technical guidance to Centers for Health Development (CHDs) and Local Government Units (LGUs) to strengthen dengue and leptospirosis prevention and response measures, particularly in high-risk and flood-prone areas during the rainy season, including integrated vector management, outbreak response, and early treatment.
- Ensure the sustained implementation of the Alas Kwatro Kontra Mosquito campaign and the 4S Strategy in coordination with local stakeholders, particularly in high-risk areas.

○ Health Promotion Bureau (HPB):

- Intensify risk communication and community engagement (RCCE) campaigns promoting the 4S Strategy, source reduction, personal protective measures, and early healthcare-seeking behavior for individuals with dengue- or chikungunya-like symptoms.
- Deploy RCCE packages to reduce leptospirosis risk in flood-prone and agricultural communities.

● Subnational Level

○ Centers for Health Development (CHDs):

- Validate clusters, threshold breaches, and corresponding response actions with LGUs and notify through ESR.
- Provide technical assistance to LGUs for epidemiologic investigation, risk assessment, vector control, and leptospirosis prevention measures.
- Conduct monitoring and supportive supervision of health facilities to ensure timely case notification and specimen referral, and reinforce heightened surveillance for 30 days following inclement weather events.

○ Local Government Units (LGUs):

- Intensify targeted vector control interventions, including source reduction, elimination of larval habitats, and community cleanup activities, in barangays with case clustering or sustained transmission.
- Implement targeted leptospirosis prevention measures in flood-prone areas and barangays with case clustering, including the use of protective equipment and appropriate provision of prophylaxis.
- Report diseases reaching epidemic thresholds via ESR per DM 2023-0434 Annex C: outbreak of a disease by either trend analysis or official declaration from the authority.

○ Health Facilities and Clinicians:

- Ensure early recognition, appropriate case management, and timely referral of suspected cases of mosquito-borne diseases and leptospirosis.
- Promote adherence to standard case definitions and complete clinical documentation to improve surveillance data quality and support public health decision-making.
- Ensure complete case investigation forms, timely notification through PIDSR-IS, and timely specimen collection and referral for laboratory confirmation.

Summary of All Other Notifiable Diseases, Philippines, Jan 4 - Sep 19, 2026

Diseases / Conditions	Cases			Deaths	CFR** (%)	No. of Barangay/s with Clustering [‡]	Region/s exceeding Thresholds [§]
	2025	2026	Change				
Mosquito-borne Diseases:							
Chikungunya	725	1,017	↑40	0	0	2	None
Dengue	233,155	161,925	↓31	634	0.4	3,445	V
Zoonotic Diseases:							
Leptospirosis	7,782	11,386	↑46	581	5.1	426	NCR, CAR, I, III, IV-A, IV-B, VI
Rabies	250	222	↓11	222	100	0	None
Mpox*	264	69	↓74	2	2.9	0	None
Food and Water-Borne Diseases:							
Acute Bloody Diarrhea	10,194	7,056	↓31	7	0.1	17	IV-A
Cholera	1,334	460	↓66	11	2.4	0	None
Rotavirus	9,039	8,742	↓3	23	0.3	17	CAR, I, III, VII
Typhoid Fever	19,648	12,174	↓38	32	0.3	50	None
Pan-Respiratory Diseases:							
COVID-19*	6,360	3,271	↓49	42	1.3	24	None
Influenza-Like Illness (ILI)	127,148	99,037	↓22	53	<0.1	1,068	III, IV-B
Severe Acute Respiratory Infection (SARI)	16,715	18,776	↑12	96	0.5	116	I, III, V, VI, X, NCR
Other Diseases/Syndromes:							
Acute Meningitis Encephalitis Syndrome (AMES)	3,736	3,206	↓14	154	4.8	5	CAR
Hand, Foot, and Mouth Disease	50,409	7,993	↓84	3	<0.1	6^	None
Meningococcal Disease	147	86	↓41	32	37	0	None

* Confirmed cases only

** Case Fatality Rate (CFR) is computed by dividing the total number of deaths per disease/condition by the total number of reported cases.

‡ Clustering is defined as three or more cases in a barangay in the past four weeks.

^ Clustering for HFMD and SED is defined as two or more cases in a barangay in one week. ("DM 2025-0451")

§ The listed regions exceeded their set (**epidemic, past 5-year average, previous year**) thresholds at least twice in the past four weeks.

Summary of Notifiable Vaccine-Preventable Diseases, Philippines, Jan 4 - Sep 19, 2026

Diseases / Conditions	Cases		% Change	No. of Confirmed Cases (%)*	Incidence Rate (per 1M population)	% Unvaccinated*	Deaths	CFR‡ (%)
	2025	2026						
Acute Flaccid Paralysis	535	572	↑7	0	-	23	4	0.7
Measles-Rubella	3,899	3,079	↓21	1,086 (35.3)	38.24	66	6	0.2
- Measles Cases	3,795	2,917	↓23	924 (85.1)§	36.23	66	6	0.2
- Rubella Cases	104	162	↑56	162 (14.9)§	2.01	26	0	0
Diphtheria	130	84	↓35	2 (2.4)	0.7	52	5	5.9
Neonatal Tetanus	73	51	↓30	17 (33)	5.8	-	20	39.2
Non-Neonatal Tetanus	755	726	↓4	248 (34)	9.0	24	152	20.9
Pertussis	339	218	↓36	3 (1.4)	1.9	45	2	0.9

^{*} Percentage is computed from the total reported cases

[§] Percentage is computed from the total confirmed MR cases

[^] per 100,000 population instead of per 1 million

[‡] Case Fatality Rate (CFR) is computed by dividing the total number of deaths per disease/condition by the total number of reported cases.

Surveillance Performance Indicators *Summary tables are provided in the Annex.*

● Acute Flaccid Paralysis

AFP surveillance as of MW37 showed adequate overall case detection, with the national annualized AFP reporting rate meeting the target at 2.31 per 100,000 population. However, the more specific non-polio AFP rate remained below target at 1.72 per 100,000 population, with 9 regions meeting the ≥ 2 per 100,000 population target. Surveillance quality indicators also remained below target: national stool adequacy was 70.63%, with 7 regions reaching the $\geq 80\%$ target, while timeliness of notification was 78.26%, not meeting the target. These findings indicate persistent regional gaps in discarding non-polio AFP cases and ensuring adequate specimen collection.

● Measles-Rubella

MR surveillance showed adequate overall sensitivity for case detection, with the national suspect measles reporting rate at 6.98 per 100,000 population and non-measles/non-rubella reporting rate at 3.15 per 100,000 population, both meeting their respective targets. However, surveillance quality and case classification showed gaps: only 4 regions met the target for adequate and timely specimen collection, and only Region II met the target for adequate and timely case investigation. Moreover, 23.05% of suspected cases were classified as measles-compatible, exceeding the $<10\%$ target, with only 5 regions meeting this indicator. Measles incidence also remained above the target of less than 1 case per million population in all regions. These findings suggest that while suspected cases are being detected and reported, timely investigation, specimen collection, and definitive case classification remain important surveillance gaps, particularly in regions with multiple indicators below target.

- This report provides data on notifiable diseases reported in EDCS as of Morbidity Week 37, covering the period January 4 to September 19, 2026. Case definitions may be accessed here: bit.ly/Latest_Case_Defs.
- The number of cases shown in the summary table above does not represent the final numbers and may change after the inclusion of delayed reports and updates in the status of a case following a review or confirmatory testing.

Note: The Epidemiology Bureau respectfully informs stakeholders of the current status of surveillance data availability due to downtime of the Philippine Integrated Disease Surveillance and Response Information System (PIDSR-IS). As of Morbidity Week 37, COVID-19 data (from TKC) and mpox data (from RITM) continue to be reported through their respective existing reporting systems. All other notifiable diseases may be reported in the PIDSR-IS through either direct per-case encoding or batch upload; thus, fluctuations in case counts may be due to delayed data encoding or reporting. For clarifications, you may contact the Public Health Surveillance Division at eb@doh.gov.ph or local 2930.

ANNEX

Regional Surveillance Performance Indicators for Acute Flaccid Paralysis, Philippines, Jan 4 - Sep 19, 2026

Regions	<15 yo Population <i>Based on 2025 Population Projections</i>	Expected AFP Cases for 2026	2026 Reported AFP Cases	Annualized AFP Reporting Rate per 100k population	Discarded as Non-Polio AFP	Annualized Non-Polio AFP Rate per 100k population	Cases reported within 7 days	% Timeliness of Notification	Cases with Adequate Stools	% Timeliness and Adequacy of Stool Collection	NPEV Stools		NPEV Rate
											Total stool submission <i>NPEV denominator</i>	Total NPEV stools	
PHL	34,783,002	697	572	2.31	426	1.72	430	78.26%	404	70.63%	975	41	4.21%
NCR	3,846,987	77	46	1.68	27	0.99	36	53.85%	25	54.35%	57	1	1.75%
CAR	529,975	11	13	3.45	11	2.92	7	92.11%	9	69.23%	26	0	0.00%
Region I	1,529,711	31	38	3.49	31	2.85	35	89.66%	36	94.74%	74	4	5.41%
Region II	1,094,552	22	29	3.72	25	3.21	26	84.91%	27	93.10%	56	3	5.36%
Region III	3,763,776	75	53	1.98	45	1.68	45	60.47%	36	67.92%	88	4	4.55%
Region IV-A	4,865,853	97	43	1.24	17	0.49	26	25.00%	17	39.53%	65	2	3.08%
Region IV-B	1,087,253	22	8	1.03	3	0.39	2	75.61%	3	37.50%	12	1	8.33%
Region V	2,178,720	44	41	2.64	33	2.13	31	42.11%	26	63.41%	73	0	0.00%
Region VI	1,379,660	28	19	1.94	13	1.32	8	72.86%	9	47.37%	33	1	3.03%
Region VII	2,095,334	42	70	4.70	55	3.69	51	64.71%	51	72.86%	122	6	4.92%
Region VIII	1,520,508	30	17	1.57	15	1.39	11	83.72%	14	82.35%	33	0	0.00%
Region IX	1,352,730	27	43	4.47	29	3.01	36	73.53%	36	83.72%	77	6	7.79%
Region X	1,753,091	35	34	2.73	27	2.16	25	72.97%	22	64.71%	56	4	7.14%
Region XI	1,713,275	34	37	3.04	31	2.54	27	87.50%	30	81.08%	67	5	7.46%
Region XII	1,527,359	31	32	2.94	28	2.58	28	80.95%	28	87.50%	57	0	0.00%
Region XIII	945,395	19	21	3.12	13	1.93	17	81.25%	12	57.14%	30	0	0.00%
BARM	2,110,384	42	16	1.07	11	0.73	13	50.00%	12	75.00%	25	4	16.00%
NIR	1,488,439	30	12	1.13	12	1.13	6	0.00%	11	91.67%	24	0	0.00%

LEGEND

	Reached or surpassed the target ($\geq 80\%$ for Timeliness and Adequacy of Stool Collection and NPEV Rate; $\geq 2.00/100,000$ for AFP Reporting and Non-Polio AFP Rate)
	Nearly-reached target ($60\text{--}79\%$ for Timeliness and Adequacy of Stool Collection and NPEV Rate; and $1.00\text{--}1.99/100,000$ for AFP Reporting and Non-Polio AFP Rate)
	Substantially below target ($<60\%$ for Timeliness and Adequacy of Stool Collection and NPEV Rate; and $\leq 0.99/100,000$ for AFP Reporting and Non-Polio AFP Rate)

Regional Surveillance Performance Indicators for Measles-Rubella, Philippines, Jan 4 - Sep 19, 2026

Region	Population	Suspect Measles-Rub ella Cases	Annualized Measles Incidence Rate	Adequacy and Timeliness of Specimen Collection	Adequacy and Timeliness of Case Investigation	Annualized Suspect Measles Reporting Rate	Annualized Non-Measles Non-Rubella Reporting Rate	Percentage of Measles Compatible
			Target: $<1/1,000,000$ Population	Target: $\geq 80\%$	Target: $\geq 80\%$	Target: $/100,000$ Populati	Target: $/100,000$ Populati	Target: $<10\%$
PHILIPPINES	113,164,233	5,618	27.73	67.16%	34.91%	6.98	3.15	23.05%
NCR	14,034,289	611	24.83	55.97%	28.15%	6.12	2.97	23.24%
CAR	1,879,443	97	5.23	85.57%	45.36%	7.25	5.91	2.06%
I	5,407,203	366	24.43	69.95%	30.60%	9.51	2.5	18.85%
II	3,795,955	149	5.18	93.29%	91.28%	5.52	4.48	1.34%
III	12,899,427	545	14.27	74.31%	20.92%	5.94	4.02	17.43%
IV-A	16,833,748	510	22.12	55.69%	23.92%	4.26	1.29	38.82%
IV-B	3,344,000	104	18.49	45.19%	17.31%	4.37	1.13	39.42%
V	6,366,505	92	9.05	61.96%	33.70%	2.03	0.6	30.43%
VI	4,794,970	145	7.03	87.59%	33.10%	4.25	3.17	5.52%
NIR	4,867,460	85	7.22	71.76%	35.29%	2.45	1.27	21.18%
VII	6,845,759	302	32.03	40.73%	9.60%	6.2	2.3	49.67%
VIII	4,668,165	181	19.27	51.93%	13.81%	5.45	3.25	34.81%
IX	4,024,887	181	25.14	61.88%	51.38%	6.32	2.1	28.73%
X	5,303,720	191	21.73	60.73%	32.46%	5.06	2.09	34.55%
XI	5,478,439	488	26.17	73.16%	72.54%	12.52	8.8	9.63%
XIII	4,617,564	722	101.96	86.84%	41.41%	21.97	9.98	9.42%
XIII	2,909,223	138	31.88	50.00%	23.91%	6.67	2.8	36.23%
BARM	5,093,476	711	127.75	66.67%	33.61%	19.62	3.53	27.57%
			≥ 1.00	$< 80.00\%$	$< 80.00\%$	$< 2/100,000$ Population	$< 2/100,000$ Population	$< 10\%$
								$\leq 50\%$
								$> 50\%$

Note: Link for the MR clustering as of MW 37: <https://tinyurl.com/3ryz84kr>

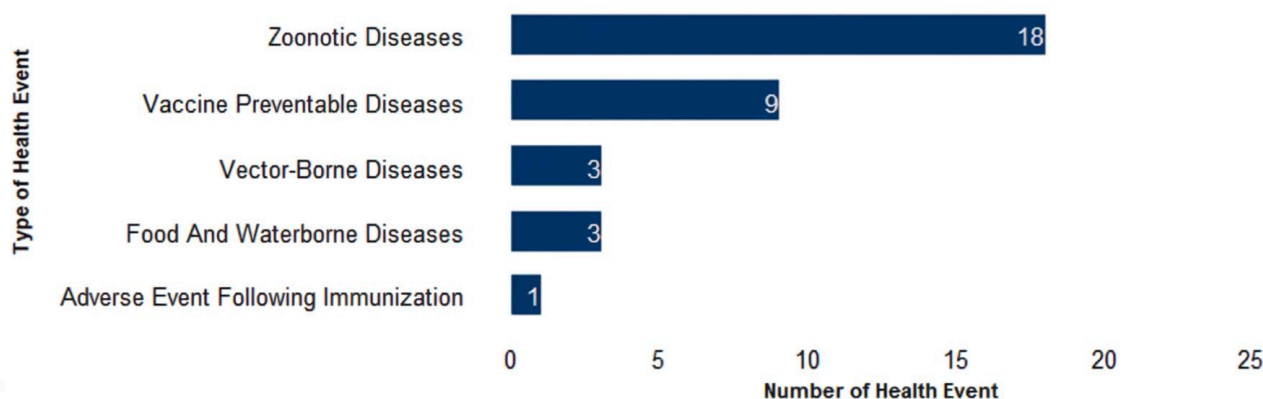
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I. Captured Health Events

From September 13-19, 2026 (Morbidity Week 37) **35 health events (HE)** were reported, of which **34** are ongoing. Of the ongoing HEs, Zoonotic Diseases accounted for the highest proportion, with 18 (53%) events, followed by Vaccine-Preventable Diseases (VPDs) (9 or 26%) and Vector-borne Diseases (VBDs) and Food and Water-borne Diseases (FWBDs) with 3 (9%) HEs each.

Ongoing Health Events by Disease Classification (n=34)
Philippines, September 13-19, 2026



II. Distribution of Event Detection and Notification by Region

Among the 35 reported HEs, 24 (69%) met the target of detection within seven (7) days of emergence. Additionally, 28 (80%) HEs were reported from the Regional Epidemiology and Surveillance Unit (RESU) to EB within one (1) day of verification.

Regional Distribution of Health Event Detection and Notification
Philippines, September 13-19, 2026

Region	Reported Health Events	No. of Health Events detected within 7 days from Emergence		No. of Health Events within 1-day Notification from Date of Verification	
		HE with ≤ 7-day detection	Percent	RESU to EB	Percent
I	1	1	100	1	100
II	15	14	93	15	100
III	6	3	50	4	67
V	2	0	0	0	0
VI	2	2	100	1	50
VII	1	0	0	0	0
VIII	1	0	0	0	0
IX	1	1	100	1	100
X	1	1	100	1	100
XI	4	2	50	4	100
XII	1	0	0	1	100
Total	35	24	69	28	80

Notes:

1. Reported HEs include both open and closed reports.
2. Regions with no reported HEs during the period of September 13-19, 2026 were not included in the analysis

I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), 572 cases have been reported. Of these, 426 cases (74%) were discarded as Non-Polio AFP, and 146 (26%) are pending classification.
- 4 deaths were reported (CFR: 0.70%), a 43% decrease from the seven deaths reported during the same period in the previous year (CFR: 1.31%).

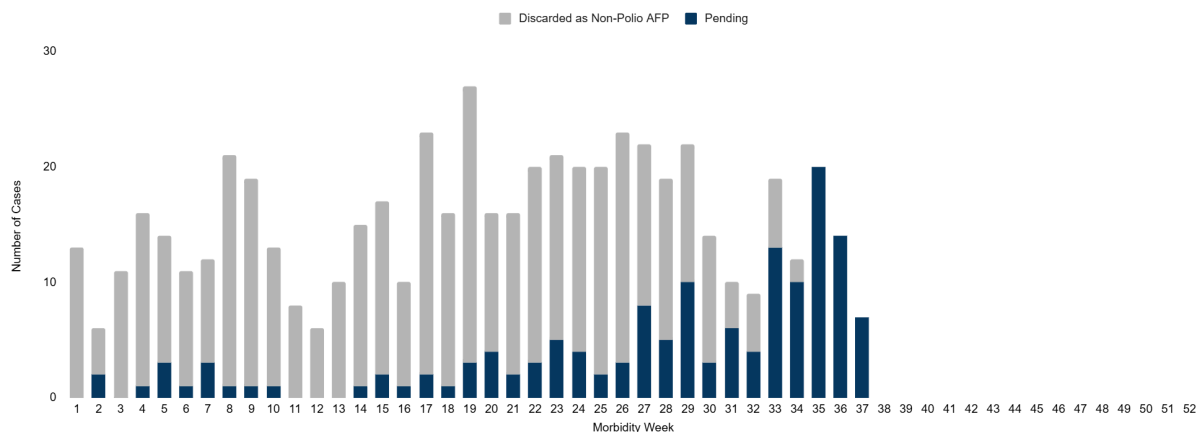


Figure 1. Acute Flaccid Paralysis Cases by Morbidity Week (N=572)
Philippines, Jan 4 to Sep 19, 2026

Note: Morbidity week is based on Symptom Onset Date; data in blue bars may still change due to pending stool specimen results and results of classification

II. Subnational Trend

For the cumulative number of cases beginning 2026, regions with the highest number of AFP cases pending classification are Regions IV-A (18%, 26 cases), NCR (13%, 19 cases), and Region VII (10%, 15 cases).

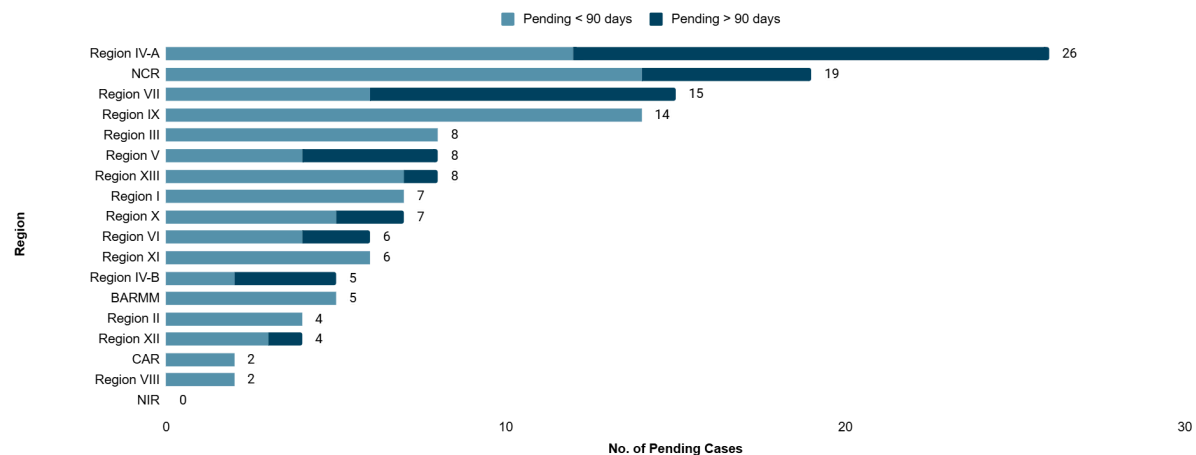


Figure 2. Proportion of Cumulative Acute Flaccid Paralysis Cases with Pending Case Classification by Region (n=146)
Philippines, Jan 4 to Sep 19, 2026

*Adequacy of stool specimens is defined as collection within 14 days from the onset of paralysis. Cases whose stool samples have not been collected are classified under no samples for testing, while cases who are awaiting a second stool sample collection are classified under no second stool, thus tagged as with pending case classification.

Of the 146 pending cases, 21 cases have adequate stool submission with pending laboratory results. The remaining are as follows: 36 cases with two stool submissions but inadequate, 20 cases with no 2nd stool submission, and 69 with no 1st or 2nd stool submission as of MW 37. These are for 60-day follow-up and laboratory testing of close contacts, or for classification by the Regional or National Expert Review Committee.

III. Surveillance Performance Indicators

- For the non-polio AFP rate, 9 out of 18 regions (CAR, I, II, V, VII, IX, X, XI, and XII) reached the target of ≥ 2 per 100,000 population as of MW37 in 2026. However, at the national level, the target was not met, at 1.72 per 100,000 population.

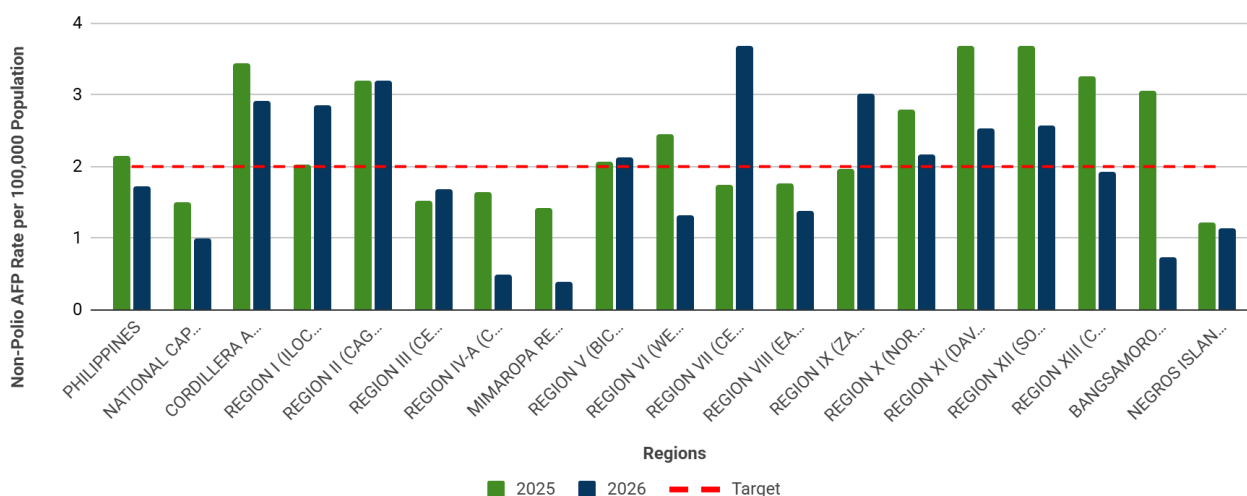


Figure 3. Non-Polio AFP Rate per 100,000 Population by Region, Philippines, Jan 4 to Sep 19, 2026 vs. 2025

- For the stool adequacy rate, 7 out of 18 regions (I, II, VIII, IX, XI, XII, and NIR) reached the target of at least 80% as of MW37 in 2026. This is similar to the 7 regions that met the target during the same period in the previous year. Nationally, the stool adequacy rate is 70.63%.

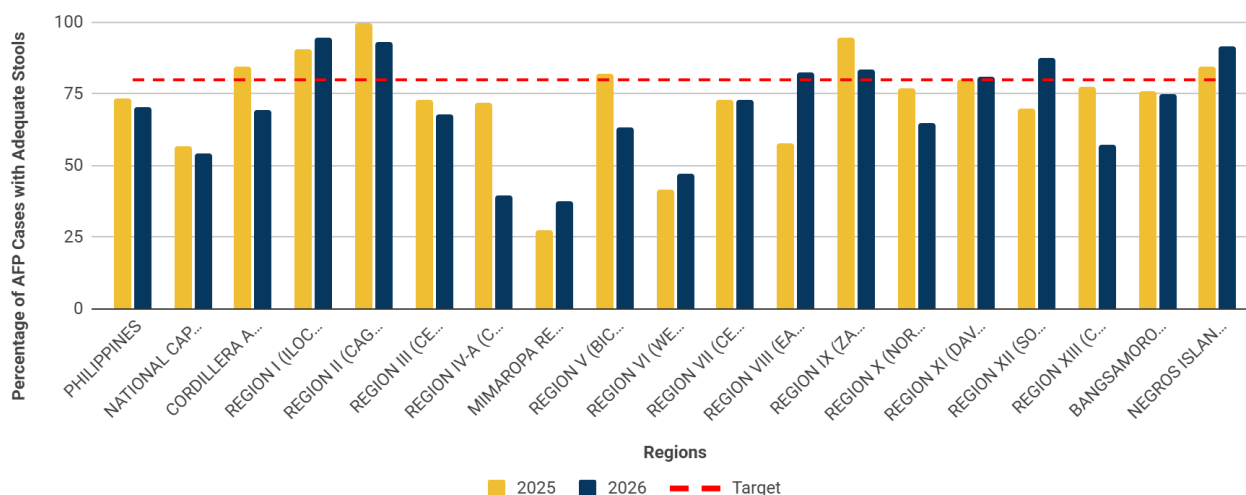


Figure 4. Percentage of AFP Cases With Adequate Stools by Region Philippines, Jan 4 to Sep 19, 2026 vs. 2025

Table 1. Summary Table of Acute Flaccid Paralysis Surveillance Performance Indicators, Philippines, Jan 4 to Sep 19, 2026

Regions	<15 yo Population <i>*Based on 2025 Population Projections</i>	Expected AFP Cases for 2026	2026 Reported AFP Cases	Annualized AFP Reporting Rate per 100k population	Discarded as Non-Polio AFP	Annualized Non-Polio AFP Rate per 100k population	Cases reported within 7 days	% Timeliness of Notification	Cases With Adequate Stools	% Timeliness and Adequacy of Stool Collection	NPEV Stools		NPEV Rate
											Total stool submission NPEV denominator	Total NPEV stools	
PHL	34,783,002	697	572	2.31	426	1.72	430	78.26%	404	70.63%	975	41	4.21%
NCR	3,846,987	77	46	1.68	27	0.99	36	54%	25	54%	57	1	1.75%
CAR	529,975	11	13	3.45	11	2.92	7	92%	9	69%	26	0	0.00%
Region I	1,529,711	31	38	3.49	31	2.85	35	90%	36	95%	74	4	5.41%
Region II	1,094,552	22	29	3.72	25	3.21	26	85%	27	93%	56	3	5.36%
Region III	3,763,776	75	53	1.98	45	1.68	45	60%	36	68%	88	4	4.55%
Region IV-A	4,865,853	97	43	1.24	17	0.49	26	25%	17	40%	65	2	3.08%
Region IV-B	1,087,253	22	8	1.03	3	0.39	2	76%	3	38%	12	1	8.33%
Region V	2,178,720	44	41	2.64	33	2.13	31	42%	26	63%	73	0	0.00%
Region VI	1,379,660	28	19	1.94	13	1.32	8	73%	9	47%	33	1	3.03%
Region VII	2,095,334	42	70	4.70	55	3.69	51	65%	51	73%	122	6	4.92%
Region VIII	1,520,508	30	17	1.57	15	1.39	11	84%	14	82%	33	0	0.00%
Region IX	1,352,730	27	43	4.47	29	3.01	36	74%	36	84%	77	6	7.79%
Region X	1,753,091	35	34	2.73	27	2.16	25	73%	22	65%	56	4	7.14%
Region XI	1,713,275	34	37	3.04	31	2.54	27	88%	30	81%	67	5	7.46%
Region XII	1,527,359	31	32	2.94	28	2.58	28	81%	28	88%	57	0	0.00%
Region XIII	945,395	19	21	3.12	13	1.93	17	81%	12	57%	30	0	0.00%
BARMM	2,110,384	42	16	1.07	11	0.73	13	50%	12	75%	25	4	16.00%
NIR	1,488,439	30	12	1.13	12	1.13	6	0%	11	92%	24	0	0.00%

LEGEND

	Reached or surpassed the target ($\geq 80\%$ for Timeliness and Adequacy of Stool Collection and NPEV Rate; $\geq 2.00/100,000$ for AFP Reporting and Non-Polio AFP Rate)
	Nearly-reached target ($60\text{--}79\%$ for Timeliness and Adequacy of Stool Collection and NPEV Rate; and $1.00\text{--}1.99/100,000$ for AFP Reporting and Non-Polio AFP Rate)
	Substantially below target ($<60\%$ for Timeliness and Adequacy of Stool Collection and NPEV Rate; and $\leq 0.99/100,000$ for AFP Reporting and Non-Polio AFP Rate)

Note: Link for the AFP Surveillance Performance Indicators as of MW 37: <https://tinyurl.com/MW37-AFP-2026-AFPIndicators>

IV. Profile of Cases

Cases were 58% male (333 cases), with 37% (212 cases) aged 10–14 years. Four deaths were reported, three of whom were male (75%). The ages of affected individuals ranged from 0 to 46 years, with a median age of 8 years and an interquartile range of 3 to 13.

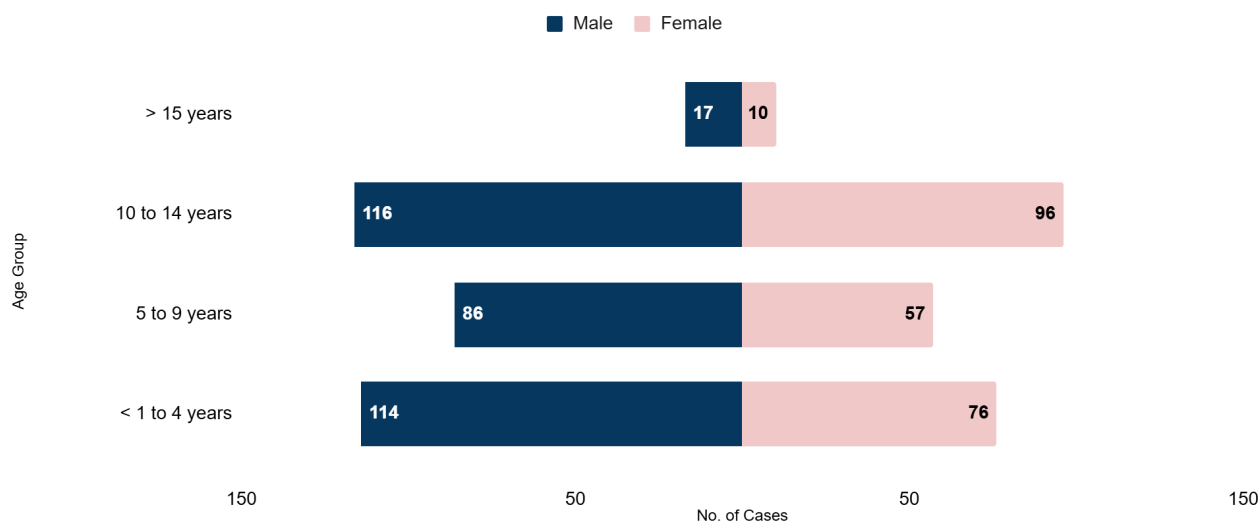


Figure 5. Distribution of Cases by Age Group and Sex (N=572)
Philippines, Jan 4 to Sep 19, 2026

No specimen was collected for 14% (74 of 572) of cases. Specimens were collected for 498 of 572 cases, and 81% (404 of 498) were adequate; 15% (73 of 498) were inadequate with 2 stools, while the remaining 4% (21 of 498) were inadequate with only 1 stool submission.

Among those with no or inadequate specimens, as well as adequate specimens with pending laboratory results, 77% (146 of 189 cases) remain pending. The remaining 23% (43 of 189 cases)

were discarded through other means (no residual paralysis and adequate negative contact stools, or expert classification).

Table 2. Laboratory Profile of Reported Acute Flaccid Paralysis Cases (N=572), Philippines, Jan 4 to Sep 19, 2026

	Cases	Percentage
Total Reported Cases	572	
With Specimen	498	87%
*Adequate Cases	404	81%
Sabin-like type 1	0	0
Sabin-like type 3	0	0
NPEV	24	6%
Negative	359	89%
Pending	21	5%
*Inadequate Cases 2 stools	73	15%
Sabin-like type 1	0	0
Sabin-like type 3	0	0
NPEV	2	3%
Negative	71	97%
Pending	0	0%
*Inadequate Cases 1 stool	21	4%
Sabin-like type 1	0	0
Sabin-like type 3	0	0
NPEV	1	5%
Negative	9	43%
Pending	11	52%
Without Specimen**	74	13%
No stool collection	74	100%

*The percentage was calculated using the total number of cases with specimens as the denominator

Note: For cases with more than one stool specimen, the final laboratory result is assigned according to the following hierarchy: Sabin-like poliovirus (Type 2, Type 1 & 3, Type 1, Type 3) → NPEV → Negative → Pending; changes in counts are due to data validation compared to previous reports

In terms of vaccination status, 61% of cases less than 5 years old (118 out of 190 cases) had received two or more vaccine doses.

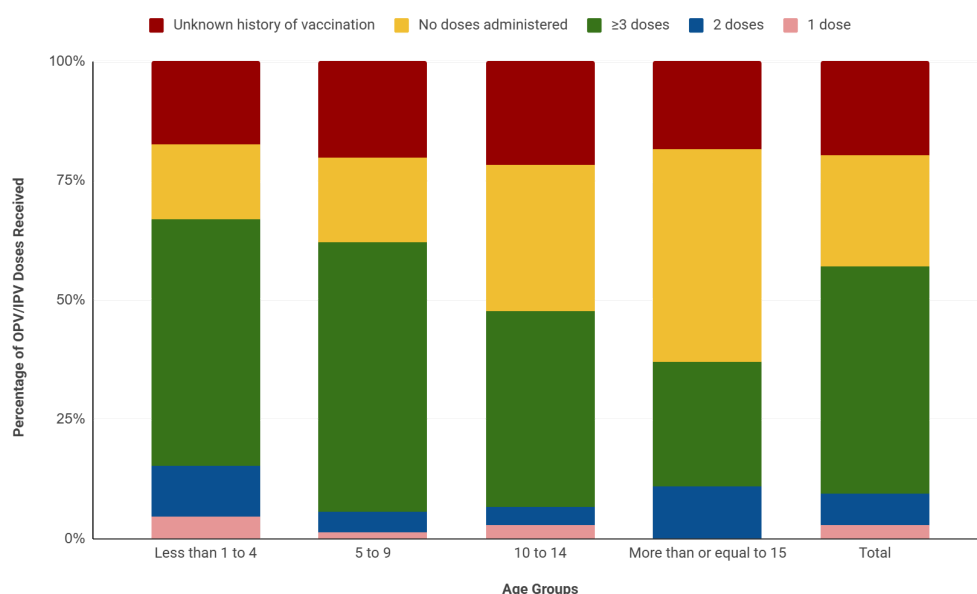


Figure 6. Proportion of Acute Flaccid Paralysis by Age Group and Vaccination Status (N=572)
Philippines, Jan 4 to Sep 19, 2026

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026.



Diphtheria

Weekly Surveillance Report 2026

I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), there were 84 reported cases, a 35% decrease from 130 cases reported in the same period last year. Of these, 28 cases (33%) underwent testing, and two (7%) tested positive for diphtheria-associated *Corynebacterium* species. The positive cases were reported from Barangay Bagong Silangan, Quezon City (NCR) and Barangay Behia, Tiaong, Quezon Province (Region IV-A).
- Five deaths were reported (CFR: 5.95%), which was 62% lower than the 13 deaths reported last year (CFR: 10%).
- Nationally, cases have increased by 29% in MW 34-35 (August 23 to September 5, 2026), with nine cases reported, compared to MW 32-33 (August 9 to 22, 2026), which had seven cases. The recent two MWs were not used for growth rate comparison due to the possibility of delayed reports.

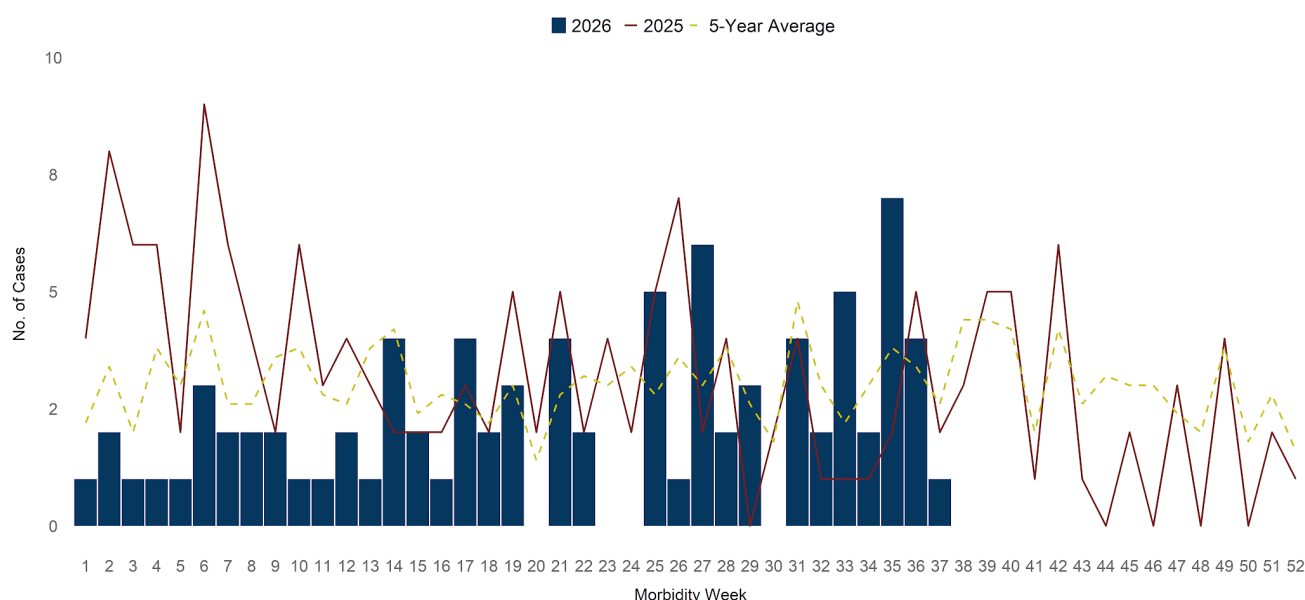


Figure 1. Cases by Morbidity Week (N=84)
Philippines, Jan 4 to Sep 19, 2026

II. Sub-national Distribution of Cases

- Among the 18 regions, NCR reported the highest cases, followed by Regions III and IV-A. The majority of the regions noted a significant decrease in cases from the same period last year.
- Case counts increased by 400% in Region IV-A and 100% in Region III in the most recent 3–4 weeks, compared with the preceding 5–6 weeks.

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR 3-4 vs 5-6	Deaths			CFR (%) [*] 2026
	2025	2026	% Change		2025	2026	% Change	
NCR	33	22	↓33	↓100	3	1	↓67	4.55
III	15	17	↑13	↑100	0	1	↑	5.88
IV-A	23	16	↓30	↑400	3	2	↓33	12.50
BARMM	9	6	↓33	↓100	1	1	-	16.67

Region	Cases			TWGR 3-4 vs 5-6	Deaths			CFR (%)*
	2025	2026	% Change		2025	2026	% Change	
XI	6	5	↓17	-	0	0	-	0
VI	2	3	↑50	-	1	0	↓100	0
VII	8	3	↓62	-	0	0	-	0
CAR	1	2	↑100	-	0	0	-	0
IV-B	4	2	↓50	-	0	0	-	0
IX	2	2	-	-	0	0	-	0
V	4	2	↓50	↓100	0	0	-	0
X	3	2	↓33	-	1	0	↓100	0
NIR	5	1	↓80	-	0	0	-	0
XIII	1	1	-	-	1	0	↓100	0
I	5	0	↓100	-	0	0	-	0
II	1	0	↓100	-	0	0	-	0
VIII	8	0	↓100	-	3	0	↓100	0
XII	0	0	-	-	0	0	-	0

*Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases

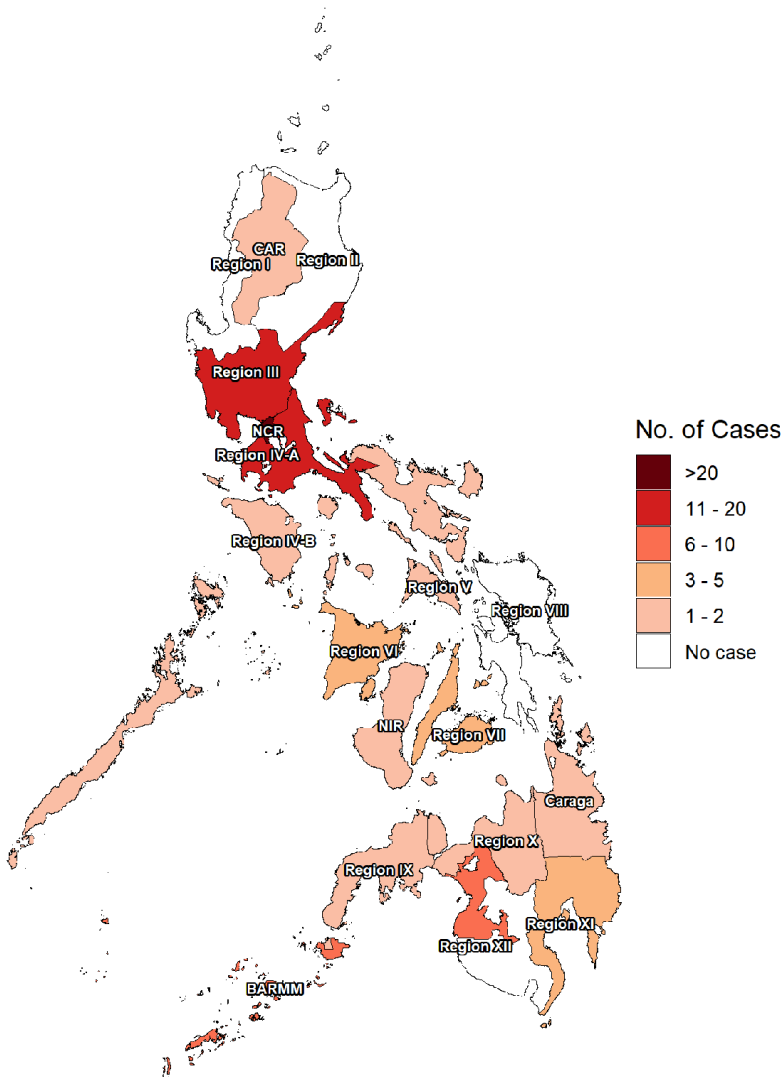


Figure 2. Regional Distribution of Cases (N=84)
Philippines, Jan 4 to Sep 19, 2026

III. Profile of Cases

a. Distribution by Age Groups and Sex

Cases were concentrated among individuals aged 5-9 years (22 cases, 26%) and those aged under 5 years (21 cases, 25%). Overall, cases ranged in age from <1 year to 68 years, with a median age of 9 years (IQR: 5-22 years). Of the cases, males (44, 52%) were higher than females (40, 48%).

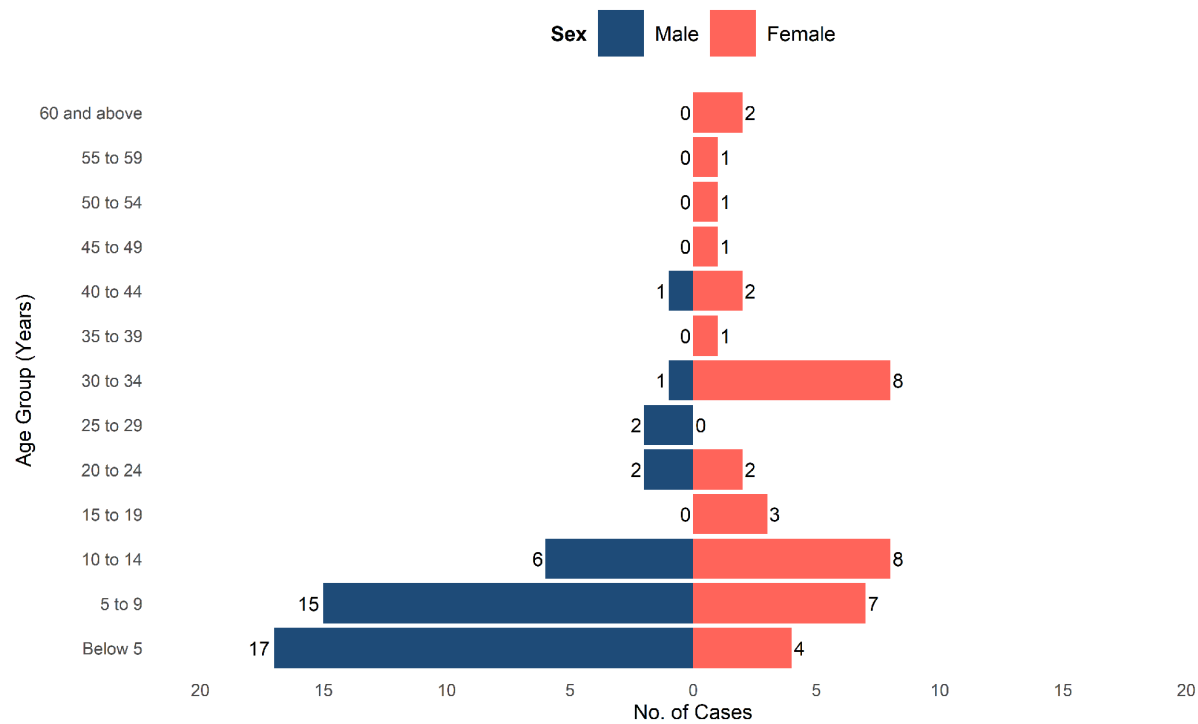


Figure 3. Cases by Age Group and Sex (N=84)
Philippines, Jan 4 to Sep 19, 2026

b. Distribution by Vaccination Status and Age Groups

Unvaccinated cases accounted for 44 (52%), while 29 (35%) had received two or more doses of diphtheria-containing vaccine. Among cases aged under 5 years, six of 21 (29%) were unvaccinated.

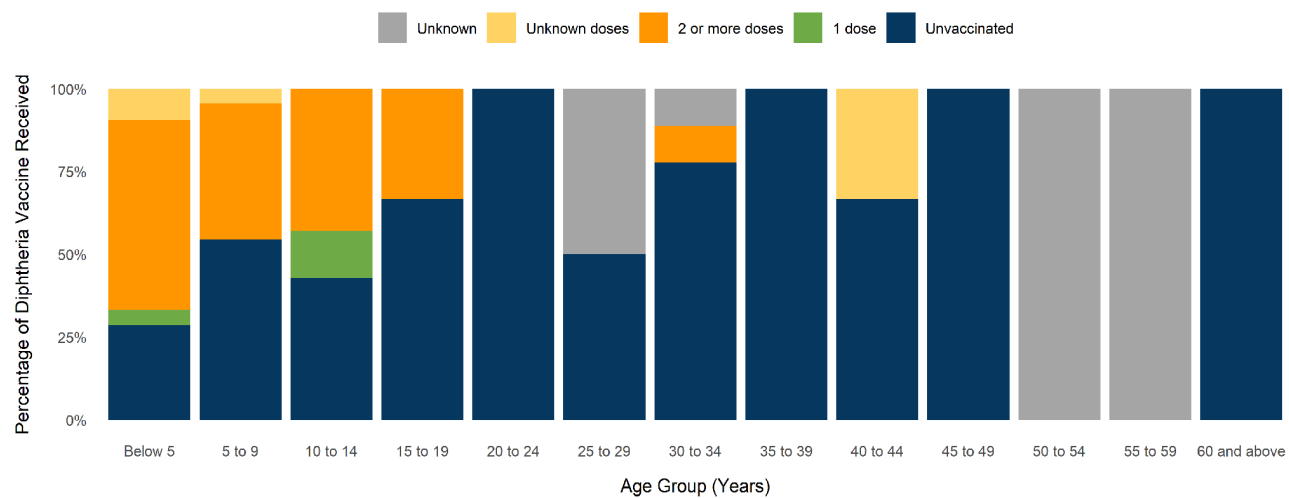


Figure 4. Cases by Age Group and Vaccination Status (N=84)
Philippines, Jan 4 to Sep 19, 2026

c. Symptoms and Hospitalization Status

- Fever, sore throat, and pseudomembrane were commonly reported among cases.
- Hospital admission was reported for 61 cases (73%).

Table 2. Symptoms and Hospitalization Profile of Cases (N=84), Philippines, Jan 4 to Sep 19, 2026

		No. of Cases	Percentage*
Symptoms*	Fever	78	93
	Sore Throat	64	76
	Pseudomembrane	58	69
	Cough	38	45
	Difficulty in Breathing	19	23
Hospitalization Status	Admitted	61	73
	Not Admitted	23	27

*Varied symptomatology and includes multiple responses

d. Laboratory Data

Among the 28 cases who underwent laboratory testing (33%), two (7%) yielded positive results for diphtheria-associated *Corynebacterium* species. The positive cases originated from Barangay Bagong Silangan, Quezon City and Barangay Behia, Tiaong, Quezon Province.

Table 3. Laboratory Profile of Cases (N=84), Philippines, Jan 4 to Sep 19, 2026

		No. of Cases	Percentage*
Laboratory Result	Tested	28	33
	Positive (<i>Corynebacterium</i> spp.)	2	7
	Positive (Other organism)	1	4
	Pending	3	11
	Negative	22	79
	Not Tested	56	67

*Total percentage may not total to 100% due to rounding off of cases

e. Antibiotic Profile

Antibiotic therapy was received by 76% of cases, while antitoxin therapy was administered to three cases (4%) – all were classified as clinically compatible cases.

Table 4. Antibiotic Profile of Cases (N=84), Philippines, Jan 4 to Sep 19, 2026

		No. of Cases	Percentage*
Antibiotic Therapy	Yes	64	76
	No	20	24
	Unknown	0	0
Diphtheria Antitoxin	Yes	3	4
	No	81	96
	Unknown	0	0

*Total percentage may not total to 100% due to rounding off of cases

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026.

Measles - Rubella

Weekly Surveillance Report 2026

I. Measles-Rubella

A. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), 3,079 measles and rubella cases have been reported, a 21% decrease from 3,899 cases during the same period last year.
- Of the overall proportion of suspect* cases (5,618 cases), the remaining 45% (2,539 cases) were discarded as non-measles/non-rubella through laboratory testing.
- Of the 3,079 reported cases, 924 (30%) were laboratory-confirmed measles, and 162 (5%) were laboratory-confirmed rubella, while 14 (1%) were epidemiologically linked confirmed measles; the remaining 1,979 (64%) were classified as clinically compatible measles cases (without specimen collected, and with specimen collected but pending laboratory results).
- There were 32 clusters* reported across 12 regions, with Region III, X, and BARMM reporting the highest clustering with four barangays each in the past four weeks.

*Suspect measles-rubella cases refer to the overall count of clinically compatible, laboratory-confirmed, epidemiologically linked, and discarded cases; reported measles-rubella cases exclude discarded cases in the total count

^A cluster is defined as two or more measles-rubella cases reported in a barangay within four consecutive weeks. Link for the Measles-Rubella Clustering as of MW 37: <https://tinyurl.com/5n7594d9>

B. Surveillance Performance Indicators

The percentage of clinically compatible measles cases under MR surveillance is 23.05% of all suspected cases (5,618 cases, including discarded classification), not achieving the target of less than 10%. Among the regions, Region VII reported the highest percentage (49%), making it the farthest from achieving the target.

Table 1. Measles-Rubella Surveillance Performance Indicators by Region, Philippines, Jan 4 to Sep 19, 2026

Region	Population	Suspect Measles-Rubella Cases	Annualized Measles Incidence Rate Target: <1/1,000,000 Population	Adequacy and Timeliness of Specimen Collection Target: ≥ 80%	Adequacy and Timeliness of Case Investigation Target: ≥80%	Annualized Suspect Measles Reporting Rate Target: ≥2/100,000 Population	Annualized Non-Measles Non-Rubella Reporting Rate Target: ≥2/100,000 Population	Percentage of Measles Compatible Target: <10%
PHILIPPINES	113,164,233	5,618	27.73	67.16%	34.91%	6.98	3.15	23.05%
NCR	14,034,289	611	24.83	55.97%	28.15%	6.12	2.97	23.24%
CAR	1,879,443	97	5.23	85.57%	45.36%	7.25	5.91	2.06%
I	5,407,203	366	24.43	69.95%	30.60%	9.51	2.5	18.85%
II	3,795,955	149	5.18	93.29%	91.28%	5.52	4.48	1.34%
III	12,899,427	545	14.27	74.31%	20.92%	5.94	4.02	17.43%
IV-A	16,833,748	510	22.12	55.69%	23.92%	4.26	1.29	38.82%
IV-B	3,344,000	104	18.49	45.19%	17.31%	4.37	1.13	39.42%
V	6,366,505	92	9.05	61.96%	33.70%	2.03	0.6	30.43%
VI	4,794,970	145	7.03	87.59%	33.10%	4.25	3.17	5.52%
NIR	4,867,460	85	7.22	71.76%	35.29%	2.45	1.27	21.18%
VII	6,845,759	302	32.03	40.73%	9.60%	6.2	2.3	49.67%
VIII	4,668,165	181	19.27	51.93%	13.81%	5.45	3.25	34.81%
IX	4,024,887	181	25.14	61.88%	51.38%	6.32	2.1	28.73%
X	5,303,720	191	21.73	60.73%	32.46%	5.06	2.09	34.55%
XI	5,478,439	488	26.17	73.16%	72.54%	12.52	8.8	9.63%
XII	4,617,564	722	101.96	86.84%	41.41%	21.97	9.98	9.42%
Caraga	2,909,223	138	31.88	50.00%	23.91%	6.67	2.8	36.23%
BARMM	5,093,476	711	127.75	66.67%	33.61%	19.62	3.53	27.57%
			≥1.00	< 80.00%	< 80.00%	<2/100,000 Population	<2/100,000 Population	<10%
								≤50%
								>50%

Notes: 1. Link for the Measles-Rubella Surveillance Performance Indicators as of MW 37: <https://tinyurl.com/3rvz84kr>

2. In the computation of the Annualized Measles Incidence Rate, the total number of reported cases was used, namely: clinically compatible measles cases without specimen collected, epidemiologically linked measles cases, and laboratory-confirmed measles cases (n = 2,166). Discarded cases were excluded. Values were updated this Morbidity Week following data validation and refinements in data analysis.

3. The Measles Compatibility Rate is calculated by dividing the number of cases without specimen collection by the total number of suspected cases

C. Laboratory Case Classification

Of the 5,618 suspected measles cases, 2,539 (45%) cases were non-measles/rubella discarded cases. Of the remaining 3,079 reported cases, 924 (30%) were laboratory-confirmed measles, and 162 (5%) were laboratory-confirmed rubella, while 14 (1%) were epidemiologically linked confirmed measles; the remaining 1,979 (64%) were classified as clinically compatible measles cases.

Of the clinically compatible measles cases, 1,295 (65%) were classified as clinically compatible cases with no collected specimen in the prescribed time period, while the remaining 684 (35%) were classified as pending laboratory results and still for reclassification.

Table 2. Measles-Rubella Laboratory Case Classification, Philippines, Jan 4 to Sep 19, 2026

Region	Total Suspected Cases	Clinically Compatible Measles*		Laboratory-Confirmed Measles	Laboratory-Confirmed Rubella	Epi-Linked Measles	Epi-Linked Rubella	Non-Measles/Rubella Discarded Cases
		Cases with Pending Result	Cases Without Specimen					
Philippines	5,618	684	1,295	924	162	14	0	2,539
NCR	611	60	142	106	6	0	0	297
CAR	97	3	2	5	8	0	0	79
I	366	159	69	25	17	0	0	96
II	149	11	2	12	3	0	0	121
III	545	18	95	36	27	0	0	369
IV-A	510	75	198	59	15	8	0	155
IV-B	104	33	41	3	0	0	0	27
V	92	23	28	13	1	0	0	27
VI	145	9	8	16	4	0	0	108
NIR	85	16	18	7	0	0	0	44
VII	302	28	150	6	6	0	0	112
VIII	181	2	63	1	7	0	0	108
IX	181	40	52	20	9	0	0	60
X	191	24	66	16	6	0	0	79
XI	488	21	47	55	22	0	0	343
XII	722	43	68	261	16	6	0	328
XIII	138	8	50	16	6	0	0	58
BARMM	711	111	196	267	9	0	0	128

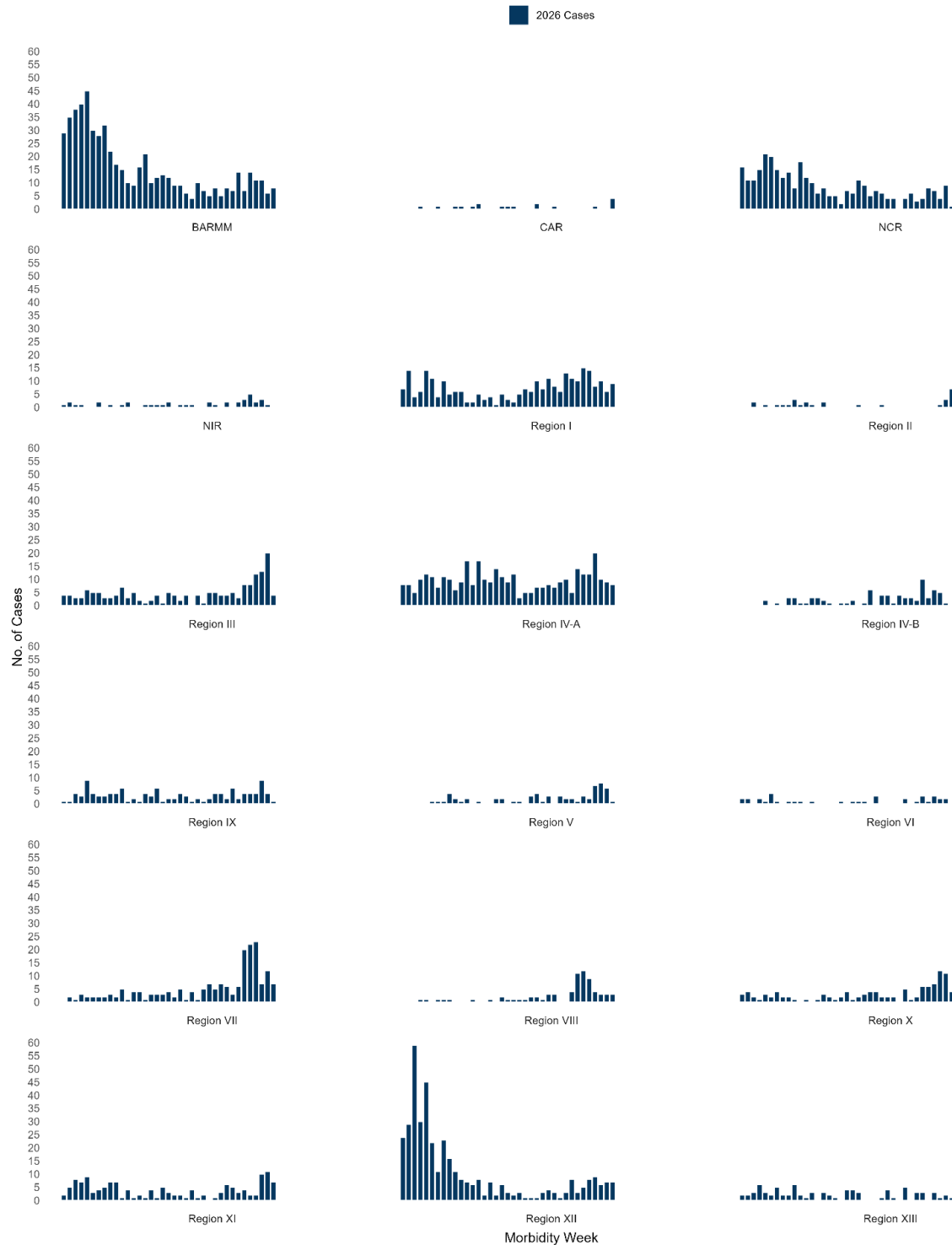
Note: Clinically compatible measles are disaggregated into cases pending laboratory results and cases without a collected specimen within the prescribed time period*

Below are the cities/municipalities with laboratory-confirmed measles-rubella cases in the **recent 21 days (August 29 to September 19, 2026)**

- Region I: Lingayen
- Region IV-A: City of Sto. Tomas
- Region VI: Sta. Barbara, San Joaquin
- NIR: City of Kabankalan
- Region XI: Bansalan, Compostela, City of Davao, Mawab

D. Subnational Distribution of Measles-Rubella Cases

Of the 18 regions, CAR, Regions II and XI showed a continuous increase in the two-week growth rate across both periods: 3–4 weeks vs. 5–6 weeks and 1–2 weeks vs. 3–4 weeks (See Annexes A-C).



**Figure 1. Regional Measles and Rubella Cases by Morbidity Week (N=3,079)
Philippines, Jan 4 to Sep 19, 2026**

II. Measles

A. National Trend

Nationally, measles cases have increased by 36% in MW 34-35 (August 23 to Sep 5, 2026), with 239 cases reported, compared to MW 32-33 (August 9 to 22, 2026), which had 225 cases. The recent two MWs were not used for growth rate comparison due to the possibility of delayed reports.

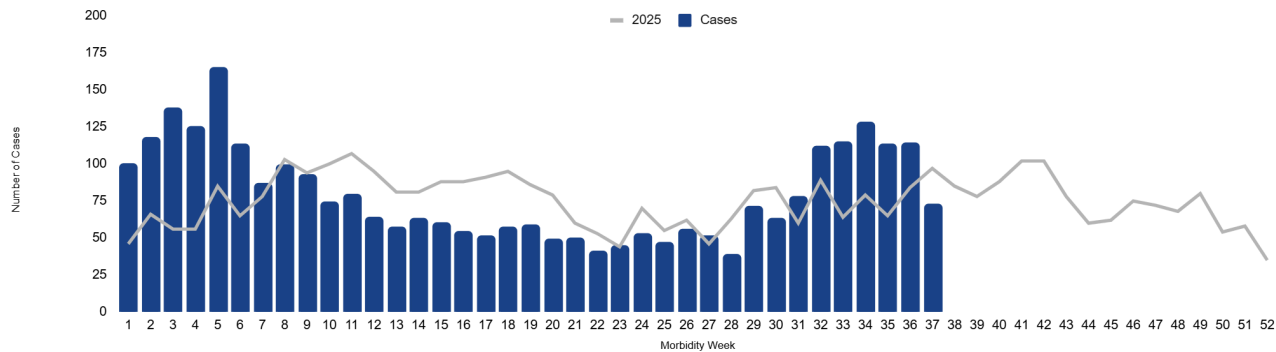


Figure 2. Measles Cases by Morbidity Week (n=2,917)
Philippines, Jan 4 to Sep 19, 2026

B. Subnational Distribution of Measles Cases

BARMM recorded the highest number of measles cases this year (574 cases, 20%). Regions VIII, XII, and XIII reported the highest increase in cases compared to the same period in the previous year.

Table 3. Cumulative Number of Measles Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025								
Region	Cases		% Change	TWGR^ 3-4 vs 5-6 (%)	Deaths		% Change	CFR (%)* 2026
	2025	2026			2025	2026		
BARMM	469	574	↑22	↑4.76	2	0	-100	0
XII	166	378	↑128	↑15.38	1	4	300	0.01
IV-A	441	340	↓23	↑26.09	6	0	-100	0.01
NCR	902	308	↓66	↓9.09	3	1	-67	0
I	170	253	↑49	↓35.71	0	0	0	0
VII	132	184	↑39	↓28.57	0	0	0	0
III	170	149	↓12	↑84.62	1	0	-100	0.01
XI	166	123	↓26	↑120.00	2	1	-50	0.01
IX	258	112	↓57	↑62.50	0	0	0	0
X	326	106	↓67	↑72.73	0	0	0	0
IV-B	189	77	↓59	↓15.38	0	0	0	0
XIII	46	74	↑61	↑33.33	0	0	0	0
VIII	32	66	↑106	↓61.11	0	0	0	0
V	78	64	↓18	↑200.00	0	0	0	0
NIR	97	41	↓58	↓37.50	0	0	0	0
VI	58	33	↓43	↑33.33	0	0	0	0
II	47	25	↓47	↑From Zero	0	0	0	0
CAR	28	10	↓64	↑From Zero	0	0	0	0

^Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)
*Case Fatality Rate (CFR) is computed by dividing the total number of deaths over the total number of reported cases

C. Profile of Measles Cases

Of the reported measles cases, 1,540, (53%) were male and 1,377 (47%) were female. The median age for measles is 5 years (IQR: 1-12 years). The most affected age group was 1 to 4 years old, with 923 cases (32%).

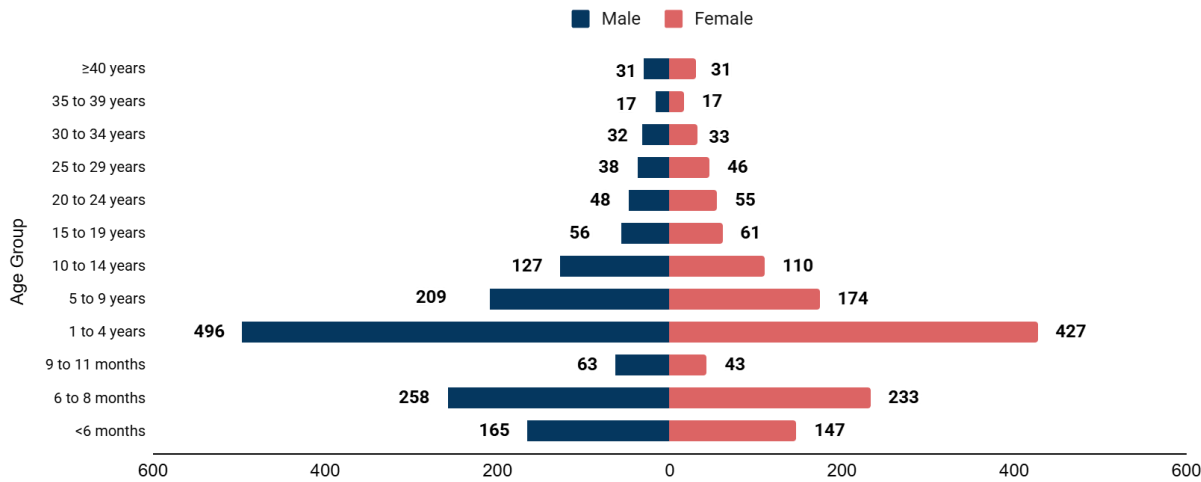


Figure 3. Age–Sex Distribution of Measles Cases (n=2,917) Philippines, Jan 4 to Sep 19, 2026

More than half (2,028, 66%) of the reported measles cases were among unvaccinated individuals. Three percent (80) presented without fever and 1% (42) without rash, both of which are part of the case definition. More than half (2,602, 89%) of cases warranted hospital admission.

Table 4. Demographic and Clinical Profile of Measles Cases* (n=3,090), Philippines, Jan 4 to Sep 19, 2026			
		No. of Cases	Percentage
Vaccination			
	unvaccinated	2,028	66
	vaccinated with unknown number of dose	181	6
	one dose	393	13
	two or more doses	446	14
	unknown	42	1
Reasons for unvaccinated cases:§			
	not eligible	395	49
	mother was busy	137	17
	child was sick during vaccination	80	10
	fear of side effects	68	8
	against belief	62	8
	forgot schedule	40	5
	vaccines not available	15	2
	medical contraindications	10	1
	no vaccinators	6	1
Clinical Profile:‡			
Signs and Symptoms			
	rash	2,886	99
	fever	2,848	97
	cough	2,332	80
	runny nose/coryza	1,816	62
	conjunctivitis	987	34
	koplik sign	506	17
	arthralgia	141	5
	lymphadenopathy	122	4
Hospitalization			
	not admitted	315	11
	admitted	2,602	89

		No. of Cases	Percentage
Travel History	without travel history	2,671	92
	with travel history	246	8
Place of Exposure			
Home		664	23
Barangay		479	16
School		85	3
Community		83	3
Health Care Facility		29	1
Day Care		16	1
Others^		95	3
Unknown		1,466	50

*Reported measles cases only (confirmed and clinically-compatible measles);

§Denominator comprised of unvaccinated cases with specified reasons only (n=813); the rest did not specify a reason (n = 1,173)

‡Cases have multiple signs and symptoms

^Other place of exposure for measles have one to four cases each

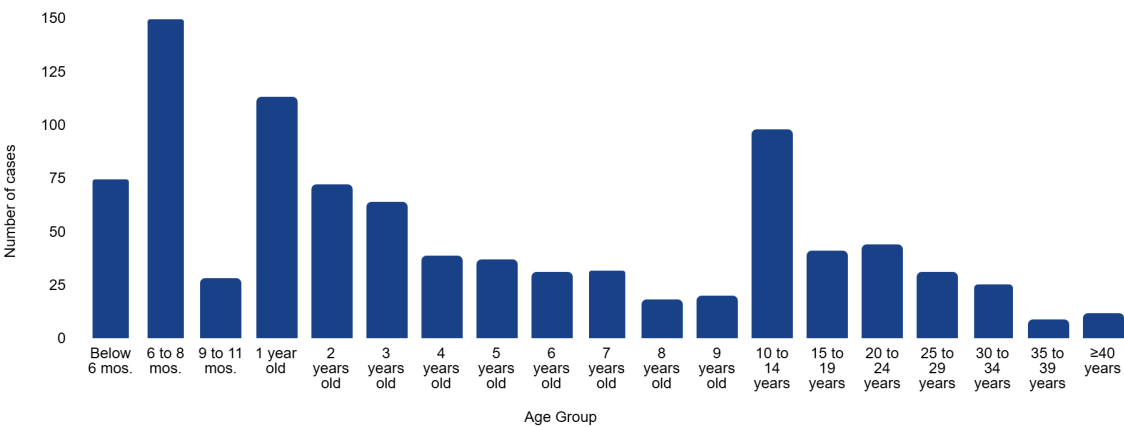


Figure 4. Laboratory-Confirmed Measles Cases by Age Group (n=924)
Philippines, Jan 4 to Sep 19, 2026

III. Rubella

A. National Trend

Nationally, rubella cases have decreased by 58% in MW 34-35 (August 23 to September 5, 2026), with five cases reported, compared to MW 32-33 (August 9 to 22, 2026), which had 12 cases. The recent two MWs were not used for growth rate comparison due to the possibility of delayed reports.

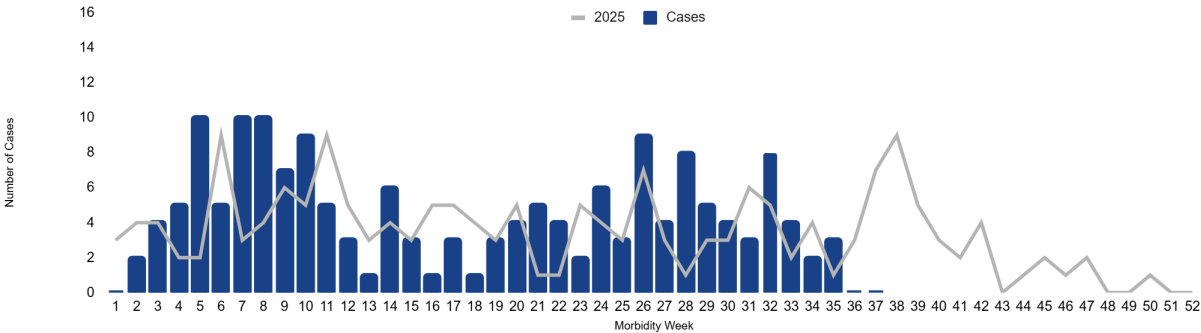


Figure 5. Rubella Cases by Morbidity Week (n=162)
Philippines, Jan 4 to Sep 19, 2026

B. Subnational Distribution of Rubella Cases

Region III reported the highest number of confirmed rubella cases this year (27 cases, 17% of the national total). Regions VIII, XI, and BARMM had the largest increases compared with the same period last year.

Table 5. Cumulative Number of Rubella Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases				Deaths				CFR (%)*	
	2025	2026	% Change	TWGR^ 3-4 vs 5-6 (%)	2025	2026	% Change		2025	2026
III	14	27	↑93	↓66.67	0	0	0		0	0
XI	3	22	↑633	0.00	0	0	0		0	0
I	10	17	↑70	↓100	1	0	-100		0	0
XII	11	16	↑45	-	0	0	0		0	0
IV-A	16	15	↓6	0.00	0	0	0		0	0
IX	4	9	↑125	-	0	0	0		0	0
BARMM	3	9	↑200	-	0	0	0		0	0
CAR	13	8	↓38	-	0	0	0		0	0
VIII	3	7	↑133	↓100	0	0	0		0	0
VII	0	6	-	-	0	0	0		0	0
X	0	6	-	↓100	0	0	0		0	0
Caraga	3	6	↑100	-	0	0	0		0	0
NCR	12	6	↓50	0.00	1	0	-100		0	0
VI	6	4	↓33	0.0	0	0	0		0	0
II	3	3	0	-	0	0	0		0	0
V	0	1	-	-	0	0	0		0	0
IV-B	0	0	0	-	0	0	0		-	-
NIR	3	0	0	-	0	0	0		-	-

^Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)
*Case Fatality Rate (CFR) is computed by dividing the total number of deaths over the total number of reported cases

C. Profile of Rubella Cases

Most of the reported rubella cases were male (90, 56%), and the median age is 1 year (IQR: 3 months to 2 years). More than half (145, 90%) had no travel history and required hospital admission.

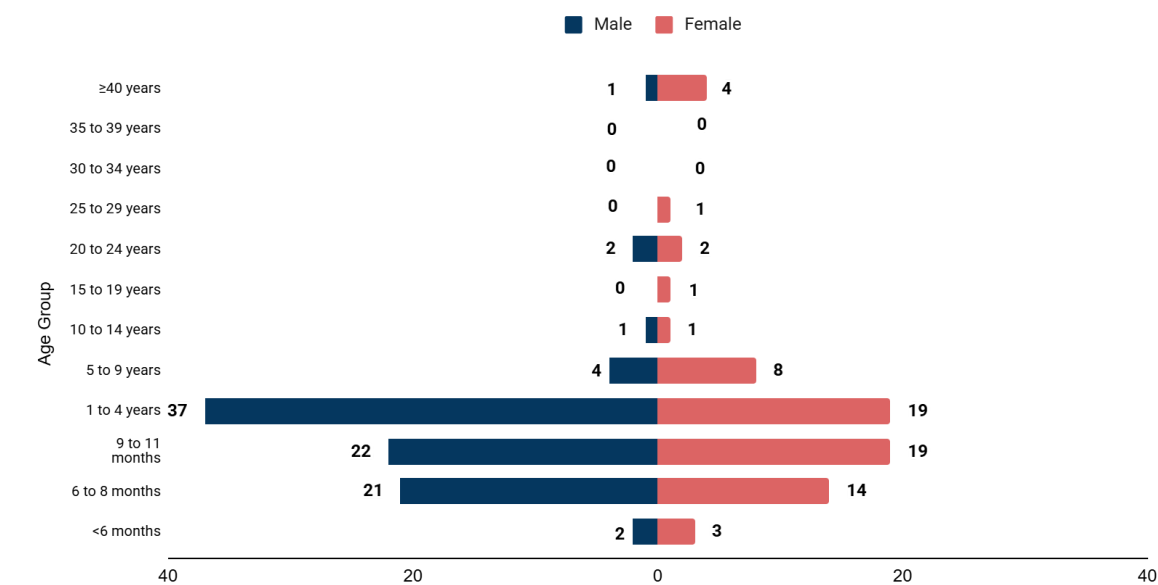


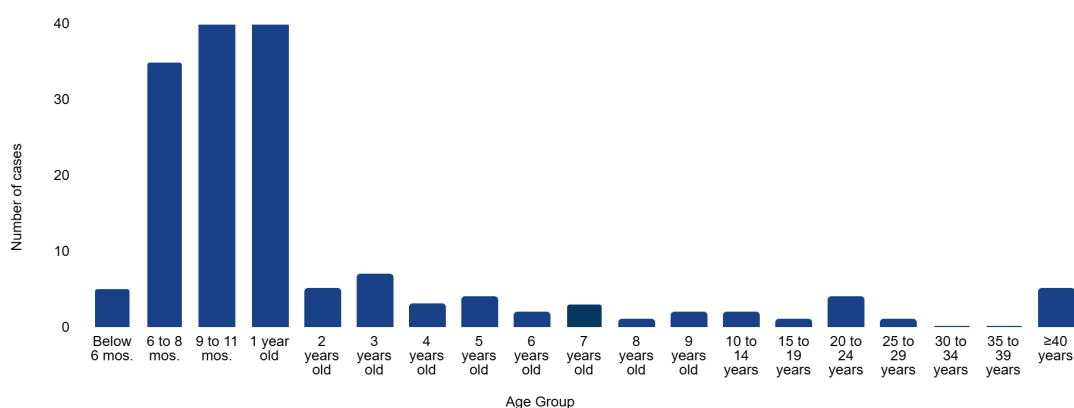
Figure 6. Age–Sex Distribution of Rubella Cases (n=162)
Philippines, Jan 4 to Sep 19, 2026

Table 6. Demographic and Clinical Profile of Rubella Cases* (n=162), Philippines, Jan 4 to Sep 19, 2026

		No. of Cases	Percentage
Vaccination	unvaccinated	42	26
	vaccinated with unknown number of dose	10	6
	one dose	67	41
	two or more doses	38	24
	unknown	5	3
Clinical Profile:*			
Signs and Symptoms	rash	161	99
	fever	157	97
	cough	111	69
	runny nose/coryza	97	60
	koplik sign	23	14
	conjunctivitis	20	12
	lymphadenopathy	8	5
	arthralgia	6	4
Hospitalization			
	not admitted	17	10
	admitted	145	90
Travel History			
	without travel history	145	90
	with travel history	17	11
Place of Exposure			
	Home	47	29
	Barangay	15	9
	Workplace	3	2
	Health Care Facility	2	1
	Day Care	2	1
	Others**	6	4
	Unknown	87	54

*Cases have multiple signs and symptoms

**Other place of exposure for rubella have one to four cases each



**Figure 7. Laboratory-Confirmed Rubella Cases by Age Group (n=162)
Philippines, Jan 4 to Sep 19, 2026**

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026

IV. Event-based Surveillance and Response

1. **Region III: Clustering of Suspect Measles-Rubella Cases in Brgy. Borol 1st, Balagtas, Bulacan (2 cases, 0 deaths)**

- Two (2) suspect cases were reported involving 6 months and 1 year old, both males. On July 31 and August 11, 2026, the cases developed fever, rash, cough, and colds and subsequently sought consultation at Balagtas Municipal Health Office (MHO) and Borol 1st Barangay Health Station (BHS) on August 3 and 12, 2026. The cases were not epidemiologically linked, with no known exposure or travel history reported. . Between July 31 and August 11, 2026, the cases developed fever, rash, cough, and colds, subsequently seeking consultation at Balagtas Municipal Health Office (MHO) and Borol 1st Barangay Health Station (BHS) on August 3 and 12, 2026. Serum specimens tested for anti-Measles/Rubella IgM at Jose B. Lingad Memorial General Hospital (JBLMGH) yielded negative results for both Measles and Rubella. As of September 1, 2026, the cases have recovered.

2. **Region IX: Clustering of Measles Cases in Brgy. Tiogan, Leon B. Postigo, Zamboanga Del Norte (2 cases, 0 deaths)**

- Two (2) cases were reported involving male and female siblings and fully immunized students of Cilago Integrated School aged 9 and 12 years old. The cases had no significant travel history. On August 30–31, 2026, the cases developed rashes, fever, cough, and colds, and subsequently sought consultation at the Leon B. Postigo Municipal Health Office (MHO) on September 1, 2026. As of September 3, 2026, both cases remain active, with other information still for verification.

Annex A

Epidemic Curves for Highly Urbanized Cities and Municipalities in CAR

- In CAR, 18 cases were reported. No death was reported in the region from MWs 1-36, 2026.
- Of these, one (6%) were unvaccinated, 11 (61%) had one dose, three (17%) had received two or more doses of measles-containing vaccine, and three (17%) had unknown vaccination status.
- No clustering was observed in the region in the recent four weeks.

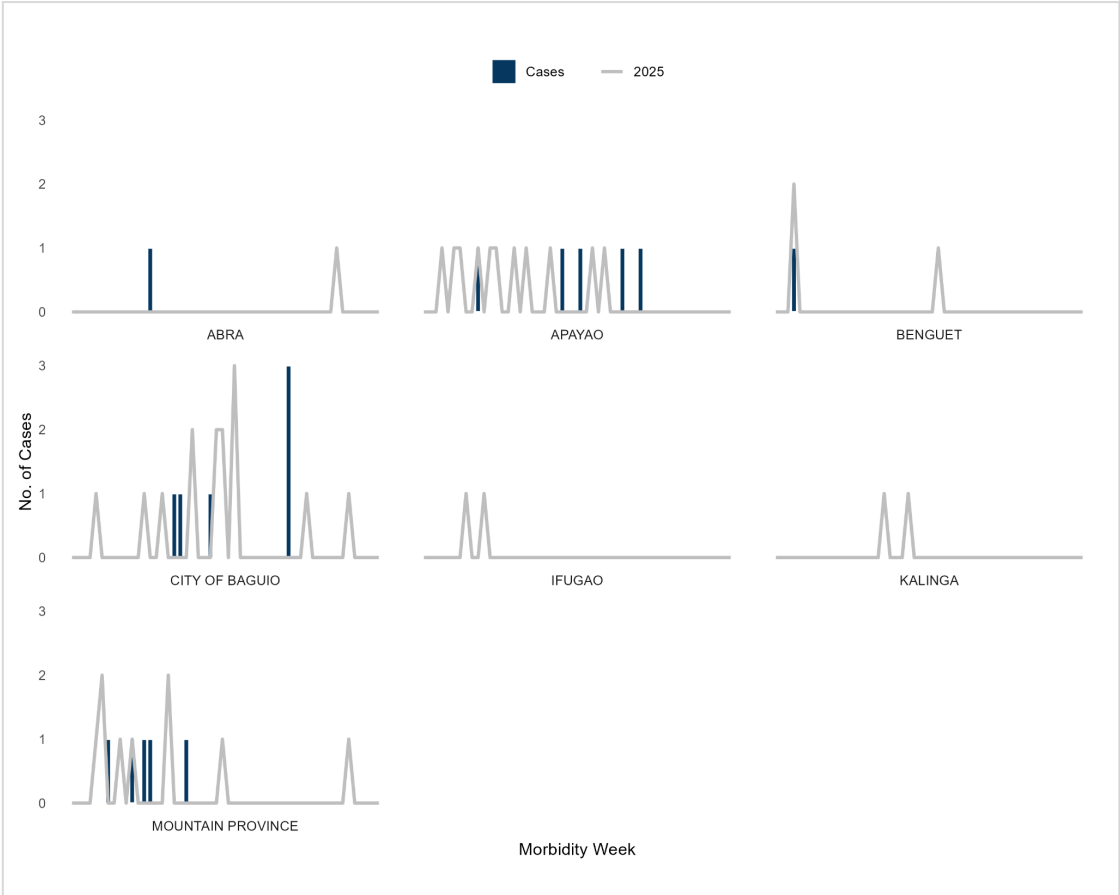


Figure A.1. Distribution of Measles and Rubella Cases by Municipality and Five-Year Average Data (n=18) CAR, Jan 4 to Sep 19, 2026

Among the 18 reported cases in these areas, five (28%) were laboratory-confirmed measles, eight (44%) were laboratory-confirmed rubella, and five (28%) were classified as clinically compatible measles.

Annex A.1. Measles-Rubella Cases (n=18), CAR, Jan 4 to Sep 19, 2026

CAR	Cases	Deaths	CFR	Disease Case Classification				
				Laboratory-Confirmed	Laboratory-Confirmed	Epi-linked confirmed	Epi-linked confirmed	Clinically Compatible
				Measles	Rubella	measles	rubella	Case
Abra	1	0	0	0	1	0	0	0
Apayao	5	0	0	0	3	0	0	2
Benguet	1	0	0	1	0	0	0	0
City of Baguio	6	0	0	1	2	0	0	3
Mountain Province	5	0	0	3	2	0	0	0

Annex B

Epidemic Curves for Highly Urbanized Cities and Municipalities in Region II

- In Region II, 28 cases were reported. No death was reported in the region.
- Of these, nine (32%) were unvaccinated, 11 (39%) had one dose, and eight (29%) had received two or more doses of measles-containing vaccine.
- No clustering was observed in the region in the recent four weeks.

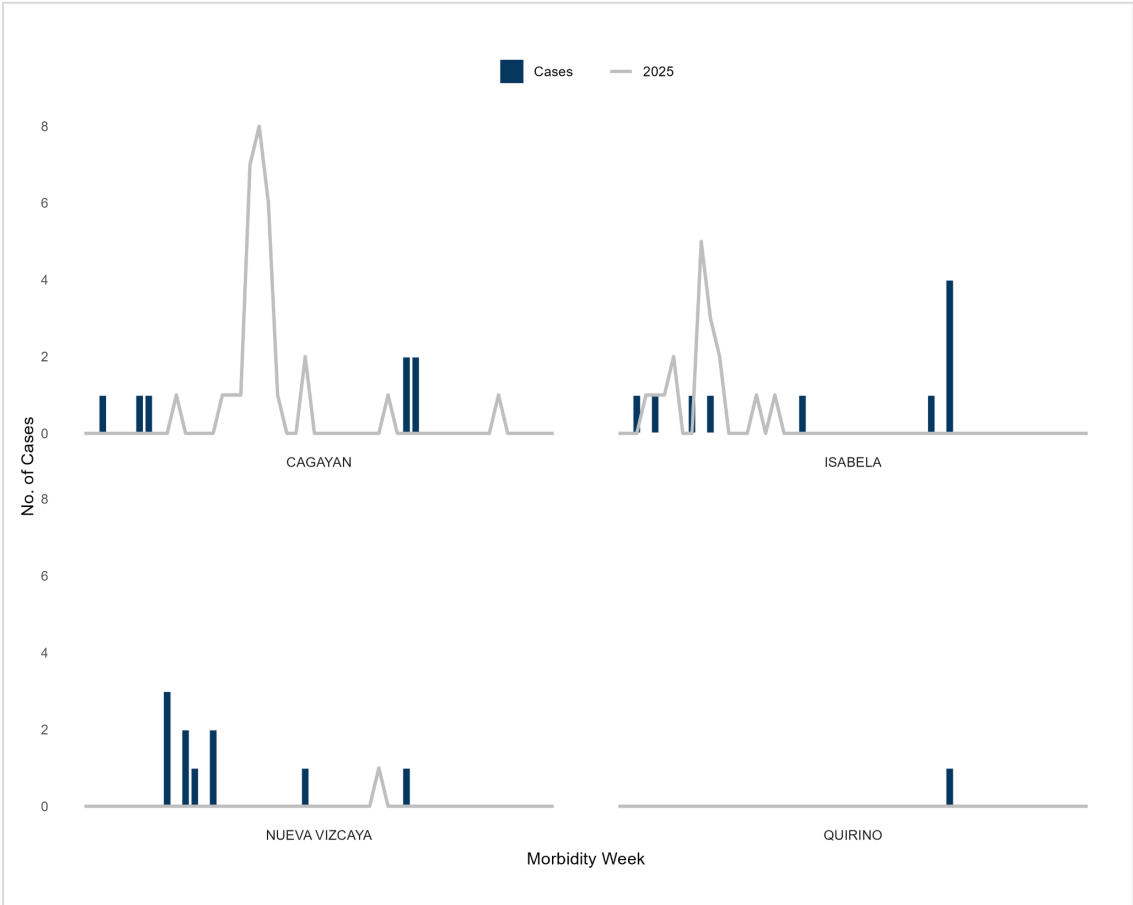


Figure B.1. Distribution of Measles and Rubella Cases by Province and Five-Year Average Data (n=28) Region II, Jan 4 to Sep 19, 2026

Among the 28 reported cases in these areas, 12 (43%) were laboratory-confirmed measles, three (11%) were laboratory-confirmed rubella, and 13 (46%) were classified as clinically compatible measles.

Annex B.1. Measles-Rubella Cases (n=28), Region II, Jan 4 to Sep 19, 2026

Region II	Cases	Deaths	CFR	Disease Case Classification		
				Laboratory-Confirmed Measles	Laboratory-Confirmed Rubella	Clinically Compatible Case
Cagayan	7	0	0	0	1	6
Camarines Norte	10	0	0	3	2	5
Camarines Sur	10	0	0	9	0	1
Sorsogon	1	0	0	0	0	1

Annex C

Epidemic Curves for Highly Urbanized Cities and Municipalities in Region XI

- In Region XI, 145 cases were reported. One death (CFR: 1%) was recorded in the City of Davao.
- Among the reported cases, 71 (49%) were unvaccinated, 26 (18%) had one dose, while 30 (21%) had received two or more doses of measles-containing vaccine, and 18 (12%) had unknown vaccination status.
- Clustering was observed in three barangays in the recent four weeks.

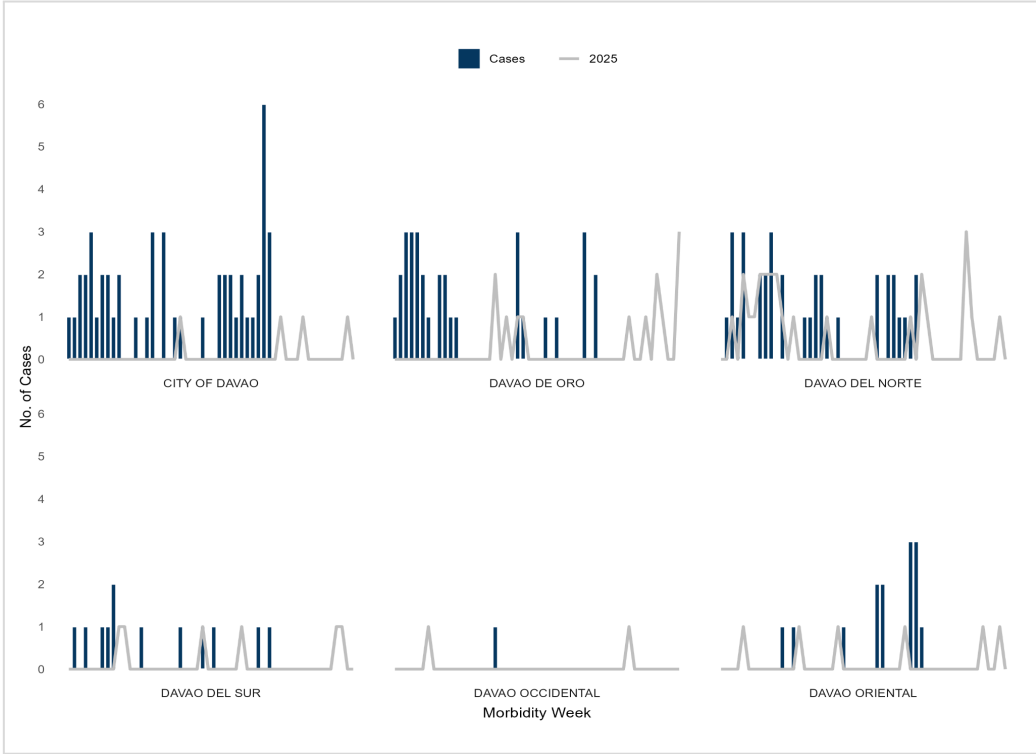


Figure C.1. Distribution of Measles and Rubella Cases by Province and Five-Year Average Data (n=145) Region XI, Jan 4 to Sep 19, 2026

Among the 145 reported cases in these areas, 55 (38%) were laboratory-confirmed measles, 22 (15%) were laboratory-confirmed rubella, and 68 (47%) were classified as clinically compatible measles.

Annex C.1. Measles-Rubella Cases (n=145), Region XI, Jan 4 to Sep 19, 2026

Region XI	Cases	Deaths	CFR	Laboratory-Confirmed Measles	Disease Case Classification		
					Laboratory-Confirmed Rubella	Epidemiologically-linked confirmed measles	Clinically Compatible Case
City of Davao	50	1	2	23	7	0	20
Davao de Oro	32	0	0	11	6	0	15
Davao del Norte	36	0	0	16	6	0	14
Davao Occidental	1	0	0	1	0	0	0
Davao Oriental	14	0	0	1	2	0	11
Davao del Sur	12	0	0	3	1	0	8



Non-Neonatal Tetanus

Weekly Surveillance Report 2026

I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), there were 726 reported cumulative non-neonatal tetanus cases, a 4% decrease from 755 cases reported in the same period last year. Of the reported cases, 248 (34%) were confirmed, and 478 (66%) were classified as suspect.
- A total of 152 deaths were reported (CFR: 20.94%), which was 12% lower than the 172 deaths reported last year (CFR: 22.78%).
- Eight new cases were reported this morbidity week.
- Nationally, cases have increased by 2% in MW34 - 35 (August 23 to September 05, 2026), with 46 cases reported, compared to MW32 - 33 (August 09 to August 22, 2026), which had 45 cases. The two most recent MWs were not used for growth rate comparison due to the possibility of delayed reports.

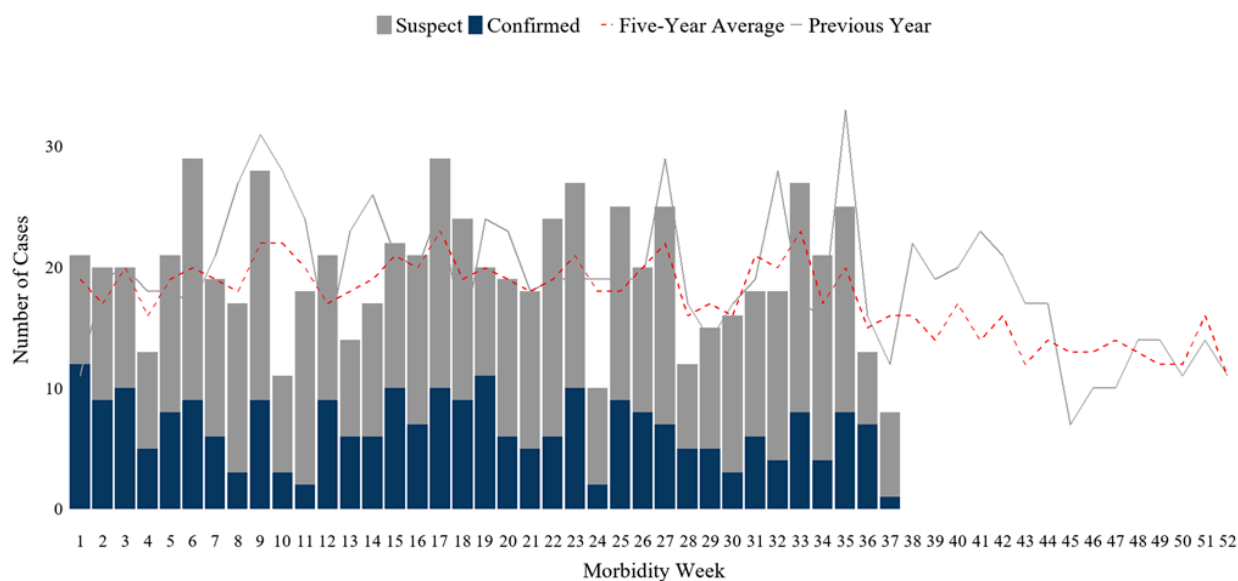


Figure 1. Cases by Morbidity Week (N = 726)
Philippines, Jan 4 to Sep 19, 2026

II. Sub-national Distribution of Cases

- Among the 18 regions, Region IV-A reported the highest number of cases, followed by Regions VII and NCR. Regions CAR, X, and VII recorded the highest percentage increase in cases across all regions from the same period in the previous year.

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR [^] 3-4 vs 5-6 weeks (%)	Deaths			CFR(%) [*] 2026
	2025	2026	% Change		2025	2026	% Change	
IV-A	71	71	-	-	16	13	↓19	18.31
VII	46	64	↑39	-	14	17	↑21	26.56
XII	63	57	↓10	↓40	15	10	↓33	17.54
NCR	56	57	↑2	↑150	17	17	-	29.82
III	70	50	↓29	↑50	18	7	↓61	14.00
BARMM	54	46	↓15	↑300	15	10	↓33	21.74
V	53	46	↓13	-	9	17	↑89	36.96
I	52	43	↓17	↓100	7	5	↓29	11.63
XI	38	40	↑5	-	11	8	↓27	20.00
II	34	37	↑9	↑100	3	5	↑67	13.51
IX	27	35	↑30	↓75	4	4	-	11.43

Region	Cases			TWGR [^] 3-4 vs 5-6 weeks (%)	Deaths			CFR(%)*
	2025	2026	% Change		2025	2026	% Change	
X	19	34	↑79	↑50	7	8	↑14	23.53
NIR	53	34	↓36	↓100	9	5	↓44	14.71
VI	41	34	↓17	↑33	11	7	↓36	20.59
IV-B	25	29	↑16	↑100	8	8	-	27.59
VIII	26	19	↓27	-	4	1	↓75	5.26
XIII	22	18	↓18	↑	4	7	↑75	38.89
CAR	5	12	↑140	↓100	0	3	↑	25.00

[^]Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)

*Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases.

III. Profile of Cases

- The age of cases ranges from less than 1 to 93 years old, with a median age of 45 (IQR: 24 - 58.5). The majority of cases were male (608, 84%).

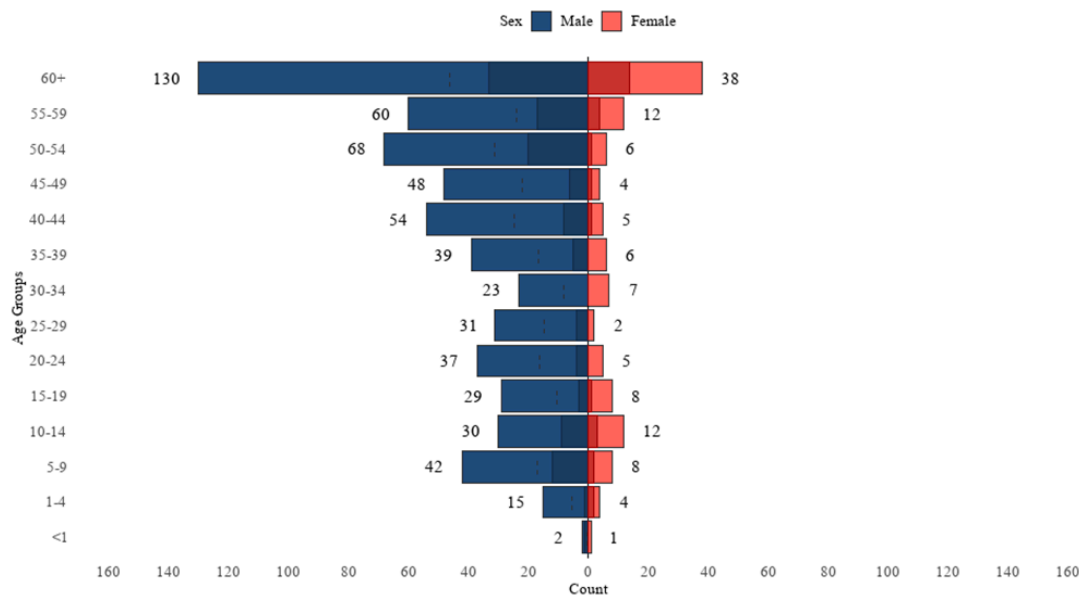


Figure 2. Cases by Age Group and Sex (N = 726)
Philippines, Jan 4 to Sep 19, 2026

- More than half of cases were given tetanus antitoxin or immunoglobulin, and 88 reported deaths had been vaccinated with tetanus toxoid.

Table 2. Vaccination Profile of Cases (N = 726), Philippines, Jan 4 to Sep 19, 2026

		Cases	Percent (%)	Deaths	Percent (%)
Tetanus Antitoxin or Immunoglobulin	Yes	470	65	93	61
	No	173	24	44	29
	Unknown	83	11	15	10
Tetanus Toxoid	Yes	441	61	88	58
	No	176	24	46	30
	Unknown	109	15	18	12

- The lower extremity was the site of the wound in 61% (440) of cases, while 38% (279) of them had puncture-type wounds. Sixty-nine percent (500) of cases did not have any skin lesions within the past three months.

Table 3. Clinical Signs Profile of Cases (N = 726), Philippines, Jan 4 to Sep 19, 2026

		No. of Cases	Percentage
Wound Site	Lower Extremity	440	61
	Upper Extremity	110	15
	Head and Neck	52	7
	Trunk	8	1
	Unknown	116	16
Type of Wounds	Puncture	279	38
	Abrasion	118	16
	Laceration	94	13
	Dental (Caries/Extraction)	39	5
	Animal Bite	14	2
	Burn	12	2
	Lesions	9	1
	Surgery	9	1
	Avulsion	7	1
	Tissue Necrosis	7	1
	Crash	5	1
	Open Fracture	3	<1
	Insect Bite	2	<1
	Others	16	2
	Unknown	112	15
Skin Lesion*	Yes	123	17
	No	500	69
	Unknown	103	14

*Skin lesions for the past 3 months, which may include abscess, ulcer, blister, gangrene, cellulitis, etc.

- Agricultural and Fishery Workers accounted for 18% (129) of total cases.

Table 4. Occupation of Cases (N = 726), Philippines, Jan 4 to Sep 19, 2026

	No. of Cases	Percentage
Agricultural and Fishery Workers	129	18
Construction and House Builders	53	7
Dependents, Students, and Children	37	5
Elementary Workers and Laborers	29	4
Driver	17	2
Stay-at-home and Business	17	2
Service and Sales Workers	13	2
Technicians and Machine Operators	11	2
Security and Police	7	1
Retiree	6	1
Others	25	3
Unemployed	168	23
Unknown	214	29

*Total percentage may not total to 100% due to rounding off of cases.

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSIR-IS batch upload feature. Data was extracted on September 21, 2026.



Neonatal Tetanus

Weekly Surveillance Report 2026

I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), 51 cases were reported, a 30% decrease from the 73 cases reported in the same period of 2025. Of the reported cases, 17 (33%) were confirmed, while 34 (67%) were classified as suspect.
- 20 fatalities were reported (CFR: 39.22%), a 43% decrease from the 35 deaths (CFR: 47.95%) recorded during the same period last year.
- Two new cases were reported for this morbidity week.
- Nationally, cases have increased by 50% in MW34 - 35 (August 23 to September 05, 2026), with 6 cases reported, compared to MW32 - 33 (August 09 to August 22, 2026), which had 4 cases. The two most recent MWs were not used for growth rate comparison due to the possibility of delayed reports.

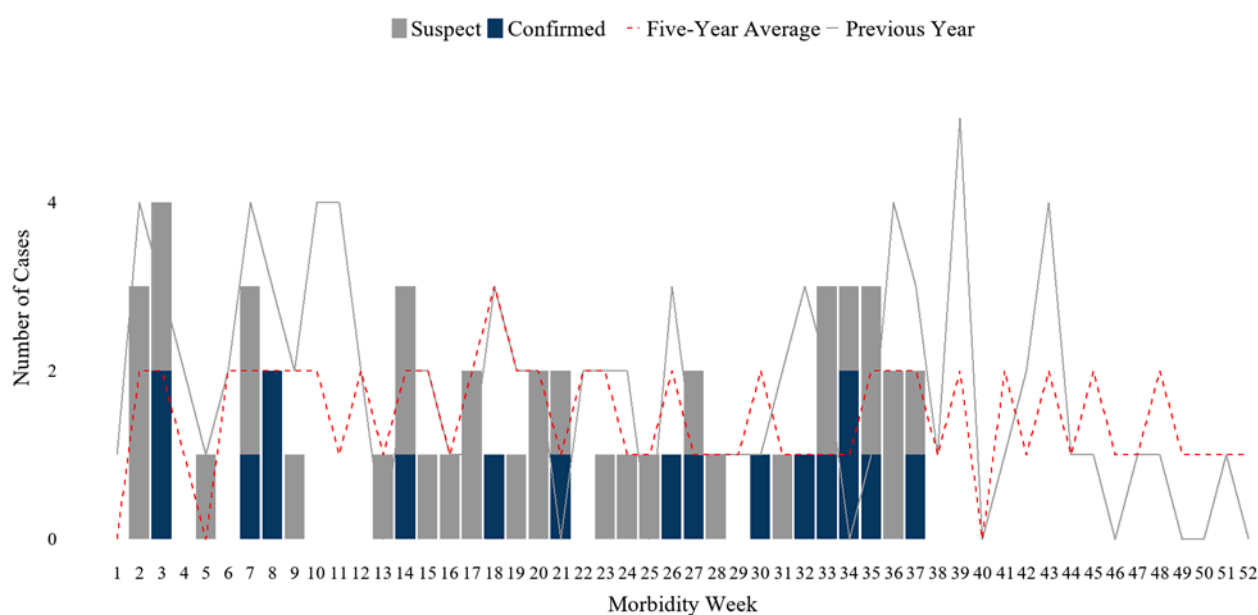


Figure 1. Neonatal Tetanus Cases by Report Date (N = 51)
Philippines, Jan 4 to Sep 19, 2026

II. Sub-national Distribution of Cases

- Among the 18 regions, BARMM reported the highest number of cases, followed by Region IV-B and XII. NCR, CAR, Regions VII, X, XI, and XIII also saw a percentage increase in cases from the same period in the previous year.
- Reported cases in Regions BARMM, XII, and X increased during the recent 3-4 weeks compared with the preceding 5-6 weeks.

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR [^] 3-4 vs 5-6 weeks (%)	Deaths			CFR(%) [*] 2026
	2025	2026	% Change		2025	2026	% Change	
BARMM	33	18	↓45	↑	14	6	↓57	33.33
IV-B	11	8	↓27	↓100	5	4	↓20	50.00
XII	11	6	↓45	↑	4	4	-	66.67
XI	1	5	↑400	↓100	0	1	↑	20.00

Region	Cases			TWGR [^] 3-4 vs 5-6 weeks (%)	Deaths			CFR(%)*
	2025	2026	% Change		2025	2026	% Change	
X	1	3	↑200	↑	1	1	-	33.33
XIII	2	3	↑50	↓100	2	1	↓50	33.33
NCR	1	2	↑100	-	0	1	↑	50.00
III	2	1	↓50	-	2	0	↓100	0.00
IV-A	2	1	↓50	-	1	1	-	100.00
IX	2	1	↓50	-	0	0	-	0.00
CAR	0	1	↑	-	0	1	↑	100.00
NIR	1	1	-	-	1	0	↓100	0.00
VII	0	1	↑	-	0	0	-	0.00

[^]Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)

*Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases.

III. Demographic Profile of Cases

- There are more male cases (30, 59%) reported than female cases (21, 41%).
- The ages of affected individuals ranged from 3 to 23 days, with a median of 7 days (IQR: 5 - 9).

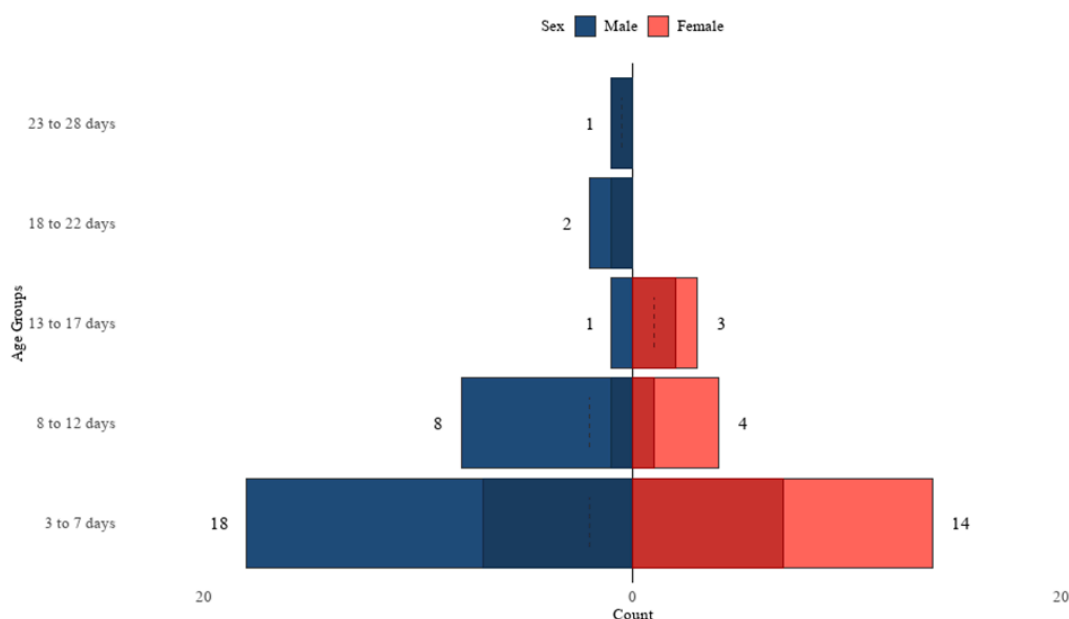


Figure 2. Cases by Age Group and Sex (N = 51)
Philippines, Jan 4 to Sep 19, 2026

- Bamboos were used in 29% of cases (15), and Traditional Birth Attendants (*Hilot*) assisted in 75% (38) of deliveries. Additionally, 92% (47) of cases were delivered at home, and 82% (42) had mothers that had unknown vaccination status

Table 2. Demographic Profile and Exposure History of Cases (N=51), Philippines, Jan 4 to Sep 19, 2026

		No. of Cases	Percentage
Cord-Cutting Instruments	Bamboo	15	29
	Scissors	14	27
	Blade	13	25
	Skin Stalk of Coconut	1	2
	Unknown	8	16

Neonatal Tetanus

EB WSR 2026

		No. of Cases	Percentage
Delivery Attendants	Traditional Birth Attendants (<i>Hilot</i>)	38	75
	Husband/Partner (Father)	4	8
	Family Member/Relative	4	8
	Healthcare Worker	2	4
	Self (Mother)	1	2
	Unknown	2	4
Place of Delivery	Home	47	92
	Hospital/Lying-in/Clinic	2	4
	Unknown	2	4
Vaccination Status of Mothers	Incomplete Dose	7	14
	Complete Dose	2	4
	Unknown	42	82

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026.



Pertussis

Weekly Surveillance Report 2026

I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), there were 218 reported cumulative pertussis cases, a 36% decrease from 339 cases reported in the same period last year.
- Of 218 cases, 158 (72%) underwent laboratory testing, of which only 3 tested positive for *Bordetella pertussis*.
- Two deaths were reported (CFR: 0.92%), which was 60% lower than the five deaths reported last year (CFR: 1.47%).
- Nationally, cases decreased by 50% in MW 34-35 (August 23 to September 5, 2026), with two cases reported, compared to MW 32-33 (August 9 to 22, 2026), which had four cases. The recent two MWs were not used for growth rate comparison due to the possibility of delayed reports.
- No new cases were reported this morbidity week.

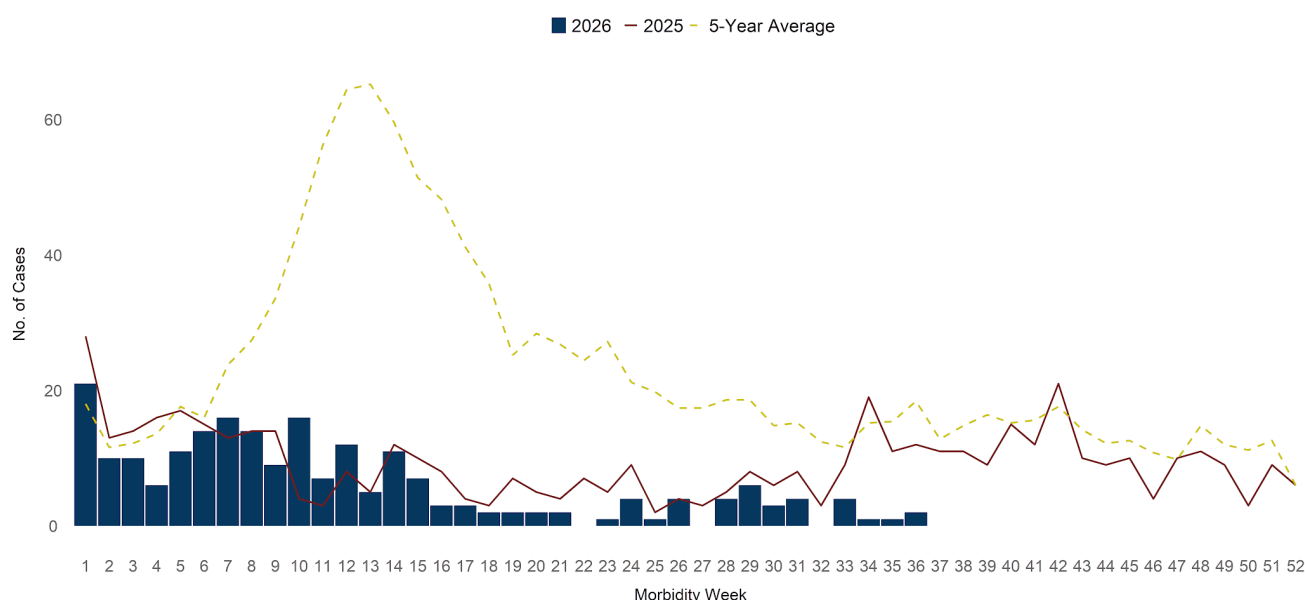


Figure 1. Cases by Morbidity Week (N=218)
Philippines, Jan 4 to Sep 19, 2026

II. Sub-regional Distribution of Cases

- CAR, Regions I, X, and XII recorded increases in case percentages across all regions. A significant 700% increase was noted in Region XII, with 8 cases this year compared to 1 last year, followed by CAR, with a 49% increase from 80 cases last year to 119 this year.
- Reported cases in Regions I and III increased during the recent 3-4 weeks compared with the preceding 5-6 weeks.
- As of MW 37, there was no reported clustering in the past four weeks.

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR 3-4 vs 5-6	Deaths			CFR (%) [*] 2026
	2025	2026	% Change		2025	2026	% Change	
CAR	80	119	↑49	↓100	1	1	-	0.84
I	18	20	↑11	↑	0	0	-	0

Region	Cases			TWGR 3-4 vs 5-6	Deaths			CFR (%)*
	2025	2026	% Change		2025	2026	% Change	
NCR	50	15	↓70	-	1	0	↓100	0
VII	21	8	↓62	-	0	0	-	0
XI	13	8	↓38	↓100	1	0	↓100	0
XII	1	8	↑700	-	0	0	-	0
III	24	7	↓71	↑	1	0	↓100	0
X	5	6	↑20	-	0	0	-	0
IV-A	21	4	↓81	-	0	0	-	0
IX	8	4	↓50	-	1	0	↓100	0
VI	12	4	↓67	-	0	0	-	0
XIII	4	4	-	-	0	1	↑	25.00
NIR	26	3	↓88	-	0	0	-	0
II	19	3	↓84	-	0	0	-	0
BARMM	7	2	↓71	-	0	0	-	0
IV-B	5	2	↓60	-	0	0	-	0
V	17	1	↓94	-	0	0	-	0
VIII	8	0	↓100	-	0	0	-	0

*Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases

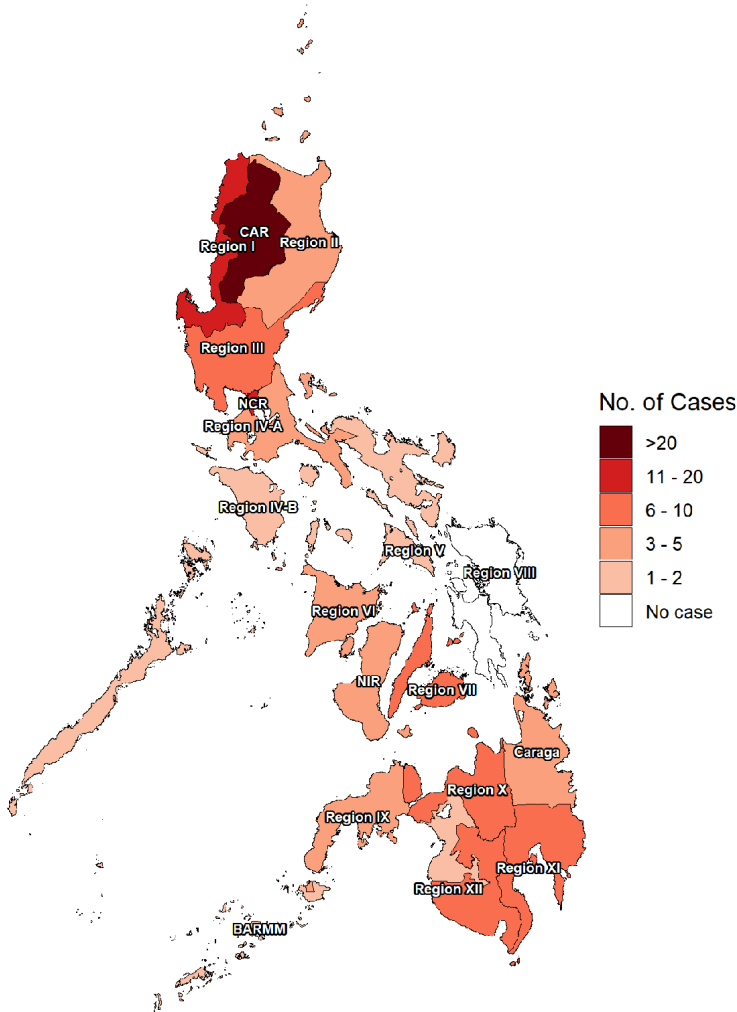


Figure 2. Regional Distribution of Cases (N=218)
Philippines, Jan 4 to Sep 19, 2026

III. Profile of Cases

a. Distribution by Age Groups and Sex

Of the reported cases, 63% (138 cases) were among individuals aged under 5 years. Reported ages ranged from <1 to 90 years, with a median age of 2 years (IQR: <1–15 years). Among these, male cases (135, 62%) were higher than female cases (83, 38%).

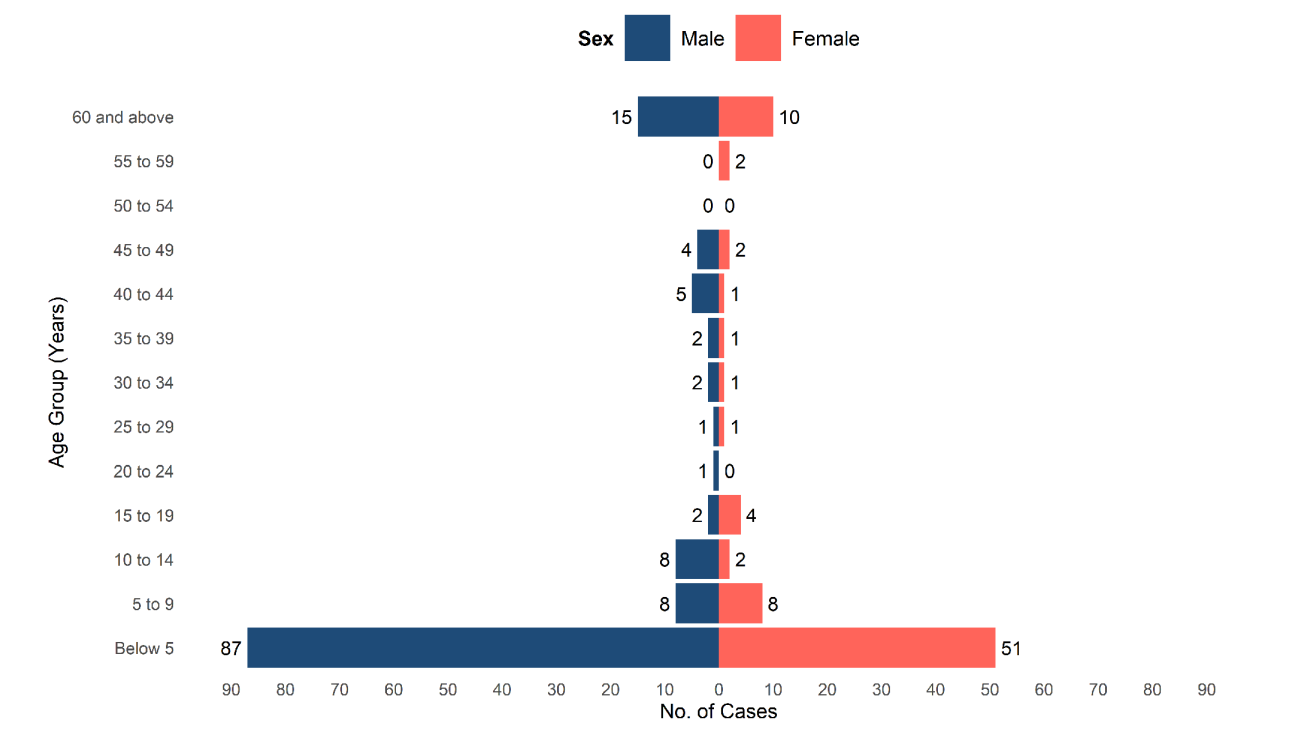


Figure 3. Age-Sex Distribution of Cases (N=218)
Philippines, Jan 4 to Sep 19, 2026

b. Distribution by Vaccination Status and Age Groups

Half (108 cases, 50%) received 2 or more doses of pertussis-containing vaccine, while a large proportion (99 cases, 45%) were unvaccinated. Of the 138 cases below five, 57 (41%) did not receive vaccination.

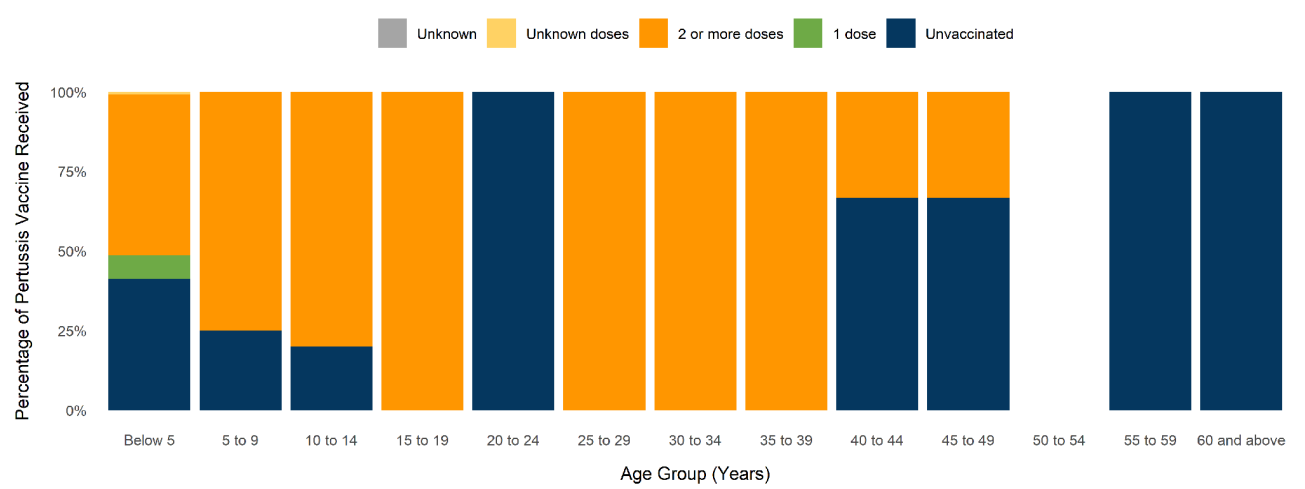


Figure 4. Age and Vaccination Status of Cases (N=218)
Philippines, Jan 4 to Sep 19, 2026

c. Symptoms and Hospitalization Status

- Of the 218 reported cases, cough lasting at least two weeks, paroxysmal cough, and posttussive vomiting (case-definition criteria) were the most common symptoms.
- Hospital admission was reported among 92% of cases.

Table 2. Symptoms and Hospitalization Profile of Cases (N=218), Philippines, Jan 4 to Sep 19, 2026

		No. of Cases	Percentage**
Symptoms*	Coughing lasting at least 2 weeks	179	82
	Paroxysmal Cough	177	81
	Posttussive Vomiting	83	38
	Inspiratory Whooping	33	15
	Apnea (Infants)	16	7
Hospitalization Status	Admitted	201	92
	Not Admitted	17	8

*Varied symptomatology and includes multiple responses

**Total percentage may not total to 100% due to rounding off of cases

d. Laboratory Data

Among the 218 cases, 158 (72%) were tested, of which only three cases tested positive for *Bordetella pertussis*.

Table 3. Laboratory Data Profile of Cases (N=218), Philippines, Jan 4 to Sep 19, 2026

		No. of Cases	Percentage*
Laboratory Result	Tested	158	72
	Positive (<i>Bordetella pertussis</i>)	3	2
	Positive (Non-pertussis organism)	5	3
	Pending	10	6
	Negative	140	89
	Not Tested/ Unknown	60	28

*Total percentage may not total to 100% due to rounding off of cases

e. Antibiotic Profile

Antibiotic therapy was received by 87% of cases.

Table 4. Antibiotic Profile of Cases (N=218), Philippines, Jan 4 to Sep 19, 2026

		No. of Cases	Percentage*
Antibiotic Therapy	Yes	189	87
	No	28	13
	Unknown	1	0

*Total percentage may not total 100% due to rounding off of cases

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026.



I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), 1,017 chikungunya cases were reported, a 40% increase from 725 cases reported in the same period last year. No deaths were reported this year, 100% decrease from one death (CFR: 0.14%) reported in the same period last year.
- Among the reported cases, 70 (7%) were confirmed while 947 (93%) were suspected.
- There is a 10% decrease from MW 34-35 (August 23 to September 05), with 36 cases reported, compared to MW 32-33 (August 09 to August 22), which had 40 cases. The number of cases recorded from the recent MW 36-37 (September 06 to September 19, 2026) was 31 cases. Cases may change as there may be delayed reports.

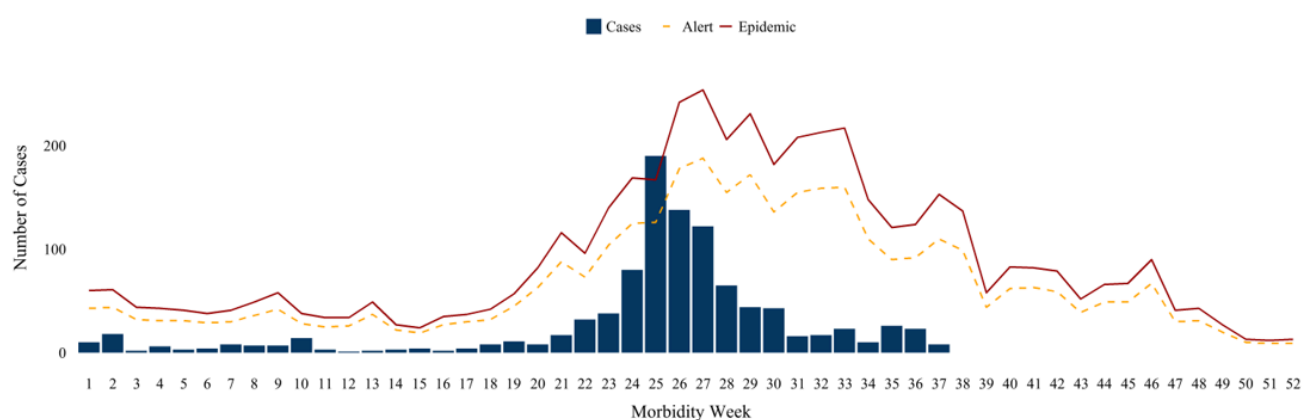


Figure 1. Cases by Morbidity Week (N=1,017)
Philippines, Jan 4 to Sep 19, 2026

II. Subnational Distribution of Cases

- Regions VIII, I and IV-A recorded the highest number of cumulative cases.
- Region VIII recorded the highest increase in cumulative number cases of 3845% from 11 cases last year to 434 this year, and the highest two-week growth rate of 267%. No regions reported deaths. (Table 2).
- No regions breached the epidemic threshold at least twice in the past four weeks (August 23 to September 19, 2026) (Figure 2).
- Clustering of cases was reported in two barangays across two regions, with Barangay Dilong, Tubo, Abra (CAR) reporting 5 cases within the past four weeks (August 23 to September 19).

Table 1. Barangays with Clustering* of Cases, Philippines, Aug 23 to Sep 19, 2026

Region	Province	Muncity	Barangay	No. of Cases
CAR	Abra	Tubo	Dilong	5
VIII	Samar	Paranas	Poblacion 3	4

*A cluster is defined as three or more chikungunya cases reported in a barangay within four consecutive weeks.

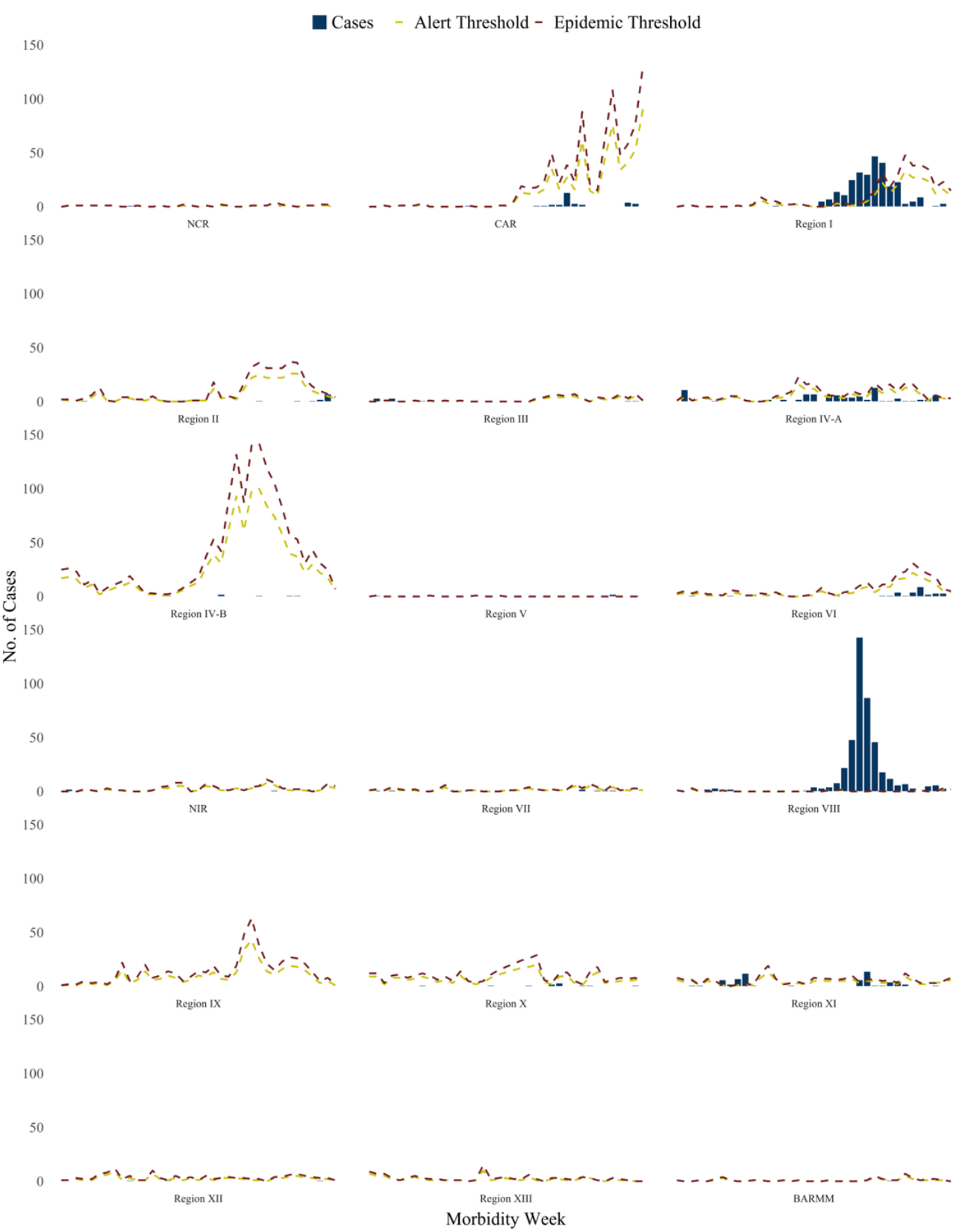


Figure 2. Regional Cases by Morbidity Week (N=1,017)
Philippines, Jan 4 to Sep 19, 2026

Table 2. Cumulative Number of Cases and Deaths (N=1,017), by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR^ 3-4 vs 5-6 weeks (%)	Deaths			CFR (%)*
	2025	2026	% Change		2025	2026	% Change	
VIII	11	434	↑3,845	↑267	0	0	-	0
I	35	276	↑689	↓92	0	0	-	0
IV-A	109	93	↓15	↑167	0	0	-	0
XI	100	67	↓33	↑	0	0	-	0
VI	132	36	↓73	↓62	0	0	-	0
CAR	24	35	↑46	↑	0	0	-	0
II	27	16	↓41	↑200	0	0	-	0
III	7	12	↑71	↑	0	0	-	0
VII	22	12	↓45	↓50	0	0	-	0
X	48	11	↓77	-	0	0	-	0
IV-B	63	6	↓90	↓100	0	0	-	0
NCR	17	6	↓65	-	0	0	-	0
V	1	4	↑300	↓100	0	0	-	0
NIR	49	3	↓94	-	0	0	-	0
XII	54	3	↓94	↑	0	0	-	0
BARMM	6	2	↓67	-	0	0	-	0
XIII	3	1	↓67	↓100	1	0	↓100	0
IX	17	0	↓100	-	0	0	-	0

^Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)
*Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases.

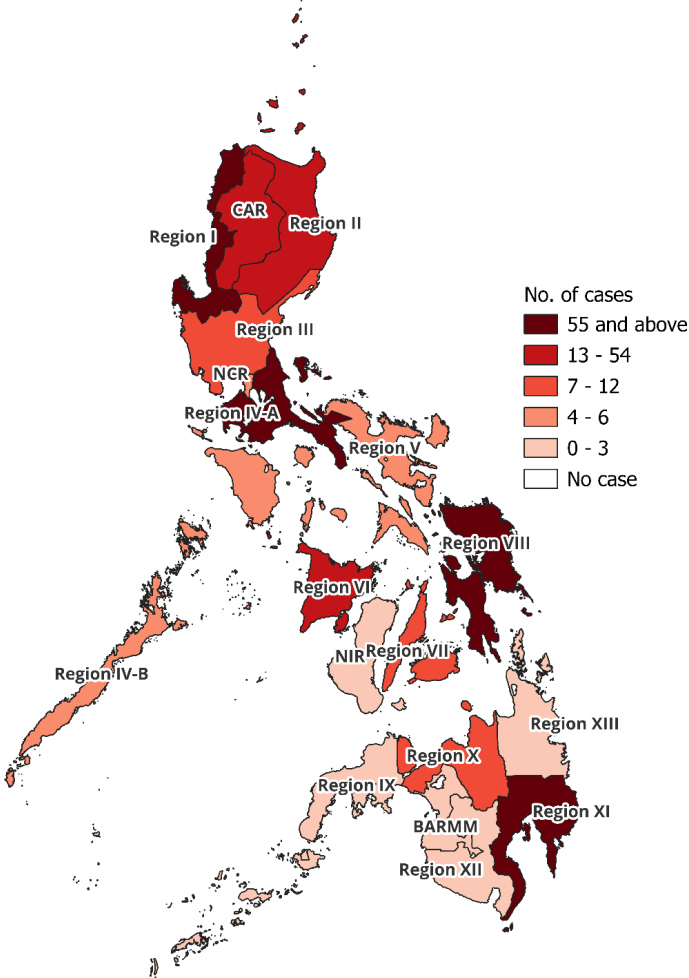


Figure 3. Regional Distribution of Cases (N=1,017)
Philippines, Jan 4 to Sep 19, 2026

III. Profile of Chikungunya Cases

- Cases were reported in all age groups, ranging from less than 1 to 90 years, with a median age of 30. Of these cases, 594 females (58%) and 423 males (42%) were reported.

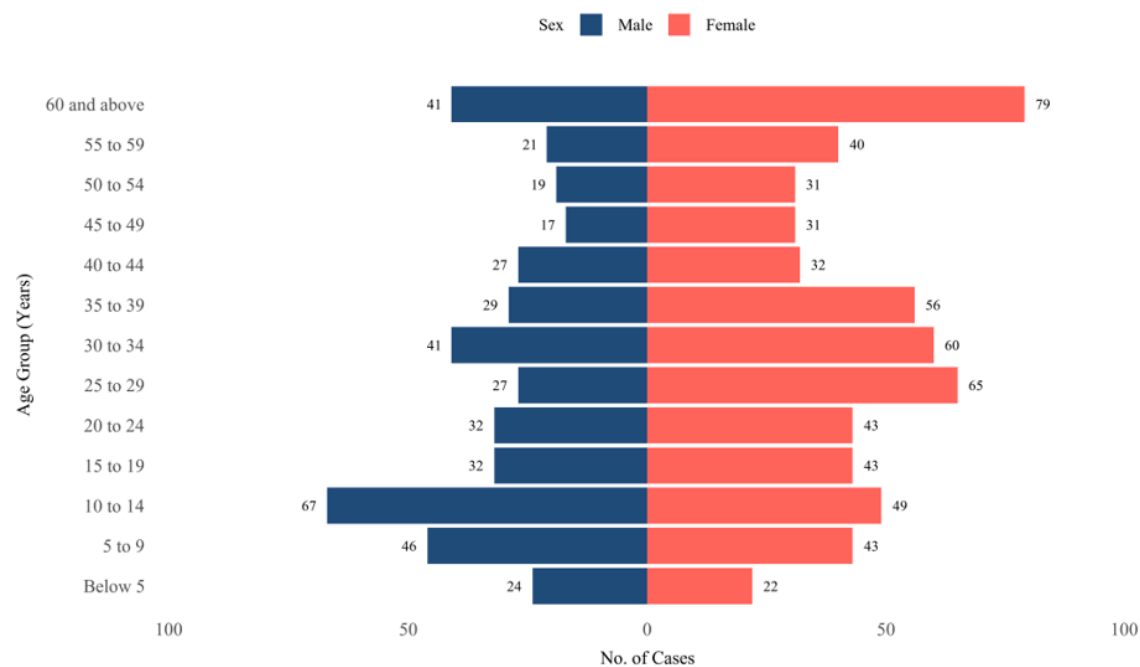


Figure 4. Age-Sex Distribution of Cases (N=1,017)
Philippines, Jan 4 to Sep 19, 2026

- Among the 1,017 cases, 190 (19%) were tested, of which 70 (37%) were positive.

Table 3. Laboratory Profile of Cases (N=1,017), Philippines, Jan 4 to Sep 19, 2026

Laboratory Result	No. of Cases		Percentage
	Tested	Not Tested	
Tested	190		19
Positive	70		37
Negative	68		36
Pending	52		27
Not Tested		827	81

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026.



Dengue

Weekly Surveillance Report 2026

I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), 161,925 dengue cases were reported, a 31% decrease from 233,155 cases reported in the same period last year. Among the reported cases, 62,837 (39%) were confirmed, 20,541 (13%) were probable, and 78,547 (49%) were suspected.
- A total of 634 deaths were reported (CFR: 0.39%), which was 29% lower than the 889 deaths reported last year (CFR: 0.38%).
- There is a 10% increase from MW 34-35 (August 23 to September 05), with 20,384 cases reported, compared to MW 32-33 (August 09 to August 22), which had 18,595 cases. The number of cases recorded from the recent MW 36-37 (September 06 to September 19, 2026) was 12,739 cases. Cases may change as there may be late consultations and reports. With the rainy season approaching, dengue cases may increase as conditions become more favorable for mosquito breeding.

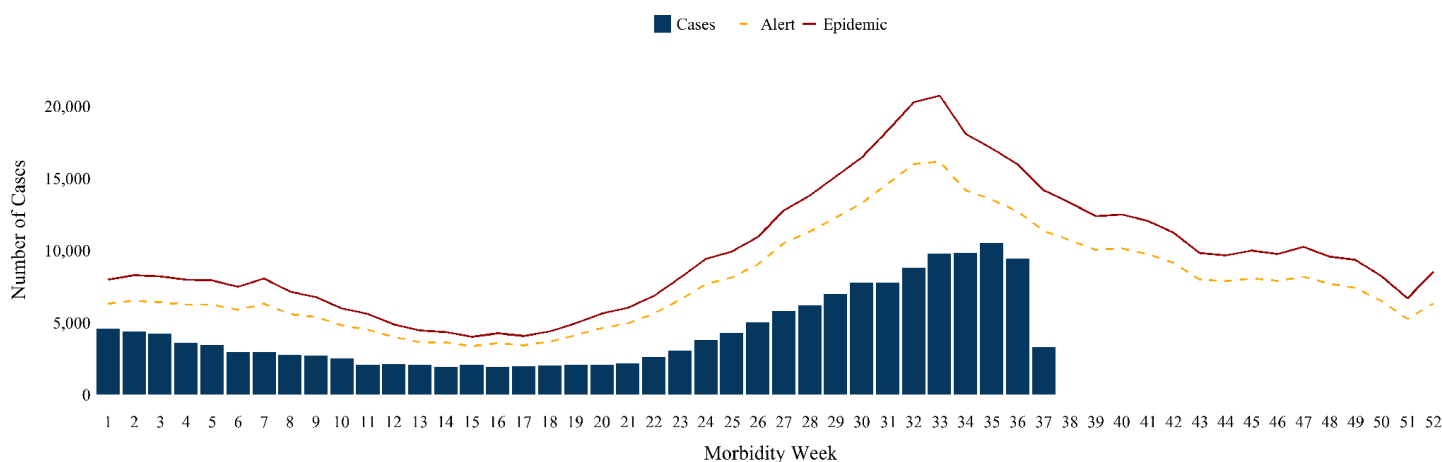


Figure 1. Dengue by Morbidity Week (N=161,925), Philippines, Jan 4 to Sep 19, 2026

II. Subnational Distribution of Cases

- Region II, V, and NCR recorded the highest increases in the recent 3-4 weeks when compared to the recent 5-6 weeks. (Table 1).
- Case fatality rate (CFR) is below 1% in all regions. However, Regions VII, V, and NIR represent the highest increases in deaths at 167%, 160%, and 50%, respectively (Table 1).
- Region V breached the epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026) (Annex A), while several provinces and highly urbanized cities across regions also exceeded alert or epidemic thresholds within the same period (Annex B).
- Clustering of cases was reported in 3,445 barangays across all regions. The regions with the highest number of barangay clusters are Region III (497, 14%), Region IV-A (372, 11%), and Region VII (337, 10%).

Note: Link for the dengue clustering as of MW 37 2026: <https://tinyurl.com/MW37-ClusteringDengue>

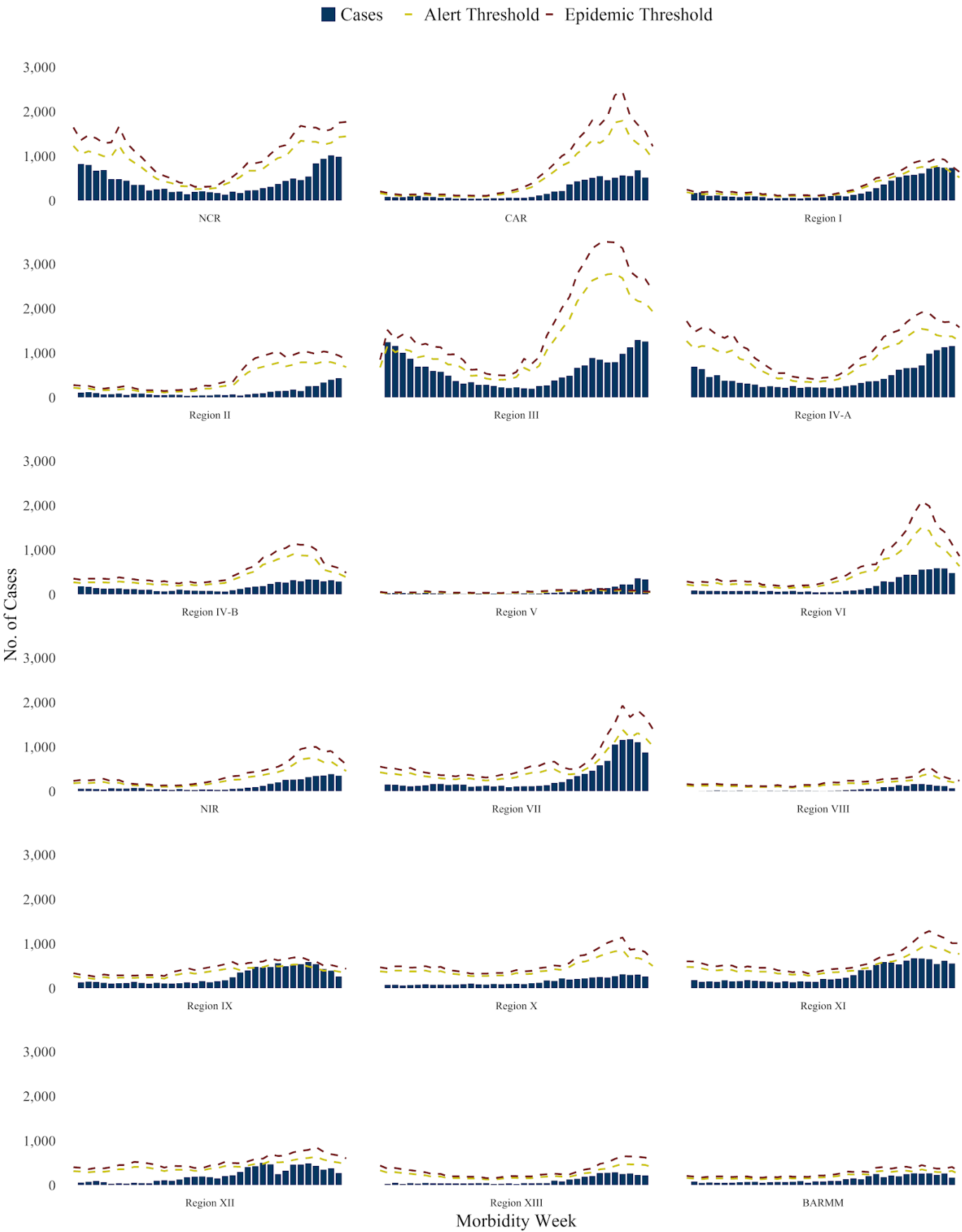


Figure 2. Regional Cases by Morbidity Week (N=161,925)
Philippines, Jan 4 to Sep 19, 2026

Table 1. Cumulative Number of Cases and Deaths (N=161,925), by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR [^]		Deaths			CFR [*]
	2025	2026	% Change	3-4 vs 5-6 weeks (%)	2025	2026	% Change	2026	
III	47,273	23,674	↓50	↑35.54	151	80	↓47	0.34	
IV-A	35,528	17,547	↓51	↑28.22	113	64	↓43	0.36	
NCR	31,707	16,380	↓48	↑41.46	121	58	↓52	0.35	
VII	8,498	12,188	↑43	↑3.10	27	72	↑167	0.59	
XI	11,372	12,011	↑6	↓12.12	84	40	↓52	0.33	
IX	6,423	9,974	↑55	↓26.08	33	46	↑39	0.46	
I	11,235	9,540	↓15	↑11.50	41	20	↓51	0.21	
XII	9,907	8,422	↓15	↓20.74	25	22	↓12	0.26	
CAR	5,904	8,326	↑41	↑14.07	14	18	↑29	0.22	
VI	12,208	7,324	↓40	↑4.64	35	32	↓9	0.44	
IV-B	11,535	6,513	↓44	↓7.68	72	40	↓44	0.61	
X	10,140	5,834	↓42	↑3.28	63	33	↓48	0.57	
BARMM	4,996	5,133	↑3	↓5.82	25	20	↓20	0.39	
NIR	7,394	4,949	↓33	↑11.64	18	27	↑50	0.55	
II	8,990	4,923	↓45	↑43.05	24	9	↓62	0.18	
XIII	5,478	4,039	↓26	↓9.20	27	25	↓7	0.62	
V	1,845	2,980	↑62	↑42.92	10	26	↑160	0.87	
VIII	2,722	2,168	↓20	↓19.76	6	2	↓67	0.09	

[^]Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)
^{*}Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases.

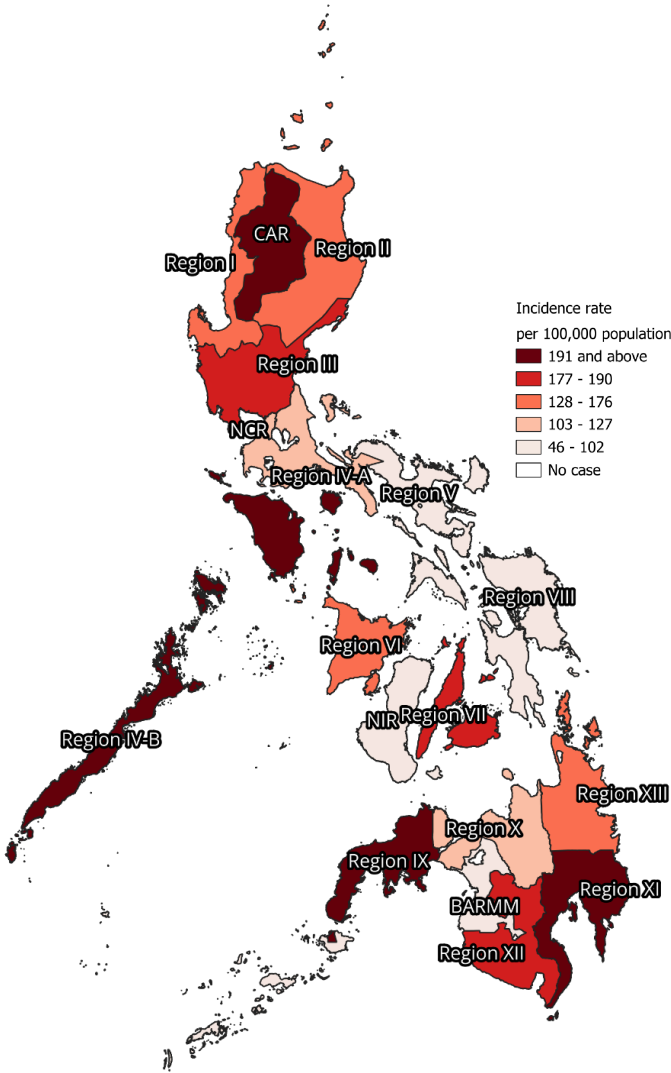
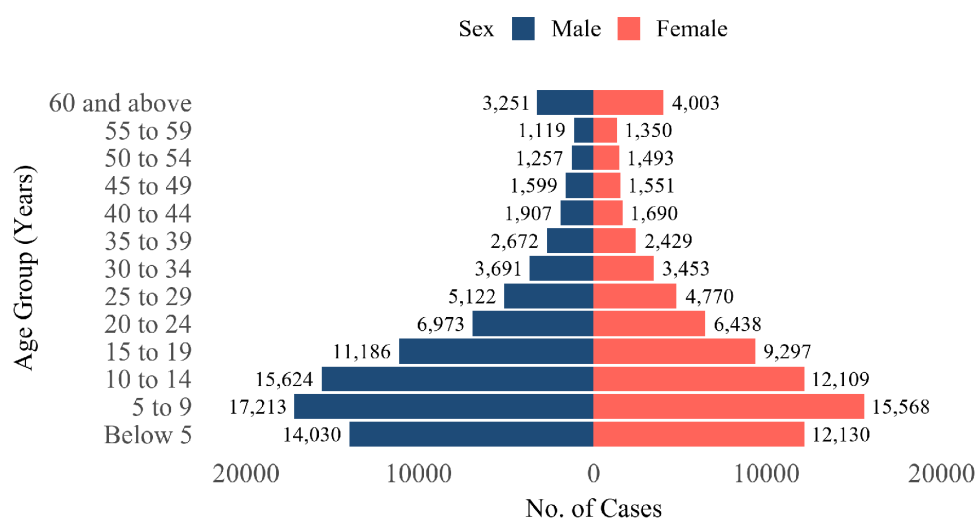


Figure 3. Regional Incidence Rates (N=161,925) Philippines, Jan 4 to Sep 19, 2026

III. Profile of Dengue Cases

- Cases were reported in all age groups, ranging from less than 1 year to 109 years, with a median age of 13. The distribution was generally comparable, with 47% females (76,281 cases) and 53% males (85,644 cases).



**Figure 4. Age-Sex Distribution of Cases (N=161,925)
Philippines, Jan 4 to Sep 19, 2026**

- Of the total 119,057 tested cases, 80% were tested for NS1, 30% for IgM antibody, and <1% via PCR.

Table 2. Laboratory Profile of Dengue Cases* (N=161,925), Jan 4 to Sep 19, 2026

	No. of Cases	Percentage
Laboratory Tested	119,057	74
Nonstructural Protein 1 (NS1) Test	94,737	80
NS1 Positive	62,332	66
NS1 Equivocal	36	<1
NS1 Negative	31,106	33
NS1 Pending	1,263	1
IgM Antibody Test	35,259	30
IgM Positive	23,109	66
IgM Equivocal	82	<1
IgM Negative	11,739	33
IgM Pending	329	1
Polymerase Chain Reaction (PCR) Test	797	1
PCR Positive	473	59
Dengue 1	103	22
Dengue 2	67	14
Dengue 3	139	29
Dengue 4	31	7
Serotype Not Specified	133	28
PCR Equivocal	2	<1
PCR Negative	291	37
PCR Pending	31	4
Not Tested	42,868	26

- Cases include suspected, probable, and confirmed cases.

-The tested cases include those that underwent IgG testing, which is not shown in the table, and some cases may have undergone more than one test. Therefore, percentages may not total 100%.

- Percentages are calculated using the total number of tests performed for each parameter; for dengue serotype results, percentages are based only on samples that tested positive by PCR.

- 150,277 (93%) cases and 616 (97%) deaths are admitted to healthcare facilities.

Table 3. Cases and Deaths by Hospitalization Status, Jan 4 to Sep 19, 2026

Hospitalization Status	Cases	%	Deaths	%
Not Admitted	11,648	7%	18*	3%
Admitted	150,277	93%	616	97%
Total	161,925	100%	634	100%

* For verification

- 91,372 (56%) cases manifested dengue with warning signs (56%), while 300 (47%) of deaths manifested severe dengue

Table 4. Cases and Deaths by Clinical Classification, Jan 4 to Sep 19, 2026

Clinical Classification	Cases	%	Deaths	%
Dengue Without Warning Signs	68,578	42%	42*	7%
Dengue With Warning Signs	91,372	56%	292	46%
Severe Dengue	1,975	1%	300	47%
Total	161,925	100%	634	100%

* For verification

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026.

IV. Event-based Surveillance and Response

Increasing Dengue Cases in Albay (1,785 cases, 14 deaths)

- In April–August 2026, 14 dengue deaths were recorded in Albay, a significant increase compared with 3 deaths reported in 2025. On September 10, 2026, the Sangguniang Panlalawigan ng Albay approved the declaration of a State of Calamity due to the increasing number of dengue cases in the province of Albay. Cases involve individuals aged 2 months to 92 years old (Median: 15 years). Nine (9) cities/municipalities reported increases in dengue cases. The largest percentage increases were observed in Malilipot (884%), Daraga (357%), Bacacay (344%), and Sto. Domingo (300%). Laboratory tests showed 967/1,129 (86%) were positive for NS1, 672/765 (88%) were positive for Dengue Duo and 38/44 (86%) were positive for PCR. In response, the hospitals, RHUs, and CHOs managed the cases and dengue testing. The CHD RESU V conducted case monitoring, reporting, coordination, and specimen collection. The PHO, Dengue Program Coordinator, and Environmental Sanitation personnel conducted Dengue Caravans, larvicide provision, and LGU assistance. The PHO also conducted health promotion activities and distributed IEC materials.

Increasing Dengue Cases in Brgy. Poblacion, Kabugao, Apayao (110 cases, 0 deaths)

- Dengue cases in Kabugao, Apayao, showed an increasing trend and breached the epidemic threshold starting MW 26 and peaked on MW 34 (17 cases). A decrease in cases was observed starting MW 35. A total of 110 dengue cases were reported involving individuals aged 1 to 62 years old (Median: 15 years). Of these, 76 were admitted while 34 were managed as outpatients, and 20 (18%) were classified as dengue with warning signs. Laboratory tests showed 69/110 (63%) were positive for NS1, while 10/18 (56%) were positive for IgM. In response, Apayao Provincial Hospital and RHU Kabugao clinically managed the cases, facilitated testing and conducted initial investigations. The MHO Kabugao conducted environmental surveillance for breeding sites, larval surveillance, fever surveillance in the communities, and health education in the community through social media posting.

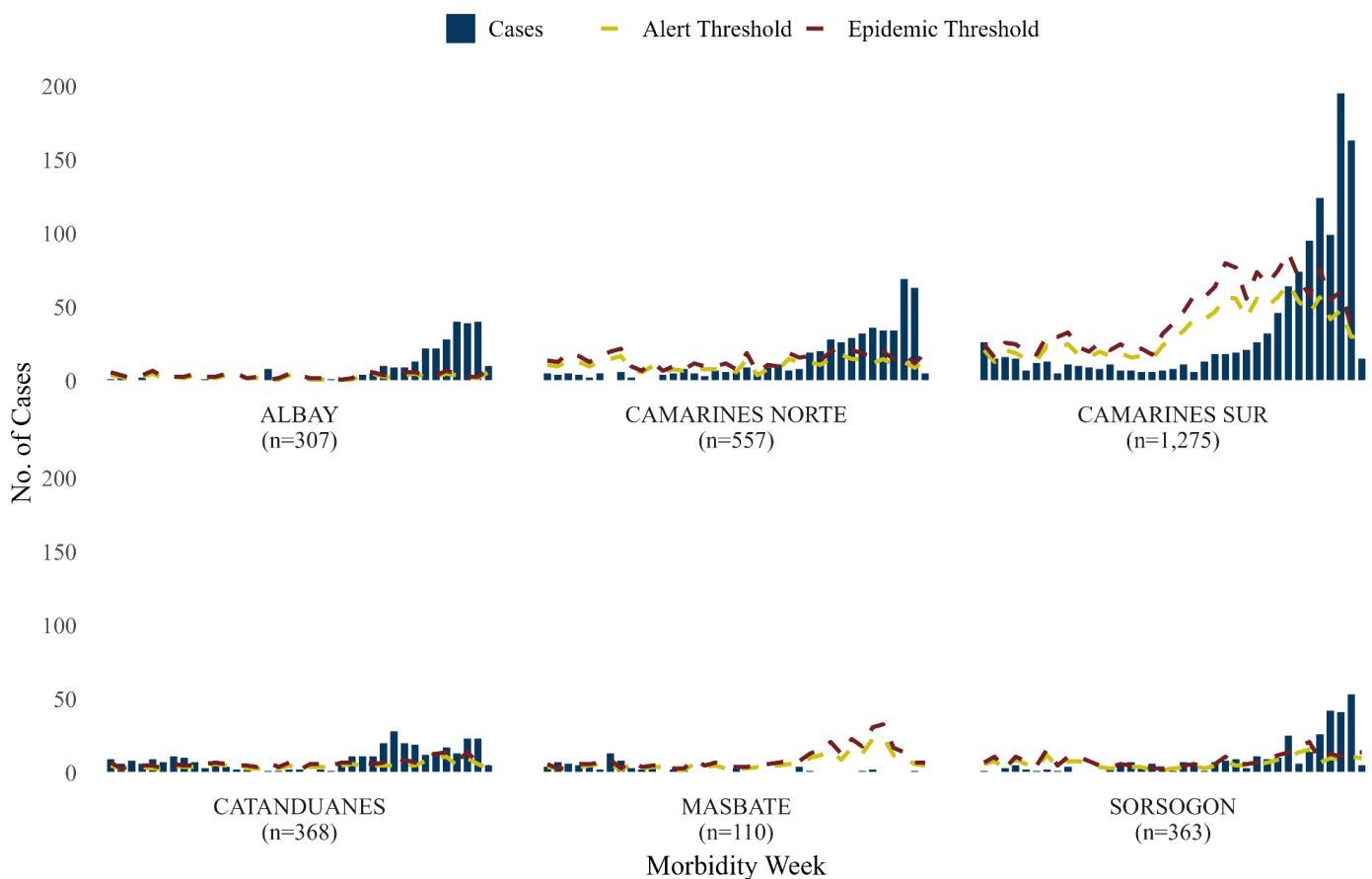
Increasing Dengue Cases in Brgy. Cogon, Tagbilaran City, Bohol (62 cases, 0 deaths)

- Dengue cases in Tagbilaran City showed an increasing trend from MW 27-35. A total of 62 dengue cases were reported involving individuals aged 1 to 35 years old (Median: 15 years). Of the reported cases, 41 were admitted while 21 were managed as outpatients. Laboratory tests showed 35/39 (90%) were positive for NS1. In response, The hospitals clinically managed the cases and facilitated collection of specimens for dengue testing. CESU Tagbilaran conducted case investigation, mapping of cases, assessment of possible risk factors and breeding sites, misting operations and health education on dengue prevention and control.

ANNEX A

Subregional Analyses of Region V

- Region V continues to breach the epidemic thresholds, and the overall trend is still increasing.
- Albay, Camarines Norte, Camarines Sur, Catanduanes, and Sorsogon breached the epidemic threshold for at least two weeks over the past four weeks.
- A cumulative total of 2,980 dengue cases and 26 deaths (CFR: 0.87%) were reported in the region from January 4 to September 19, 2026.
- Clustering was reported in 103 barangays, with Barangay Cararayan, City of Naga reporting the most, with 23 cases in the past four weeks.
- Among the reported cases, 1,505 (51%) are suspected while 1,218 cases (41%) were confirmed.
- Of the total 1,789 tested cases, 82% were tested for NS1, 21% for IgM antibody, and <1% via PCR (some cases may have undergone more than one test; therefore, percentages may not total 100%).
 - Of those tested for NS1, 1,207 (83%) are positive
 - Of those tested for IgM, 313 (85%) are positive
 - Of those tested for PCR, 57 (69%) are positive
- Cases reported ranged in age from less than 1 year to 95 years (median: 40 years). Among the reported cases, 1,534 (51%) are males.



**Figure A.1. Provincial Cases by Morbidity Week (n=2,980)
Region V, Jan 4 to Sep 19, 2026**

ANNEX B

Provinces and Highly Urbanized Cities (P/HUCs) with Threshold Breaches, Philippines, Aug 23 to Sep 19, 2026

Geographic Area	Thresholds Breached in Past 4 MWs
Region I	
Ilocos Norte	Epidemic
Ilocos Sur	Alert
La Union	Epidemic
Region III	
Aurora	Epidemic
Bataan	Alert
Region IV-A	
Quezon	Epidemic
Region V	
Albay	Epidemic
Camarines Norte	Epidemic
Camarines Sur	Epidemic
Catanduanes	Epidemic
Sorsogon	Epidemic
Region VI	
Aklan	Epidemic
Iloilo	Epidemic
Region VII	
Bohol	Epidemic
City of Cebu	Alert
City of Lapu-Lapu	Alert
Region IX	
City of Zamboanga	Epidemic
Zamboanga Sibugay	Epidemic
Region XI	
Davao Oriental	Alert
BARMM	
Basilan	Epidemic
Sulu	Epidemic
CAR	
Abra	Alert
Apayao	Epidemic
Kalinga	Alert
Mountain Province	Epidemic
Region XIII	
Agusan Del Sur	Epidemic
NCR	
City of Mandaluyong	Epidemic
City of Manila	Alert
Quezon City	Alert

**Note: Only P/HUCs with at least two alert or epidemic threshold breaches in the past four weeks are shown.*

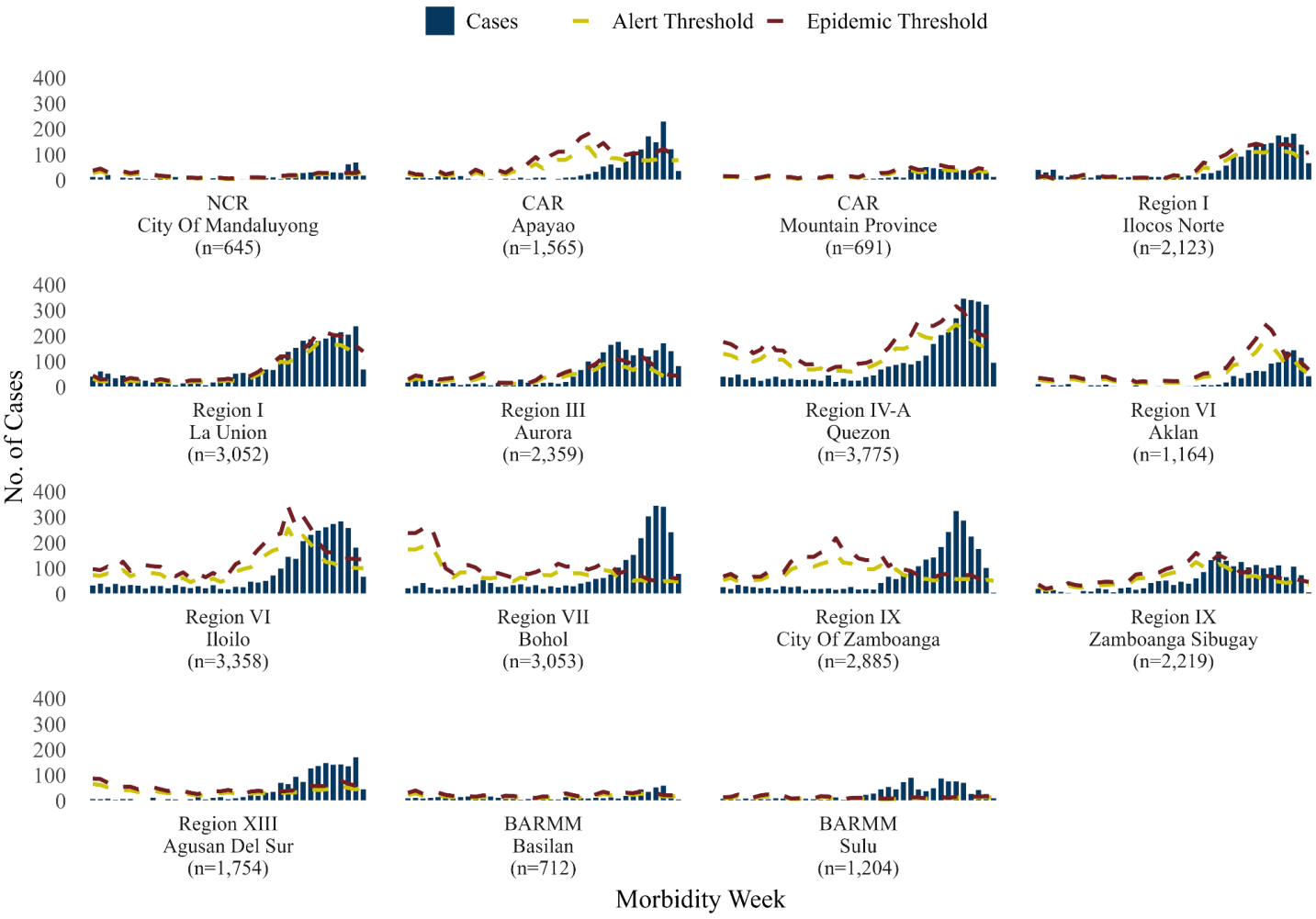


Figure B.1. Provinces and Highly Urbanized Cities Exceeding Epidemic Thresholds
Philippines, Jan 4 to Sep 19, 2026



Leptospirosis

Weekly Surveillance Report 2026

I. National Trend

As of Morbidity Week 37 (January 4 to September 19, 2026), 11,386 cases have been reported, a 46% increase from 7,782 cases during the same period last year. A total of 581 deaths were reported (CFR: 5.1%), a 29% decrease from 821 deaths (CFR: 10.55%) in the same period last year.

There is a 38% increase from MW 34-35 (August 23 to September 5, 2026) with 3,480 cases reported, compared to MW 32-33 (August 9 to August 22, 2026), which had 2,522 cases. The recent two MWs were not used for growth rate comparison due to the possibility of delayed reports.

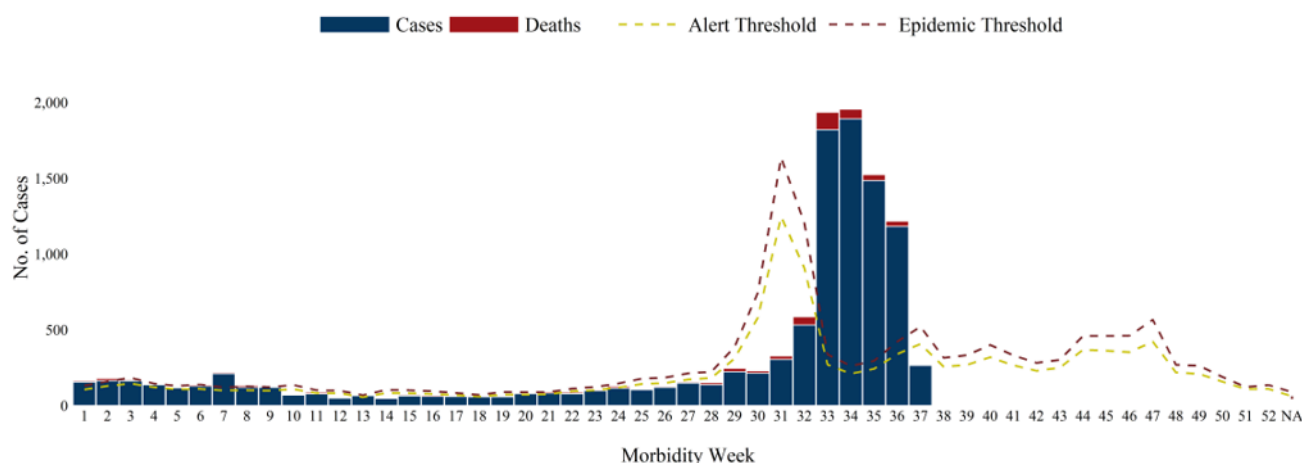


Figure 1b. Cases and Deaths by Morbidity Week (N = 11,386)
Philippines, Jan 4 to Sep 19, 2026
Deaths are a subset of reported cases; bar height is the total cases for the week.

II. Subnational Distribution of Cases

- Regions III, X, and NCR recorded the highest percentage increase in cases compared with the same period last year (*Table 1*).
- NCR, CAR, Regions I, III, IV-A, IV-B, and VI breached their epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026) (*Figure 2*), as further described in Annexes A, B, C, D, E, F, and G.
- NCR (331.27), Region II (167.28), and CAR (157.49) reported the highest incidence rates per 1,000,000 population as of Morbidity Week 37 (*Figure 3*).
- Clustering was reported in 426 barangays across 7 regions over the past four weeks, with a maximum of 154 cases recorded in a barangay

Note: Link for the Clustering as of MW 37: <https://tinyurl.com/ClusteringMW37-Leptospirosis>

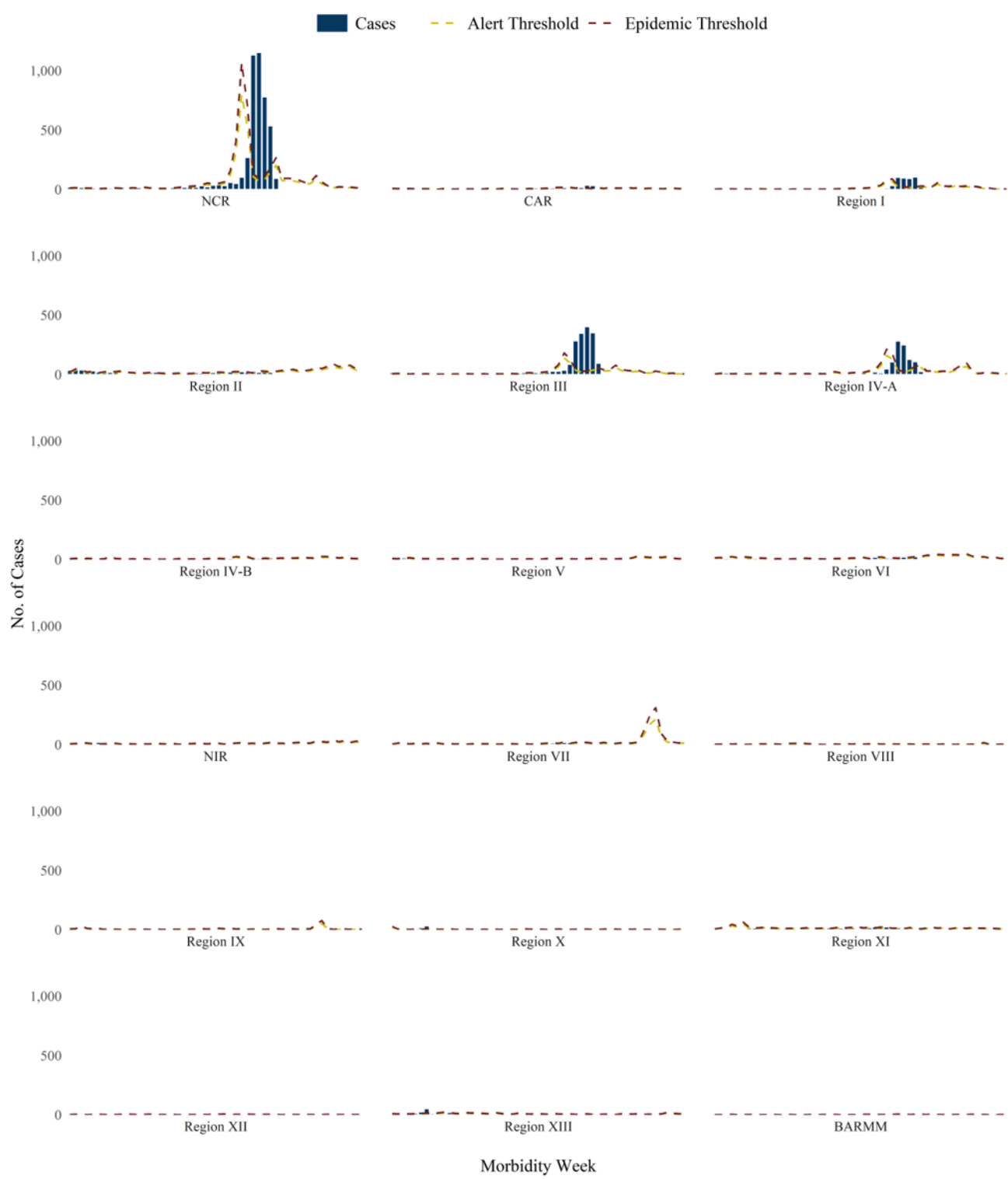


Figure 2. Regional Cases by Morbidity Week (N = 11,386)
Philippines, Jan 4 to Sep 19, 2026

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR^ 3-4 vs 5-6 weeks (%)	Deaths			CFR (%)*
	2025	2026	% Change		2025	2026	% Change	
NCR	2,885	4,643	↑61	↑39%	276	188	↓32	4.05
III	658	1,894	↑188	↑103%	88	88	-	4.65
IV-A	823	1,135	↑38	↓3%	79	70	↓11	6.17
II	602	635	↑5	↓9%	31	17	↓45	2.68
I	395	556	↑41	↑40%	56	37	↓34	6.65
XI	443	416	↓6	↓32%	69	31	↓55	7.45
VI	276	345	↑25	↑42%	27	38	↑41	11.01
XIII	323	310	↓4	↓40%	48	20	↓58	6.45
CAR	197	296	↑50	↑83%	12	14	↑17	4.73
VII	234	282	↑21	↓38%	34	28	↓18	9.93
IV-B	232	219	↓6	↓12%	31	13	↓58	5.94
NIR	196	196	-	↓50%	19	13	↓32	6.63
V	107	115	↑7	↑300%	8	9	↑12	7.83
X	60	114	↑90	↑20%	3	4	↑33	3.51
IX	132	76	↓42	↓75%	17	3	↓82	3.95
XII	55	69	↑25	↑20%	11	3	↓73	4.35
VIII	129	65	↓50	-	9	4	↓56	6.15
BARMM	35	20	↓43	↓100%	3	1	↓67	5.00

^Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)
*Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases.

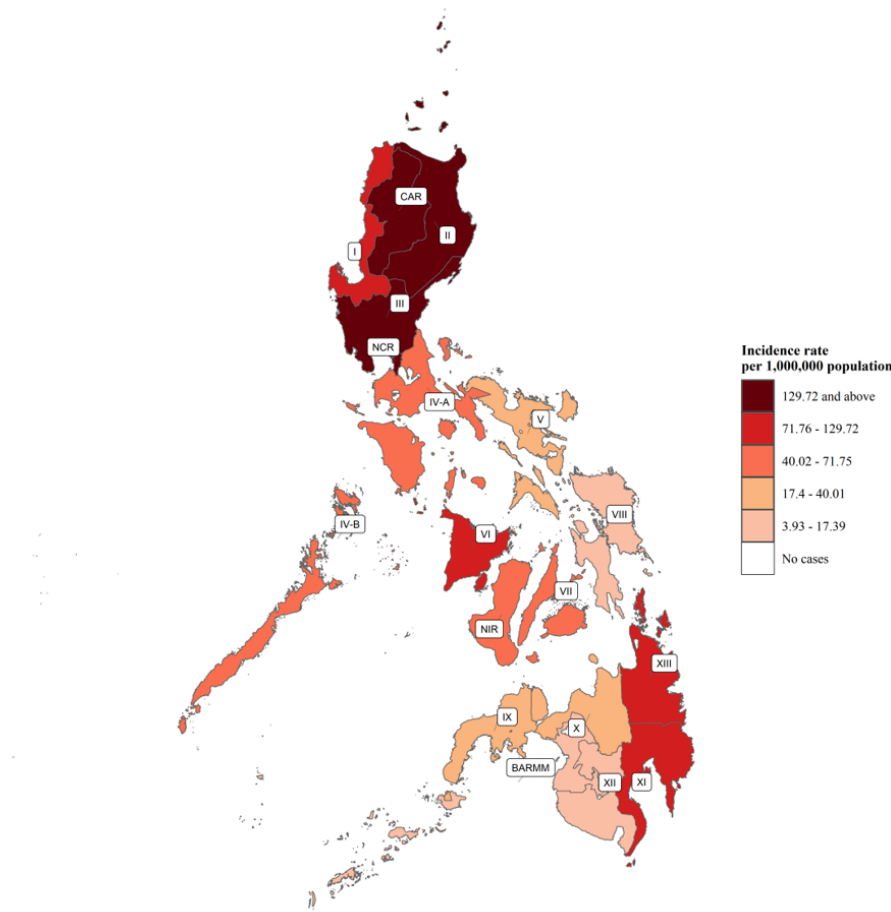


Figure 3. Regional Incidence Rates
Philippines, Jan 4 to Sep 19, 2026

III. Profile of Cases

The youngest reported age was less than 1 year, the oldest was 91 years, and the median age was 33 years. Among the reported cases, 79% were male, 67% had flood-related exposure, and 78% were classified as suspect cases.

Table 2. Profile of Cases (N = 11,386), Philippines, Jan 4 to Sep 19, 2026

		Median	Range
Age (in years)		33	0-91
		No. of Cases	Percentage
Sex			
	Male	8,986	79
	Female	2,400	21
Exposure History			
	Flood related	7,613	67
	Agriculture related	1,104	10
	Mud exposure	393	3
	Ingestion of contaminated foods/drinks	65	1
	Others	1,201	11
	Unknown	1,010	9
Case Classification			
	Suspect	8,866	78
	Probable	2,216	19
	Confirmed	304	3

Table 3. Top 10 Disease Reporting Units by Case Count, Philippines, Jan 4 to Sep 19, 2026

Disease Reporting Unit (DRU)*	DRU Address			2026	
	Region	Province	Municipality/City	Cases	Deaths
San Lazaro Hospital	NCR	Manila	Santa Cruz	772	24
Jose B. Lingad Memorial General Hospital	III	Pampanga	San Fernando City	423	24
East Avenue Medical Center	NCR	Quezon City	Quezon City	338	16
Ospital ng Maynila Medical Center	NCR	Manila	Malate	267	13
Dr. Paulino J. Garcia Memorial Research and Medical Center	III	Nueva Ecija	Cabanatuan City	260	19
Region I Medical Center	I	Pangasinan	Dagupan City	220	16
National Kidney And Transplant Institute	NCR	Quezon City	Quezon City	210	17
Taguig-Pateros District Hospital	NCR	Taguig City	Taguig City	193	1
Quirino Memorial Medical Center	NCR	Quezon City	Quezon City	191	4
Amang Rodriguez Memorial Medical Center	NCR	Marikina City	Marikina City	164	13

* List includes only the top ten disease reporting units by case count

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026.

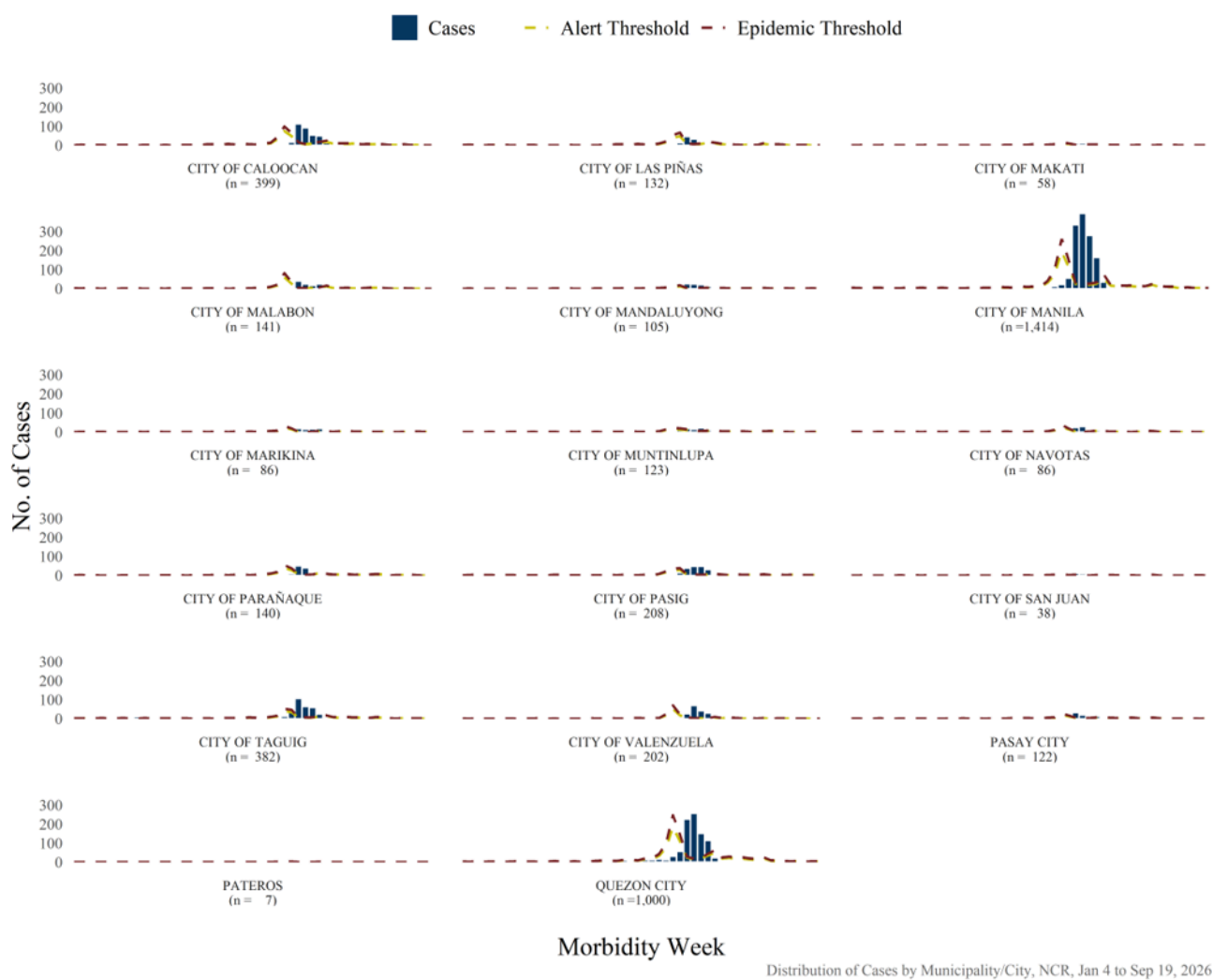
IV. Event-based Surveillance and Response

- Increasing Leptospirosis Cases in Guimaras (23 cases, 0 deaths)**
 - From January 4 to September 14, 2026 (MW 1-36), Guimaras recorded 23 cases representing a 130% increase compared with the 10 cases during the same period in 2025. Epidemic threshold was breached during MW 32. Cases involved individuals aged 6 to 60 years old (Median: 23 years). Of the 23 cases, 15 (65%) were probable, 7 (30%) were suspect, and 1 (5%) was confirmed. Cases’ reported exposures included flood (6), agriculture (5), other exposures (9), and unknown exposures (3). Laboratory tests showed that 16/22 (73%) samples were positive by Rapid Diagnostic Test (RDT). One case tested for Microscopic Agglutination Test (MAT) had positive result. In response, DRUs, hospitals, and RHUs provided clinical management to the cases and collected specimens for testing. The Western Visayas Medical Center – Sentinel Laboratory (WVMC-SNL) conducted confirmatory testing. RHUs also provided prophylaxis to farmers and other exposed individuals and conducted case investigations while the PHO Guimaras distributed IEC materials on leptospirosis to the communities.

ANNEX A

Subregional Analyses of NCR

- As of Morbidity Week 37 (January 4 to September 19, 2026), 4,643 leptospirosis cases were reported, a 61% increase from 2,885 cases reported in the same period last year. Cases peaked in MW 33 and 34, but cases have shown a downward trend since.
- NCR exceeded its epidemic threshold in 3 of the past 4 morbidity weeks (*Figure 2*). Within the region, the City of Caloocan, City of Makati, City of Malabon, City of Mandaluyong, City of Manila, City of Marikina, City of Muntinlupa, City of Navotas, City of Pasig, City of San Juan, City of Taguig, City of Valenzuela, Pasay City, Pateros, Quezon City, and City of Parañaque exceeded their epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026).
- A total of 188 deaths were reported (CFR: 4.05%), which was 32% lower than the 276 deaths reported last year (CFR: 9.57%).
- Clustering of cases was reported in 123 barangays. Of these, Barangay 649 reported the most cases, with 96 in the past four weeks.



- Cases were reported across all age groups, ranging from less than 1 year to 89 years, with a median age of 32. The distribution was 74% males (3,444 cases) and 26% females (1,199 cases).
- Among the reported cases, 81% were suspect cases, 60% were unemployed, and 82% had flood-related exposure.

Annex A.1. Profile of Cases (n=4,643), NCR, Jan 4 to Sep 19, 2026

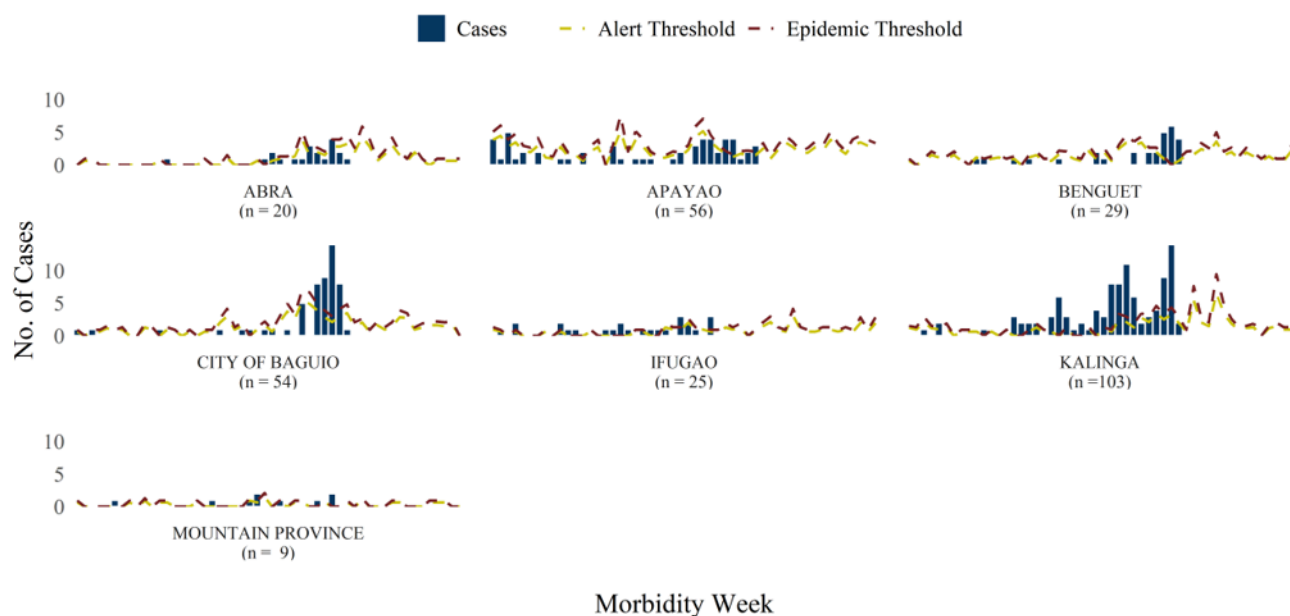
	No. of Cases	Percentage
Case Classification		
Suspect	3,775	81
Probable	765	16
Confirmed	103	2
Exposure History		
Flood related	3,806	82
Ingestion of contaminated foods/drinks	23	<1
Mud exposure	22	<1
Agriculture related	15	<1
Others	469	10
Unknown	308	7
Occupation*		
None	2,776	60
Student	426	9
Driver/rider	280	6
Construction worker	219	5
Market worker/vendor	137	3
Laborer	105	2
Factory worker	51	1
Others	649	14

*Occupations directly extracted from encoded data

**Other occupations one percent or below the total number of cases are grouped under the 'Others' category

ANNEX B Subregional Analyses of CAR

- As of Morbidity Week 37 (January 4 to September 19, 2026), 296 leptospirosis cases were reported, a 50% increase from 197 cases reported in the same period last year. Cases have consistently been higher compared to past years, with an observed increasing trend.
- CAR exceeded its epidemic threshold in 3 of the past 4 morbidity weeks (*Figure 2*). Within the region, Benguet, City of Baguio, and Kalinga exceeded their epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026).
- A total of 14 deaths were reported (CFR: 4.73%), which was 17% higher than the 12 deaths reported last year (CFR: 6.09%).
- Clustering of cases was reported in 5 barangays. Of these, Barangay Bulanao reported the most cases, with 6 in the past four weeks.



Distribution of Cases by Province/City, CAR, Jan 4 to Sep 19, 2026

- Cases were reported across all age groups, ranging from 1 years to 86 years, with a median age of 29. The distribution was 77% males (227 cases) and 23% females (69 cases).
- Among the reported cases, 50% were probable cases, 28% were unemployed, and 26% had agriculture-related exposure.

Annex B.1. Profile of Cases (n=296), CAR, Jan 4 to Sep 19, 2026

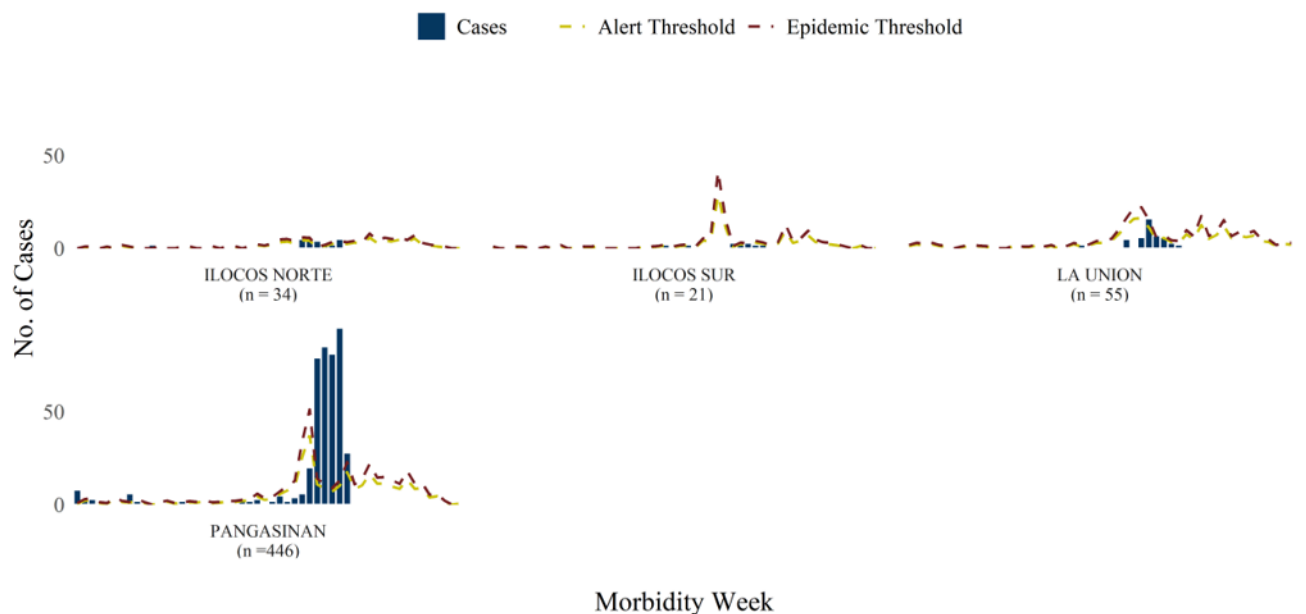
	No. of Cases	Percentage
Case Classification		
Probable	149	50
Suspect	139	47
Confirmed	8	3
Exposure History		
Agriculture related	76	26
Flood related	46	16
Mud exposure	9	3
Others	91	31
Unknown	74	25
Occupation*		
None	84	28
Farmer	76	26
Student	57	19
Construction worker	16	5
Laborer	14	5
Miner	11	4
Driver/rider	6	2
Housekeeper	5	2
Others	27	9

*Occupations directly extracted from encoded data

**Other occupations one percent or below the total number of cases are grouped under the 'Others' category

ANNEX C Subregional Analyses of Region I

- As of Morbidity Week 37 (January 4 to September 19, 2026), 556 leptospirosis cases were reported, a 41% increase from 395 cases reported in the same period last year. Recent cases started to peak last MW 33, forming a sustained plateau.
- Region I exceeded its epidemic threshold in 4 of the past 4 morbidity weeks. (*Figure 2*). Within the region, Pangasinan and Ilocos Norte exceeded their epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026).
- A total of 37 deaths were reported (CFR: 6.65%), which was 34% lower than the 56 deaths reported last year (CFR: 14.18%).
- Clustering of cases was reported in 10 barangays. Of these, Barangay Bonuan Gueset reported the most cases, with 13 in the past four weeks.



Distribution of Cases by Province/City, Region I, Jan 4 to Sep 19, 2026

- Cases were reported across all age groups, ranging from less than 1 year to 91 years, with a median age of 35. The distribution was 82% males (454 cases) and 18% females (102 cases).
- Among the reported cases, 82% were suspect cases, 35% were unemployed, and 77% had flood-related exposure.

Annex B.1. Profile of Cases (n=556), Region I, Jan 4 to Sep 19, 2026

	No. of Cases	Percentage
Case Classification		
Suspect	456	82
Probable	97	17
Confirmed	3	1
Exposure History		
Flood related	430	77
Agriculture related	73	13
Mud exposure	13	2
Ingestion of contaminated foods/drinks	1	<1
Others	23	4
Unknown	16	3
Occupation*		
None	196	35
Farmer	92	17
Construction worker	66	12
Market worker/vendor	32	6
Driver/rider	25	5
Student	25	4
Laborer	18	3
Fisherman	17	3
Restaurant worker	8	1
Others	77	14

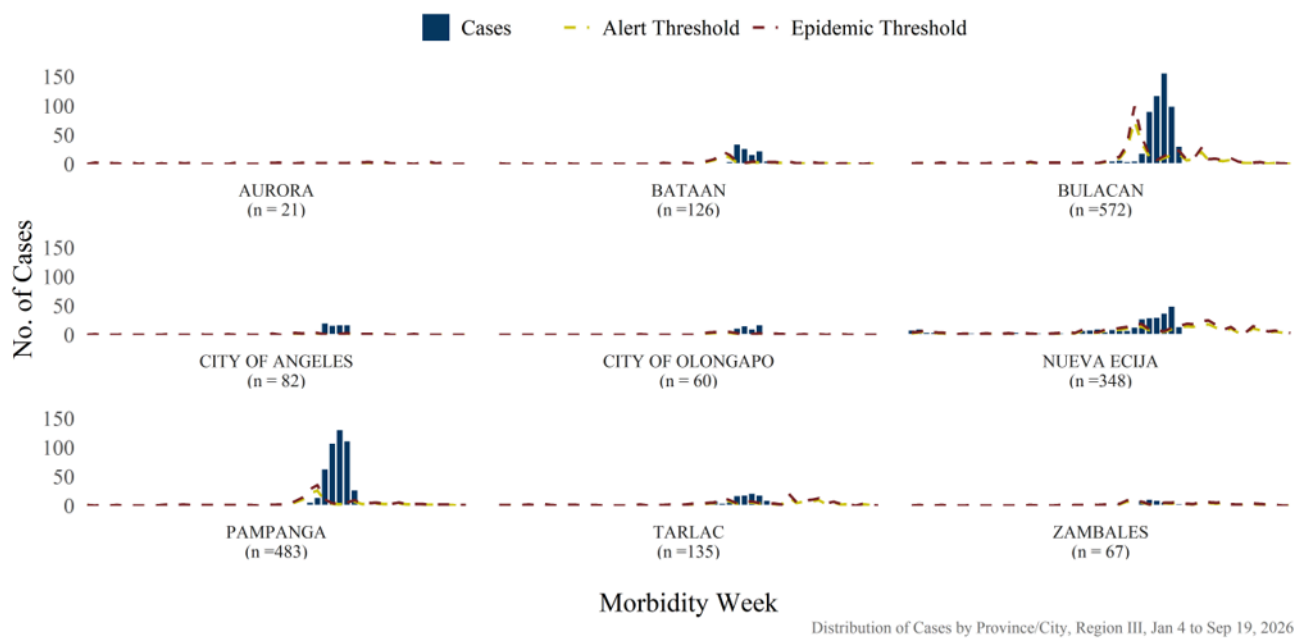
*Occupations directly extracted from encoded data

**Other occupations one percent or below the total number of cases are grouped under the 'Others' category

ANNEX D

Subregional Analyses of Region III

- As of Morbidity Week 37 (January 4 to September 19, 2026), 1,894 leptospirosis cases were reported, a 188% increase from 658 cases reported in the same period last year. Cases peaked in MW 33, showing an upward trend.
- Region III exceeded its epidemic threshold in 4 of the past 4 morbidity weeks (*Figure 2*). Within the region, Bataan, Bulacan, City of Olongapo, Pampanga, Tarlac, City of Angeles, Nueva Ecija, Zambales, and Aurora exceeded their epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026).
- A total of 88 deaths were reported (CFR: 4.65%), which was equal to the 88 deaths reported last year (CFR: 13.37%).
- Clustering of cases was reported in 70 barangays. Of these, Barangay San Jose reported the most cases, with 14 in the past four weeks.



- Cases were reported across all age groups, ranging from less than 1 year to 86 years, with a median age of 32. The distribution was 79% males (1,495 cases) and 21% females (399 cases).
- Among the reported cases, 81% were suspect cases, 48% were unemployed, and 78% had flood-related exposure.

Annex C.1. Profile of Cases (n=1,894), Region III, Jan 4 to Sep 19, 2026

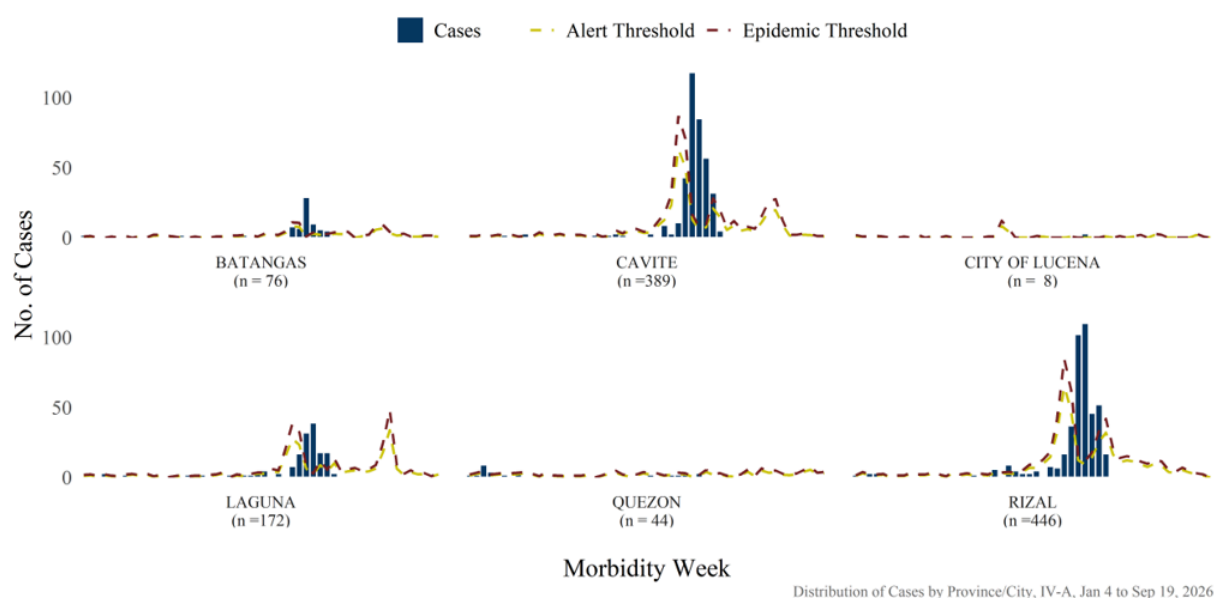
	No. of Cases	Percentage
Case Classification		
Suspect	1,534	81
Probable	336	18
Confirmed	24	1
Exposure History		
Flood related	1,470	78
Mud exposure	180	10
Agriculture related	94	5
Ingestion of contaminated foods/drinks	8	<1
Unknown	95	5
Others	47	2
Occupation*		
None	749	48
Farmer	227	15
Student	113	7
Construction worker	95	6
Driver/rider	62	4
Market worker/vendor	43	3
Laborer	34	2
Fisherman	19	1
Health worker	19	1
Others	208	13

*Occupations directly extracted from encoded data

**Other occupations one percent or below the total number of cases are grouped under the 'Others' category

ANNEX E Subregional Analyses of Region IV-A

- As of Morbidity Week 37 (January 4 to September 19, 2026), 1,135 leptospirosis cases were reported, a 38% increase from 823 cases reported in the same period last year. Cases peaked in MW 33, but cases have shown a downward trend since.
- Region IV-A exceeded its epidemic threshold in 3 of the past 4 morbidity weeks (*Figure 2*). Within the region, Batangas, Cavite, Laguna, and Rizal exceeded their epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026).
- A total of 70 deaths were reported (CFR: 6.17%), which was 11% lower than the 79 deaths reported last year (CFR: 9.6%).
- Clustering of cases was reported in 20 barangays. Of these, Barangay San Jose reported the most cases, with 16 in the past four weeks.



- Cases were reported across all age groups, ranging from less than 1 year to 83 years, with a median age of 34. The distribution was 80% males (910 cases) and 20% females (225 cases).
- Among the reported cases, 75% were suspect cases, 55% were unemployed, and 74% had flood-related exposure.

Annex D.1. Profile of Cases (n=1,135), Region IV-A, Jan 4 to Sep 19, 2026

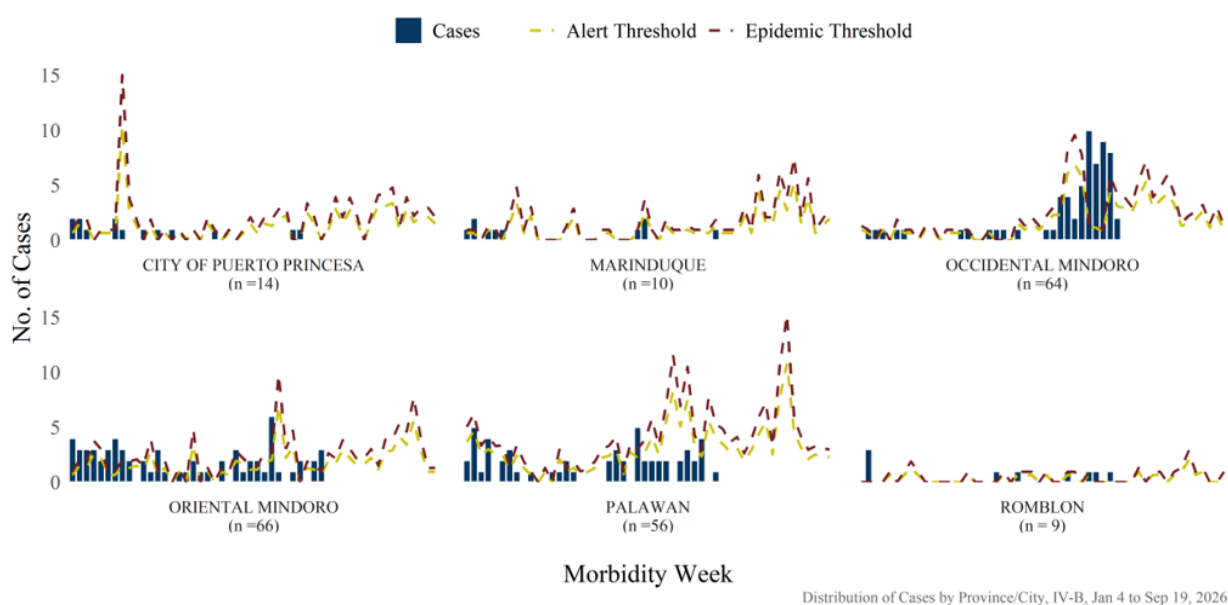
	No. of Cases	Percentage
Exposure History		
Suspect	855	75
Probable	238	21
Confirmed	42	4
Exposure History		
Flood related	842	74
Agriculture related	26	2
Ingestion of contaminated foods/drinks	11	1
Mud exposure	11	1
Others	147	13
Unknown	98	9
Occupation*		
None	621	55
Construction worker	106	9
Student	79	7
Driver/rider	73	6
Market worker/vendor	30	3
Laborer	22	2
Factory worker	19	2
Farmer	15	1
Office worker	13	1
Sales worker	12	1
Others	145	13

*Occupations directly extracted from encoded data

**Other occupations one percent or below the total number of cases are grouped under the 'Others' category

ANNEX F Subregional Analyses of Region IV-B

- As of Morbidity Week 37 (January 4 to September 19, 2026), 219 leptospirosis cases were reported, a 6% decrease from 232 cases reported in the same period last year. Recent cases remained at elevated levels, forming a sustained plateau.
- Region IV-B exceeded its epidemic threshold in 2 of the past 4 morbidity weeks (*Figure 2*). Within the region, Occidental Mindoro, Oriental Mindoro, and Romblon exceeded their epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026).
- A total of 13 deaths were reported (CFR: 5.94%), which was 58% lower than the 31 deaths reported last year (CFR: 13.36%).
- Clustering of cases was reported in 2 barangays. Of these, Barangay Tayamaan reported the most cases, with 3 in the past four weeks.



- Cases were reported across all age groups, ranging from 6 years to 79 years, with a median age of 39. The distribution was 88% males (192 cases) and 12% females (27 cases).
- Among the reported cases, 51% were probable cases, 28% were farmers, and 27% had flood-related exposure.

Annex E.1. Profile of Cases (n=219), Region IV-B, Jan 4 to Sep 19, 2026

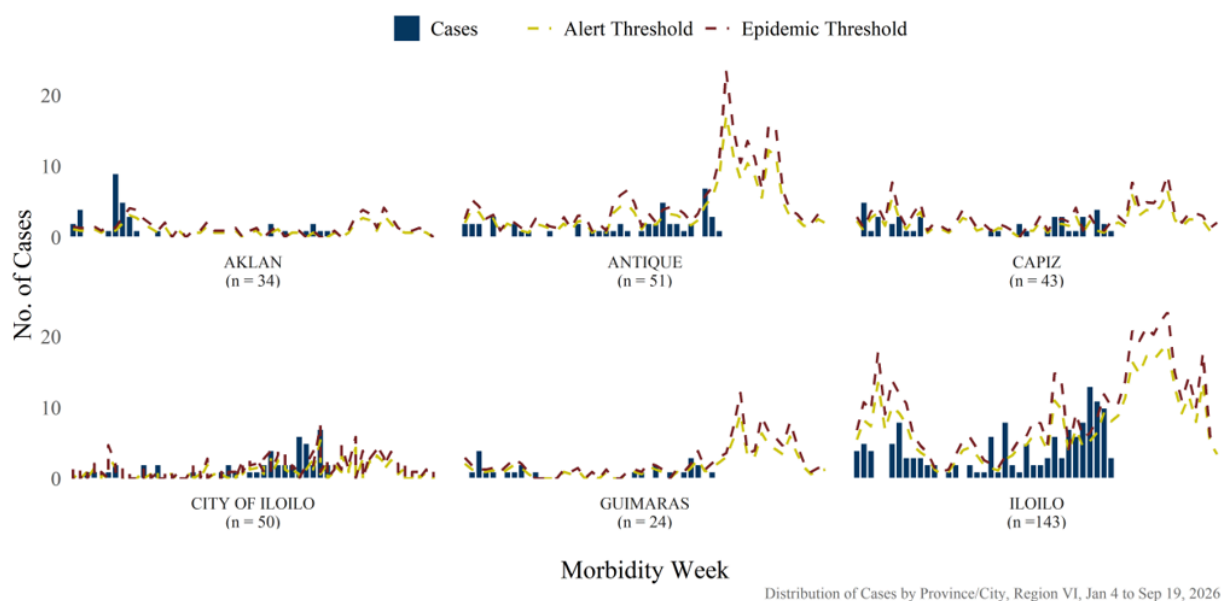
	No. of Cases	Percentage
Exposure History		
Probable	111	51
Suspect	107	49
Confirmed	1	<1
Exposure History		
Flood related	60	27
Agriculture related	56	26
Mud exposure	16	7
Ingestion of contaminated foods/drinks	3	1
Unknown	62	28
Others	22	10
Occupation*		
Farmer	61	28
None	57	26
Student	18	8
Fisherman	17	8
Construction worker	12	6
Driver/rider	10	5
Laborer	8	4
Market worker/vendor	5	2
Plumber	3	1
Restaurant worker	3	1
Others	25	11

*Occupations directly extracted from encoded data

**Occupations one percent or below the total number of cases are grouped under the 'Others' category

ANNEX G Subregional Analyses of Region VI

- As of Morbidity Week 37 (January 4 to September 19, 2026), 345 leptospirosis cases were reported, a 25% increase from 276 cases reported in the same period last year.
- Region VI exceeded its epidemic threshold in 2 of the past 4 morbidity weeks (*Figure 2*). Within the region, Aklan, Capiz, and Iloilo exceeded their epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026).
- A total of 38 deaths were reported (CFR: 11.01%), which was 41% higher than the 27 deaths reported last year (CFR: 9.78%).
- No clustering of cases was reported over the past four weeks.



- Cases were reported across all age groups, ranging from less than 1 year to 89 years, with a median age of 33. The distribution was 86% males (297 cases) and 14% females (48 cases).
- Among the reported cases, 63% were suspect cases, 29% were unemployed, and 32% had flood-related exposure.

Annex F.1. Profile of Cases (n=345), Region VI, Jan 4 to Sep 19, 2026

	No. of Cases	Percentage
Exposure History		
Suspect	218	63
Probable	112	32
Confirmed	15	4
Exposure History		
Flood related	111	32
Agriculture related	95	28
Mud exposure	15	4
Others	75	22
Unknown	49	14
Occupation*		
None	100	29
Farmer	78	23
Student	37	11
Construction worker	27	8
Driver/rider	19	5
Laborer	18	5
Market worker/vendor	10	3
Fisherman	4	1
Housekeeper	4	1
Technician	4	1
Others	44	13

*Occupations directly extracted from encoded data

**Other occupations one percent or below the total number of cases are grouped under the 'Others' category

ANNEX H

Provinces and Highly Urbanized Cities (P/HUCs) with Threshold Breaches, Philippines, Aug 23 to Sep 19, 2026

Geographic Area	Thresholds Breached at least Twice in Past 4 MWs
Region I	
Pangasinan	Epidemic
La Union	Alert
Ilocos Norte	Epidemic
Region II	
Cagayan	Alert
Nueva Vizcaya	Alert
Region III	
Nueva Ecija	Epidemic
Bulacan	Epidemic
Tarlac	Epidemic
Aurora	Epidemic
Pampanga	Epidemic
Bataan	Epidemic
Zambales	Epidemic
City of Angeles	Epidemic
City of Olongapo	Epidemic
Region IV-A	
Rizal	Epidemic
Laguna	Epidemic
Cavite	Epidemic
Batangas	Epidemic
Region IV-B	
Oriental Mindoro	Epidemic
Occidental Mindoro	Epidemic
Romblon	Epidemic
Region V	
Albay	Alert
Region VI	
Iloilo	Epidemic
Aklan	Epidemic
Capiz	Epidemic
City of Iloilo	Alert
Region XII	
City of General Santos	Epidemic
Region XIII	
Surigao del Sur	Alert
Agusan del Sur	Epidemic
CAR	
Benguet	Epidemic
City Of Baguio	Epidemic
Kalinga	Epidemic
Apayao	Alert
NCR	
Quezon City	Epidemic
City of Manila	Epidemic
City of Caloocan	Epidemic
City of Muntinlupa	Epidemic
Pasay City	Epidemic
City of Malabon	Epidemic
City of Valenzuela	Epidemic
City of Pasig	Epidemic
City of Makati	Epidemic
City of Mandaluyong	Epidemic
City of Marikina	Epidemic
City of Parañaque	Epidemic
City of Las Piñas	Alert
City of Navotas	Epidemic
City of San Juan	Epidemic
Pateros	Epidemic

**Note: Only P/HUCs with at least two alert or epidemic threshold breaches in the past four weeks are shown.*

**I. National Trend**

As of Morbidity Week 37 (January 4 to September 19, 2026), 69 confirmed cases were reported, a 74% decrease from 264 cases reported during the same period last year. Two deaths (CFR: 2.90%) have been reported this year, compared with five deaths (CFR: 1.89%) during the same period last year. No Clade I cases have been detected to date.

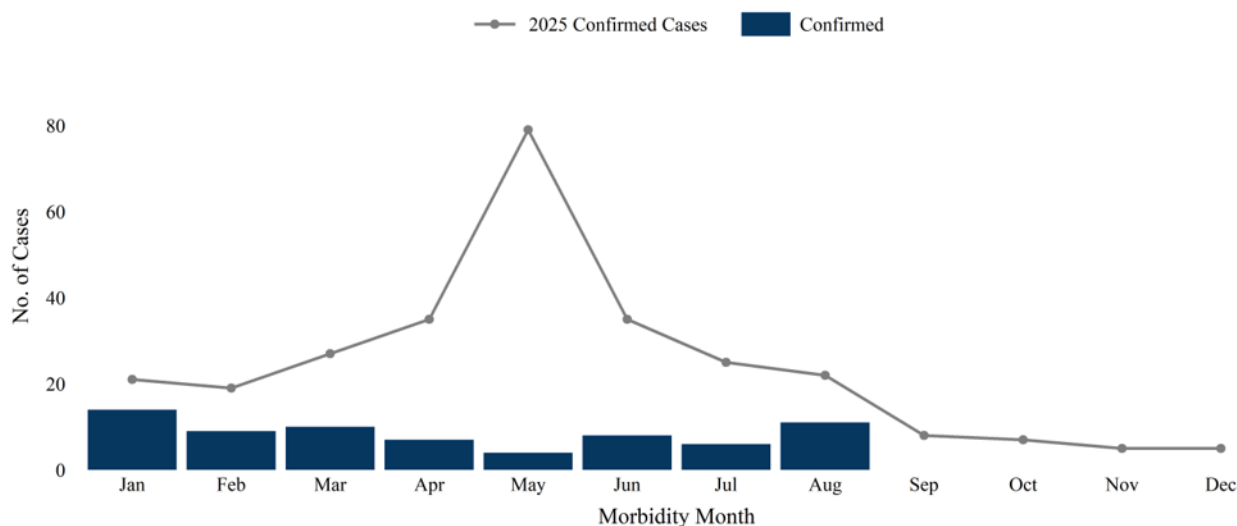


Figure 1. Confirmed Cases by Month of Onset (n=69)
Philippines, Jan 4 to Sep 19, 2026

II. Subnational Distribution of Cases

NCR, and Regions VII and XI had the highest number of confirmed cases. No clusters were reported in the past four weeks.

Table 1. Cumulative Number of Cases by Classification and Region (N=216), Philippines, Jan 4 to Sep 19, 2026

Region	Suspect	Probable	Confirmed	Percentage*
NCR	33	4	22	32
VII	18	1	17	25
XI	7	1	17	25
IV-A	18	1	6	9
XII	7	0	3	4
II	0	0	2	3
I	4	0	1	1
NIR	1	0	1	1
CAR	13	0	0	0
III	13	0	0	0
BARMM	11	0	0	0
IX	6	0	0	0
VI	3	0	0	0
IV-B	2	0	0	0
X	2	0	0	0
VIII	1	0	0	0
XIII	1	0	0	0
V	0	0	0	0
Total	140	7	69	100

*The percentage was calculated using the total confirmed cases as the denominator.

III. Profile of Confirmed Cases

- Cases were **all males** ranging in age from **16 to 51 years (median of 31 years)**.
- Office workers accounted for 45% of reported occupations.
- Forty-three cases (62%) had male-to-male intimate close contact while 22 cases (32%) reported no sexual activity.
- Thirty-five cases (51%) disclosed that they are persons living with HIV (PLHIV).
- 2 cases (3%) reported a history of foreign travel and 55 (80%) reported no known contact.

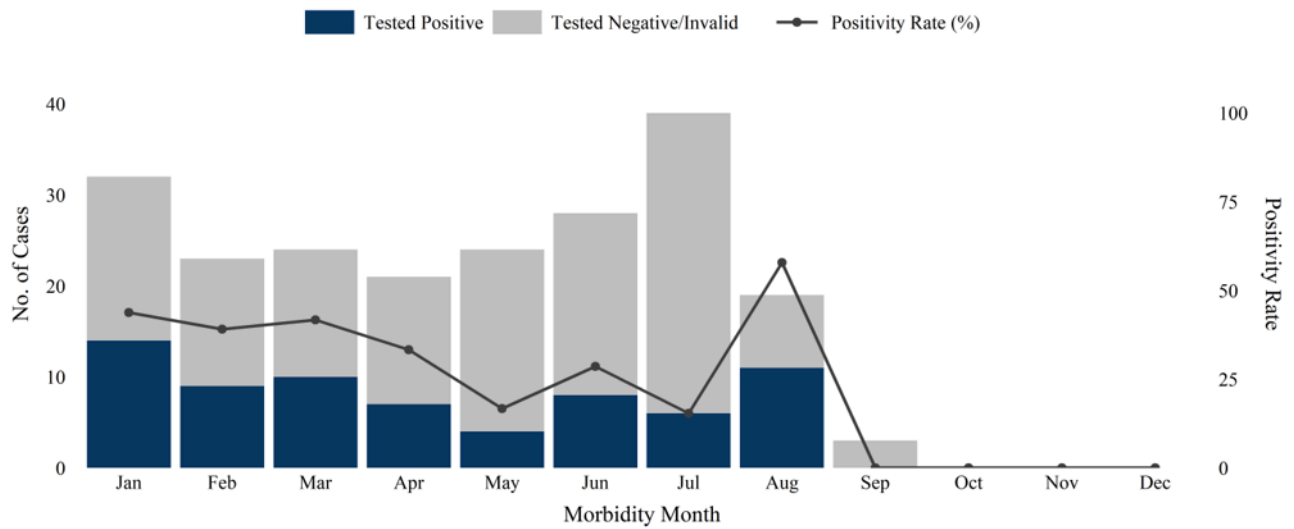
Table 2. Profile of Confirmed Mpox Cases (n=69), Philippines, Jan 4 to Sep 19, 2026

	No. of Cases	Percentage
Age Group		
Children (< 1-9 years)	0	0
Adolescents (10-19 years)	1	1
Adults (20-59 years)	68	99
Elderly (≥ 60 years)	0	0
Occupation Classification		
Office worker	31	45
Laborer	2	3
Student	4	6
Unemployed	8	12
Healthcare Worker	2	3
Aviation/Air Transport Worker	5	7
Coast Guard	1	1
Self-employed	1	1
Sex Worker	1	1
For verification	14	20
HIV Status (as disclosed)		
PLHIV*	28	41
Non-PLHIV	35	51
For verification	6	9
History of Contact		
With a confirmed/probable case	3	4
With someone having similar signs & symptoms	11	16
No known contact	55	80
History of Intimate Close Contact		
Male to male	43	62
Male to female	1	1
Both male and female	3	0
No sexual activity	22	32
Number of Sexual Partner/s		
Single partner	26	38
Multiple sexual partners	19	28
None	22	32
For verification	2	3
History of Foreign Travel		
With history of foreign travel	2	3
Without history of foreign travel	67	97

*PLHIV (People Living with HIV)

IV. Positivity Rate

As of MW37, 69 out of 216 reported cases tested positive, corresponding to a positivity rate of 32%.



**Figure 2. Test Positivity Rate of Confirmed Mpox Cases (N=216)
Philippines, Jan 4 to Sep 19, 2026**

Note: Reported cases may still change due to late consultations, delayed reporting, and ongoing verification. Data was extracted on September 21, 2026.



Rabies

Weekly Surveillance Report 2026

I. National Trend

As of Morbidity Week 37 (January 4 to September 19, 2026), 222 cases were reported, a 11% decrease from 250 cases reported in the same period last year. Six cases have been reported under MW 37.

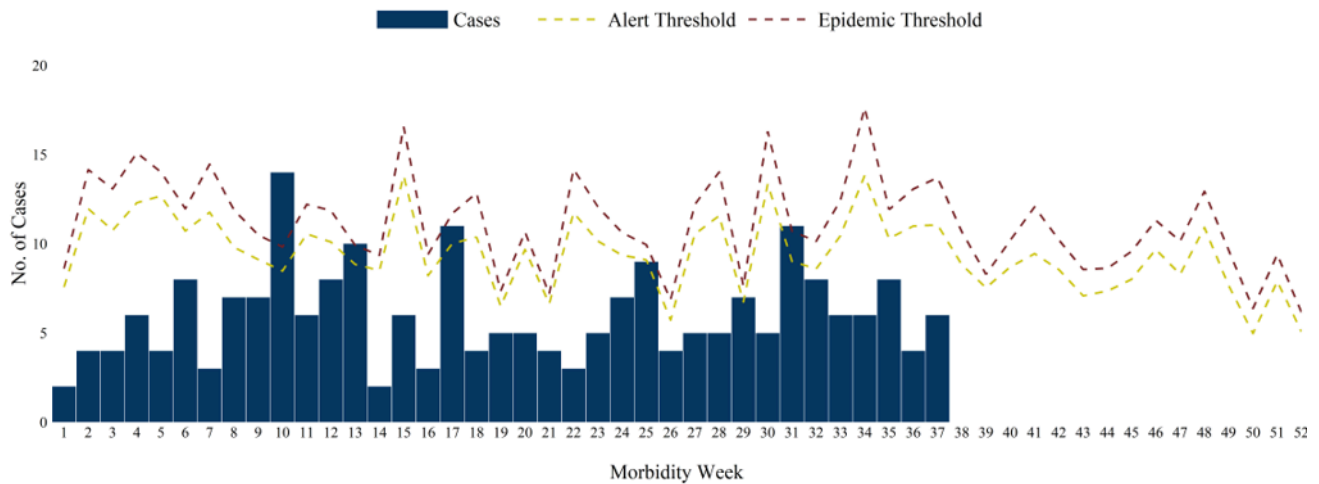


Figure 1. Cases by Morbidity Week (N = 222)
Philippines, Jan 4 to Sep 19, 2026

II. Subnational Distribution of Cases

- Six out of 18 regions showed an increase in cases, with Regions XI, V, and I recording the top 3 highest percentage increases (*Table 1*).
- No region breached its epidemic thresholds at least twice in the past four weeks (August 23 to Sep 19, 2026), and no clusters were reported during that same period.
- Regions IX (7.70), XI (4.38), and XII (4.33) reported the highest incidence rates per 1,000,000 population (*Figure 3*).

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			Deaths			CFR (%)*
	2025	2026	% Change	2025	2026	% Change	
IX	25	31	↑24	25	31	↑24	100.00
IV-A	32	30	↓6	32	30	↓6	100.00
XI	13	24	↑85	13	24	↑85	100.00
III	34	22	↓35	34	22	↓35	100.00
XII	21	20	↓5	21	20	↓5	100.00
V	12	18	↑50	12	18	↑50	100.00
NCR	13	13	↓0	13	13	↓0	100.00
I	9	13	↑44	9	13	↑44	100.00
VII	11	13	↑18	11	13	↑18	100.00
XIII	7	9	↑29	7	9	↑29	100.00
X	11	7	↓36	11	7	↓36	100.00
BARMM	13	6	↓54	13	6	↓54	100.00
II	9	5	↓44	9	5	↓44	100.00
NIR	18	4	↓78	18	4	↓78	100.00
CAR	5	2	↓60	5	2	↓60	100.00
IV-B	4	2	↓50	4	2	↓50	100.00
VI	5	2	↓60	5	2	↓60	100.00
VIII	8	1	↓88	8	1	↓88	100.00

*Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases.

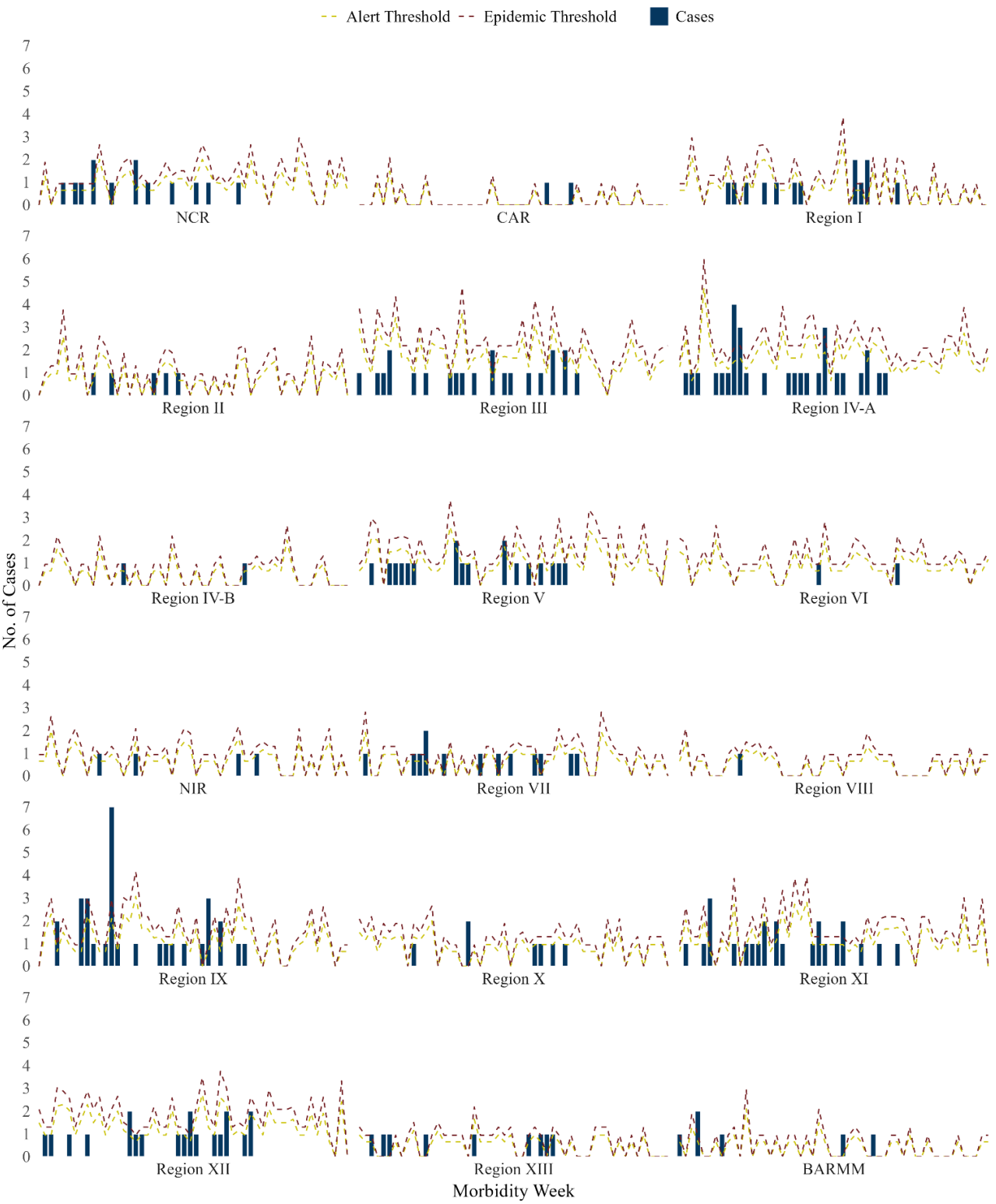


Figure 2. Regional Cases by Morbidity Week (N=222)
Philippines, Jan 4 to Sep 19, 2026

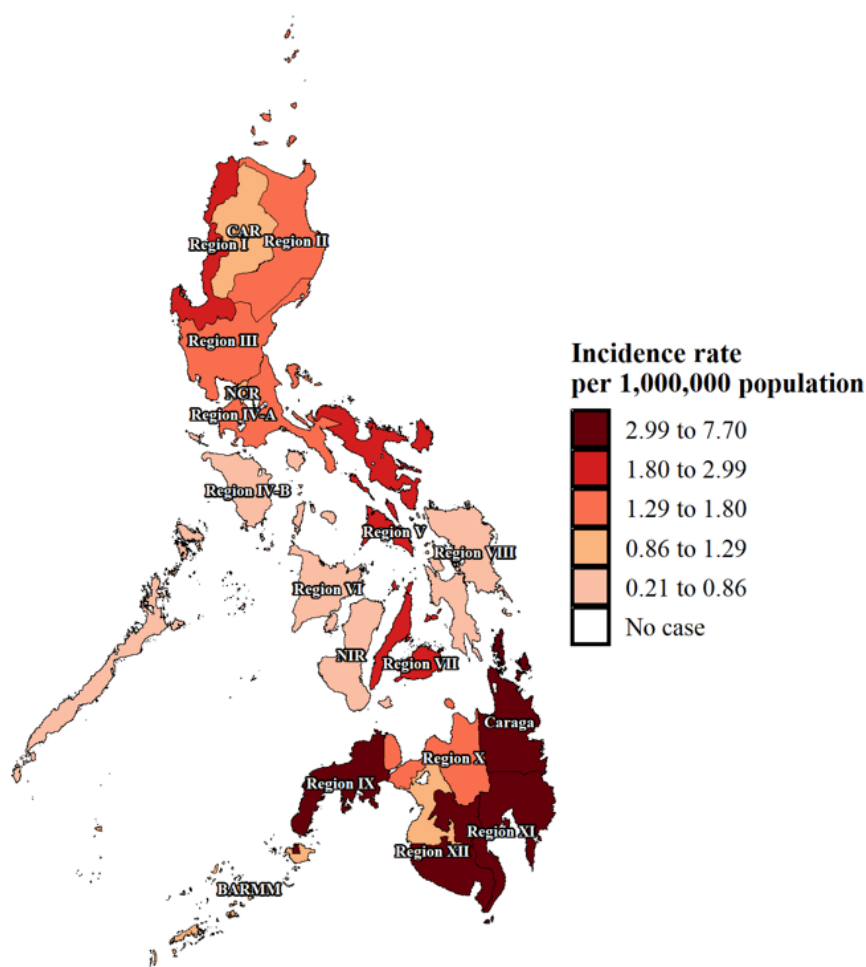


Figure 3. Regional Incidence Rates (N=222)
Philippines, Jan 4 to Sep 19, 2026

III. Profile of Cases

- Cases were reported in individuals aged 2 to 82 years, with a median age of 43, and were more common in males (175 cases, 79%).

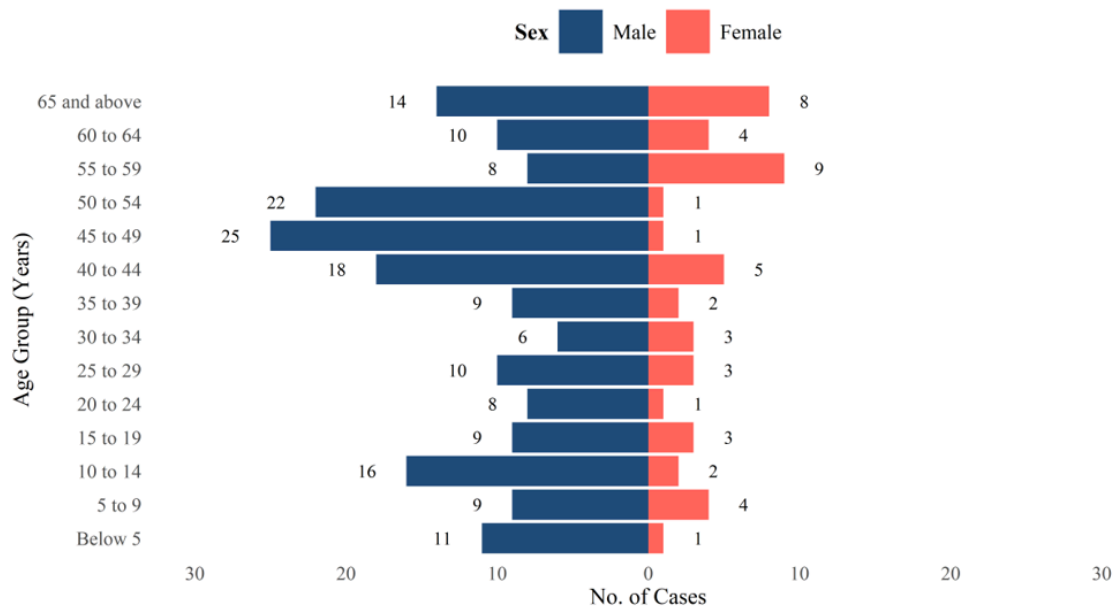


Figure 4. Cases by Age Group and Sex (N=222)
Philippines, Jan 4 to Sep 19, 2026

- Of the cases, 155 (70%) were suspect, 62 (28%) probable, and 5 (2%) confirmed. Category III exposure (148 cases, 67%) was the most common. Of the cases, 203 (91%) were either unvaccinated or incompletely vaccinated.

Table 2. Case Classification and Clinical Profile of Cases (N=222), Philippines, Jan 4 to Sep 19, 2026

		No. of Cases	Percentage
Case Classification	Suspect	155	70
	Probable	62	28
	Confirmed	5	2
Exposure Category*	Category I	39^	18
	Category II	34	15
	Category III	148	67
	Unknown	1	0
Post-exposure Prophylaxis	Completed	19^	9
	Not Vaccinated/incomplete	203	91

**Category I is defined as touching or feeding animals, animal licks on intact skin (no exposure); Category II is nibbling of uncovered skin, minor scratches or abrasions without bleeding (exposure); and Category III is defined as single or multiple transdermal bites or scratches, contamination of mucous membrane or broken skin with saliva from animal licks, exposures due to direct contact with bats (severe exposure).*

^For verification

- One hundred and ninety-three cases (87%) involved dogs, while 217 (98%) involved animals that were either unvaccinated or had an unknown vaccination status. 112 (50%) cases involved domestic animals.

Table 3. Profile of Animals Involved in Rabies Cases (N=222), Philippines, Jan 4 to Sep 19, 2026

		No. of Cases	Percentage
Type	Dog	193	87
	Cat	23	10
	Bat	1	0
	For verification	5	2
Ownership	Domestic	112	50
	Stray	87	39
	Wild	4	2
	For verification	19	9
Vaccination History	Vaccinated	5	2
	Not vaccinated	96	43
	Unknown	121	55

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026.



Acute Bloody Diarrhea

Weekly Surveillance Report 2026

I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), 7,056 cumulative cases were reported, a 31% decrease from the 10,194 cases reported in the same period last year.
- A total of 7 deaths were reported, resulting in a case fatality rate of 0.10%.
- Nationally, cases are on a downward trend with cases in MW 34 - 35 (Aug 23 to Sep 5, 2026) lower by 21%, with 344 cases reported, compared to MW 32 - 33 (Aug 9 to Aug 22, 2026), which had 438 cases. The recent two MWs were not used for growth rate comparison due to the possibility of delayed reports.

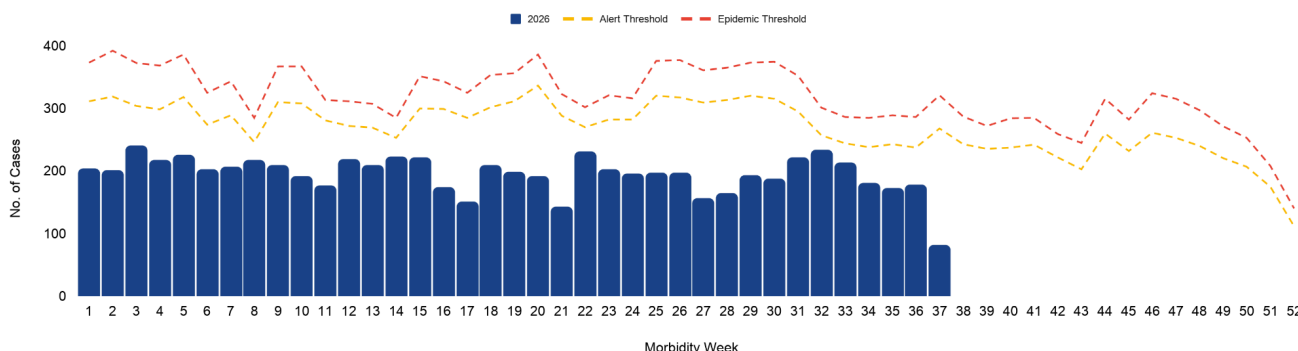


Figure 1. Cases by Morbidity Week (N=7,056)
Philippines, Jan 4 to Sep 19, 2026

II. Sub-national Distribution of Cases

- Regions IX and VII remain regions with the highest number of reported cases, with VII showing a 14% increase versus the same time period last year.
- Regions IV-A and VI also showed an increase in cases compared to the previous year.
- BARMM reported the highest number of deaths, with four cases, and the highest case fatality rate (CFR) at 1.47%. Despite this, BARMM reported the same number of deaths compared to the previous year. Meanwhile, Region V, XIII, and NCR recorded an increase in deaths, with one death reported in each region.
- Region IV-A cases, despite a decline in TWGR in the recent 3-4 weeks, are now on an upward trend and have also exceeded the epidemic threshold. NCR exceeded the alert threshold, with both regions reaching their respective thresholds at least twice during the past four weeks.
- Several provinces and highly urbanized cities across different regions also exceeded either the alert or epidemic threshold during the same period (Annex B).
- 7 regions showed an increase in cases in the recent 3-4 vs 5-6 weeks.
- Clusters were also reported in 17 barangays in XX PHUCs across 5 regions, with a maximum of 9 cases in a barangay over the past 4 weeks.

Note: Link for the ABD Clustering as of MW 37: <https://tinyurl.com/MW37Clustering-ABD>



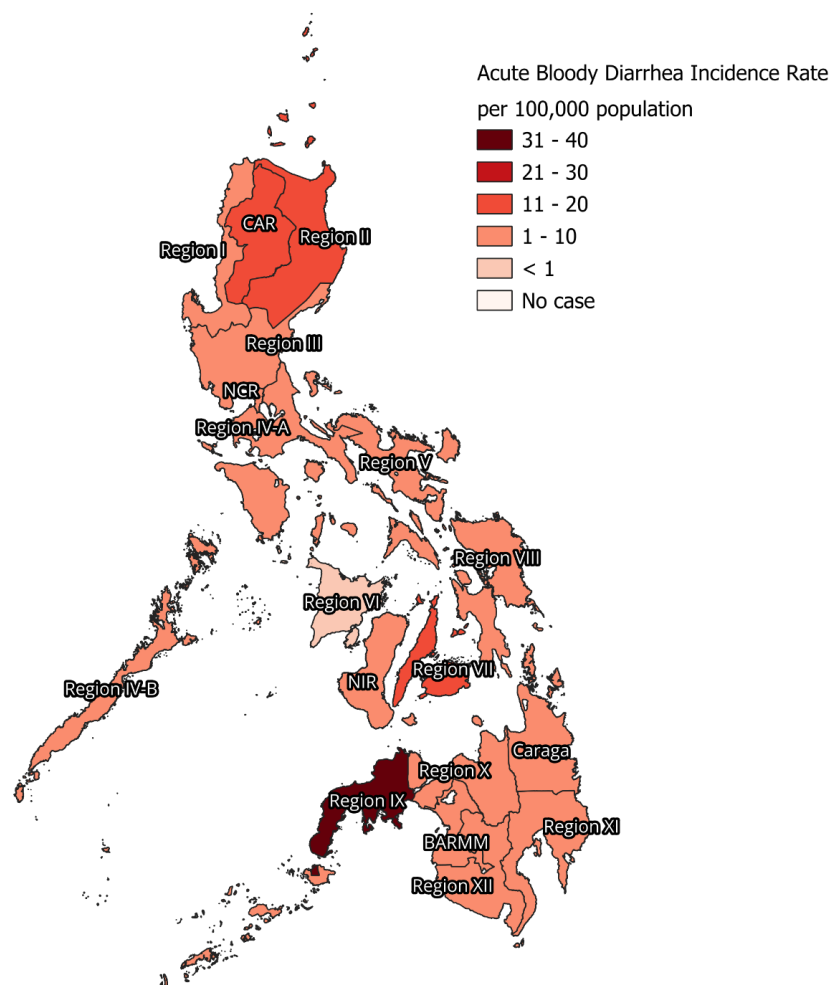
Figure 2. Regional Cases by Morbidity Week (N=7,056)
Philippines, Jan 4 to Sep 19, 2026

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR [^] 3-4 vs 5-6 weeks (%)	Deaths			CFR (%) [*] 2026
	2025	2026	% Change		2025	2026	% Change	
IX	2399	1,636	↓32	↑7.14	2	0	↓100	0
VII	1,236	1,406	↑14	↓40.00	3	0	↓100	0
IV-A	563	648	↑15	↓24.44	2	0	↓100	0
X	847	482	↓43	↓17.39	0	0	-	0
II	1,277	445	↓65	↓27.50	0	0	-	0
NCR	400	364	↓9	↑17.39	0	1	↑	0.27
BARMM	558	272	↓51	↑60.00	4	4	-	1.47
XI	286	262	↓8	↓54.55	1	0	↓100	0
CAR	473	255	↓46	↓25.00	0	0	-	0
III	286	233	↓19	↑35.71	0	0	-	0
NIR	350	204	↓42	↑28.57	0	0	-	0
VIII	455	180	↓60	↑100.00	1	0	↓100	0
XII	200	169	↓16	↑36.36	1	0	↓100	0
Caraga	324	161	↓50	↓40.91	0	1	↑	0.62
IV-B	293	132	↓55	↓61.54	2	0	↓100	0
V	155	132	↓15	↓72.73	0	1	↑	0.76
I	91	66	↓27	↓20.00	0	0	-	0
VI	1	9	↑800	-	0	0	-	0

[^]Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)

^{*}Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases.



**Figure 3. Acute Bloody Diarrhea Incidence Cases by Region (N=7,056)
Philippines, Jan 4 to Sep 19, 2026**

III. Demographic and Clinical Profile of Cases

- The cases' ages ranged from less than 1 to 99 years, with a median of 15 years and an almost equal sex distribution.
- Of the cases tested, the highest number of positive results was reported for *Entamoeba histolytica* with 1,634 (56%) cases, followed by *Escherichia coli* with 54 (2%) cases, and *Ascaris lumbricoides* with 23 (1%) cases.
- Among the reported cases, 73% required hospitalization, with consultation occurring within less than a day to 35 days after symptom onset with a median of 1 day.

Table 2. Demographic and Clinical Profile of ABD Cases* (N=7,056), Philippines, Jan 4 to Sep 19, 2026

		Median	Range
Age (in years)		15	<1-99
Onset-to-consultation interval (in days)		1	<1- 35
		No. of Cases	Percentage
Sex			
	Female	3,452	49
	Male	3,604	51
Laboratory Testing			
Tested**		2,944	42
	<i>Entamoeba histolytica</i>	1,634	56
	<i>Escherichia coli</i>	54	2
	<i>Ascaris lumbricoides</i>	23	1
	<i>Salmonella group</i>	9	<1
	<i>Helicobacter pylori</i>	5	<1
	<i>Giardia lamblia</i>	4	<1
	<i>Aeromonas veronii</i>	3	<1
	<i>Salmonella typhi</i>	2	<1
	<i>Shigella</i>	2	<1
	<i>Trichuris trichiura</i>	2	<1
	<i>Plesiomonas shigelloides</i>	1	<1
	<i>Salmonella non-typhi</i>	1	<1
	Hookworm	1	<1
	Multiple organism	15	1
	Unspecified organism	578	20
	Negative	600	20
	Pending	10	<1
Not done / Unknown		4,112	58
Hospitalization			
	Admitted	5,176	73
	Not Admitted	1,880	27

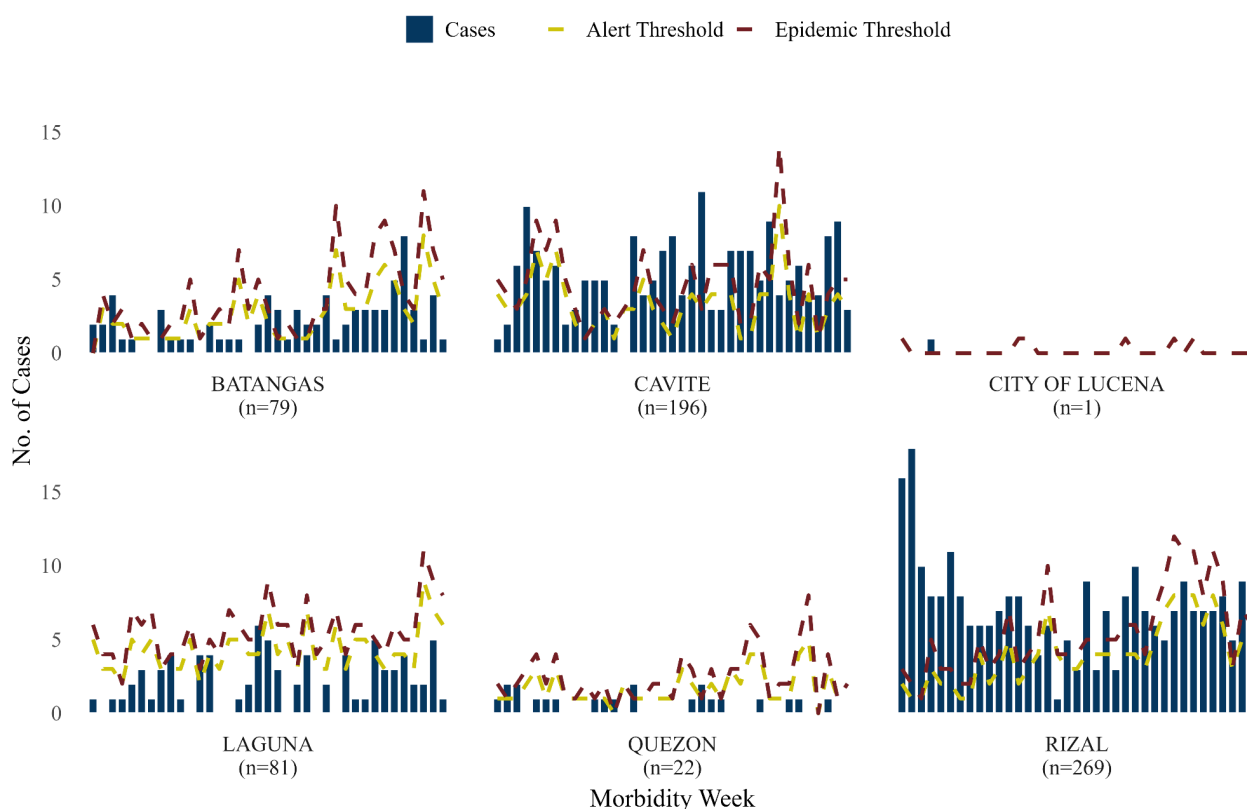
*includes suspected and confirmed cases

**denominator used for percentages under Tested is the number of tested cases (n=2,944)

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026

Annex A Subregional Analyses of Region IV-A

- A cumulative total of 648 cases and no deaths were reported in the region from January 4 to September 19, 2026.
- Region IV-A began breaching its epidemic threshold from MW 32 to MW 33, followed by a sudden decline in cases in MW 34 and a subsequent increase in MW 36. Although the number of reported cases remains low for the current morbidity week, case counts may still change due to late consultations and delayed reporting.
- In Region IV-A, Cavite and Rizal exceeded the epidemic thresholds, each at least twice over the past four weeks.
- Of the cases, 346 (54%) are males, and 298 (43%) are females, with ages ranging from less than 1 year to 99 years.
- Of the 648 cases from the region, 552 (85%) were suspected, while 96 (15%) cases were confirmed.
- Among the 182 tested cases in the region, 83 (45%) cases tested positive for *Entamoeba histolytica*, 51 (28%) cases for *Aeromonas veronii*, and 2 (1%) cases for *Escherichia coli*; 45 (24%) cases tested negative, and 1 (1%) case had a pending result.
- Clustering was reported in 3 barangays, reporting 3 to 4 cases each in the past four weeks.



**Figure A.1. Provincial Cases by Morbidity Week (n=648)
Region V, Jan 4 to Sep 19, 2026**

ANNEX B

Provinces and Highly Urbanized Cities (P/HUCs) with Threshold Breaches, Philippines, Aug 23 to Sep 19, 2026

Geographic Area	Thresholds Breached in Past 4 MWs
NCR	
City of Makati	Epidemic
City of Pasig	Epidemic
City of Taguig	Epidemic
City of Valenzuela	Alert
Pasay City	Epidemic
Quezon City	Alert
CAR	
Apayao	Alert
City Of Baguio	Epidemic
Region I	
Pangasinan	Epidemic
Region II	
Quirino	Alert
Region III	
Bataan	Epidemic
Pampanga	Epidemic
Region IV-A	
Cavite	Epidemic
Rizal	Epidemic
Region VII	
City of Cebu	Epidemic
Region X	
Misamis Oriental	Alert
Region XI	
Davao Del Norte	Epidemic
Region XII	
City of General Santos	Epidemic
Sarangani	Epidemic
South Cotabato	Epidemic

**Note: Only P/HUCs with at least two alert or epidemic threshold breaches in the past four weeks are shown.*



Cholera

Weekly Surveillance Report 2026

I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), 460 cases have been reported, a 66% decrease from 1,334 cases reported in the same period last year.
- A total of 11 deaths were reported, resulting in a case fatality rate of 2.39%.
- Nationally, cases have decreased by 48% in MW 34-35 (August 23 to September 5, 2026), with 12 cases reported, compared to MW 32-33 (August 9 to August 22, 2026), which had 23 cases. The recent two MWs were not used for growth rate comparison due to the possibility of delayed reports.

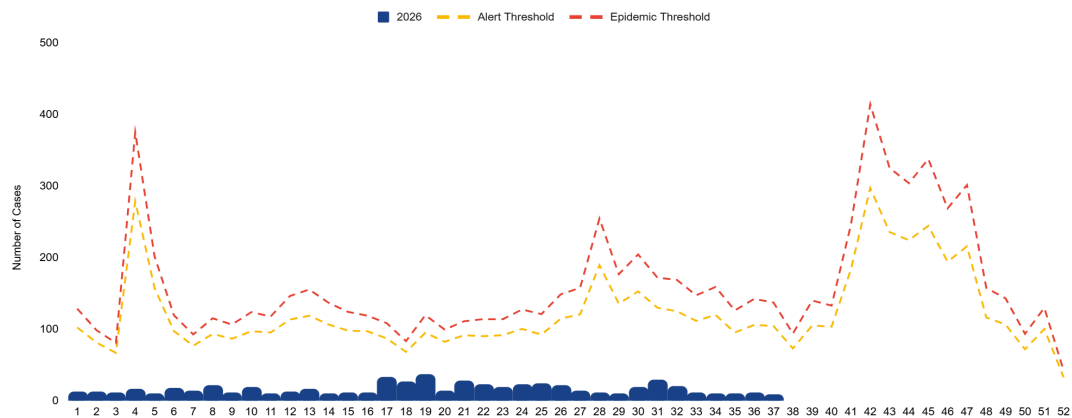


Figure 1. Cases by Morbidity Week (N=460)
Philippines, Jan 4 to Sep 19, 2026

II. Sub-regional Distribution of Cases

- No region exceeded the epidemic/alert thresholds at least twice in the past four weeks.
- Regions I, IV-A, V, and VII reported an increase in cases compared to the previous year.
- No clustering was reported over the past four weeks.

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR [^] 3-4 vs 5-6 weeks (%)	Deaths		CFR (%) [*] % Change
	2025	2026	% Change		2025	2026	
V	42	160	↑281	↑100	0	1	↑
VII	8	89	↑1013	↓100	0	4	↑
NIR	97	73	↓25	↓40	6	0	↓100
IX	105	45	↓57	0	2	0	↓100
VIII	682	28	↓96	-	5	0	↓100
BARMM	58	21	↓64	↓100	0	0	-
CAR	24	10	↓58	↓50	0	2	↑
XI	14	9	↓36	-	0	0	-
X	22	7	↓68	-	1	0	↓100
IV-A	3	5	↑67	-	0	0	-
I	0	5	-	-	0	2	↑
NCR	29	4	↓86	↓100	4	2	↓50
III	5	3	↓40	-	0	0	-
VI	3	1	↓67	-	0	0	-
II	4	0	↓100	-	1	0	↓100
XII	232	0	↓100	-	1	0	↓100
XIII	6	0	↓100	-	4	0	↓100
IV-B	0	0	-	-	0	0	-

[^]Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)

^{*}Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases.

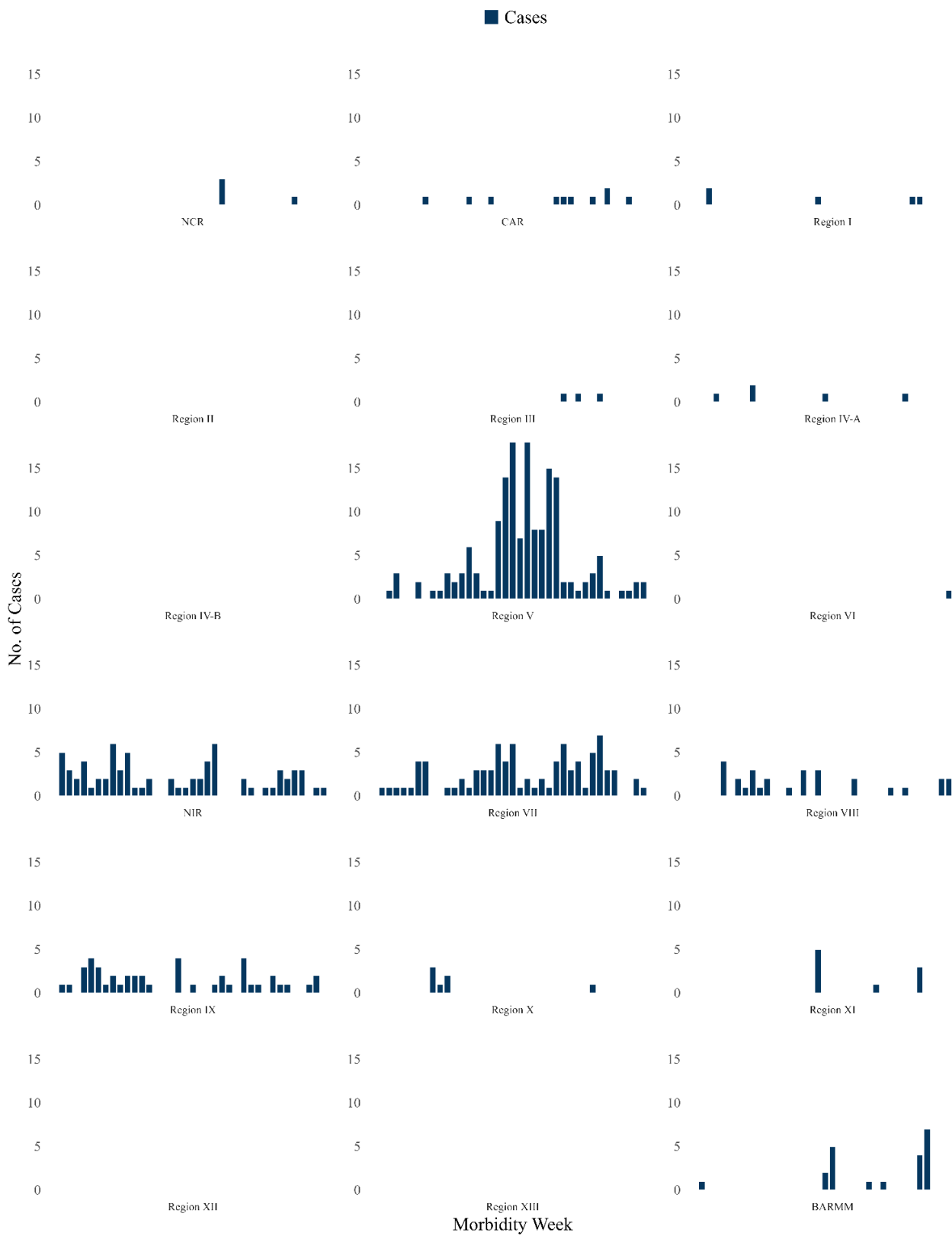


Figure 2. Regional Cases by Morbidity Week (N=460)
Philippines, Jan 4 to Sep 19, 2026

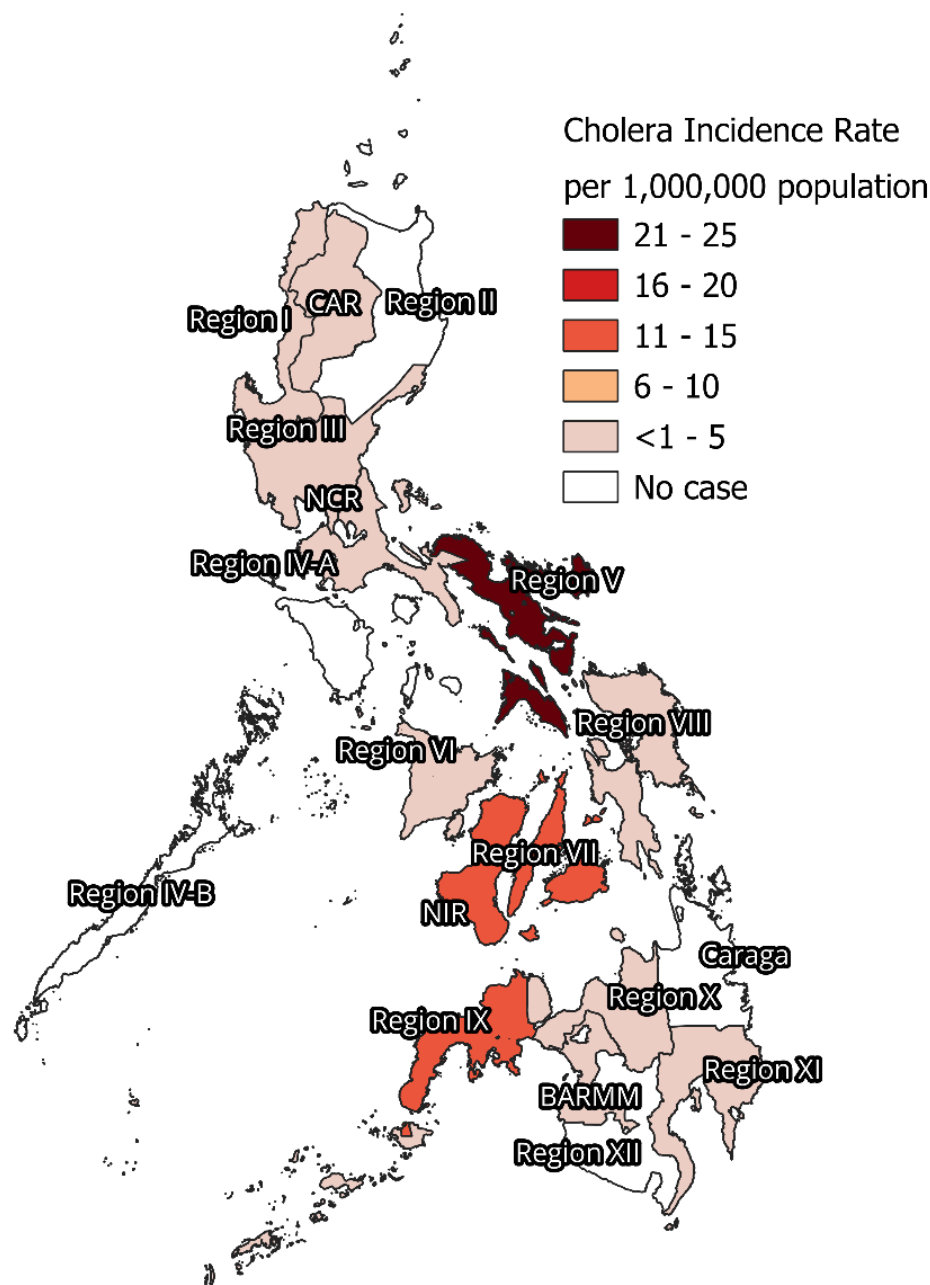


Figure 2. Cholera Cases by Morbidity Region (N=460)
Philippines, Jan 4 to Sep 19, 2026

III. Demographic, Laboratory, and Vaccination Profile of Cases

- The cases ranged in age from 2 to 94 years, with an almost equal distribution of males to females. A total of 15 cases (4%) tested positive for *Vibrio cholerae* by culture, RDT, or PCR, most of whom reported water consumption from unknown sources. Among the reported cases, 91% required hospitalization, and medical consultation was sought within 1–14 days after symptom onset.

Table 2. Demographic and Clinical Profile of Cholera Cases* (N=460), Philippines, Jan 4 to Sep 19, 2026

		Median	Range
Age (in years)		26	2-94
Onset-to-consult (days)		1	<1- 14
		No. of Cases	Percentage
Sex	Female	237	52
	Male	223	48
Main Source of Water			
	Commercial Water	292	63
	Local Water System	60	13
	Well	32	7
	Spring	19	4
	Others (Unspecified)	57	12
Laboratory Result			
	Positive	15	3
	Culture	11	73
	Rapid Diagnostic Test	2	13
	PCR	2	13
	Negative	38	8
	Pending	19	4
	Not Tested/ Unknown	388	84
Hospitalization			
	Admitted	418	91
	Not Admitted	42	9

*includes suspected and confirmed cases

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026.

I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), 8,742 cases have been reported, a 3% decrease from 9,039 cases reported in the same period last year.
- A total of 23 deaths were reported, resulting in a case fatality rate of 0.26%.
- Nationally, cases have increased by 1% in MW 34-35 (August 23 to September 5, 2026), with 337 cases reported, compared to MW 32-33 (August 9 to August 22, 2026), which had 333 cases. The recent two MWs were not used for growth rate comparison due to the possibility of delayed reports.

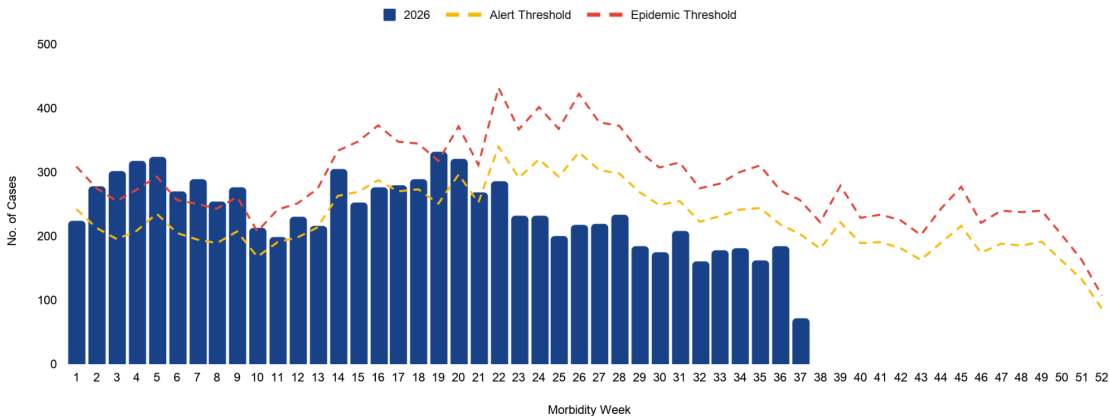


Figure 1. Cases by Morbidity Week (N=8,742)
Philippines, Jan 4 to Sep 19, 2026

II. Sub-regional Distribution of Cases

- Regions I, III, VII, and CAR exceeded epidemic thresholds, while Regions IV-A and XI exceeded alert thresholds, each at least twice in the past four weeks.
- Regions I, VII, and XI reported the highest increase in cases compared to the previous year.

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR [^]		Deaths		CFR (%) [*]	
	2025	2026	% Change	3-4 vs 5-6	2025	2026	% Change	2026	
I	375	1,643	↑338	↓16	0	10	↑	0.61	
IV-A	1,627	1,607	↓1	↑14	2	1	↓50	0.06	
VIII	3,472	1,180	↓66	↓11	11	0	↓100	0	
VII	302	857	↑184	0	0	1	↑	0.12	
BARMM	946	642	↓32	↑107	2	6	↑200	0.93	
CAR	314	553	↑76	↓19	1	0	↓100	0	
II	260	460	↑77	↓20	0	1	↑	0.22	
III	402	417	↑4	↑143	2	1	↓50	0.24	
IV-B	287	334	↑16	↓52	2	0	↓100	0	
NCR	209	246	↑18	↓57	1	3	↑200	1.22	
XI	94	223	↑137	↑67	0	0	-	0	
NIR	284	202	↓29	↓75	2	0	↓100	0	
IX	147	197	↑34	0	1	0	↓100	0	
VI	108	78	↓28	↑25	0	0	-	0	
V	148	74	↓50	↓50	0	0	-	0	
XII	37	16	↓57	-	0	0	-	0	
XIII	27	11	↓59	-	0	0	-	0	
X	0	2	-	-	0	0	-	0	

[^]Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)

^{*}Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases.



Figure 2. Regional Cases by Morbidity Week (N=8,742)
Philippines, Jan 4 to Sep 19, 2026

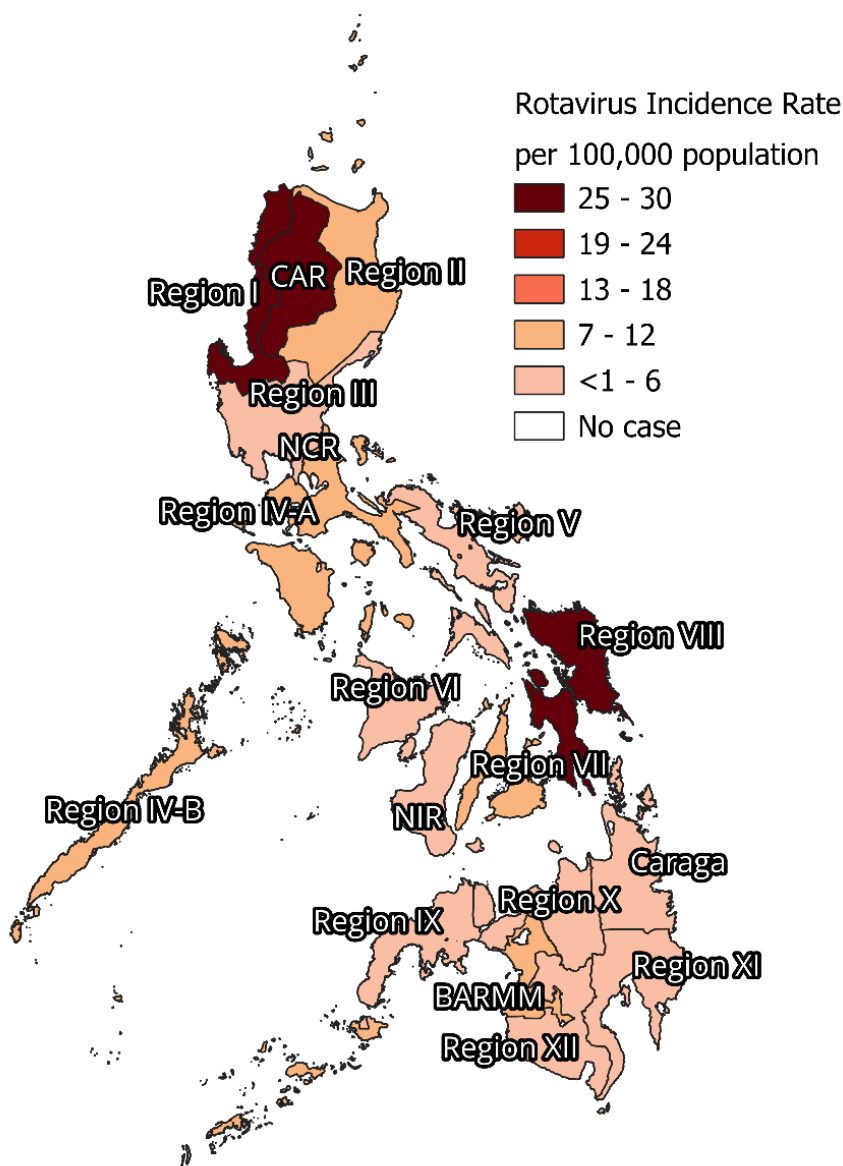


Figure 2. Rotavirus Cases* by Morbidity Region (N=8,742)
Philippines, Jan 4 to Sep 19, 2026
* Includes suspected and confirmed cases

- Clustering was reported in 17 barangays across 5 regions over the past four weeks, with a maximum of 5 cases recorded in a barangay.

Note: Link for the Rotavirus Clustering as of MW 37: <https://tinyurl.com/ClusteringMW37-Rotavirus>

III. Demographic, Laboratory, and Vaccination Profile of Cases

- Cases ranged in age from under 1 year to 5 years, with males comprising the majority. A total of 333 cases (4%) tested positive via ELISA or RT-PCR. Among the reported cases, 71% presented with vomiting, and 70% with some degree of dehydration. Notably, **60 (1%) had been vaccinated against rotavirus.**

Table 2. Demographic and Clinical Profile of Rotavirus Cases* (N=8,742), Philippines, Jan 4 to Sep 19, 2026

		Median	Range
Age (in years)		1	<1 to 5
		No. of Cases	Percentage
Sex	Female	3,692	42
	Male	5,050	58
Fever	Present	4,105	47
	Unknown	4,637	53
Vomiting	Present	6,214	71
	Unknown	2,528	29
Degree of Dehydration	Severe dehydration	153	2
	Some dehydration	6,162	70
	No dehydration	318	4
	Unknown	2,109	24
Laboratory Testing	Positive	333	4
	Negative	658	8
	Pending	89	1
	Equivocal	7	<1
	Not done/ Unknown	7,655	88
Health Facility	Sentinel Site	778	9
	Non Sentinel Site	7,964	91

*includes suspected and confirmed cases

- Out of the 23 reported deaths, ages ranged from under 1 year to 3 years, with mostly males. Common symptoms included fever, vomiting, and some degree of dehydration. **Two cases tested positive for rotavirus**, three were negative, and the testing status of the remaining cases remains unknown. Notably, **none had received rotavirus vaccination**.

Table 3. Demographic and Clinical Data of Rotavirus Deaths (n=23), Philippines, Jan 4 - Sep 19, 2026

		Median	Range
Age (in years)		1	<1 to 3
Onset-to-admit (days)		2	<1 to 135
		No. of Cases	Percentage
Sex	Female	11	48
	Male	12	52
Fever	Present	18	78
	Unknown	5	22
Vomiting	Present	20	87
	Unknown	3	13
Degree of Dehydration	Severe dehydration	9	39
	Some dehydration	10	43
	No dehydration	2	9
	Unknown	2	9

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on Sept 21, 2026.

Annex A

Subregional Analyses of CAR

- As of Morbidity Week 37 (January 4 to September 19, 2026), 553 rotavirus cases were reported, a 76% increase from 314 cases reported in the same period last year. Cases peaked in MW 28, but cases have shown a downward trend since.
- CAR exceeded its epidemic threshold at least twice in the past 4 morbidity weeks (*Figure 2*). Within the region, Benguet and Baguio City exceeded their epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026).
- No deaths have been reported.
- Clustering of cases was reported in Brgy. Irisan, Baguio City with 3 cases in the past four weeks.
- Out of the 553 cases, the sex distribution was predominantly male (58%), with ages ranging from less than 1 year to 4 years. Notably, less than 1% of the cases had been vaccinated.
- Among the reported cases from the region, all were suspect cases.

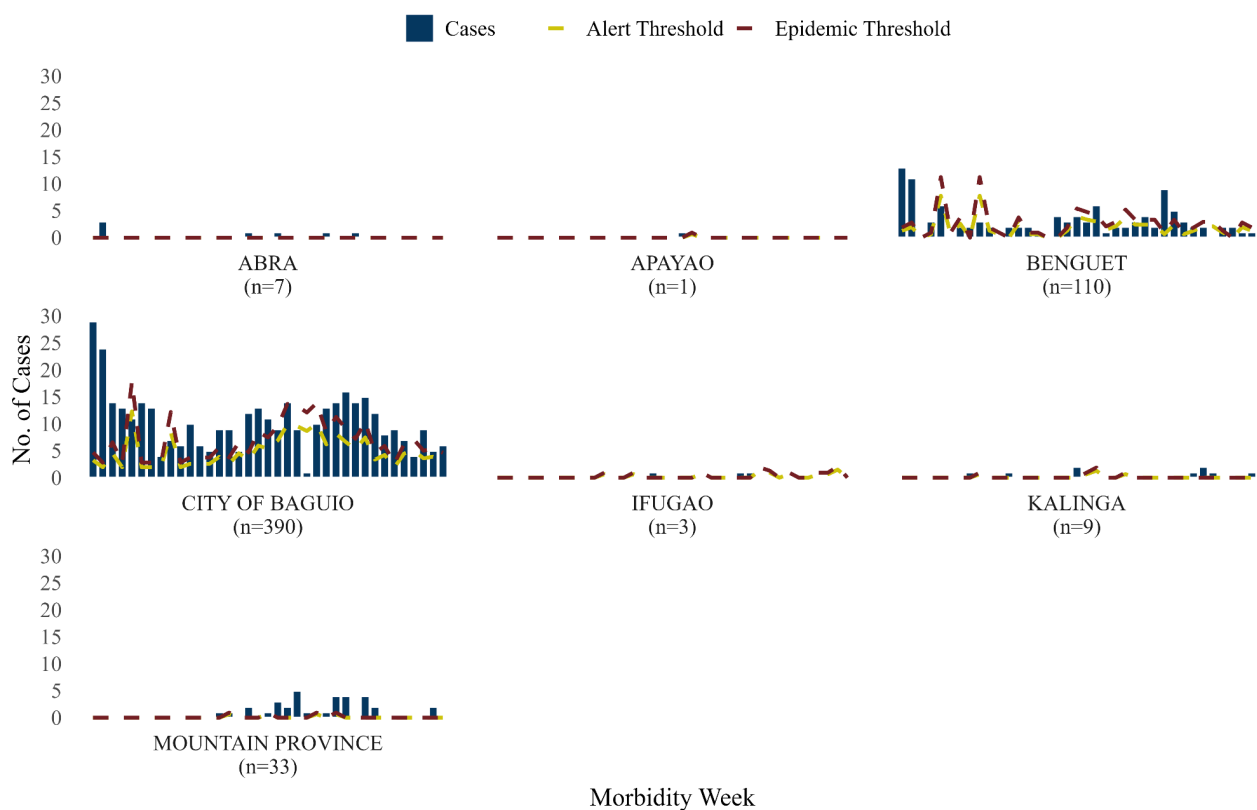


Figure A.1. Provincial/City Cases by Morbidity Week (N=553)
CAR, Jan 4 to Sep 19, 2026

Annex B

Subregional Analyses of Region I

- As of Morbidity Week 37 (January 4 to September 19, 2026), 1,643 rotavirus cases were reported, a 338% increase from 375 cases reported in the same period last year. Cases have consistently been higher compared to 2025 trends, with fluctuations observed throughout the year and no sustained upward or downward trend.
- Region I exceeded its epidemic threshold at least twice in the past 4 morbidity weeks (*Figure 2*). Within the region, all provinces exceeded their epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026).
- A total of 10 deaths were reported, resulting in a case fatality rate of 0.61%.
- Clustering of cases was reported in 6 barangays. Of these, Brgy. San Julian West reported the most cases, with 4 in the past four weeks.
- Out of the 1,643 cases, the sex distribution was predominantly male (58%), with ages ranging from less than 1 year to 5 years. Notably, 3% of the cases had been vaccinated.
- Among the reported cases from the region, 19 (1%) were confirmed cases, while all others were suspect cases.

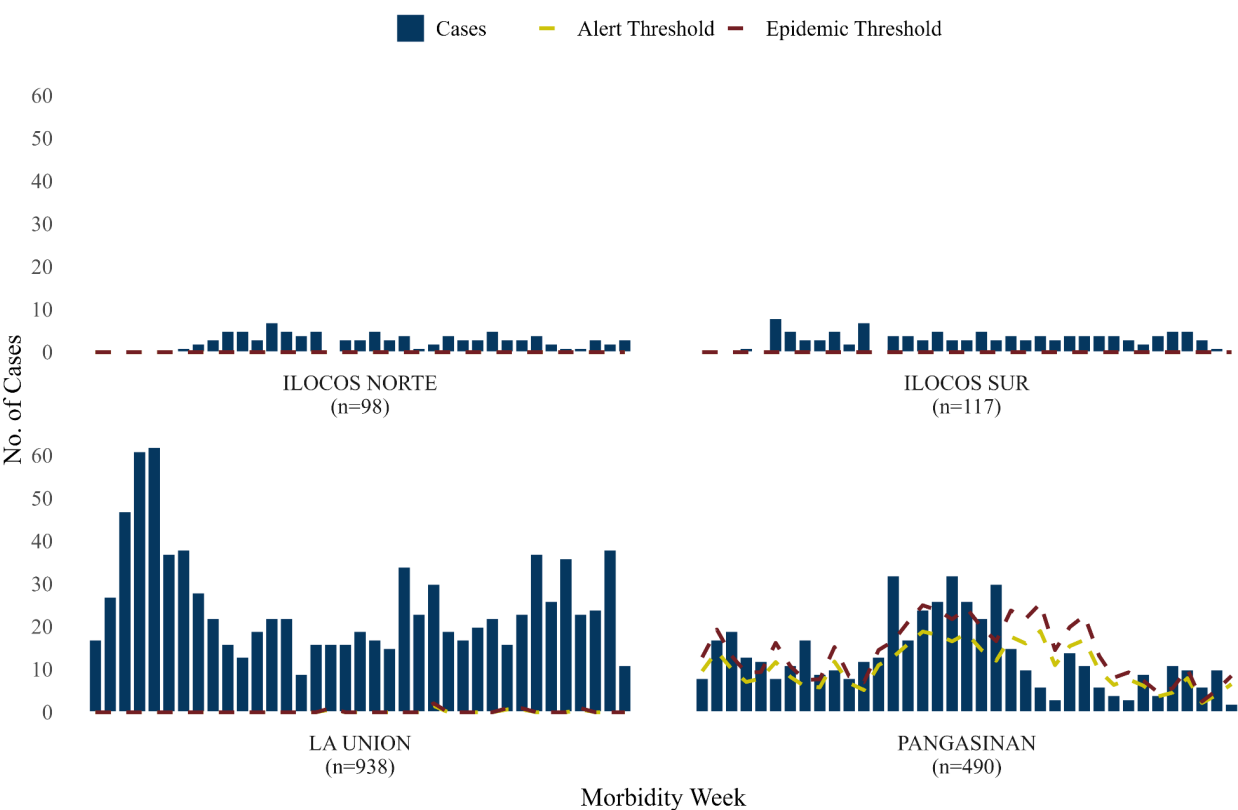


Figure B.1. Provincial/City Cases by Morbidity Week (N=1,643)
Region I, Jan 4 to Sep 19, 2026

Annex C

Subregional Analyses of Region III

- As of Morbidity Week 37 (January 4 to September 19, 2026), 417 rotavirus cases were reported, a 4% increase from 402 cases reported in the same period last year. Recent cases remained at elevated levels, forming a sustained plateau.
- Region III exceeded its epidemic threshold at least twice in the past 4 morbidity weeks (*Figure 2*). Within the region, Aurora, Bataan, and Pampanga exceeded their epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026).
- One death was reported, resulting in a case fatality rate of 0.24%.
- Clustering of cases was reported in 4 barangays. Of these, Brgy. Marcos Village, Mabalacat City reported the most cases, with 5 in the past four weeks.
- Out of the 417 cases, the sex distribution was predominantly male (57%), with ages ranging from less than 1 year to 5 years. Notably, less than 1% of the cases had been vaccinated.
- Among the reported cases from the region, 56 (13%) were confirmed cases, while all others were suspect cases.



Figure C.1. Provincial/City Cases by Morbidity Week (N=417)
Region III, Jan 4 to Sep 19, 2026

Annex D

Subregional Analyses of Region VII

- As of Morbidity Week 37 (January 4 to September 19, 2026), 857 rotavirus cases were reported, a 184% increase from 302 cases reported in the same period last year. Recent cases remained at elevated levels, forming a sustained plateau.
- Region VII exceeded its epidemic threshold at least twice in the past 4 morbidity weeks (*Figure 2*). Within the region, Bohol and Cebu exceeded their epidemic thresholds at least twice in the past four weeks (August 23 to September 19, 2026).
- One death was reported, resulting in a case fatality rate of 0.12%.
- No clustering of cases was reported in the past four weeks.
- Out of the 857 cases, the sex distribution was predominantly male (58%), with ages ranging from less than 1 year to 5 years. Notably, less than 1% of the cases had been vaccinated.
- Among the reported cases from the region, all were suspect cases.

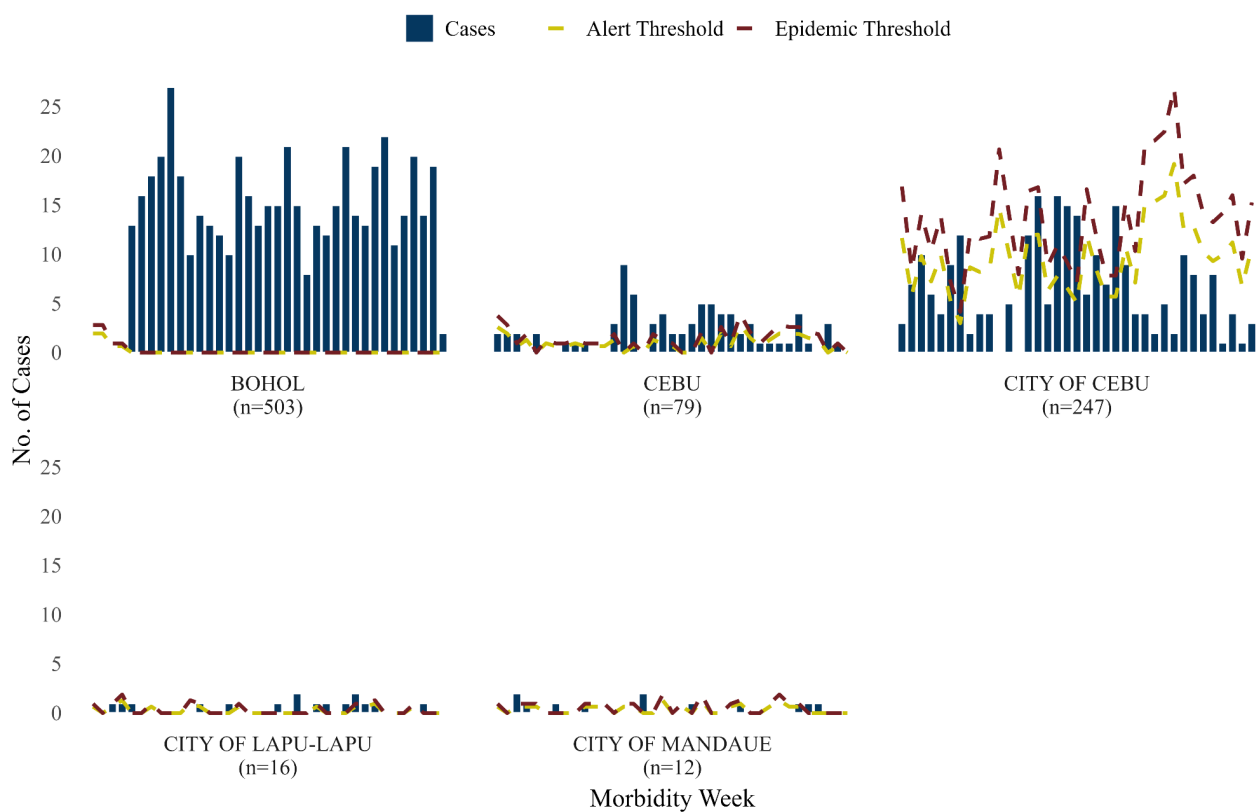


Figure D.1. Provincial/City Cases by Morbidity Week (N=857)
Region VII, Jan 4 to Sep 19, 2026



Typhoid Fever

Weekly Surveillance Report 2026

I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), 12,174 cumulative cases were reported, a 38% decrease from the 19,648 cases reported in the same period last year.
- A total of 32 deaths were reported, resulting in a case fatality rate of 0.26%.
- Nationally, cases have increased by 17% in MW 34 - 35 (Aug 23 to Sep 5, 2026), with 806 cases reported, compared to MW 32 - 33 (Aug 9 to Aug 22, 2026), which had 688 cases. The recent two MWs were not used for growth rate comparison due to the possibility of delayed reports.

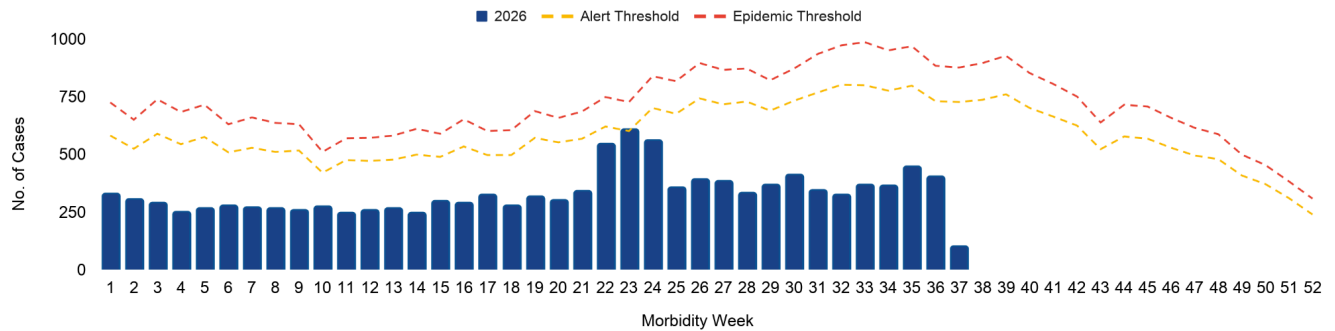


Figure 1. Cases by Morbidity Week (N=12,174)
Philippines, Jan 4 to Sep 19, 2026

II. Sub-national Distribution of Cases

- Regions V, IX, and NCR showed an increase in cases compared to the previous year.
- NCR reported the highest number of deaths, with nine (9) deaths, as well as the highest increase in deaths at 80% compared with the same period last year. Meanwhile, Region V recorded the highest case fatality rate (CFR) at 1.21%.
- While no region exceeded the epidemic threshold, Region XI exceeded the alert threshold at least twice during the past four weeks. Several provinces and highly urbanized cities across different regions also exceeded either the alert or epidemic threshold during the same period (Annex A).
- All regions, except I, X, XI, XIII, and BARMM, showed an increase in cases in the recent 3-4 weeks vs. 5-6 weeks.
- Clusters were reported in 50 barangays across 12 regions, with a maximum of 10 cases in a barangay over the past 4 weeks.

Note: Link for the Typhoid Clustering as of MW37: <https://tinyurl.com/MW37Clustering-Typhoid>



Figure 2. Regional Cases by Morbidity Week (N=12,174)
Philippines, Jan 4 to Sept 19, 2026

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR^ 3-4 vs 5-6 weeks (%)	Deaths			CFR (%)*
	2025	2026	% Change		2025	2026	% Change	
X	2397	1,622	↓32	↓11.48	3	1	↓67	0.06
IV-B	1,430	1,035	↓28	↑35.38	12	3	↓75	0.29
BARMM	1,789	1,019	↓43	↓12.12	3	1	↓67	0.10
NCR	427	971	↑127	↑11.11	5	9	↑80	0.93
CAR	1,595	913	↓43	↑47.92	2	0	↓100	0
II	1,827	723	↓60	↑95.83	0	1	↑	0.14
NIR	1,756	711	↓60	↑27.66	8	6	↓25	0.84
VI	1,109	671	↓39	↑50.00	6	1	↓83	0.15
IX	1,399	652	↓53	↑41.38	11	1	↓91	0.15
XI	357	603	↑69	↓30.77	1	1	-	0.17
III	1,002	568	↓43	↑76.00	3	0	↓100	0
XIII	611	531	↓13	↓27.59	5	1	↓80	0.19
VII	990	528	↓47	↑37.04	4	3	↓25	0.57
XII	887	482	↓46	↑47.37	4	0	↓100	0
I	848	416	↓51	↓10.87	1	0	↓100	0
IV-A	735	344	↓53	↑32.00	2	0	↓100	0
V	261	331	↑27	↑300.00	3	4	↑33	1.21
VIII	228	54	↓76	↑500.00	2	0	↓100	0

^Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)

*Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases.

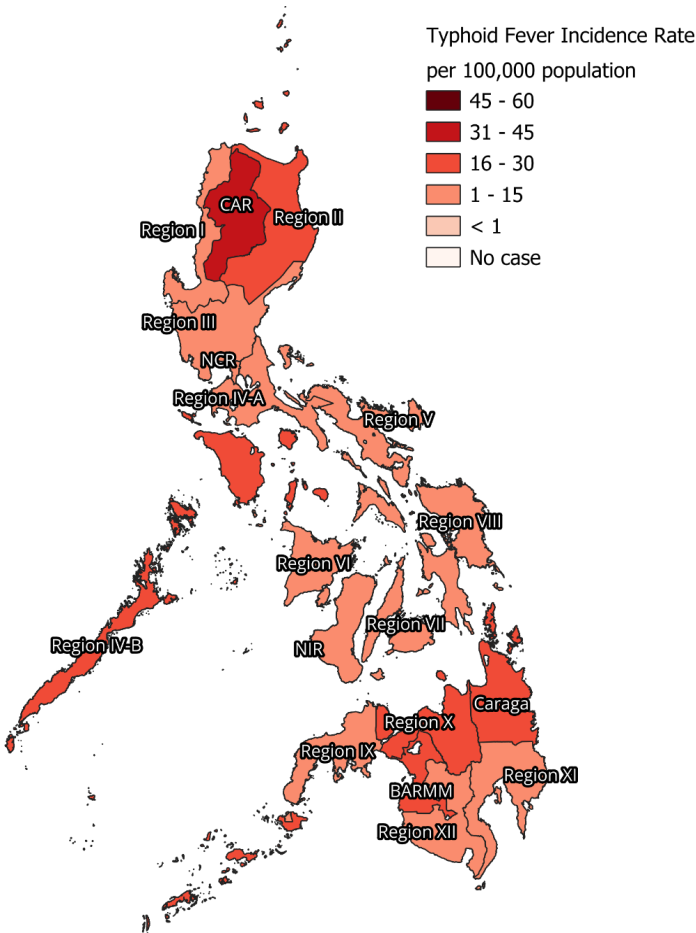


Figure 3. Typhoid Fever Cases by Region (N=12,174)
Philippines, Jan 4 to Sep 19, 2026

III. Demographic and Clinical Profile of Cases

- The cases' ages ranged from less than 1 to 103 years, with a median of 23 years, and the sex distribution was almost equal.
- A total of 22 cases were reported positive via culture and were tagged as confirmed cases, 6,779 (56%) cases were probable, while 5,373 (44%) cases were suspected.
- 10,763 (88%) cases and 30 (<1%) deaths are admitted to healthcare facilities, with consultation occurring within 1 to 86 days after symptom onset.
- Of the admitted cases, 63% (6,828) underwent laboratory testing.

Table 2. Demographic and Clinical Profile of Typhoid Fever Cases* (N=12,174), Philippines, Jan 4 to Sep 19, 2026

		Median	Range
Age (in years)		23	<1- 103
Onset-to-consult (days)		3	<1- 86
		No. of Cases	Percentage
Sex			
	Female	6,185	51
	Male	5,989	49
Laboratory Results			
	Positive Culture	22	<1
	<i>Salmonella typhi</i>	19	86
	<i>Salmonella paratyphi</i>	0	0
	<i>Salmonella</i> group	3	14
	Positive TyphiDot	5,767	47
	Positive Tubex	626	5
	Positive Widal	386	3
	Negative	1,341	11
	Not Tested/ Unknown	4,032	33
Case Classification			
	Confirmed	22	<1
	Probable	6,779	56
	Suspect	5,373	44
Hospitalization			
	Admitted	10,763	88
	Not Admitted	1,411	12

*includes suspected, probable, and confirmed cases

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026.

ANNEX A

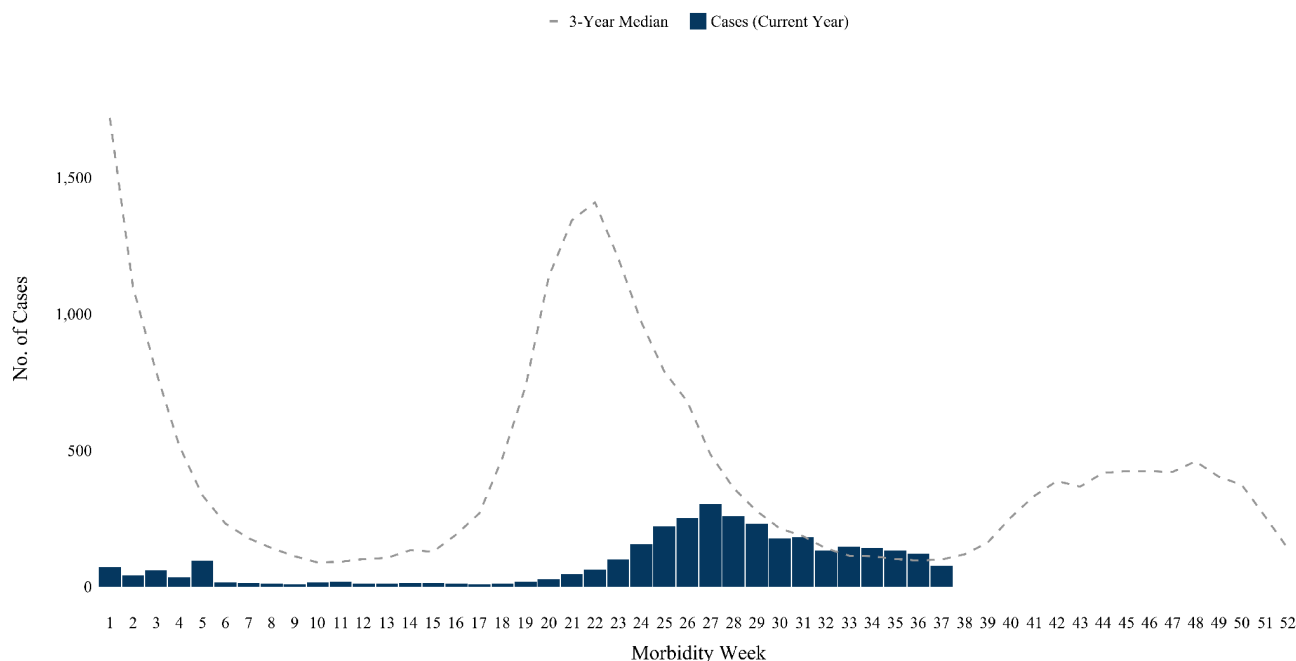
Province and Highly Urbanized Cities (P/HUCs) with Threshold Breaches, Philippines, Aug 23 to Sep 19, 2026

Geographic Area	Thresholds Breached in Past 4 MWs
NCR	
City Of Mandaluyong	Alert
City Of Valenzuela	Epidemic
Pasay City	Epidemic
CAR	
City Of Baguio	Epidemic
Region II	
Quirino	Epidemic
Region III	
Bulacan	Epidemic
Region IV-A	
Laguna	Alert
Region IV-B	
City Of Puerto Princesa	Epidemic
Occidental Mindoro	Epidemic
Romblon	Alert
Region V	
Albay	Alert
Sorsogon	Epidemic
Region VI	
Capiz	Alert
Region VII	
Bohol	Alert
Region XI	
Davao Del Norte	Epidemic
Davao Oriental	Epidemic
Region XIII	
Surigao Del Sur	Epidemic

**Note: Only P/HUCs with at least two alert or epidemic threshold breaches in the past four weeks are shown.*

I. National Trends

- As of Morbidity Week 37 (Jan 4 to Sep 19, 2026), 3,271 cumulative confirmed COVID cases were reported, a 49% decrease from the 6,360 cases in 2025. Of these, 42 deaths were reported, resulting in a case fatality rate (CFR) of 1.28%, a 50% decrease from the 84 deaths last year (CFR: 1.32%).
- 76 new confirmed COVID cases were reported this week, resulting in an average of 11 cases reported per day. Among the confirmed cases with reported health status, the majority were mild (66%)
- Reported cases have decreased by 2% in MW 34-35 (August 23, 2026 to September 05, 2026), with 275 cases compared to MW 32-33 (August 09, 2026 to August 22, 2026), which had 281 cases. *It is important to note that the recent two MWs are included in the total counts but not used for growth rate comparison due to the possibility of delayed reports.*



**Figure 1. Confirmed Cases by Morbidity Week (N=3,271)
Philippines, Jan 4 to Sep 19, 2026**

II. Sub-national Distribution of Cases

- Cumulatively, NCR has the highest number of confirmed COVID-19 cases (1,078, 33%), followed by IV-A (496, 15%), and III (355, 11%). No new deaths were reported.
- NCR has shown a continuous increase in cases in the recent six-week period, with cases ranging from 41 to 57 per two weeks.
- In terms of clustering, a total of 24 barangays have reported clusters across 10 regions.

Note: Link for clustering as of MW 37 2026: <https://tinyurl.com/MW37-ClusteringC19>

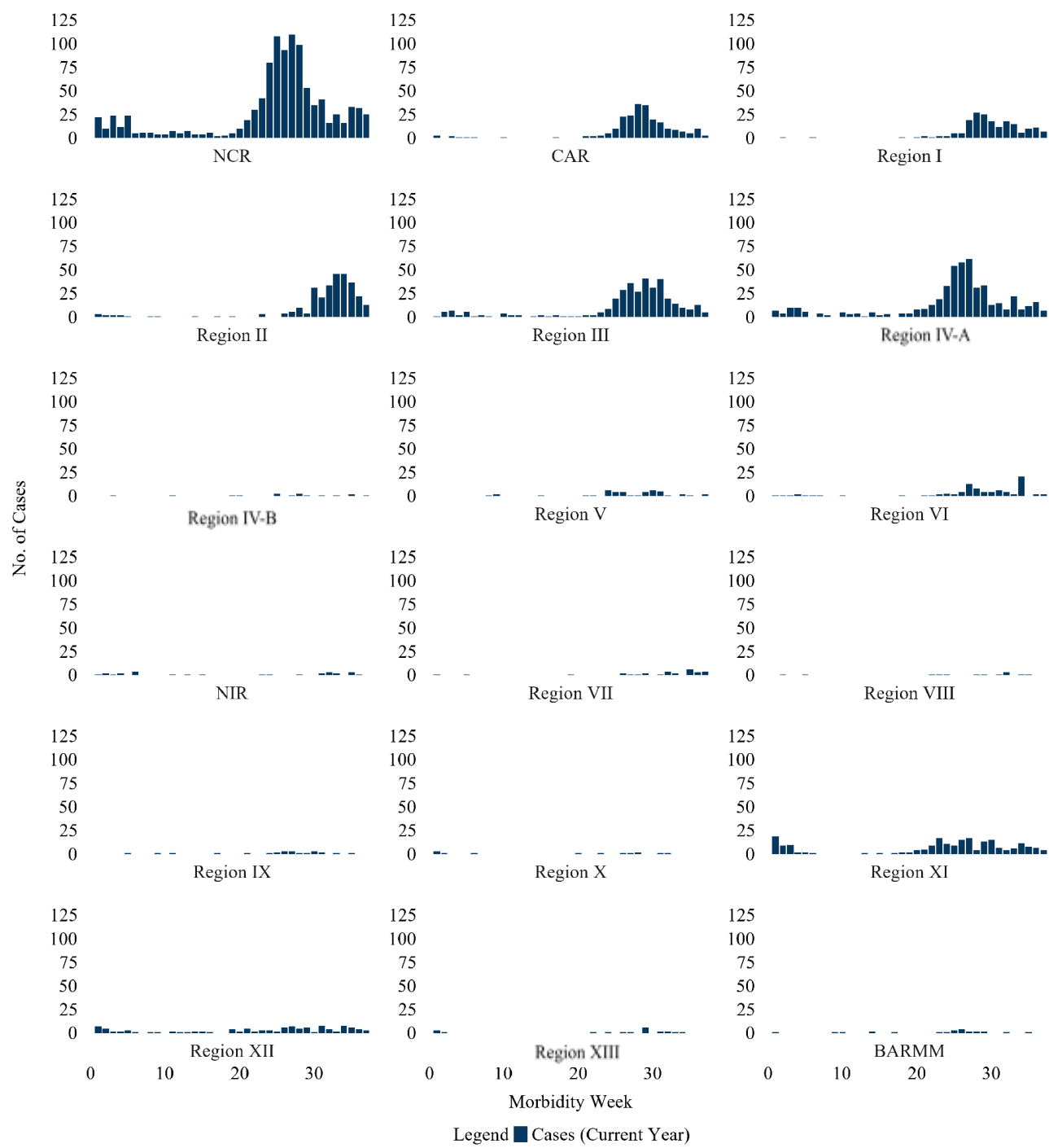


Figure 2. Regional Confirmed Cases by Morbidity Week (N=3,271)
Philippines, Jan 4 to Sep 19, 2026

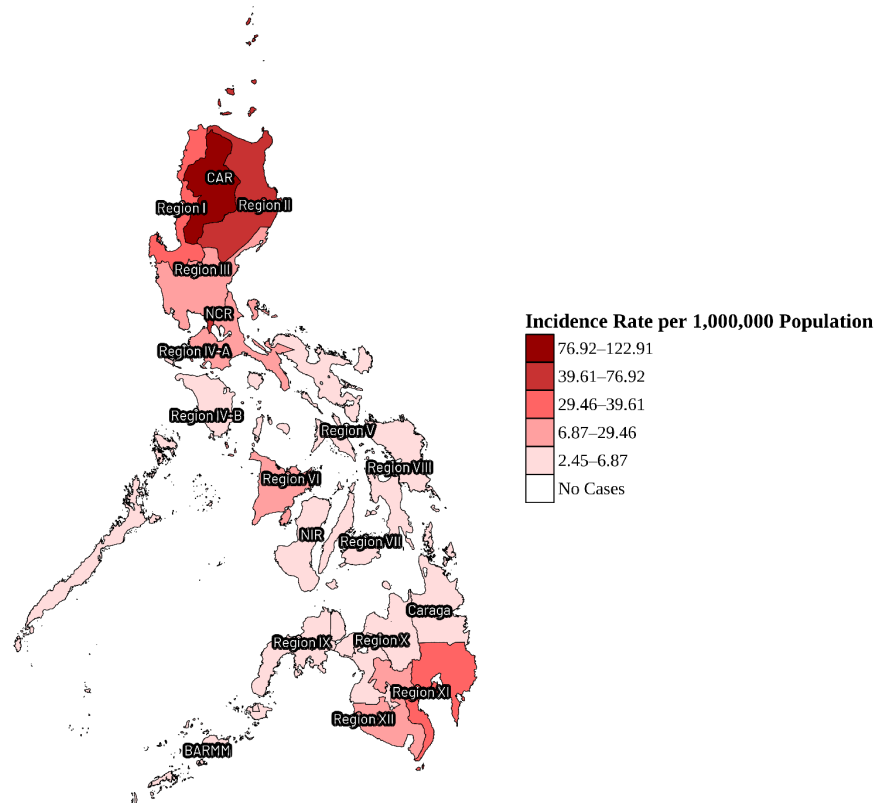
Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases		TWGR [^]		Deaths		CFR (%)**	
	2025*	2026	% Change	3-4 vs 5-6 weeks (%)	2025	2026	% Change	2026
NCR	1,875	1,078	↓43	↑19.51%	38	26	↓32	2.41
IV-A	1,044	496	↓52	↓33.33%	18	8	↓56	1.61
III	923	355	↓62	↓47.06%	10	1	↓90	0.28
II	385	292	↓24	↑3.75%	3	0	↓100	0.00
CAR	140	231	↑65	↓36.84%	3	0	↓100	0.00
XI	213	217	↑2	↑100.00%	1	4	↑300	1.84
I	207	189	↓9	↓51.52%	3	0	↓100	0.00
XII	321	112	↓65	↑133.33%	0	1	↑	0.89
VI	263	89	↓66	↑250.00%	2	0	↓100	0.00
V	140	43	↓69	↑200.00%	1	0	↓100	0.00
VII	396	29	↓93	-	2	0	↓100	0.00
NIR	68	27	↓60	↓40.00%	1	0	↓100	0.00
BARMM	94	23	↓76	-	1	0	↓100	0.00
IX	52	23	↓56	-	0	0	-	0.00
IV-B	59	21	↓64	↑400.00%	0	0	-	0.00
XIII	91	20	↓78	↓66.67%	0	2	↑	10.00
X	62	13	↓79	↓100.00%	0	0	-	0.00
VIII	27	13	↓52	↓33.33%	1	0	↓100	0.00

*Data is based on the final 2025 reconciled database from the Regional Epidemiology and Surveillance Units

**Case Fatality Rate (CFR) is computed by dividing the total number of deaths per disease/condition by the total number of reported cases

[^]Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)



**Figure 3. Regional Incidence Rate (N=3,271)
Philippines, Jan 4 to Sep 19, 2026**

III. Demographic Profile of Cases

a. Distribution of Cases by Sex and Age Groups

Most cases were reported among individuals aged 15 to 49 years (1,436), 44%, with age ranging from children below one year to 98 years old, and a median age of 32 years. Among these, female cases (1,852, 57%) were higher compared to male (1,419, 43%).

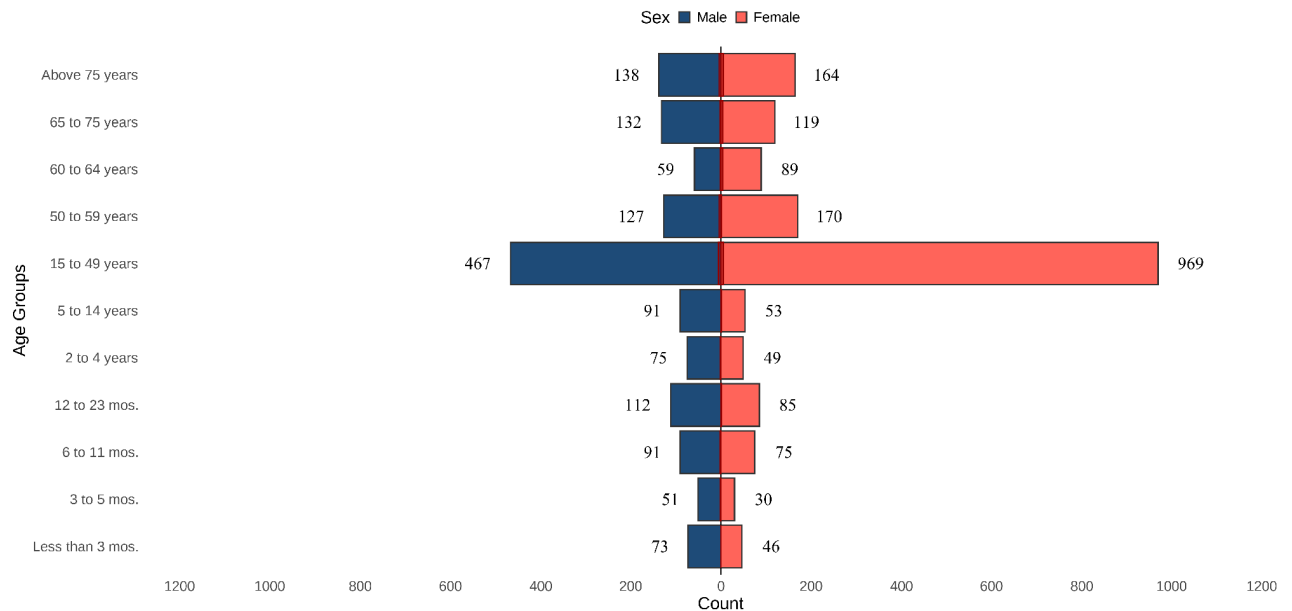


Figure 4. Age-Sex Distribution of Cases (N=3,271)*
Philippines, Jan 4 to Sep 19, 2026

**Note: The age group of six cases is still for verification.*

b. Case Distribution by Health Status

Among the reported confirmed COVID-19 cases, almost half are classified with a “resolved” health status. Of these resolved cases, the majority have recovered. However, more than half of the cases are still tagged as active across the country.

Table 2. Distribution of COVID-19 Cases by Health Status (N=3,271), Philippines, Jan 4 to Sep 19, 2026

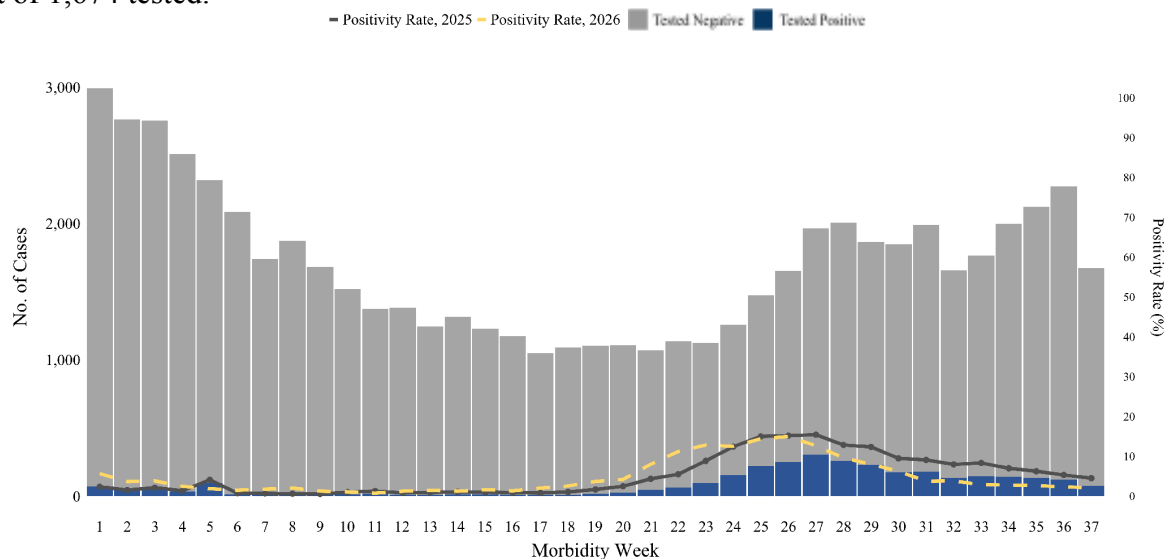
Health Status	Number of Cases	Percentage (%)
Active Cases	1,667	51
Resolved Cases	1,604	49
Recovered Cases	1,562	97*
Death Cases	42	3*
For Verification	0	0
Total Cases	3,271	100

**The percentage of recovered and death cases is computed by dividing the total number of recovered OR death cases by the total number of resolved cases.*

IV. Laboratory Profile

Cumulatively, among the 63,274 tested cases that met the case definition, 3,271 (5.17%) were positive for SARS-CoV-2, while for the current reporting week (MW 37), 76 (4.54%) were positive out of 1,674 tested.

Z



**Figure 5. Test Positivity Rate of Confirmed COVID (N=3,271)
Philippines, Jan 4 to Sep 19, 2026**

IV. Genomic Surveillance

Table 4. Summary of SARS-CoV-2 Variants Classification of WHO and ECDC, as of September 19, 2026

Classification	World Health Organization (WHO) ¹	European Centre for Disease Prevention and Control (ECDC) ²
Variant of Interest (VOI)	JN. 1	BA.2.86
Variant under Monitoring (VUM)	PQ.16.1.1; NB.1.8.1; XFG; BA. 3.2	NB.1.8.1; XFG; BA.3.2

- As of September 21, 2026, new sequencing results were received from the Research Institute for Tropical Medicine (RITM).
 - All 7 samples were assigned a SARS-CoV-2 lineage, with collection dates ranging from April 8, 2026 to June 30, 2026, from Regions IV-A (3 samples) and NCR (4 samples).
 - Of these, five samples were classified as XFG.6.13, while the remaining samples were classified as XFG.14.1 and RF.5.3.
 - XFG.6.13 and XFG.14.1 were under the XFG sublineages, and have been designated by the WHO as Variant Under Monitoring (VUM) as of February 23, 2026. Current overall risk evaluation by the WHO remains low as there are no reports to suggest that the associated disease severity is higher in comparison to other circulating variants (see <https://tinyurl.com/XFGRiskAssessmentWHO>).

Note: Reported cases may still change due to late consultations and delayed reporting. Data was extracted on September 21, 2026.

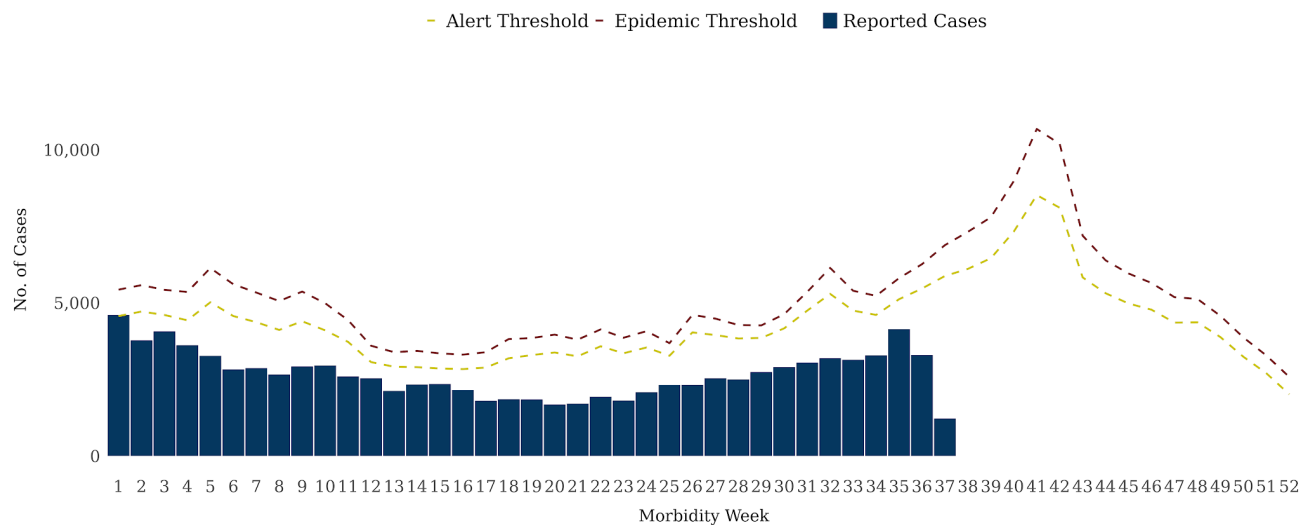
¹ World Health Organization. (n.d.). Tracking SARS-CoV-2 variants. Retrieved September 21, 2026, from <https://www.who.int/activities/tracking-SARS-CoV-2-variants>

² European Centre for Disease Prevention and Control. (n.d.). Variants of concern. Retrieved September 21, 2026, from <https://www.ecdc.europa.eu/en/covid-19/variants-concern>



I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), there were 99,037 reported cumulative cases, a 22% decrease from 127,148 cases reported in the same period last year.
- The case trend is increasing, with 7,427 cases reported in the recent 3–4 weeks (August 23 to September 05), 17% higher than the 6,334 cases reported two weeks prior (August 09 to August 22). The recent two MWs are included in the total counts but not used for growth rate comparison due to the possibility of delayed reports.
- A total of 53 deaths were reported (CFR: 0.05%), 10% lower than the 59 deaths reported last year (CFR: 0.05%).



**Figure 1. Cases by Morbidity Week (N=99,037)
Philippines, Jan 4 to Sep 19, 2026**

II. Sub-national Distribution of Cases

- Regions III and IV-B have breached their epidemic threshold in the past four weeks.
- Regions IV-B, VI, and VII recorded the highest percentage increase in cases across all regions.
- One thousand and sixty-eight (1,068) clusters were identified in the past four weeks, with Region IV-A (168, 16%) identified with the highest number of clusters reported.

Link for the ILI Clustering as of MW 37: <https://tinyurl.com/bdzj5kbbk>

Influenza-like Illness

EB WSR 2026

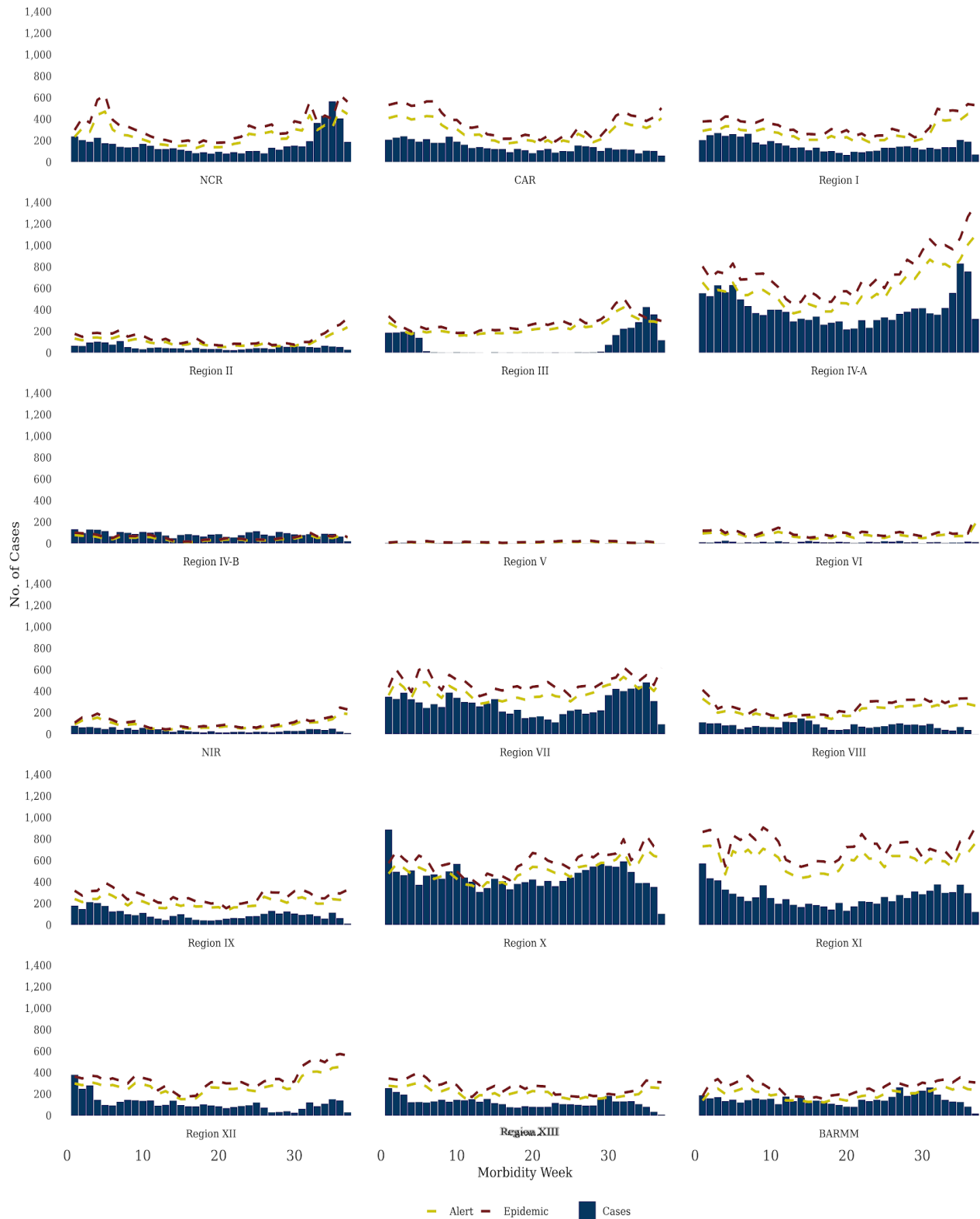


Figure 2. Regional Cases by Morbidity Week (N=99,037)
Philippines, Jan 4 to Sep 19, 2026

Influenza-like Illness

EB WSR 2026

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR [^]		Deaths			CFR 2026
	2025	2026	% Change	3-4 vs 5-6 weeks (%)		2025	2026	% Change	
X	14,765	16,537	↑12	↓	28.11	0	0	-	0.00
IV-A	20,576	14,921	↓27	↑	80.16	9	3	↓67	0.02
VII	7,835	10,201	↑30	↑	12.33	2	2	↓0	0.02
XI	13,044	9,727	↓25	↑	0.89	0	2	↑	0.02
NCR	7,725	6,263	↓19	↑	78.04	3	2	↓33	0.03
I	7,735	5,668	↓27	↑	32.31	3	1	↓67	0.02
BARMM	8,491	5,500	↓35	↓	24.56	12	20	↑67	0.36
CAR	7,622	5,196	↓32	↓	18.88	0	1	↑	0.02
XIII	5,162	4,490	↓13	↓	28.57	4	16	↑300	0.36
XII	8,073	4,213	↓48	↑	24.64	3	2	↓33	0.05
IX	7,033	3,531	↓50	↓	4.42	2	0	↓100	0.00
IV-B	1,546	3,240	↑110	↑	20.67	10	4	↓60	0.12
III	5,043	2,882	↓43	↑	55.80	4	0	↓100	0.00
VIII	6,667	2,809	↓58	↑	2.02	6	0	↓100	0.00
II	3,789	1,915	↓49	↑	23.00	1	0	↓100	0.00
NIR	1,692	1,367	↓19	↓	6.25	0	0	-	0.00
VI	206	481	↑133	↓	10.53	0	0	-	0.00
V	144	96	↓33	↑	266.67	0	0	-	0.00

[^]Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)

*Case Fatality Rate (CFR) is computed by dividing total deaths by total reported cases.

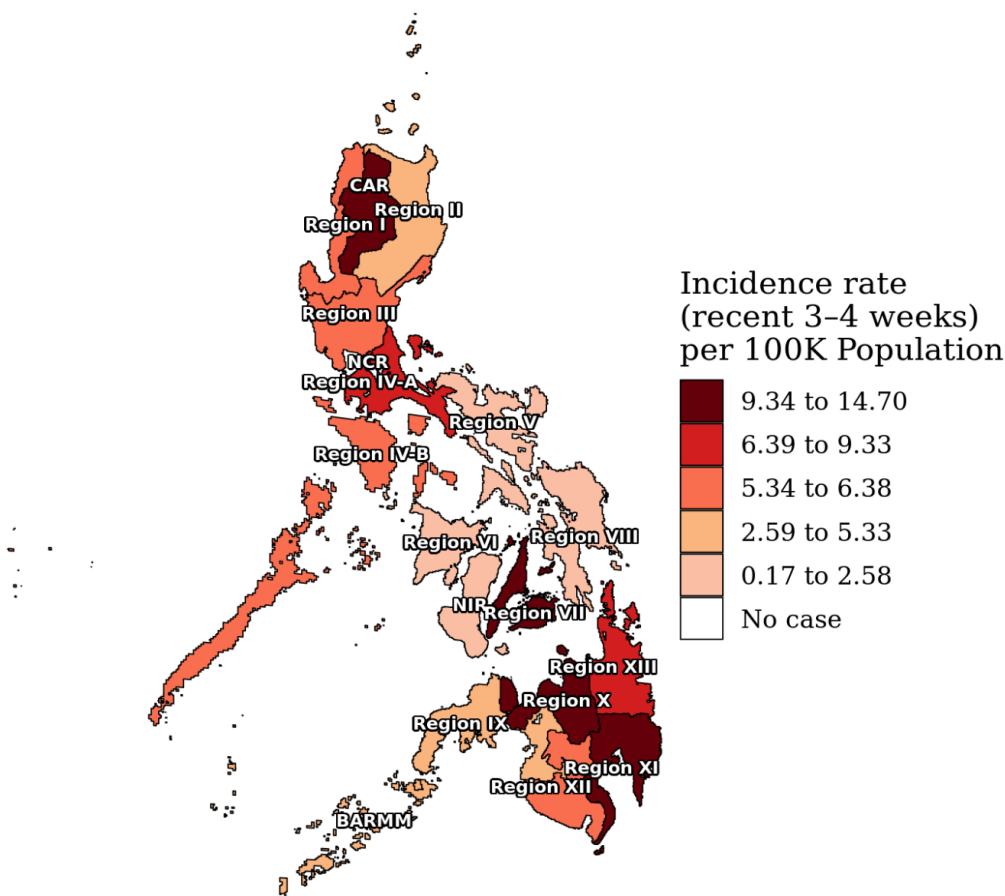
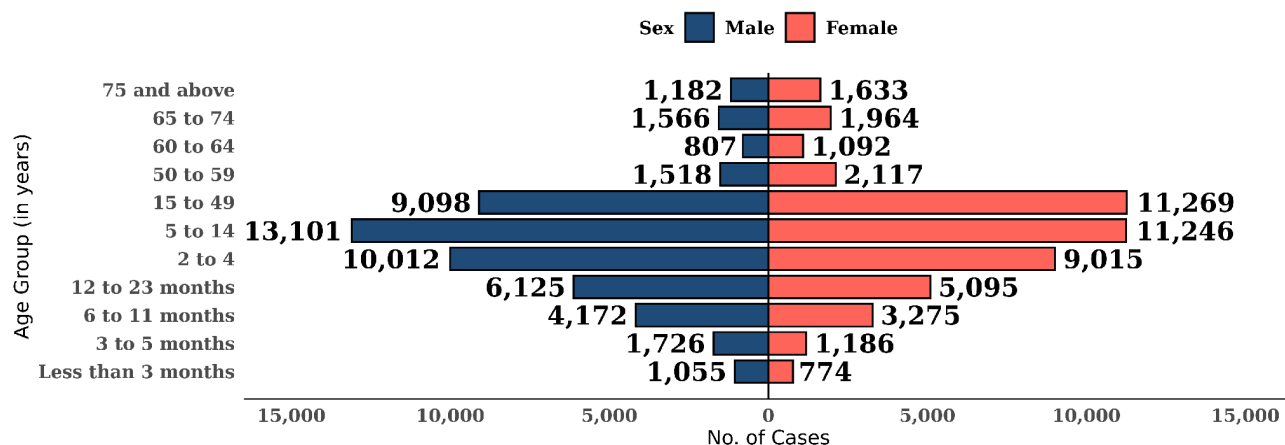


Figure 3. Regional Incidence Rate (n=7,427)*
Philippines, Aug 23 to Sep 5, 2026

*The previous 3-4 weeks were from the MW 34 - 35, 2026 reported cases.

III. Profile of Cases

A. Reported cases were generally comparable between sexes, with 51% (50,366) males and 49% (48,671) females. Ages ranged from <1 month to 109 years, with a median of 6. The age group with the highest case contribution was children 5 to 14 years of age (24,347, 25%).



**Figure 4. Age and Sex Distribution of Cases (N=99,037*)
Philippines, Jan 4 to Sep 19, 2026**

*The ages of 9 cases are still for verification.

B. Of the 1,166 cases tested for influenza, SARS-CoV-2, and Respiratory Syncytial Virus (RSV) from sentinel sites, 149 (12.8%) had positive laboratory results. Cumulatively, influenza B accounted for the highest proportion of positive detections (62, 42%). Of the 131 cases tested this month, 10 had positive results, with influenza A and B accounting for 70% of positive results.

Table 2. Laboratory Results of Tested ILI Cases (n=1,166), Philippines, Jan 4 to Sep 19, 2026

Result	Causative Agent*	Cumulative Total	August 2026	September 1 to 17, 2026
		(Jan 4 to Sep 17, 2026)**		
Positive		149 (12.8%)	36 (27.5%)	10 (24.4%)
	Influenza B	62 (42%)	19 (53%)	3 (30%)
	SARS-CoV-2	45 (30%)	7 (19%)	3 (30%)
	Respiratory Syncytial Virus (RSV)	29 (19%)	3 (8%)	0 (0%)
	Influenza A	13 (9%)	7 (19%)	4 (40%)
Negative		1,017 (87.2%)	95 (72.5%)	31 (75.6%)
Total Cases Tested‡		1,166 (100.0%)	131 (11.2%)	41 (3.5%)

*For causative agents, the percentage is computed as the total number of samples for a specific respiratory virus divided by the total number of positive laboratory results. Hence, co-infection with multiple viruses may cause pathogen breakdown to exceed 100% of positive cases.

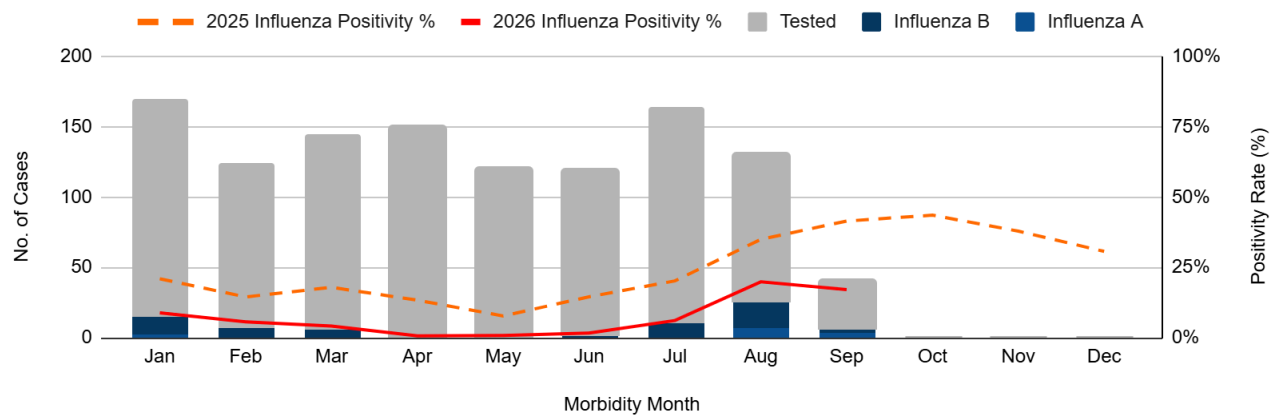
** Reporting period is based on sample collection and not the onset of symptoms

‡ Reported results are as of Sep 17, 2026.

Influenza-like Illness

EB WSR 2026

C. At sentinel sites, the influenza positivity rate has been increasing since May and is at 17% this September, comparable to the previous month (20%). This is still below the rate last year (41%) and below the average monthly positivity rate of 23% in 2025, excluding the peak influenza positivity observed in October.

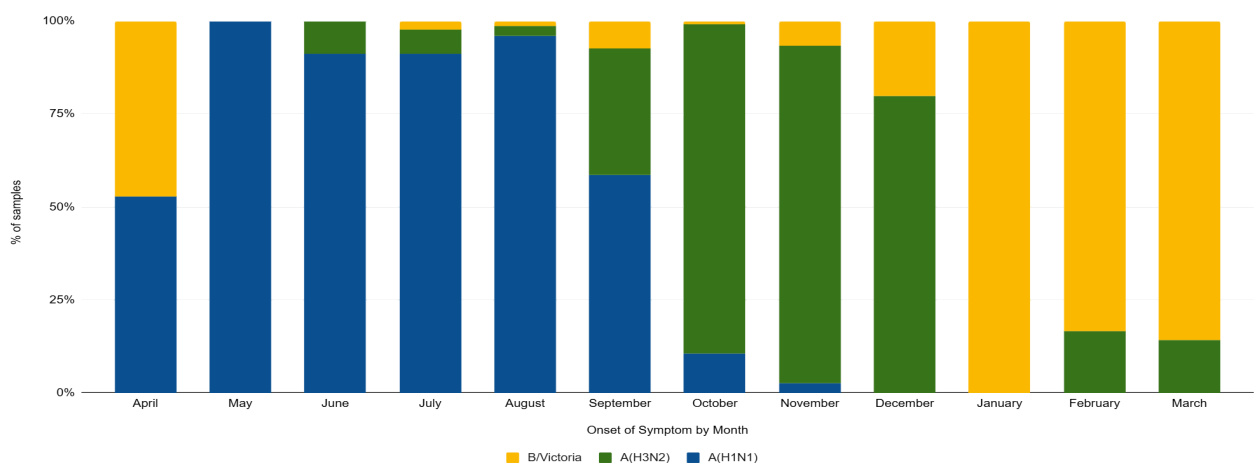


**Figure 5. Positivity Rate of Influenza A/B by Month (n=1,166)
Philippines, Jan 4 to Sep 17, 2026**

D. A total of 437 samples were sequenced year-round. The majority were Influenza A (91%), including 195 (45%) A(H1N1) and 204 (47%) A(H3N2).

Among A(H3N2) cases, 184 belonged to the J.2.4.1 lineage (alias subclade K). This is 61% of all influenza cases sequenced nationwide from August 6 to March 27, 2026. No subclade K viruses were detected prior to this period.

Per WHO Risk Assessment as of December 10, 2025, current epidemiological data do not suggest an increase in disease severity among subclade K cases. Vaccination is still expected to protect against severe illness and prevent hospitalization.



**Figure 6. Proportion of Influenza Genomic Surveillance Samples Sequenced by Month (n=437)
Philippines, Apr 2025 - Mar 2026**

**No new updates as of this report. For detailed distribution of subclade K cases, please refer to the morbidity week 20 report.*

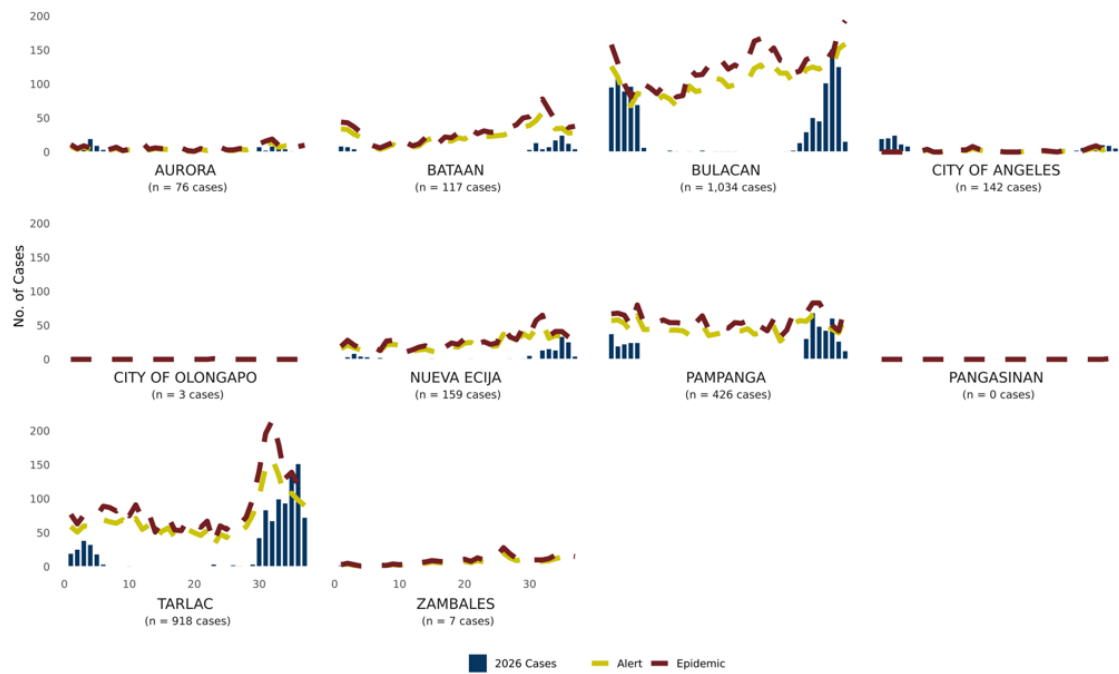
LINK: <https://tinyurl.com/WSRmw202026>

Note: Reported cases may still change due to late consultations and delayed reporting. Data was extracted on September 21, 2026.

Annex A

Subregional Analyses of Region III (Central Luzon)

In Region III, Tarlac (461 cases) breached its epidemic thresholds for at least two morbidity weeks in the recent four weeks (Annex A.1). Of the 61 cases tested, 56 tested positive, with no cases testing positive for influenza (INF), SARS-CoV-2, or RSV.



**Annex A.1 . Provincial/City Cases by Morbidity Week (n=2,882)
Region III, Jan 4 to Sep 19, 2026**

The most affected age group was 15–49 years, with 810 cases reported. No deaths were identified among the reported cases.

Table A.1. Profile of Cases (N=2,882), Region III, Philippines, Jan 4 to Sep 19, 2026

		Median	Range
Age		8.76 years	1 month - 92 years
		No. of Cases	Percentage
Sex			
	Male	1,509	52
	Female	1,373	48
		No. of Cases	%
Laboratory Results			
Positive		56	92
	Influenza A	0	0
	Influenza B	0	0
	RSV	0	0
	SARS-CoV-2	0	0
Negative		5	8
Total tested		61	100

One hundred and thirty-seven (13%) clusters were identified from this region with Barangay Palmayo, Floridablanca in Pampanga reporting the highest cases with 20 cases in a cluster.

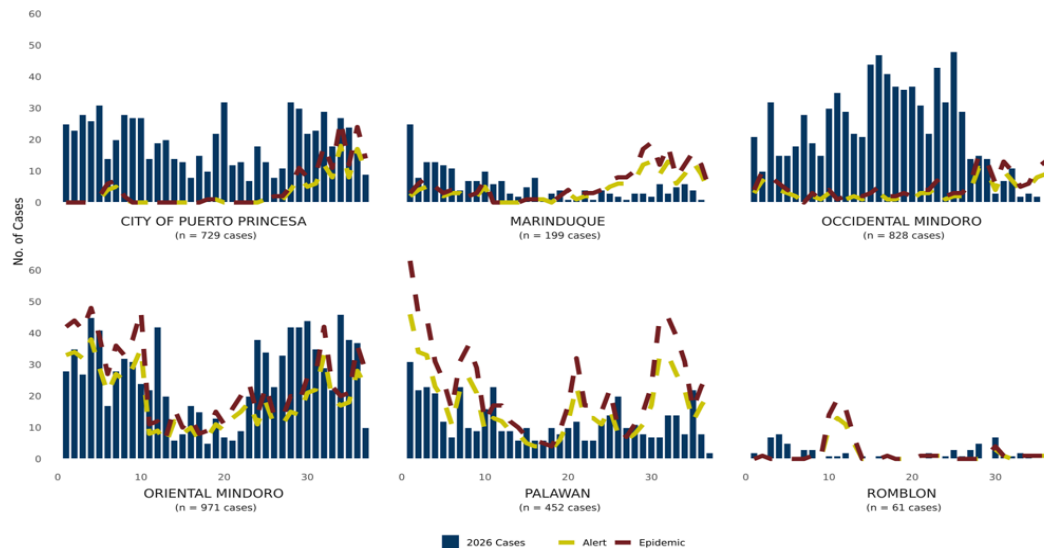
Influenza-like Illness

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Annex B

Subregional Analyses of Region IV-B (MIMAROPA Region)

In Region IV-B, City of Puerto Princesa (77 cases) and Oriental Mindoro (131 cases) breached their epidemic thresholds for at least two morbidity weeks in the recent four weeks (Annex B.1). Of the five cases tested, 4 tested positive, with no cases testing positive for influenza (INF), SARS-CoV-2, or RSV.



Annex B.1 . Provincial/City Cases by Morbidity Week (n=3,240)
Region IV-B, Jan 4 to Sep 19, 2026

The most affected age group is 5-14 years old with 727 cases.

Table B.1. Profile of Cases (N=3,240), Region IV-B, Philippines, Jan 4 to Sep 19, 2026

		Median	Range
Age		6.94 years	<1 month - 9 years
		No. of Cases	Percentage
Sex			
	Male	1641	51
	Female	1599	49
		No. of Cases	Deaths (CFR)
Case Fatality Rate (per province)			
	Oriental Mindoro	971	0 (0.0%)
	Occidental Mindoro	828	0 (0.0%)
	City of Puerto Princesa	729	2 (0.3%)
	Palawan	452	0 (0.0%)
	Marinduque	199	0 (0.0%)
	Romblon	61	0 (0.0%)
Laboratory Results			
	Positive	4	80
	Influenza A	0	0
	Influenza B	0	0
	RSV	0	0
	SARS-CoV-2	0	0
	Negative	1	20
	Total tested	5	100

Seventeen (2%) clusters were identified from this region with Barangay San Miguel in City of Puerto Princesa reporting the highest cases with 13 cases in a cluster.

Severe Acute Respiratory Infection

Weekly Surveillance Report 2026

I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), 18,776 cases were reported, a 12% increase from 16,715 cases reported in the same period last year.
- The case trend is increasing, with 1,291 cases reported in the recent 3–4 weeks (August 23 to September 05), 21% higher than the 1,067 cases reported two weeks prior (August 09 to August 22). The recent two MWs are included in the total counts but not used for growth rate comparison due to the possibility of delayed reports.
- A total of 96 deaths were reported (CFR: 0.51%), 20% higher than the 80 deaths reported last year (CFR: 0.48%).

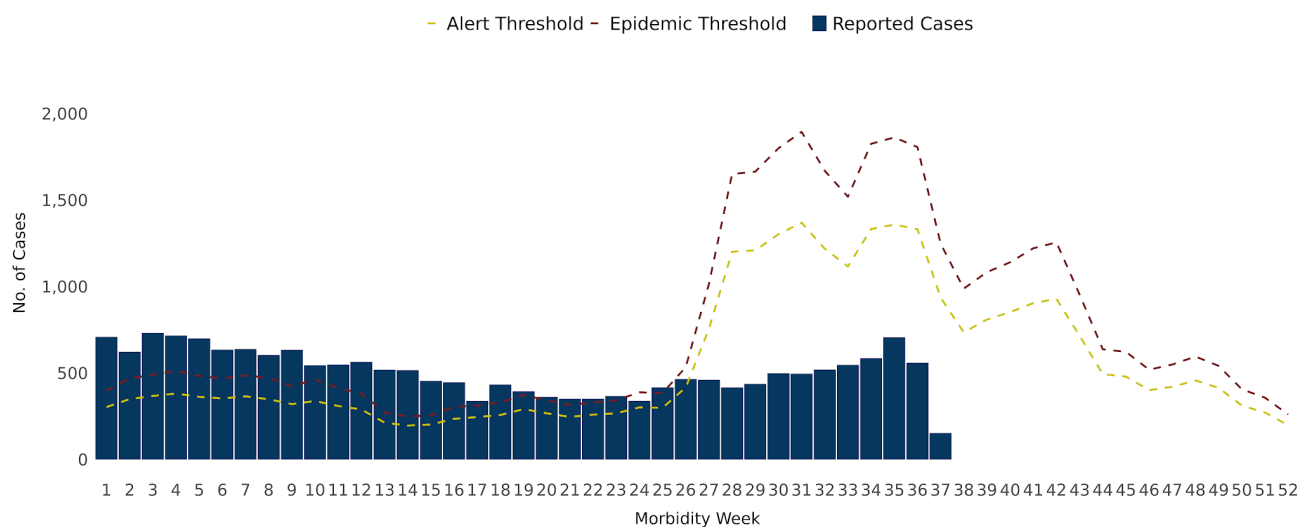


Figure 1. Cases by Morbidity Week (N=18,776)
Philippines, Jan 4 to Sep 19, 2026

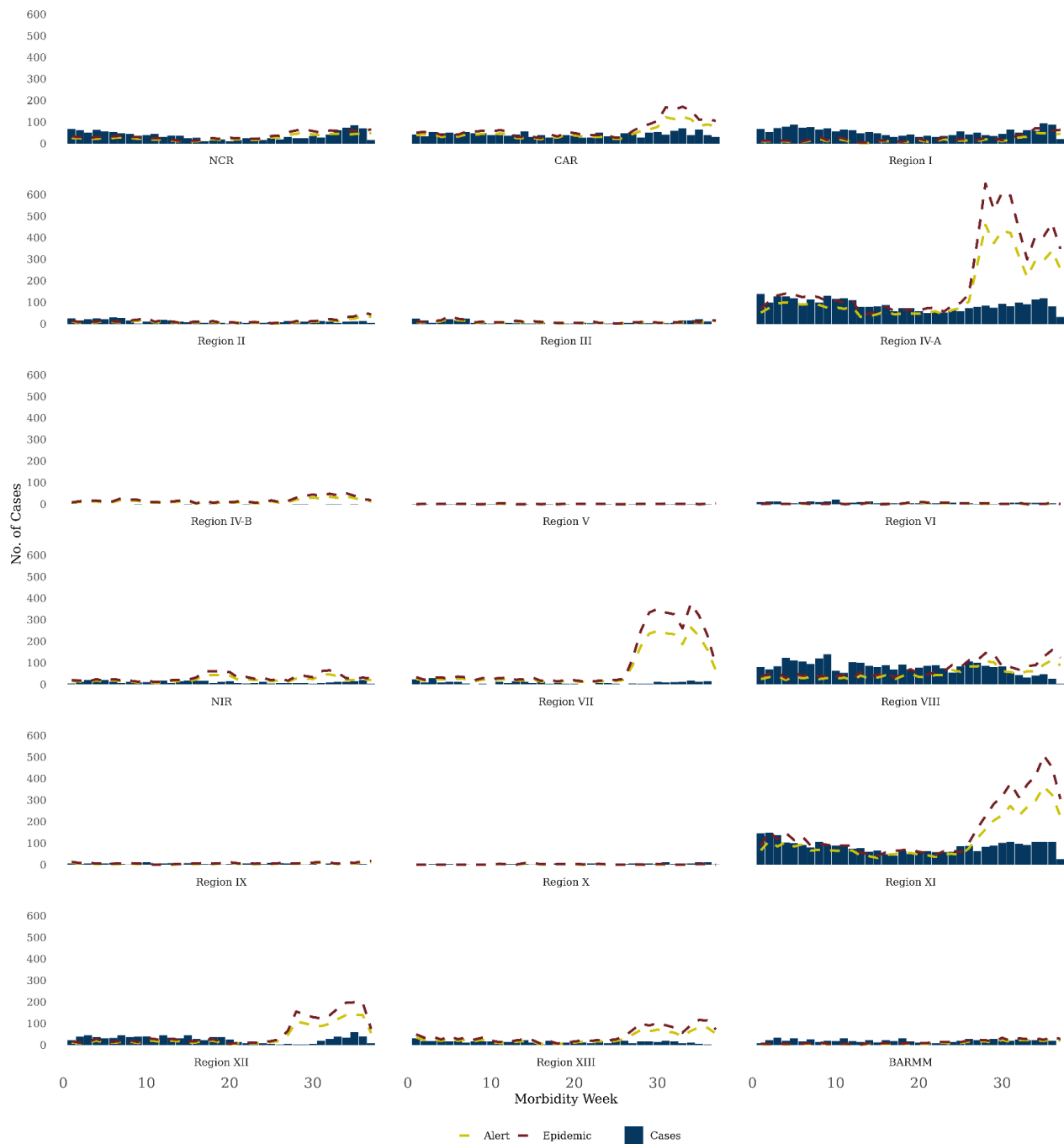
II. Sub-national Distribution of Cases

- Regions I, III, V, VI, X, and NCR have breached their epidemic thresholds at least twice in the recent four weeks.
- Regions X, XIII, and VI recorded the highest percentage increase in cases across all regions.
- One-hundred sixteen clusters were identified over the past four weeks, with Region XI (39, 34%) identified with the highest number of clusters reported.

Link for the SARI Clustering as of MW 37: <https://tinyurl.com/vmrvvedvw>

Severe Acute Respiratory Infection

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**Figure 2. Regional Cases by Morbidity Week (N=18,776)
Philippines, Jan 4 to Sep 19, 2026**

Severe Acute Respiratory Infection

EB WSR 2026

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR [^] 3-4 vs 5-6 weeks (%)	Deaths			CFR 2026
	2025	2026	% Change		2025	2026	% Change	
IV-A	4,120	3,294	↓20	↑21.99	18	20	↑11	0.61
XI	3,451	3,126	↓9	↑12.63	0	0	-	0.00
VIII	2,494	2,927	↑17	↑18.42	7	7	↓0	0.24
I	887	2,016	↑127	↑35.96	4	8	↑100	0.40
CAR	1,495	1,612	↑8	↓18.46	2	3	↑50	0.19
NCR	1,093	1,403	↑28	↑53.85	12	22	↑83	1.57
XII	892	1,016	↑14	↑39.13	3	4	↑33	0.39
BARMM	254	757	↑198	↑14.29	15	8	↓47	1.06
XIII	94	560	↑496	↓29.63	0	6	↑	1.07
II	514	445	↓13	↑37.5	1	3	↑200	0.67
NIR	425	428	↑1	↑31.82	11	7	↓36	1.64
VII	251	332	↑32	↑43.48	1	1	↓0	0.30
III	108	263	↑144	↑90	2	0	↓100	0.00
VI	81	246	↑204	↓50	3	0	↓100	0.00
IX	219	186	↓15	↑16.67	1	5	↑400	2.69
X	16	131	↑719	↑81.82	0	1	↑	0.76
V	24	24	↓0	↑200	0	0	-	0.00
IV-B	297	10	↓97	0	0	1	↑	10.00

[^] Two-week growth rate from MW32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)

*Case Fatality Rate (CFR) is computed by dividing total deaths by total reported cases.

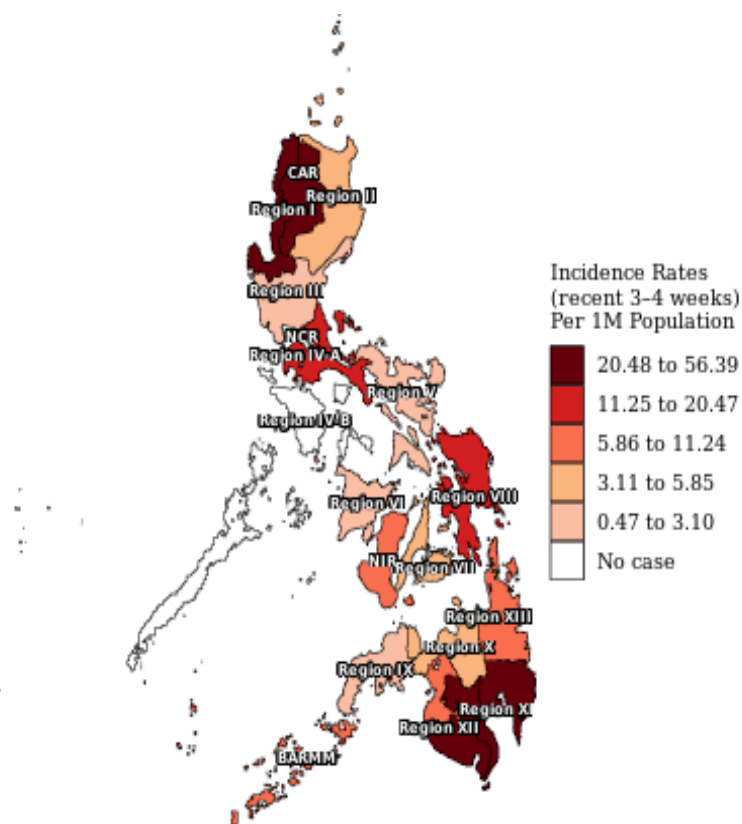


Figure 3. Regional Incidence Rates (n=1,291*)
Philippines, Aug 23 to Sep 5, 2026

*The previous 3-4 weeks were from the MW 34 - 35, 2026 reported cases

Severe Acute Respiratory Infection

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III. Profile of Cases

- A. Of the reported cases, 55% (10,235) were males. Ages ranged from <1 month to 99 years, with a median of 4. The age group with the highest case contribution was children 2 to 4 years of age (3,276, 17%).

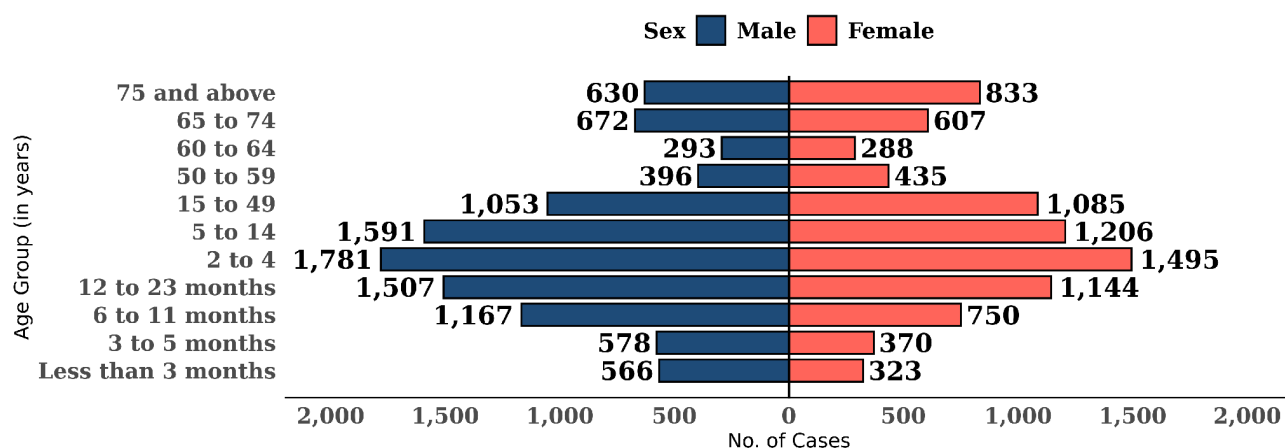


Figure 4. Age and Sex Distribution of Cases (N=18,776*)
Philippines, Jan 4 to Sep 19, 2026

*The ages of 6 cases are still for verification.

- B. From sentinel sites, 125 (6%) had positive laboratory results. Cumulatively, RSV accounted for the highest proportion of positive detections (45, 38%). Among the 29 cases tested this September, two (100%) tested positive for influenza A.

Table 2. Laboratory Results of Tested SARI Cases (n=2,041), Philippines, Jan 1 to Sep 17, 2026

Result	Causative Agent*	Cumulative Total (Jan 1 to Sep 17, 2026) ^{#^}	Aug 2026	Sep 1-17, 2026 [^]
Positive		125 (6%)	34 (15%)	2 (7%)
	RSV	48 (38%)	4 (12%)	0 (0%)
	SARS-CoV-2	34 (27%)	12 (35%)	0 (0%)
	Influenza B	29 (23%)	10 (29%)	0 (0%)
	Influenza A	14 (11%)	8 (24%)	2 (100%)
Negative		1,916 (94%)	199 (85%)	27 (93%)
Pending		0 (0%)	0 (0%)	0 (0%)
Total Cases Tested[§]		2,041 (11%)	233 (10%)	29 (2%)

*For causative agents, the percentage is computed as the total number of samples for a specific respiratory virus divided by the total number of positive laboratory results. Hence, co-infection with multiple viruses may cause pathogen breakdown to exceed 100% of positive cases.

[#] Reporting period is based on sample collection and not the onset of symptoms

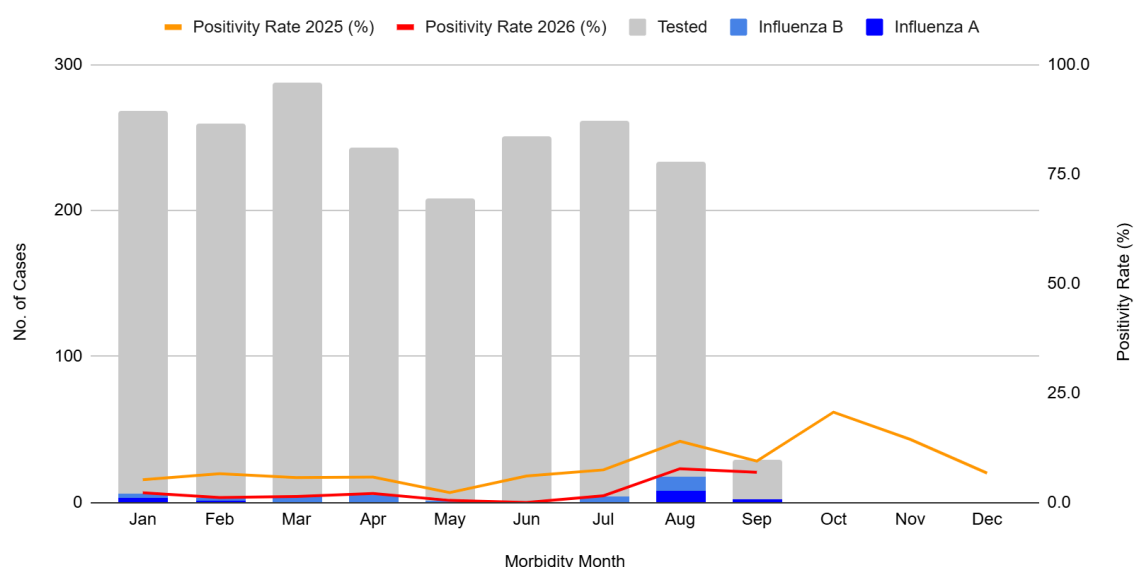
[^] Reported results are as of September 17, 2026.

[§] Total cases tested against total reported SARI cases for the specified period

Severe Acute Respiratory Infection

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- C. At sentinel sites, the influenza positivity rate has been increasing since June and is at 6.9% this September, comparable to the previous month (7.7%). This is still below the rate last year (9.5%) and remains below the average monthly positivity rate in 2025 at 7.6%, excluding the peak influenza positivity in October.



**Figure 5. Positivity Rate of Influenza A/B by Month (n=2,041)
Philippines, Jan 4 to Sep 17, 2026**

- D. Between August 6, 2025 and March 27, 2026, a total of 186 A(H3N2) J.2.4.1 lineage (subclade K) cases were detected. These cases accounted for 61% of all influenza cases sequenced nationwide during this period. Forty-nine were identified as SARI cases. The regional distribution of cases is shown below.

Table 3. Regional Distribution of SARI Cases Detected with K Subclades (n = 49), Philippines, Jan 2025 - Mar 2026

Region	Cases	Percentage
CAR	12	24.49
Region XII	10	20.41
Region VI	8	16.33
Region VII	8	16.33
Region XI	5	10.20
Region III	3	6.12
Region IV-A	2	4.08
Region VIII	1	2.04
Total	49	100

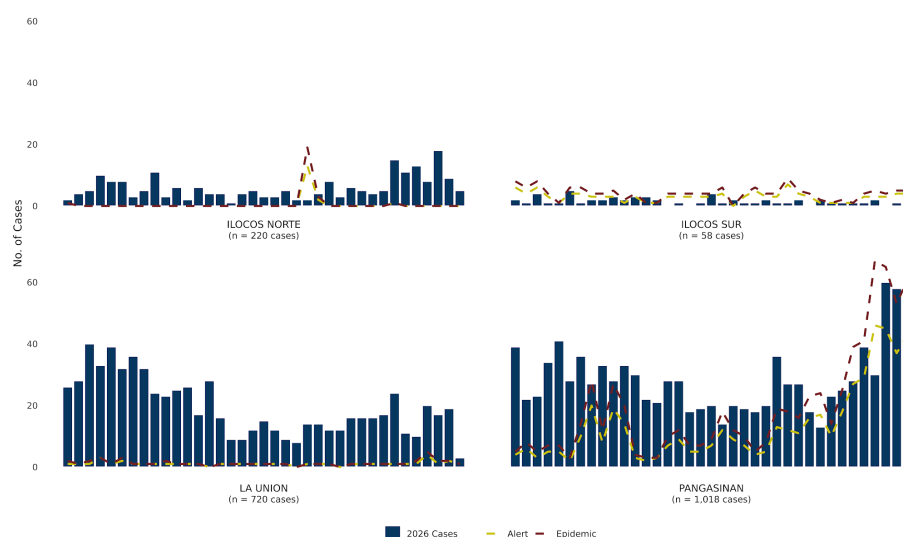
Note: Reported cases may still change due to late consultations and delayed reporting. Data was extracted on September 21, 2026.

Severe Acute Respiratory Infection

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Annex A Subregional Analyses of Region I (Ilocos Region)

In Region I, cases are increasing with 155 cases reported in the recent 3–4 weeks (August 23 to September 05), 36% higher than the 114 cases reported two weeks prior (August 09 to August 22). Ilocos Norte (40 cases), and La Union (59 cases) breached the epidemic thresholds for at least two morbidity weeks in the recent four weeks (Annex A.1).



**Annex A.1 . Provincial/City Cases by Morbidity Week (n=2,016)
Region I, Jan 4 to Sep 19, 2026**

The most affected age group is 2-4 years old with 397 cases. Of the 10 cases tested, 6 tested positive with one testing positive for influenza A (Table A.1).

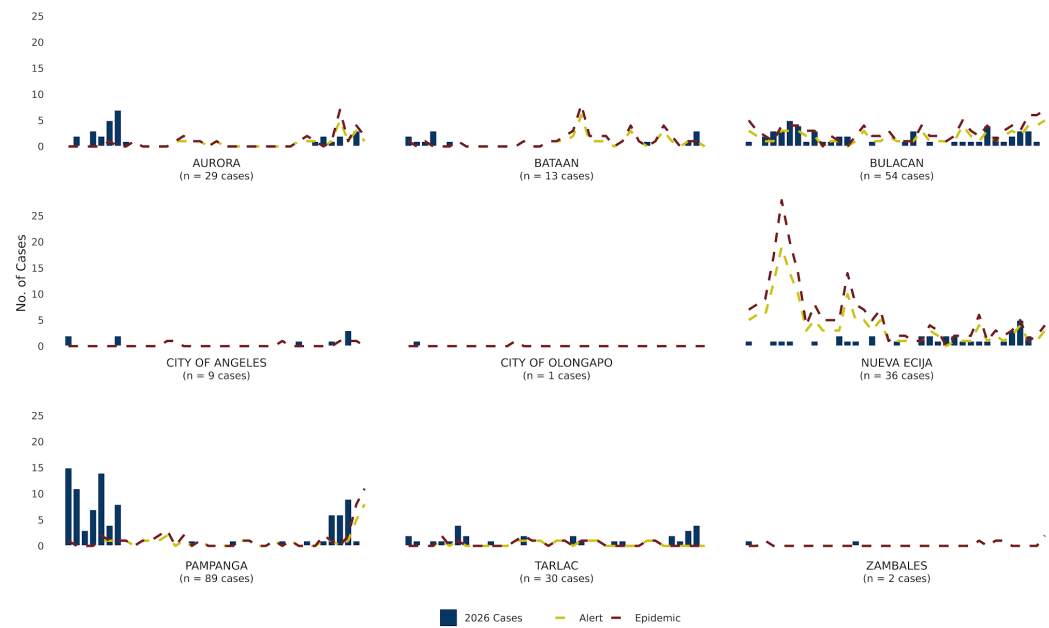
Table A.1. Profile of Cases (N=2,016), Region I, Philippines, Jan 4 to Sep 19, 2026

		Median	Range
Age		5 years	1 month - 99 years
		No. of Cases	Percentage
Sex			
	Male	1,090	54
	Female	926	46
		No. of Cases	Deaths (CFR)
Case Fatality Rate (per province)			
	Pangasinan	1018	7 (0.7%)
	La Union	720	1 (0.1%)
	Ilocos Norte	220	0 (0.0%)
	Ilocos Sur	58	0 (0.0%)
		No. of Cases	Percentage
Laboratory Results			
	Positive	6	60
	Influenza A	1	17
	Influenza B	0	0
	SARS-CoV-2	0	0
	RSV	0	0
	Negative	4	40
	Total tested	10	100

Ten (9%) clusters were identified from this region; all reporting 3 cases each cluster.

Annex B Subregional Analyses of Region III (Central Luzon)

In Region III, cases are increasing with 38 cases reported in the recent 3–4 weeks (August 23 to September 05), 90% higher than the 20 cases reported two weeks prior (August 09 to August 22). Bataan (4 cases), Pampanga (16 cases), and Tarlac (8 cases) breached the epidemic thresholds for at least two morbidity weeks in the recent four weeks (Annex B.1).



Annex B.1 . Provincial/City Cases by Morbidity Week (n=263)
Region III, Jan 4 to Sep 19, 2026

The most affected age group is 15-49 years old, with 36 cases. No deaths were reported. Of the 30 cases tested, 19 tested positive, with 2 (11%) testing positive for influenza A (Table B.1).

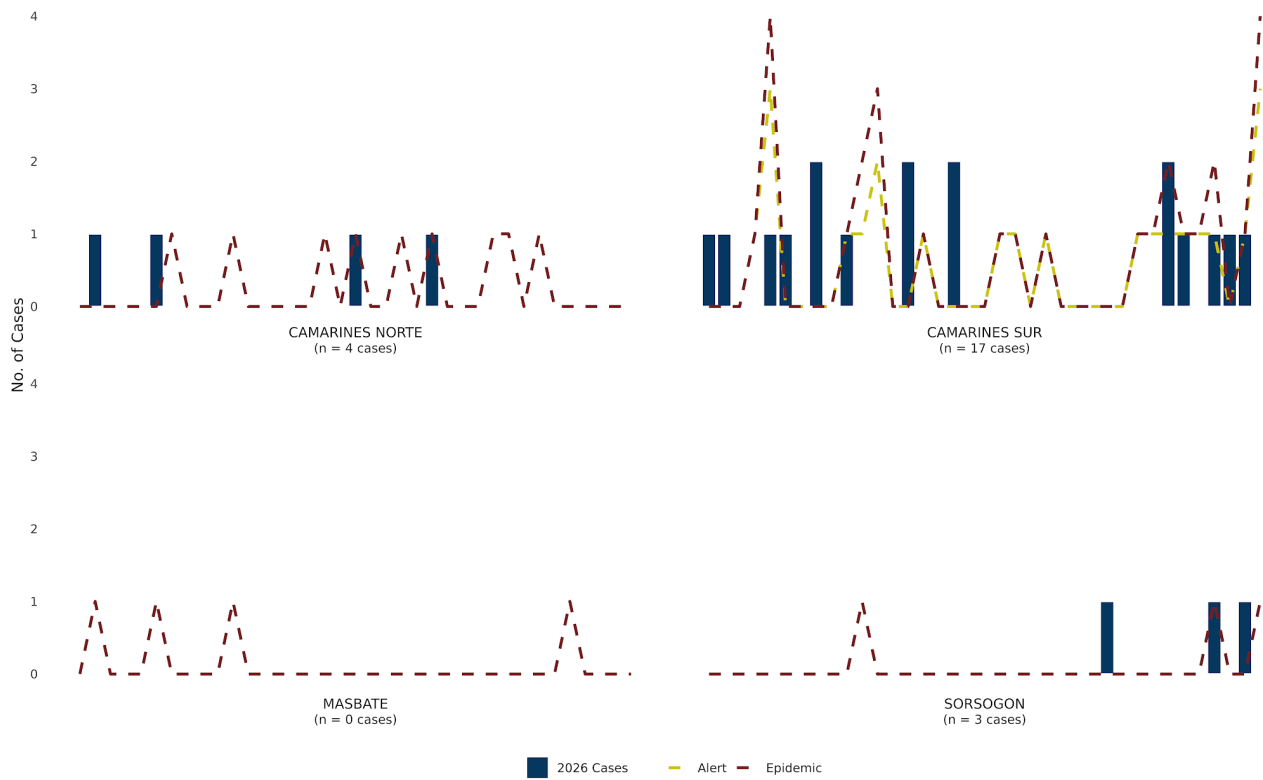
Table B.1. Profile of Cases (N=263), Region III, Philippines, Jan 4 to Sep 19, 2026

		Median	Range
Age		6 years	1 month - 93 years
		No. of Cases	Percentage
Sex	Male	144	55
	Female	119	45
		No. of Cases	Percentage
Laboratory Results			
Positive		19	63
Influenza A		2	11
Influenza B		0	0
SARS-CoV-2		0	0
RSV		0	0
Negative		10	34
Pending		1	3
Total tested		30	100

No cluster was identified from this region.

Annex C
Subregional Analyses of Region V (Bicol Region)

In Region V, cases are increasing with 3 cases reported in the recent 3–4 weeks (August 23 to September 05), 200% higher than the 1 case reported two weeks prior (August 09 to August 22). Camarines Sur (3 cases) and Sorsogon (2 cases) breached the epidemic thresholds for at least two morbidity weeks in the recent four weeks (Annex C.1).



Annex C.1 . Provincial/City Cases by Morbidity Week (n=24)
Region V, Jan 4 to Sep 19, 2026

The most affected age group is 15-49 years, with 5 cases. No deaths were reported, and no cases were tested (Table C.1).

Table C.1. Profile of Cases (n=24), Region V, Philippines, Jan 4 to Sep 19, 2026

		Median	Range
Age		7 years	1 month - 88 years
		No. of Cases	Percentage
Sex	Male	11	46
	Female	13	54

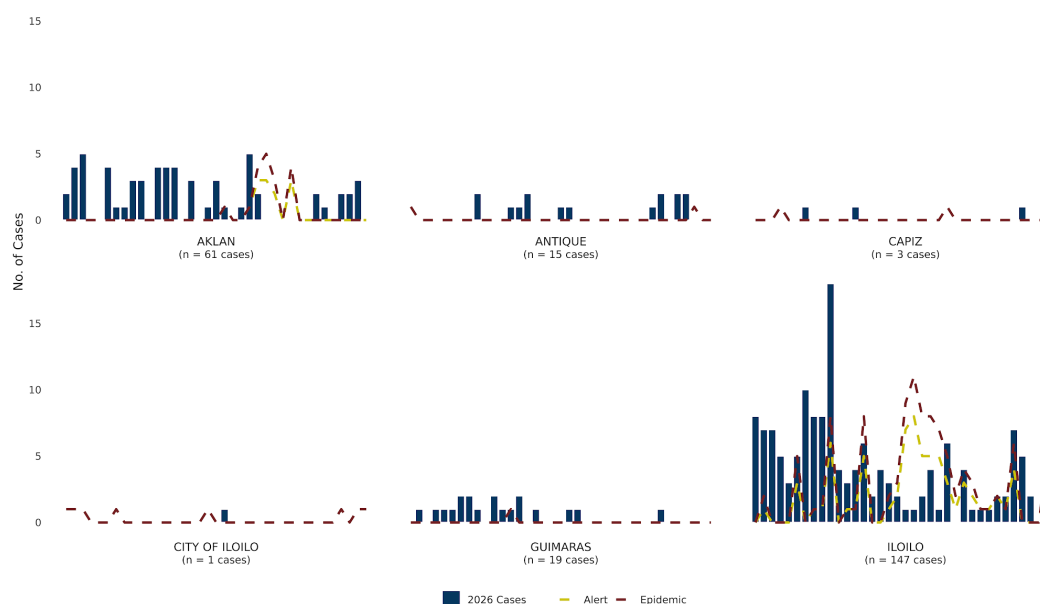
No cluster was identified from this region.

Severe Acute Respiratory Infection

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Annex D Subregional Analyses of Region VI (Western Visayas)

In Region VI, cases are decreasing with 8 cases reported in the recent 3–4 weeks (August 23 to September 05), 50% lower than the 16 cases reported two weeks prior (August 09 to August 22). Aklan (7 cases) breached the epidemic thresholds for at least two morbidity weeks in the recent four weeks (Annex D.1).



**Annex D.1 . Provincial/City Cases by Morbidity Week (n=246)
Region VI, Jan 4 to Sep 19, 2026**

The most affected age group is 12-23 months old, with 49 cases. No deaths were reported. Of the 46 cases tested, five tested positive, with 4 (80%) testing positive for either influenza A/B or SARS-CoV-2 (Table D.1).

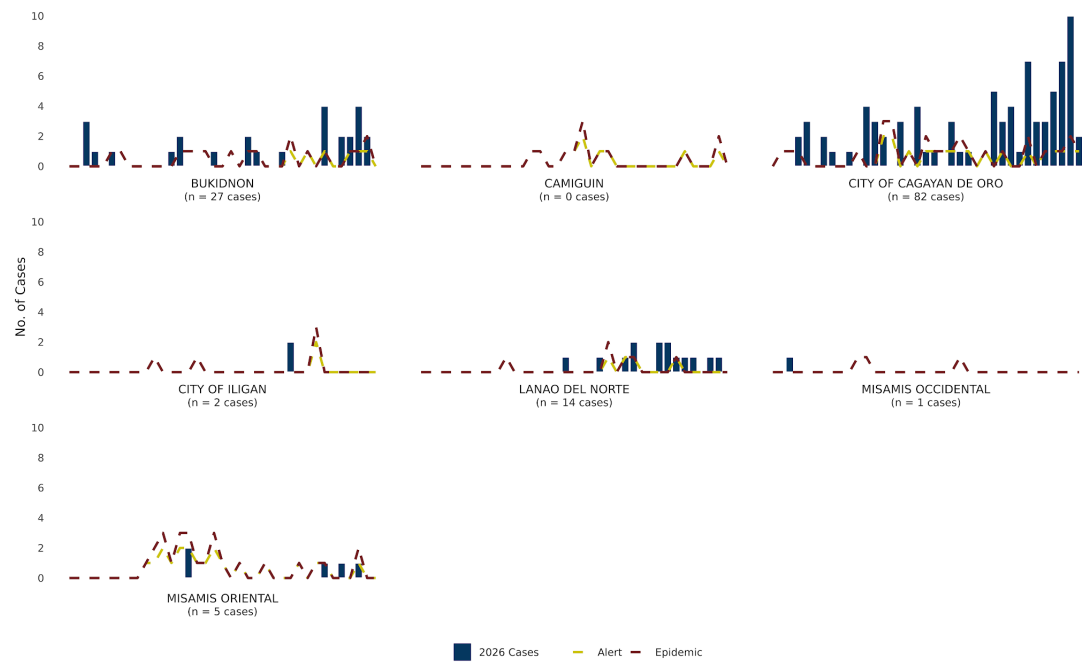
Table D.1. Profile of Cases (n=246), Region VI, Philippines, Jan 4 to Sep 19, 2026

		Median	Range
Age		2 years	1 month - 94 years
		No. of Cases	Percentage
Sex			
Male		126	51
Female		120	49
		No. of Cases	Percentage
Laboratory Results			
Positive		5	11
Influenza A		2	40
Influenza B		1	20
SARS-CoV-2		1	20
RSV		0	0
Negative		41	89
Total tested		46	100

No cluster was identified from this region.

Annex E
Subregional Analyses of Region X (Northern Mindanao)

In Region X, cases are increasing with 20 cases reported in the recent 3–4 weeks (August 23 to September 05), 82% higher than the 11 cases reported two weeks prior (August 09 to August 22). Bukidnon (8 cases), Lanao del Norte (2 cases), and City of Cagayan De Oro (24 cases) breached the epidemic thresholds for at least two morbidity weeks in the recent four weeks (Annex E.1).



Annex E.1 . Provincial/City Cases by Morbidity Week (n=131)
Region X, Jan 4 to Sep 19, 2026

The most affected age group is 12-23 months old, with 39 cases. No case tested positive for influenza A/B, SARS-CoV-2, or RSV (Table E.1).

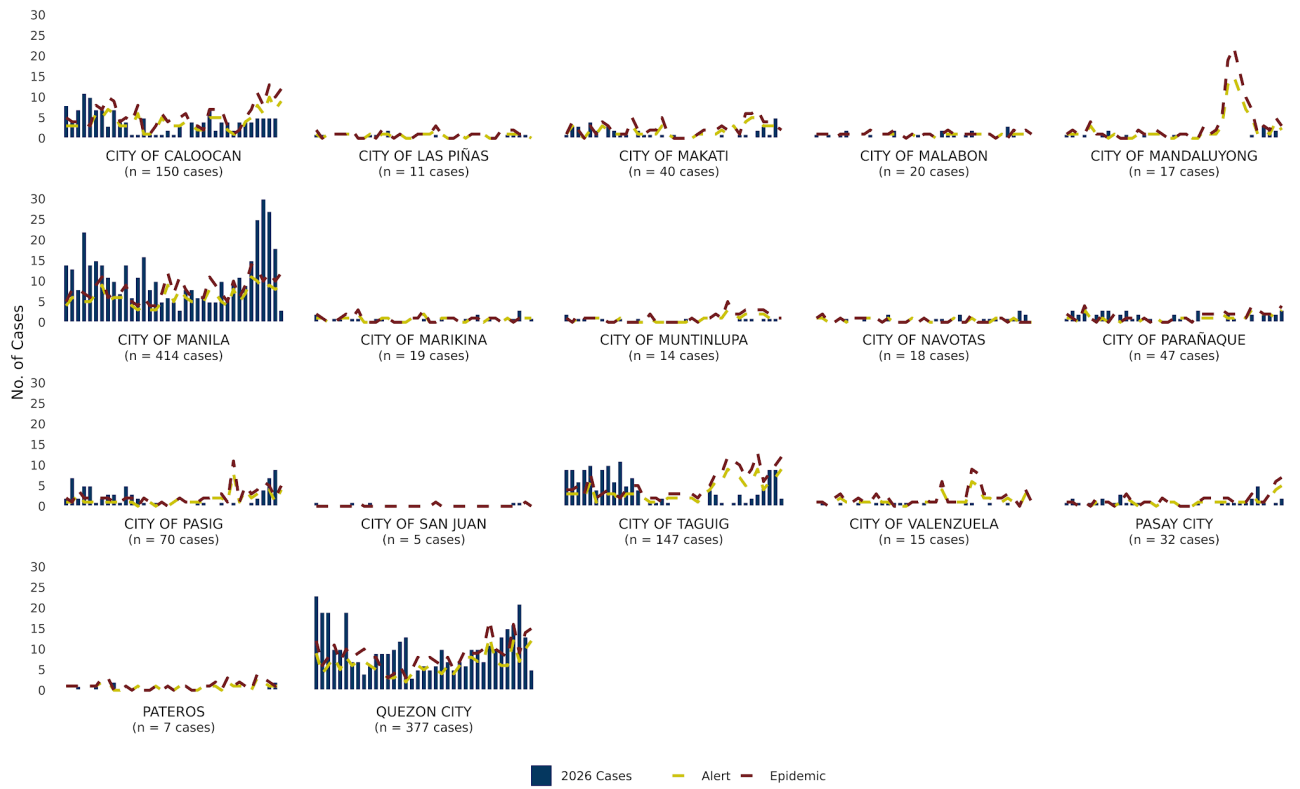
Table E.1. Profile of Cases (n=131), Region X, Philippines, Jan 4 to Sep 19, 2026

		Median	Range
Age		1 years	1 month - 79 years
		No. of Cases	Percentage
Sex	Male	78	59
	Female	53	41
		No. of Cases	Deaths (CFR)
Case Fatality Rate (per province)			
City of Cagayan De Oro		82	0 (0.0%)
Bukidnon		27	0 (0.0%)
Lanao del Norte		14	1 (7.1%)
Misamis Oriental		5	0 (0.0%)
City Of Iligan		2	0 (0.0%)
Misamis Occidental		1	0 (0.0%)

Two (2%) clusters were identified from this region, from Brgy. Balulang and Brgy Carmen, City of Cagayan de Oro, reporting 3 and 4 cases each cluster, respectively.

Annex F Subregional Analyses of National Capital Region (NCR)

In NCR, cases are increasing with 160 cases reported in the recent 3–4 weeks (August 23 to September 05), 54% higher than the 104 cases reported two weeks prior (August 09 to August 22). City of Las Piñas (3 cases), City of Manila (78 cases), City of Marikina (5 cases), City of Navotas (6 cases), City of Pasig (20 cases), and City of San Juan (2 cases) breached the epidemic thresholds for at least two morbidity weeks in the recent four weeks (Annex F.1).



Annex F.1 . Provincial/City Cases by Morbidity Week (n=1,403)
NCR, Jan 4 to Sep 19, 2026

The most affected age group is 5-14 years old, with 250 cases. Of the 527 cases tested, 423 (78%) tested positive, with 71 (17%) testing positive for either influenza A/B or SARS-CoV-2 (Table F.1).

Severe Acute Respiratory Infection

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Table F.1. Profile of Cases (n=1,403), NCR, Philippines, Jan 4 to Sep 19, 2026

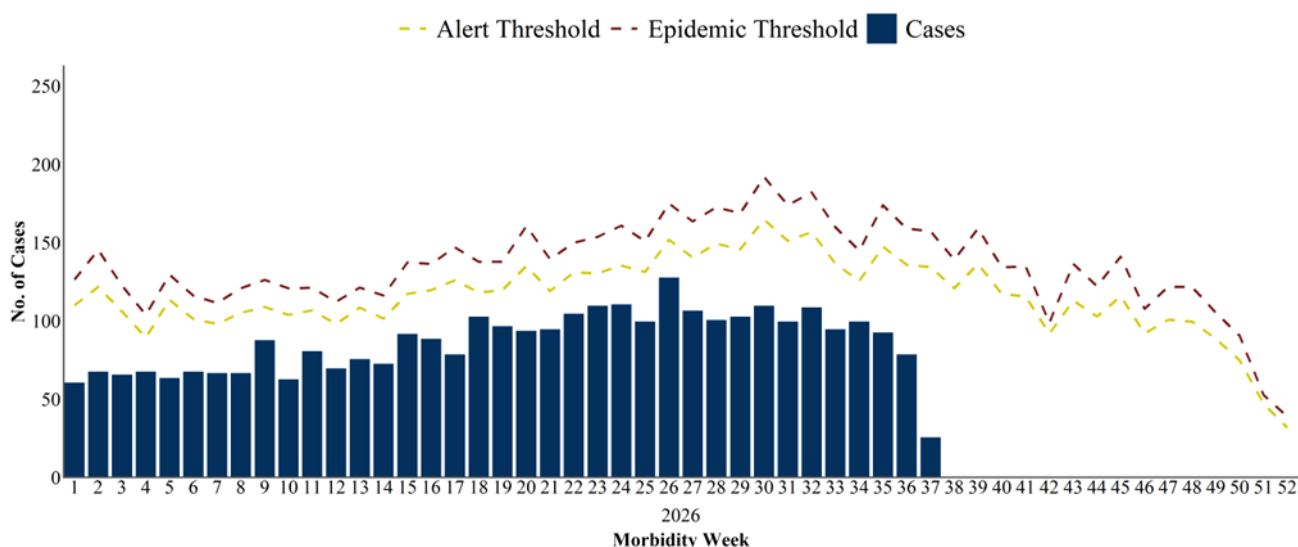
		Median	Range
Age		8 years	1 month - 98 years
		No. of Cases	Percentage
Sex			
Male		758	54
Female		645	46
		No. of Cases	Deaths (CFR)
Case Fatality Rate (per province)			
City of Manila		414	3 (0.7%)
Quezon City		377	6 (1.6%)
City of Caloocan		150	2 (1.3%)
City of Taguig		147	1 (0.7%)
City of Pasig		70	1 (1.4%)
City of Parañaque		47	1 (2.1%)
City of Makati		40	0 (0.0%)
Pasay City		32	0 (0.0%)
City of Malabon		20	1 (5.0%)
City of Marikina		19	1 (5.3%)
City of Navotas		18	1 (5.6%)
City of Mandaluyong		17	1 (5.9%)
City of Valenzuela		15	3 (20.0%)
City of Muntinlupa		14	0 (0.0%)
City of Las Piñas		11	1 (9.1%)
Pateros		7	0 (0.0%)
City of San Juan		5	0 (0.0%)
		No. of Cases	Percentage
Laboratory Results			
Positive		423	80
Influenza A		71	17
Influenza B		18	4
SARS-CoV-2		3	<1
RSV		0	0
Pending		25	5
Negative		79	15
Total tested		527	100

Nine (11%) clusters were identified from this region, with Brgy. Fort Bonifacio, City of Taguig, reporting the highest cases with 9 cases in a single cluster.

Link for the SARI Clustering as of MW 37: <https://tinyurl.com/ymvvedvw>

**I. National Trend**

- As of Morbidity Week 37 (January 4, 2026 to September 19, 2026), the overall AMES case trend has been decreasing since MW 32. Nationally, 3,206 cumulative cases have been reported, a 14% decrease from the 3,736 cases reported last year.
- Cases have decreased by 5% in MW 34-35, with 193 cases reported, compared to MW 32-33, which had 204 cases. The recent two MWs are included in the total counts but not used for growth rate comparison due to the possibility of delayed reports.
- One hundred and fifty-four (154) deaths have been reported in the current period (CFR: 4.8%), a 47% decline from the 292 deaths reported in 2025 (CFR: 7.82%). Among all regions, Region VII had the highest number of deaths with 18.



**Figure 1. Cases by Morbidity Week (N=3,206)
Philippines, Jan 4 to Sep 19, 2026**

II. Sub-national Distribution of Cases

- Nine regions, NCR, CAR, Regions V, VII, NIR, XI, XII, XIII, and BARMM, have reported case increases within the past three to four weeks, while **Regions II, IV-B, and X** have reported case increases in the past two weeks. **Region II** had the highest number of cases with 42 in the past four weeks.
- CAR** has breached epidemic thresholds during MWs 34 and 35. In this region, the highest number of cases were observed in Kalinga with 7 cases in the past four weeks. Details on provincial cases and thresholds are outlined in the Annex.
- Region XII** has breached alert thresholds during MWs 34 and 35. In this region, the highest number of cases were observed in South Cotabato, with 4 cases in the past four weeks. Details on provincial cases and thresholds are outlined in the Annex.

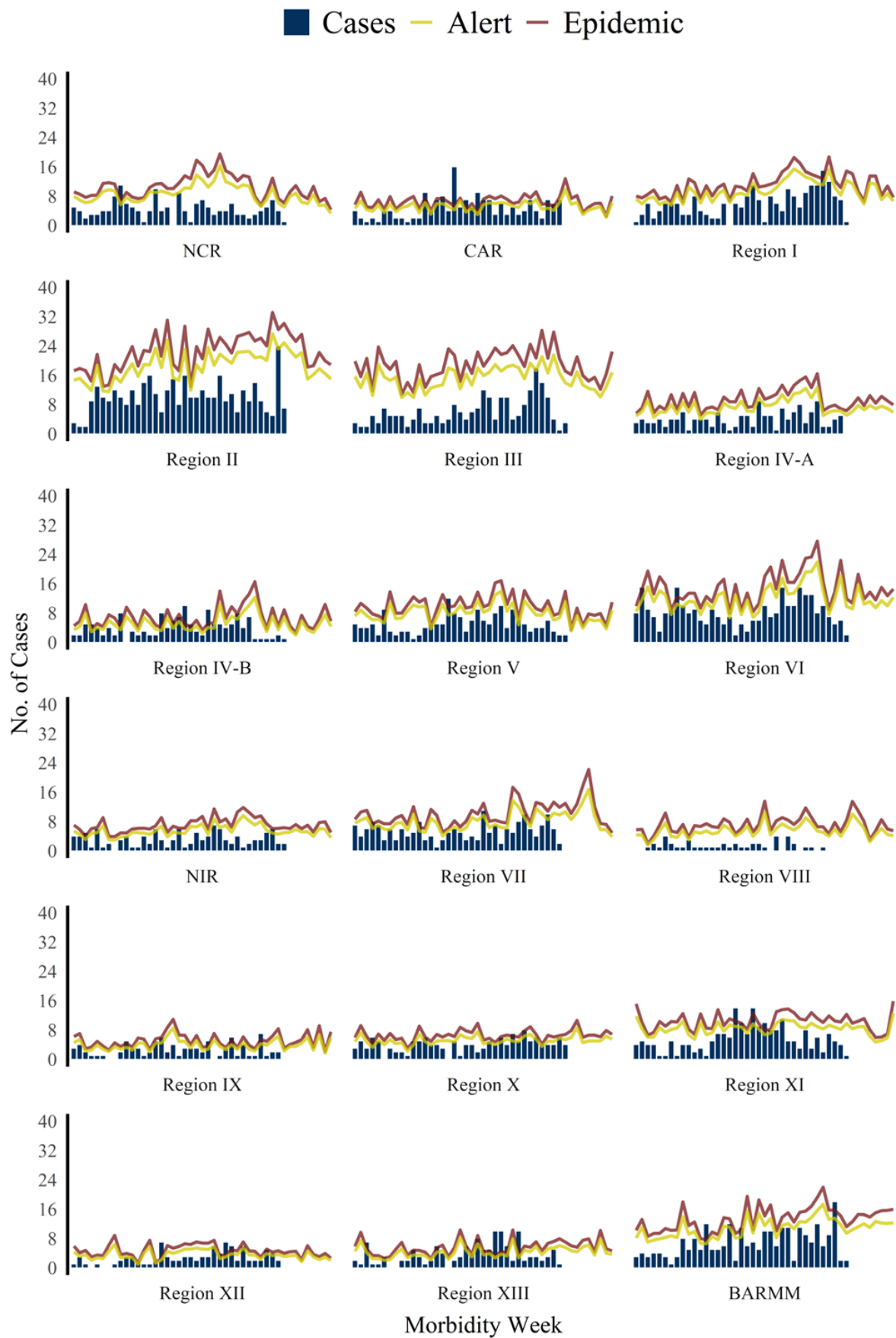


Figure 2. Regional AMES Cases by Morbidity Week (N=3,206)
Philippines, Jan 4 to Sep 19, 2026

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR [^]		Deaths			CFR (%)
	2025	2026	% Change	3-4 vs 5-6 weeks (%)		2025	2026	% Change	
II	561	376	↓32.98	↓52.17		20	4	↓80.00	1.06
VI	353	301	↓14.73	↓33.33		13	8	↓38.46	2.66
BARMM	297	248	↓16.50	↑50.00		23	13	↓43.48	5.24
III	352	224	↓36.36	↓56.25		60	8	↓86.67	3.57
I	207	218	↑5.31	↓23.08		6	2	↓66.67	0.92
XI	175	204	↑16.57	↑50.00		33	17	↓48.48	8.33
VII	145	193	↑33.10	↑45.45		12	18	↑50.00	9.33
V	209	184	↓11.96	↑12.50		12	6	↓50.00	3.26
CAR	155	172	↑10.97	↑116.67		15	4	↓73.33	2.33
NCR	208	164	↓21.15	↑71.43		15	12	↓20.00	7.32
IV-A	115	148	↑28.70	↓45.45		9	7	↓22.22	4.73
X	103	148	↑43.69	↓20.00		9	9	0.00	6.08
IV-B	159	145	↓8.81	0.00		18	13	↓27.78	8.97
XIII	98	130	↑32.65	↑60.00		9	16	↑77.78	12.31
NIR	198	118	↓40.40	↑83.33		12	4	↓66.67	3.39
XII	115	97	↓15.65	↑125.00		17	9	↓47.06	9.28
IX	92	91	↓1.09	↓62.50		6	4	↓33.33	4.40
VIII	194	45	↓76.80	↓100.00		3	0	↓100.00	0.00

[^]Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)

*Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases.

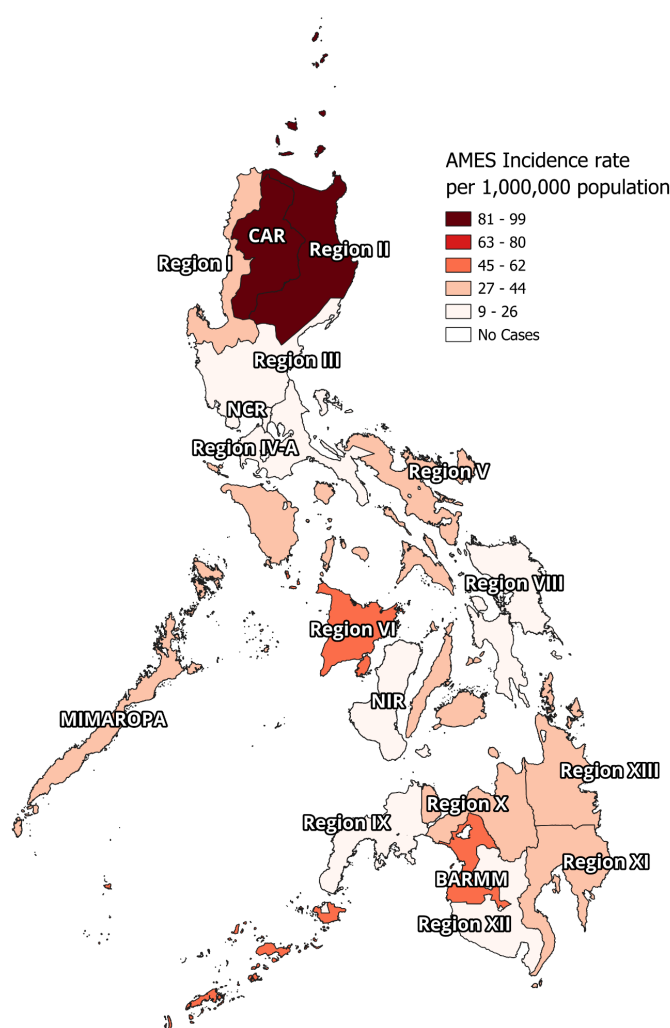


Figure 3. Regional AMES Incidence Rates (N=3,206) Philippines, Jan 4 to Sep 19, 2026

- **Fifty-five (55) areas** showed an increase in cases in the recent 1-2 and/or 3-4 weeks, with 20 areas reporting an increase in cases in the recent 1-2 weeks. These flagged areas are as follows, with **underlined** cases also reporting increases in the recent 1-2 weeks:

NCR:	City of Caloocan, City of Manila, City of Parañaque, <u>Pasay City</u> , <u>City of Pasig</u> , Quezon City, City of Valenzuela
CAR:	Abra, Ifugao, Kalinga, Mountain Province, <u>Apayao</u> , City of Baguio
Region I:	<u>La Union</u> , Pangasinan
Region II:	<u>Cagayan</u> , <u>Isabela</u>
Region III:	<u>Tarlac</u> , Aurora
Region IV-A:	<u>Cavite</u> , <u>Rizal</u> , <u>City of Lucena</u>
Region IV-B:	<u>Occidental Mindoro</u> , <u>Romblon</u> , City of Puerto Princesa
Region V:	<u>Albay</u> , Masbate, Sorsogon
Region VI:	Aklan, Capiz, <u>Iloilo</u>
NIR:	Negros Occidental, City of Bacolod
Region VII:	Bohol, City of Cebu, City of Mandaue
Region IX:	<u>Zamboanga del Norte</u> , <u>Zamboanga Sibugay</u>
Region X:	Bukidnon, Camiguin, Lanao del Norte, <u>Misamis Oriental</u> , <u>City of Cagayan de Oro</u>
Region XI:	<u>Davao Oriental</u> , Davao de Oro, City of Davao
Region XII:	Cotabato, South Cotabato, City of General Santos
Region XIII:	<u>Agusan del Norte</u> , Agusan del Sur, Surigao del Norte
BARMM:	Lanao del Sur, Sulu, Special Geographic Area

- Barangay clustering of two cases each within the past four weeks was reported in 5 barangays across 4 regions.

Table 1. Barangay Clustering by Region and City/Municipality, Philippines, Aug 23 to Sep 19, 2026

Region	City/Municipality	Barangay	No. of Cases
Region VII	Trinidad	Catoogan	2
CAR	City of Tabuk	Bulanao	2
Region XIII	San Francisco	Alegria	2
	City of Surigao	Canlanipa	2
BARMM	City of Marawi	Mipaga Proper	2

III. Japanese Encephalitis (JE)

- As of MW 37, there were a total of 133 AMES cases with positive JE results reported. This marks a 2% decrease from the 136 cases reported during the same period last year.
- Seven JE deaths have been reported in 2026 (CFR: 5.26%), more than the six total deaths reported in 2025 (CFR: 4.41%).

**Table 2. Cumulative Laboratory-Confirmed Japanese Encephalitis Cases (n=133)
Philippines, Jan 4 - Sep 19, 2026**

Region	Province	Cases	Deaths
CAR	Benguet	4	0
	City of Baguio	1	0
	Ifugao	6	1
	Kalinga	10	1
	Mountain Province	2	1
Region I	Ilocos Sur	3	0
	La Union	4	0
	Pangasinan	6	0
Region II	Cagayan	1	0
	Isabela	10	0
	Nueva Vizcaya	6	0
Region IV-A	Batangas	1	0
Region VI	Antique	2	0
	Capiz	4	1
	City of Iloilo	3	0

Region	Province	Cases	Deaths
	Guimaras	2	0
	Iloilo	18	0
Region VII	Bohol	7	0
	Cebu	2	0
Region X	Bukidnon	5	0
	Camiguin	1	0
	City of Cagayan de Oro	1	0
	Misamis Oriental	7	0
Region XI	Davao del Norte	1	0
	Davao Oriental	1	0
Region XII	Cotabato	8	2
	Sarangani	1	0
	South Cotabato	3	0
	Sultan Kudarat	4	0
Region XIII	Agusan del Norte	1	0
	Dinagat Islands	1	0
	Surigao del Norte	1	0
	Surigao del Sur	1	0
BARMM	Lanao del Sur	1	0
	Maguindanao del Sur	3	1
	Special Geographic Area	1	0

IV. Nipah Virus

- No AMES case has tested positive for Nipah Virus.

V. Profile of Cases

- Sixty percent (1,939 cases) of AMES cases are male, while forty percent (1,267 cases) are female. The majority of cases are in the 1 to 3 years age group, which accounts for 23% (757) of the cases.

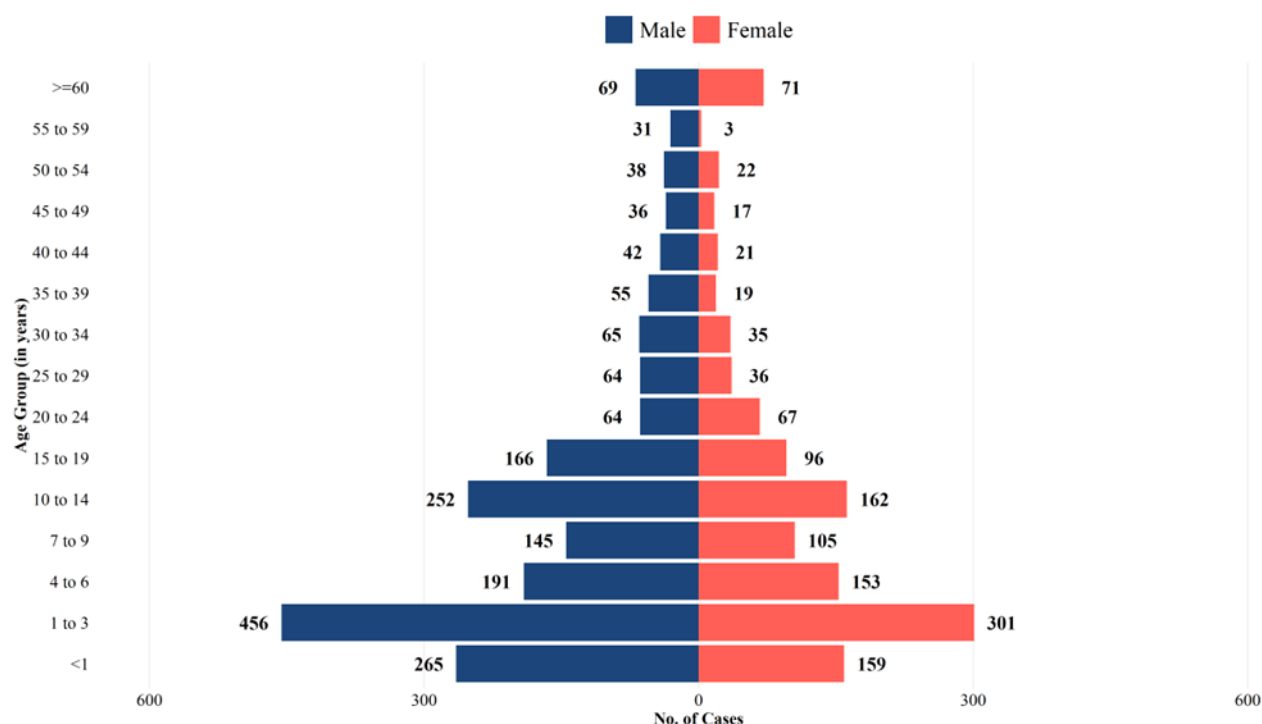


Figure 4. AMES Cases by Age and Sex (N=3,206)
Philippines, Jan 4 to Sep 19, 2026

- A total of 995 samples have been tested, resulting in 133 cases confirmed with Japanese encephalitis, 10 cases confirmed with Bacterial Meningitis, 49 cases with unknown pathogens, and three cases with other pathogens (*Klebsiella pneumoniae*, *Burkholderia cepacia*, and *Cytomegalovirus*). Common clinical symptoms in all cases were new-onset seizures (61%), fever, and change in mental status (48%). The most common exposure source was at home, followed by barangay, and vaccination rates were all at or below 11%.

Table 3. Clinical Profile of AMES Cases (N=3,206), Philippines, Jan 4 to Sep 19, 2026

	No. of Cases	Percentage
Case Classification		
Suspect Cases	2,995	93
Probable Japanese Encephalitis	7	<1
Confirmed Japanese Encephalitis	133	4
Probable Bacterial Meningitis	2	<1
Confirmed Bacterial Meningitis	10	<1
AES, Other Agents	3	<1
AES, Unknown Agents	56	2
Laboratory Testing		
Tested	995	31
Positive*	202	20
Japanese encephalitis	133	66
<i>Streptococcus pneumoniae</i>	5	2
<i>Neisseria meningitidis</i>	3	1
<i>Haemophilus influenzae</i>	2	<1
Others	3	1
Unknown	56	28
Negative	775	78
Equivocal	18	2
Pending	0	0
Indeterminate	0	0
Not Tested	2,211	69
Clinical Information [^]		
New Onset Seizures	1,957	61
Fever and Change In Mental Status**	1,549	48
Fever only	1,374	43
Change In Mental Status only	105	3
Exposure History [^]		
Home	720	22
Barangay	351	11
Healthcare Facility	148	5
School	103	3
Workplace	36	1
Daycare	18	<1
Dormitory	9	<1
Others	2,301	72
Vaccination History		
Japanese encephalitis	65	2
Penta-Hib	345	11
Pneumococcal	99	3
Meningococcal	20	<1

* The percentage of organisms under positive cases was computed using the total number of positive AMES cases as the denominator.

[^]Clinical information and exposure history are based on multiple entries.

** Cases grouped under Fever and Change In Mental Status are those who have shown both symptoms at the same time, while the aggregates below are cases who have shown signs of the individual symptom without the presence of the other. The percentages were computed using the total number of AMES cases as the denominator.

Note: Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026.

Annex A. Provinces from Regions that have Breached Epidemic Thresholds Twice in the Past Four Weeks

Province	Cases from the Same Period Last Year	Cases in the Past 4 Weeks	Incidence Rate per 100,000 for the Past 4 Weeks	Breached Thresholds Twice in the Past 4 Weeks
CORDILLERA ADMINISTRATIVE REGION (CAR)				
Abra	1	1	0.40	None
Benguet	4	2	0.41	None
Ifugao	1	4	1.81	Alert
Kalinga	5	7	2.86	None
Mountain Province	1	2	1.22	None
Apayao	3	2	1.55	None
City of Baguio	4	2	0.52	None

- Overall, Region CAR had a total of 172 AMES cases in 2026, compared to the 155 reported in 2025. Cases have increased by 116.67% in the past three to four weeks compared to the past five to six weeks.
- Kalinga had the highest two-week growth rate with a 400% increase, followed by Mountain Province with a 200% increase.
- Among all provinces, Mountain Province (CFR: 10.00%), City of Baguio (CFR: 4.00%), Kalinga (CFR: 2.78%), and Ifugao (CFR: 2.50%) all had one death in 2026.
- Kalinga (10), Ifugao (6), and Benguet (4) had the highest number of laboratory-confirmed JE cases in 2026.
- One barangay (Bulanao, City of Tabuk, Kalinga) had a clustering of two cases in the past four weeks.

Annex B. Provinces from Regions that have Breached Alert Thresholds Twice in the Past Four Weeks

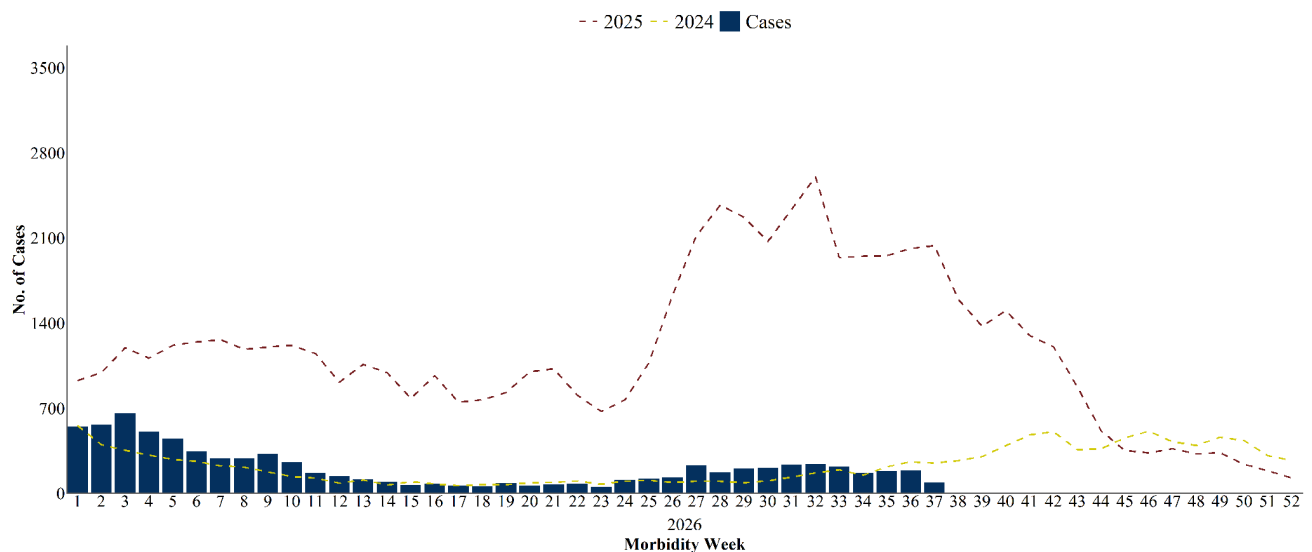
Province	Cases from the Same Period Last Year	Cases in the Past 4 Weeks	Incidence Rate per 100,000 for the Past 4 Weeks	Breached Thresholds Twice in the Past 4 Weeks
REGION XII (SOCCSKSARGEN)				
Cotabato	5	3	0.22	None
South Cotabato	0	4	0.39	Epidemic
Sultan Kudarat	4	1	0.11	None
Sarangani	1	2	0.34	Epidemic
City of General Santos	1	1	0.13	None

- Overall, Region XII had a total of 97 AMES cases in 2026, compared to the 115 reported in 2025. Cases have increased by 125% in the past three to four weeks compared to the past five to six weeks.
- Cotabato and South Cotabato had the highest two-week growth rate with a 200% increase, followed by City of General Santos with a 100% increase.
- Among all provinces, Cotabato (CFR: 10.26%) had the highest number of deaths with four in 2026.
- Cotabato had the highest number of laboratory-confirmed JE cases with eight cases. Followed by Sultan Kudarat with four laboratory-confirmed JE cases, and South Cotabato with three laboratory-confirmed JE cases.



I. National Trend

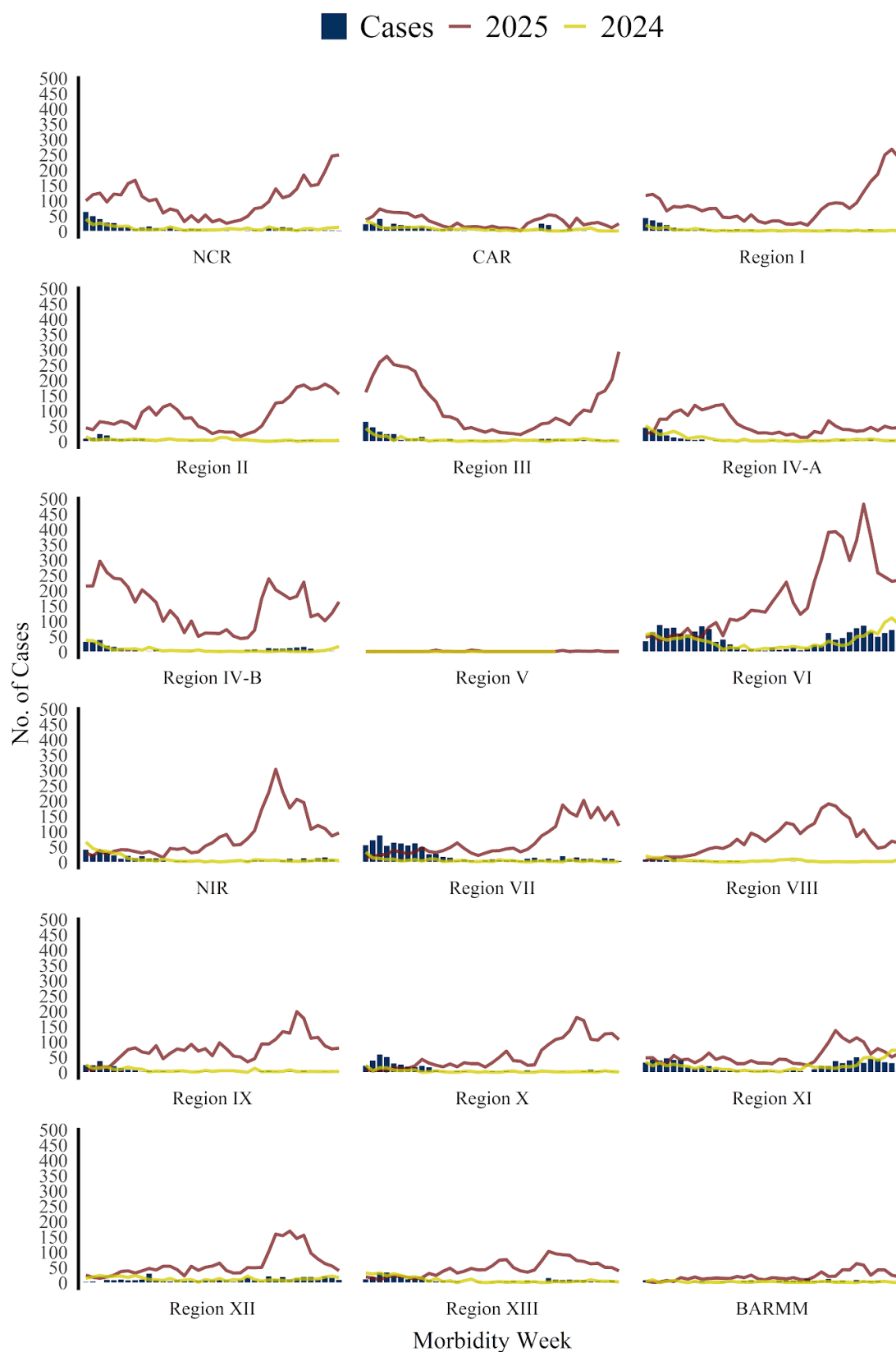
- As of Morbidity Week 37 (January 4 to September 19, 2026), 7,993 cumulative cases had been reported, an 84% decrease from the 50,409 cases reported last year.
- Three (3) deaths have been reported in the current period (CFR: 0.04%), a 25% decline from the 4 deaths reported in 2025 (CFR: 0.01%).
- Cases have decreased by 24% in MW 34-35, with 358 cases reported, compared to MW 32-33, which had 473 cases. The recent two MWs were not used for growth rate comparison due to the possibility of delayed reports.



**Figure 1. Cases by Morbidity Week (N=7,993)
Philippines, Jan 4 to Sep 19, 2026**

II. Sub-national Distribution of Cases

- **Three (3) regions, III, NIR, and XI**, have reported case increases within the past three to four weeks, and **NCR, CAR, Regions IV-A, and IX** have reported case increases in the past two weeks. **Region VI** had the highest number of cases with 224 in the past four weeks. Figure 2 shows the regional distribution of cases.
- **Region VI** reported the highest incidence rate within the past four weeks, with 4.67 per 100,000, followed by **Region XI** with 2.26 per 100,000 and **Region XII** with 1.32 per 100,000. Figure 3 illustrates the cumulative cases within 1-4 weeks, and Table 2 shows the detailed changes in cases and deaths per region.
- Barangay clustering of two or more cases from MW 37 as reported in 6 barangays across 4 regions. See details here: <https://tinyurl.com/MW37Clustering>



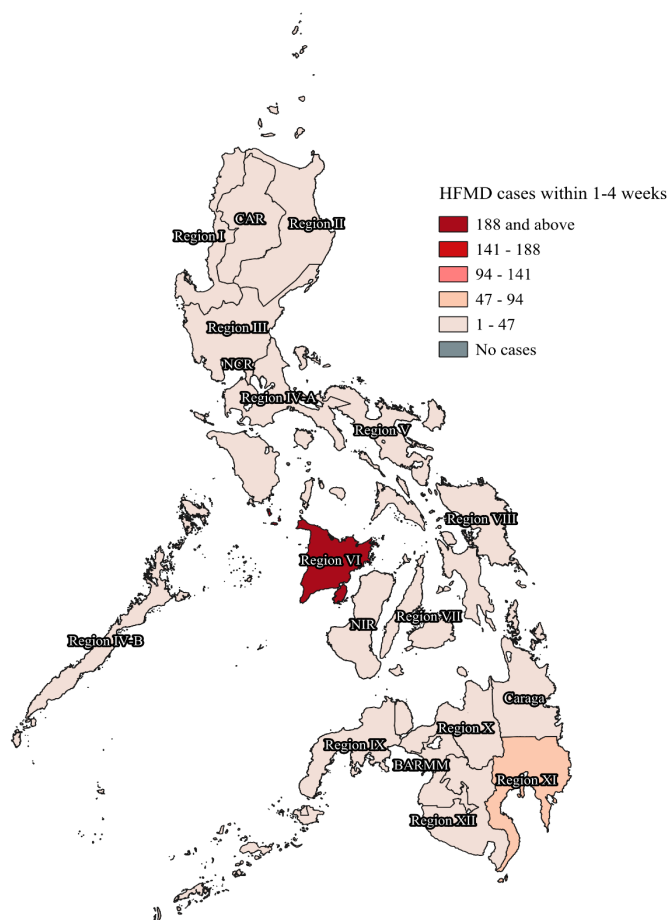
**Figure 2. Regional HFMD Cases by Morbidity Week (N=7,993)
Philippines, Jan 4 to Sep 19, 2026**

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025 data

Region	Cases			TWGR [^] 3-4 vs 5-6 weeks (%)	Deaths			CFR (%)
	2025	2026	% Change		2025	2026	% Change	
VI	6,618	1,638	↓75	↓26.62	0	0	0	0
VII	2,661	909	↓66	↓8	2	0	↓100	0
XI	2,017	842	↓58	↑1.27	1	0	↓100	0
NCR	3,836	463	↓88	↓61.54	0	0	0	0
XII	2,250	440	↓80	↓19.05	0	0	0	0
X	2,094	421	↓80	↓37.5	0	0	0	0
NIR	3,189	416	↓87	↑60	0	0	0	0
III	4,393	390	↓91	↑30	0	0	0	0
XIII	1,730	388	↓78	↓31.25	0	0	0	0
CAR	1,162	361	↓69	↓25	0	0	0	0
IV-B	5,441	346	↓94	↓80	0	0	0	0
IV-A	2,029	297	↓85	↓69.23	0	2	↑200	0.67
I	3,195	280	↓91	↓75	0	0	0	0
IX	2,831	262	↓91	↓42.86	0	0	0	0
BARMM	769	219	↓72	↓23.08	0	0	0	0
II	3,287	197	↓94	↓57.14	0	1	↑100	0.51
VIII	2,855	118	↓96	↓66.67	0	0	0	0
V	52	6	↓88	0	1	0	↓100	0

[^]Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)

*Case Fatality Rate (CFR) is computed by dividing the total number of deaths per disease/condition over the total number of reported cases.



**Figure 3. Regional HFMD Cases within 1-4 Weeks (n=642)
Philippines, Jul 26 to Sep 19, 2026**

- Fifty (50) areas showed an increase in cases in the recent 1-2 and/or 3-4 weeks, with 27 areas reporting an increase in cases in the recent 1-2 weeks. These flagged areas are as follows, with underlined cases also reporting increases in the recent 1-2 weeks:

NCR:	<u>City of Makati</u> , <u>City of Manila</u> , <u>City of Pasig</u> , Quezon City, <u>City of Taguig</u>
CAR:	<u>Benguet</u> , Ifugao, <u>Mountain Province</u> , Apayao, <u>City of Baguio</u>
Region I:	<u>Ilocos Norte</u> , Pangasinan
Region II:	<u>Cagayan</u> , Isabela
Region III:	Bataan, <u>Bulacan</u> , Pampanga, Tarlac, Zambales, <u>Aurora</u>
Region IV-A:	<u>Batangas</u> , <u>Quezon</u>
Region VI:	<u>Occidental Mindoro</u> , <u>City of Puerto Princesa</u>
NIR:	<u>Aklan</u> , Antique, <u>Capiz</u> , <u>Iloilo</u> , City of Iloilo
Region VII:	Negros Occidental, Negros Oriental
Region VIII:	Bohol, <u>City of Cebu</u> , <u>City of Lapu-Lapu</u> , <u>City of Mandaue</u>
Region IX:	Eastern Samar, <u>City of Tacloban</u>
Region X:	<u>Zamboanga del Sur</u> , <u>City of Zamboanga</u>
Region XI:	Misamis Occidental
Region XII:	Davao del Norte, Davao del Sur, Davao Oriental, City of Davao
CARAGA:	Sultan Kudarat, <u>Sarangani</u> , City of General Santos
BARMM:	Agusan del Sur, <u>Surigao del Norte</u> , <u>Surigao del Sur</u>

III. Profile of Cases

- Four thousand four hundred and seventy-four (56%) are children aged 1 to 3 years, followed by 1646 (21%) in the 4 to 6 years age group. Of the total cases, 4,458 (56%) are male.

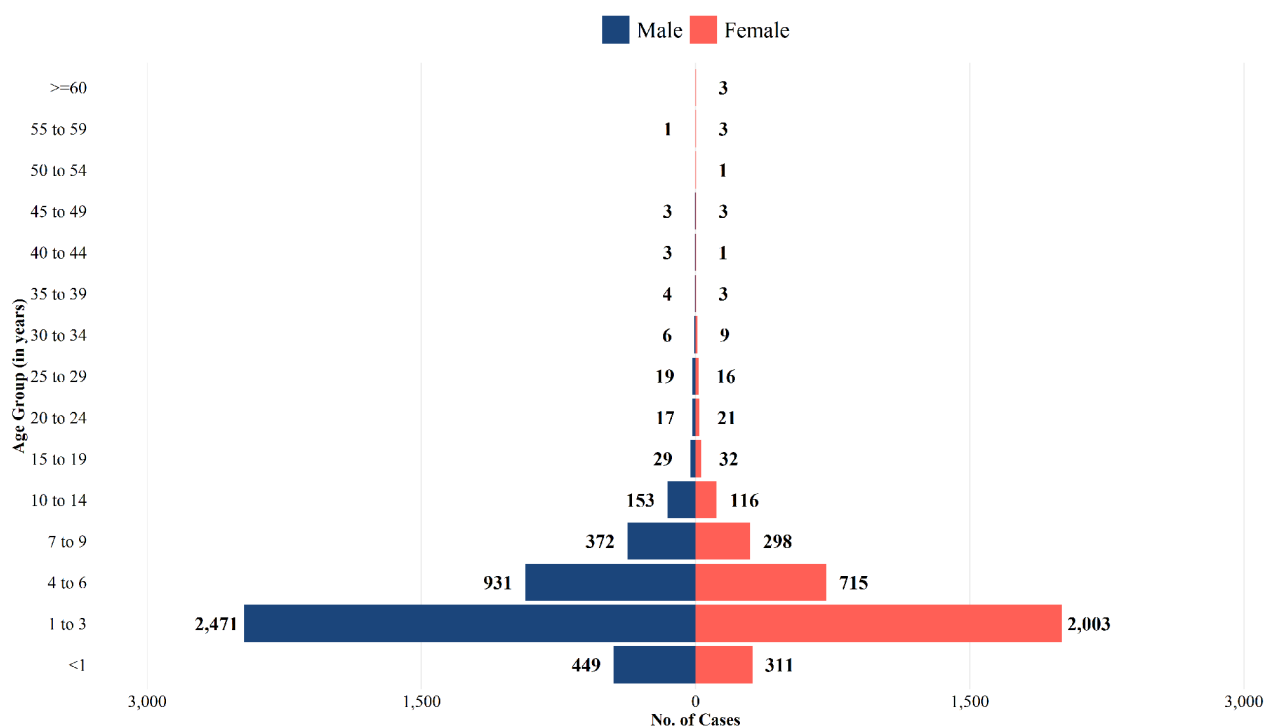


Figure 4. Regional HFMD Cases by Age and Sex (N=7,993)
Philippines, Jan 4 to Sep 19, 2026

- Among the reported cases, no case underwent laboratory testing due to suspension of HFMD testing by the national reference laboratory since January 4, 2026. Reported symptoms included fever, sore throat, rash, and mouth ulcers for HFMD cases and paralysis, meningitis, difficulty in breathing, and other complications such as seizures for SED cases (“DM 2025-0451”). Nineteen percent of cases were hospitalized, and exposures were primarily community-based (34%) and at home (32%).

Table 2. Clinical Profile of HFMD Cases (N=7,993), Philippines, Jan 4 - September 19, 2026

	No. of Cases	Percentage
Case Classification		
Suspect HFMD	7,886	99
Suspect Severe Enteroviral Disease	107	1
Laboratory Testing		
Not Tested	7,993	100
Clinical Information		
Fever	6,530	82
Sore Throat	1,222	15
Rash	7,903	99
Maculopapular	4,810	60
Papulovesicular	3,330	42
Mouth Ulcers	4,806	60
Hospital Admission / Consultation Status		
Admitted	1,519	19
Seen / Consulted	1,317	16
Unknown	5,157	65
Exposures		
Community	2,694	34
Home	2,547	32
School	588	7
Daycare	225	3
Health Facility	79	<1
Dormitory	22	<1
Unknown	5,157	65

*Clinical information and exposures are based on multiple entries.

Notes:

1. Reported cases and trends may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026.
2. Definitions based on DM No. 2025-0451 are applied as of October 25, 2025 (MW 43).
3. Changes in the reported surveillance data were due to revisions in data validation codes and the re-extraction of 2025 HFMD and SED cases, which included updates to case classification and testing data.



I. National Trend

- As of Morbidity Week 37 (January 4 to September 19, 2026), 86 cumulative cases were reported, a 41% decrease from 147 cases during the same period in 2025.
- Thirty-two deaths (CFR: 37%) have been reported, a 44% decrease compared to the 57 deaths (CFR: 39%) in the same period last year.

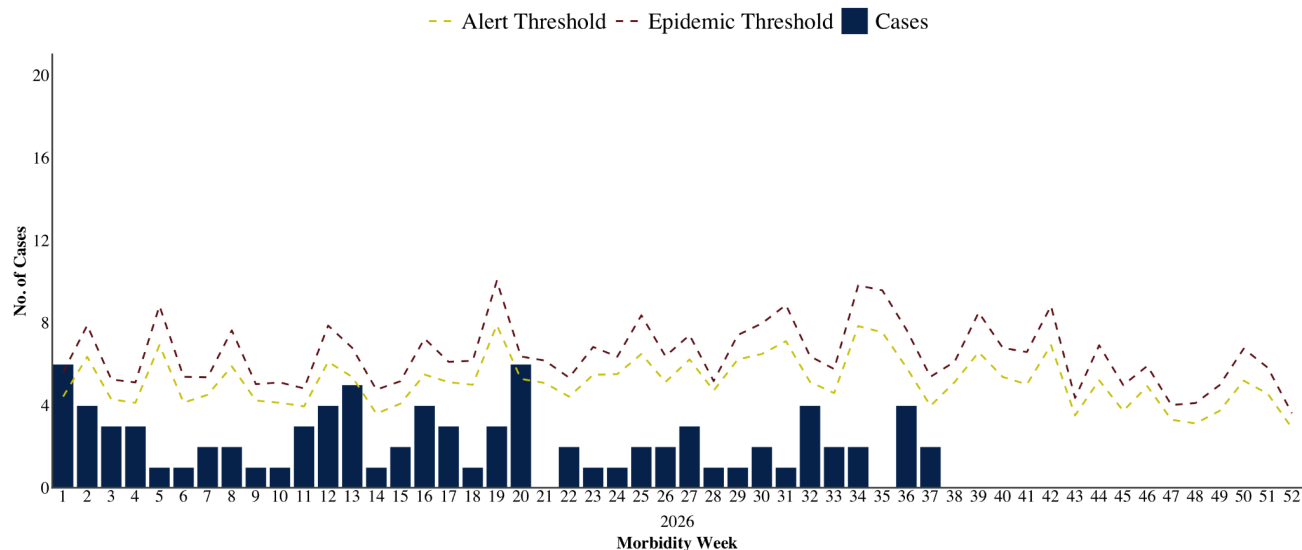


Figure 1. Cases by Morbidity Week (N=86)
Philippines, Jan 4 to Sep 19, 2026

II. Sub-national Distribution of Cases

- NCR, Regions III, IV-A, IX, and XI reported an increase in cases in the past four weeks.
- Regions IX and XI reported the highest number of cases (**one case each**) in the past four weeks with the following areas showing an increase in cases in the recent 3-4 weeks.

Region IX: City of Zamboanga

Region XI: City of Davao

- NCR, Regions IX, and XI breached epidemic thresholds once in the past 4 weeks. *Annex A* outlines provincial cases and thresholds.
- Region IV-A has breached alert thresholds once within the past 4 weeks. *Annex B* outlines provincial cases and thresholds.
- No clustering of meningococcal disease cases has been detected in any barangay in the past four weeks.

Meningococcal Disease

EB WSR 2026

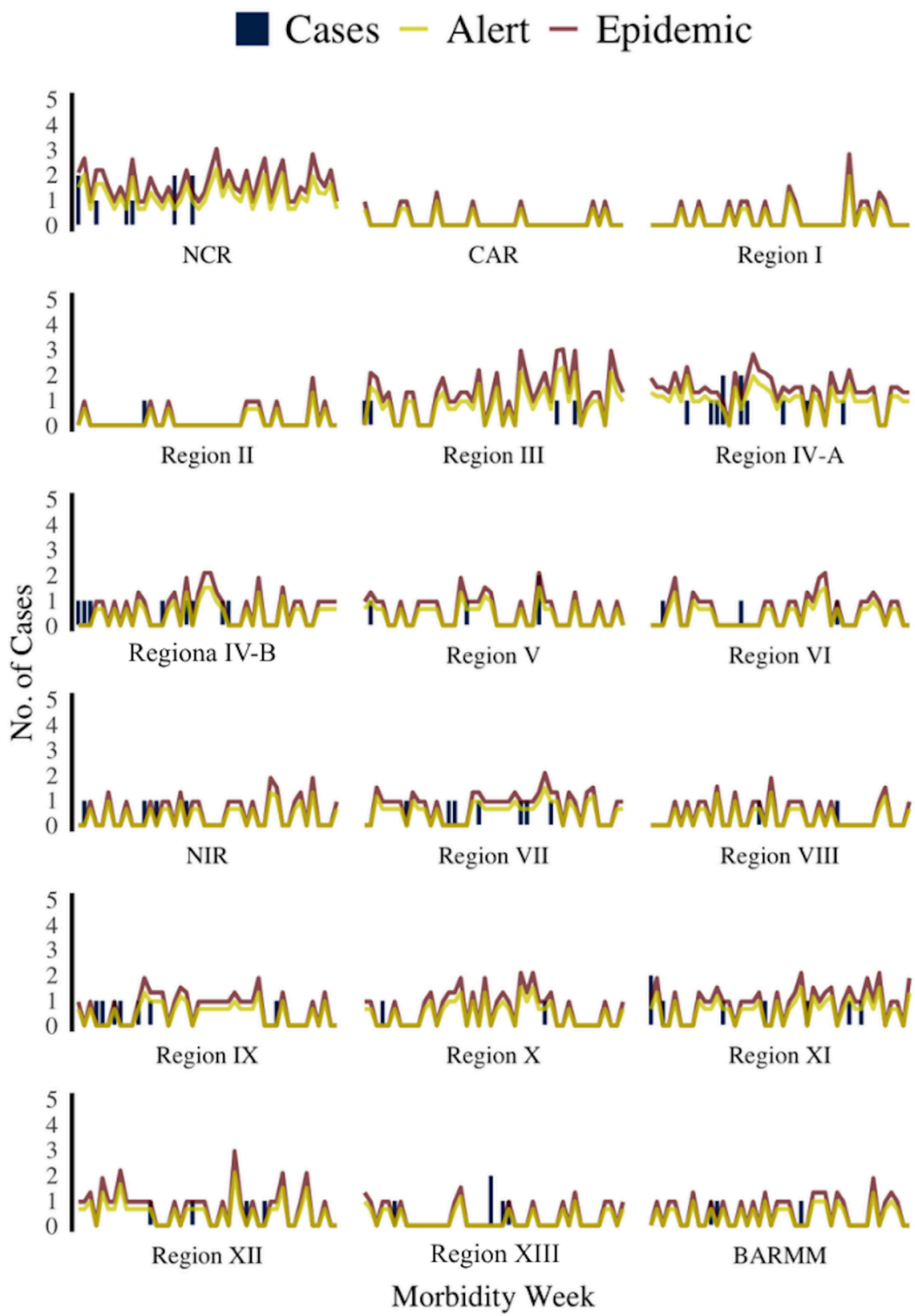


Figure 2. Regional Meningococcal Disease Cases by Morbidity Week (N=86)
Philippines, Jan 4 to Sep 19, 2026

Table 1. Cumulative Number of Cases and Deaths by Region, Philippines, Jan 4 to Sep 19, 2026 vs 2025

Region	Cases			TWGR [^] 3-4 vs 5-6 weeks (%)	Deaths			CFR (%) [*] 2026
	2025	2026	% Change		2025	2026	% Change	
IV-A	20	13	↓35	↓100	6	8	↑33	61.54
NCR	25	10	↓60	-	12	3	↓75	30.00
XI	7	9	↑29	-	5	1	↓80	11.11
IV-B	6	8	↑33	-	1	4	↑300	50.00
VII	7	7	-	↓100	1	4	↑300	57.14
IX	6	7	↑17	-	3	1	↓67	14.29
XIII	7	5	↓29	-	3	1	↓67	20.00
III	12	4	↓67	↓100	1	1	-	25.00
V	5	4	↓20	-	3	2	↓33	50.00
NIR	7	4	↓43	↓100	2	3	↑50	75.00
XII	11	4	↓64	-	3	1	↓67	25.00
VI	6	3	↓50	↓100	3	2	↓33	66.67
BARMM	8	3	↓63	-	4	0	↓100	0
VIII	4	2	↓50	↓100	3	0	↓100	0
X	10	2	↓80	↓100	5	0	↓100	0
II	2	1	↓50	-	1	1	-	100.00
I	2	0	↓100	-	1	0	↓100	-
CAR	2	0	↓100	-	0	0	-	-

[^]Two-week growth rate from MW 32-33 (August 09 to August 22) to MW 34-35 (August 23 to September 05)

^{*}Case Fatality Rate (CFR) is computed by dividing the total number of deaths by the total number of reported cases.

III. Demographic and Laboratory Profile of Cases

- Of the 86 reported cases, males accounted for 53 cases (63%). The 1 to 3-year age group recorded the highest number of cases (19, 22%), followed by the ages less than 1 year old (14, 16%).

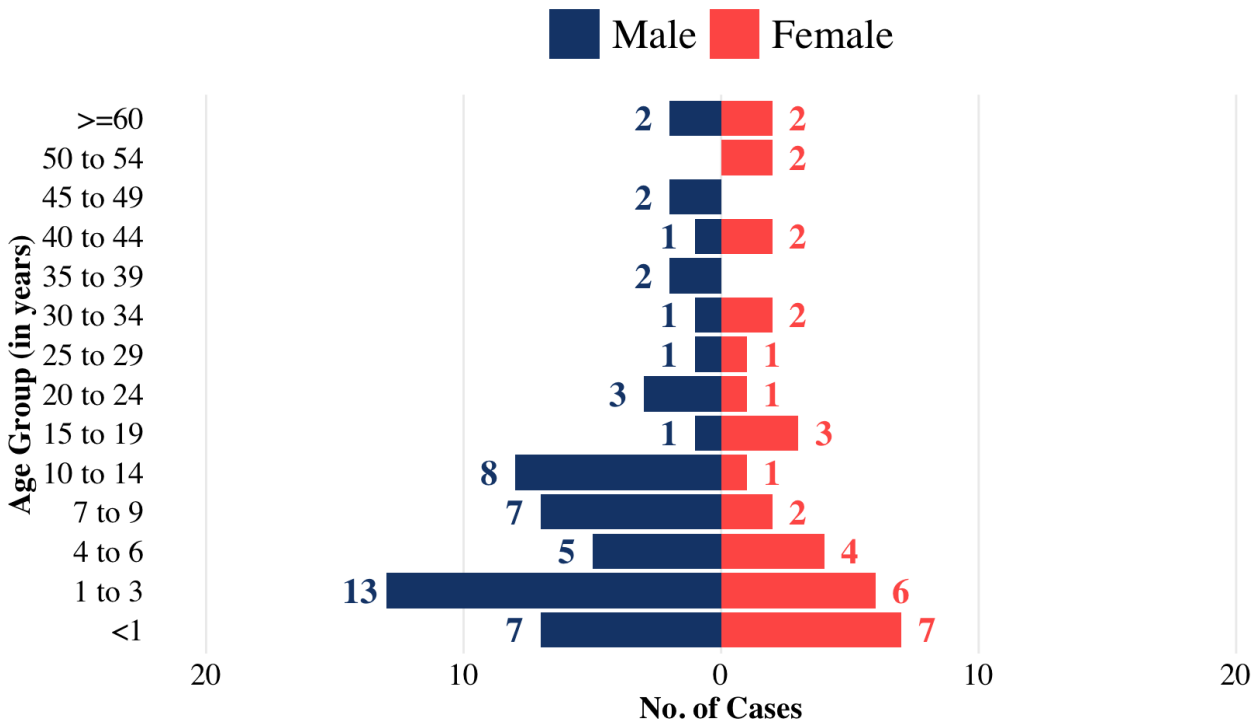
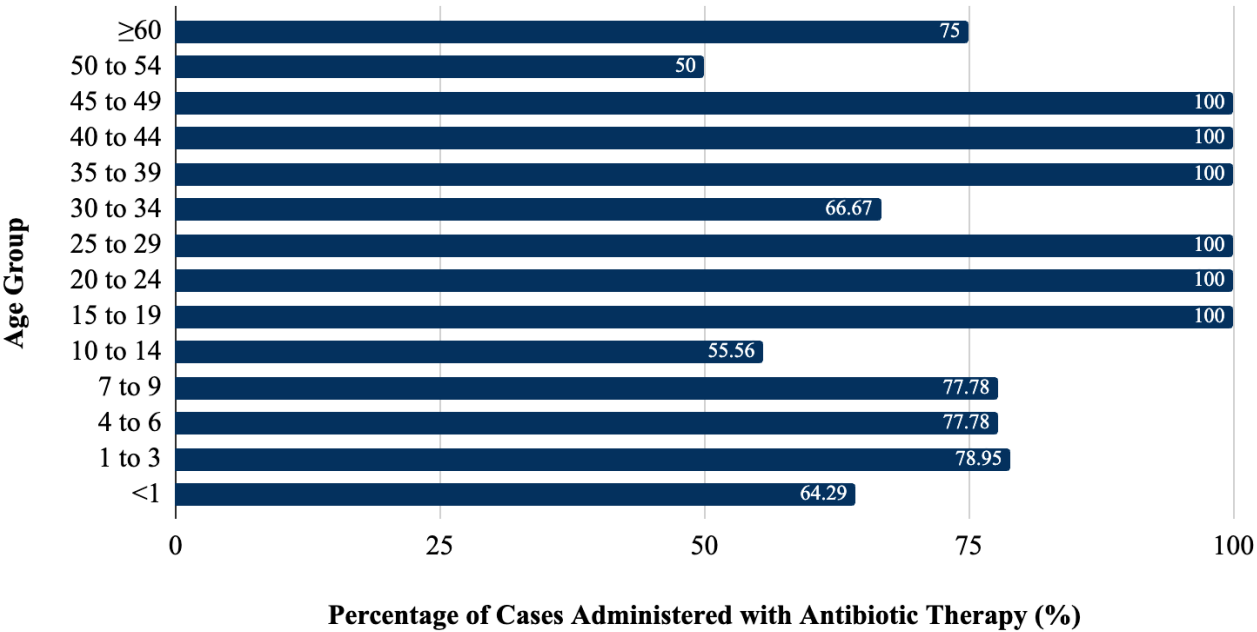


Figure 3. Age-Sex Distribution of Cases (N=86)
Philippines, Jan 4 to Sep 19, 2026

- Among the 86 reported cases, 25 cases underwent laboratory testing, of which **one (1) tested positive**, fifteen tested negative, and nine still have pending test results.

- Commonly reported symptoms included fever, purpura, petechiae, seizure, and vomiting. Fourteen (14) cases had an upper respiratory tract infection (URTI) within 2 weeks before illness onset, and five (5) close contacts of a case had URTI within 2 weeks before illness onset.
- Across all age groups, 50% and above received antibiotic therapy. The 1-to-3-year age group, which accounted for the largest number of cases, had 79% antibiotic therapy coverage.



**Figure 4. Antibiotic Therapy Administered to Meningococcal Disease Cases by Age Group
Philippines, Jan 4 to Sep 19, 2026**

- Notes:
1. Reported cases may still change due to late consultations and delayed reporting through the PIDSR-IS batch upload feature. Data was extracted on September 21, 2026.
 2. For epidemic thresholds, a single breach is sufficient for an area to be considered under “epidemic” status.

Annex A

Provinces and Highly Urbanized Cities from Regions that have Breached Epidemic Thresholds in the Past Four Weeks

Province	Cases from the Same Period Last Year	Cases in the Past 4 Weeks	Breached Thresholds in the Past 4 Weeks
NATIONAL CAPITAL REGION (NCR)			
Pasay City	1	1	Epidemic (once)
REGION IX (ZAMBOANGA PENINSULA)			
City of Zamboanga	0	1	Epidemic (once)
REGION XI (DAVAO REGION)			
City of Davao	0	1	Epidemic (once)
Davao del Norte	0	1	Epidemic (once)
Davao Occidental	0	1	Epidemic (once)

**Note: Only areas with alert or epidemic threshold breaches in the past four weeks are shown.*

Annex B

Provinces that have Breached Alert Thresholds Once in the Past Four Weeks

Province	Cases from the Same Period Last Year	Cases in the Past 4 Weeks	Breached Thresholds in the Past 4 Weeks
REGION IV-A (CALABARZON)			
Rizal	3	1	Epidemic (once)
Batangas	0	1	Epidemic (once)

**Note: Only areas with alert or epidemic threshold breaches in the past four weeks are shown.*